South East London Integrated Care System

NHS South East London

Integrated Care Board – Meeting in Public

12.30 to 15.30 on 22 July 2026
Coin Street Neighbourhood Centre 108 Stamford St, London SE1 9NH
Chair: Sir Richard Douglas Chair SEL ICB

Agenda

No.	Item	Paper	Presenter	Timing
	Opening Business and Introduction			
1	**Welcome** • Apologies for absence • Declaration of Interest. • Minutes of previous meeting actions & matters arising	A	RD	12.30
	Borough Showcase			
2	**Bexley Borough showcase**	-	DB	12.35
	Corporate business			
3	**ICB Governance Framework**	B	TF	13.05
4	**Update on Bromley community services contract readiness** p227	C	ABh	13.20
5	**Annual Report and Accounts** p236	D	TF	13.25
	Report for Assurance and discussion of current issues			
6	**Chief Executive Officer's report** p240	E	AB	13.30
7	**Board Assurance Framework** p262	F	TF	13.40
8	**Overall Report of the ICB Committees and Provider Collaboratives** p286	G	TF	13.50
9	**Performance Report** p407	H	AA	14.00
10	**Finance Report** p428	I	DM	14.15

No.	Item	Paper	Presenter	Timing
11	**Quality and Safeguarding Report** p438	J	EC	14.30
12	**Maternity update** p452	K	EC	14.45
13	**Diagnostic services** p482	L	AA	14.55
	Closing Business			
14	**Any other business**	-	RD	15.20
15	**Public Questions and Answers**	-	RD	15.20
	CLOSE 15.30			

Presenters

RD	**Sir Richard Douglas**	**ICB Chair**
DB	**Diana Braithwaite**	**Place Executive Lead Bexley**
AB	**Andrew Bland**	**ICB CEO**
ABh	**Dr Angela Bhan**	**Place Executive Lead Bromley**
AA	**Annabel Appleby**	**ICB Acting Director of Planning**
TF	**Tosca Fairchild**	**ICB Chief of Staff**
DM	**Dinah McLannahan**	**ICB CFO**
EC	**Elaine Clancy**	**ICB Chief Nurse**

NHS South East London Integrated Care Board
Register of Interests declared by ICB Senior Leadership Team
Date: 22/07/2026

Name	Position Held	Declaration of Interest	Type of interest	Date interest commenced	Date interest ceased
Sir Richard Douglas, CB	Chair	1. Chair South West London ICB	Financial interest	01-Feb-26	Current
		2. Senior Counsel for Evoke Incisive, a healthcare policy and communications consultancy	Financial interest	01-Apr-16	Current
		3. Trustee, Place2Be, an organisation providing mental health support in schools	Non-financial professional interest	01-Jul-22	Current
		4. Trustee, Demelza Hospice Care for Children, non-remunerated role.	Non-financial professional interest	01-Sep-22	Current
		5. Non-executive member, Department of Health and Social Care Board	Financial interest	01-Apr-24	Current
Andrew Bland	Chief Executive	1. Chief Executive South West London ICB	Financial interest	09 Feb 26	Current
		2. Partner is a Programme Director Employed by Guys and St Thomas NHSFT	Indirect interest	01-Apr-22	Current
Peter Matthew	Non executive director	None	n/a	n/a	n/a
Paul Najsarek	Non executive director	1. Advisor to Care Quality Commission on their approach to local authority assurance	Non-financial professional interest	01-May-22	Current
		2. Non-executive director - Health Foundation (health research charity)	Non-financial professional interest	01-Mar-23	Current
		3. Trustee of Waythrough (formally Recovery Focus and Humankind), which is commissioned the provide NHS services.	Non-financial professional interest	01-Apr-22	Current
		4. Director of Paul Policy Practice Ltd	Financial Interest	24-Dec-21	Current
		5. Advisor to DA Languages Ltd	Non-financial professional interest	01-Jul-24	Current
		6. Improvement advisor to Blackpool Council	Non-financial professional interest	22-Sep-25	Current
		7. Chair of London Neighbourhood Delivery Board (NHSE London appointment)	Non-financial professional interest	01-Sep-25	Current
		8. Governor, Christ The King Primary School	Non-financial professional interest	03-Aug-25	Current
		9. Chair of NHS North Central London Integrated Care Board	Financial interest	07-Oct-24	30-Aug-25
Anu Singh	Non executive director	1. Independent Chair of Lambeth Adult Safeguarding Board.	Non-financial professional interest	01-Apr-21	Current
		2. Member of the advisory committee on Fuel Poverty.	Non-financial professional interest	01-Jan-20	Current
		3. Non-executive director on the Parliamentary and Health Ombudsman.	Non-financial professional interest	01-Apr-20	Current
		4. Chair of Black Country Integrated Care Board	Financial interest	01-Sep-24	30-Sep-25
		5. Chair of Northamptonshire ICB and Leicester, Leicestershire & Rutland ICB cluster	Financial interest	01-Oct-25	Current
Georgina Fekete	Non executive director	None	-	-	-
Crystal Akass	Chief People Officer	1. Chief People Officer at Guys & St Thomas's NHS Foundation Trust	Financial interest	05-Aug-24	Current
Dr. Angela Bhan	Director of Place, Bromley	1. Professional Public Health advice for London Borough of Bromley.	Non-financial professional interest	01-Apr-20	Current
		2. Professional public health appraiser for NHS England	Non-financial professional interest	01-Jul-22	Current
		3. Occasional assessor for CESR applications for GMC, on behalf of Faculty of Public Health	Financial interest	01-Jul-22	Current
Diana Braithwaite	Director of Place, Bexley	None	-	-	-

Name	Position Held	Declaration of Interest	Type of interest	Date interest commenced	Date interest ceased
Annabel Appleby	Director of Planning	1. Governor on the Local Governing Body of Knole Academy, which is part of Aletheia Academies Trust	Non-Financial personal interest	1 Sep 2024	Current
Jennifer Daothong	Partner member, Local Authority	1. Chief Executive, London Borough of Lewisham	Financial interest	24-Sep-25	-
Gabi Darby	Director of Place, Greenwich	None	-	-	-
Holly Eden	Director of Delivery - Neighbourhood and Population Health	None	-	-	-
Andrew Eyres	Director of Place, Lambeth	1. Director of Lambeth, Southwark and Lewisham LIFTco, representing the class B shares on behalf of Community Health Partnerships Ltd for several LIFT companies in the boroughs. 3. Spouse is the Managing Partner at SHINE Executive Coaching & Consultancy Ltd, providing life sciences and partnerships strategic advisory and coaching services. 4. Corporate Director of Integrated Health and Adult Social Care – role spans ICB and Lambeth Council.	Non-financial professional interest Non-financial Personal interest Non-financial professional interest	01-Apr-13 05-Jan-26 01-Oct-19	Current Current Current
Tosca Fairchild	Chief of Staff	1. Partner is a Consultant in Emergency Medicine & Deputy Medical Director UHDB NHS FT. Potential to undertake locum work. 2. Bale Crocker Associates Consultancy – Client Executive 3. Director, The Flavoured Sourdough Company Ltd	Non-Financial Professional Interest Financial Interest Financial Interest	01-May-22 01-Oct-25 01-May-26	Current 31-Mar-25 Current
Dinah McLannahan	Chief Finance Officer	1. Independent Trustee Your trust Charity west midlands 2. Treasurer Minis and Juniors section Old Halesonians Rugby Football Club	Non-financial personal interest Financial	1-Apr-25 1-Apr-25	Current Current
Dr. Toby Garrood	Medical Director	1. Shareholding in Serac Healthcare 2. Consultant rheumatologist at Guy's and St Thomas' NHS Foundation Trust (GSTT) 3. In my role at GSTT I have received research and service development grant funding from Versus Arthritis, Guy's and St Thomas' Charity, Pfizer, Gilead and NHSx 4. I undertake private practice at London Bridge Hospital 5. Honorary Treasurer for British Society for Rheumatology	Financial interest Financial interest Financial interest Financial interest Non-financial professional interest	01-Apr-20 07-Oct-09 01-Jan-15 01-Jan-12 01-Apr-20	Current Current Current Current Current
Ceri Jacob	Director of Place, Lewisham	None	n/a	n/a	n/a
Ranjeet Kaile	Director of Communications & Engagement	1. Director of Communications & Engagement, South London & Maudsley NHS FT 2. Non-executive Trustee of Peoples Health Trust Charity.	Financial interest Non-financial professional interest	02-Apr-24	Current Current

Name	Position Held	Declaration of Interest	Type of interest	Date interest commenced	Date interest ceased
Matthew Trainer	Partner member, Acute	None	-	-	
Ade Odunlade	Partner member, Mental Health Services	1. Interim Chief Executive, South London and Maudsley NHS Foundation Trust	Financial interest	01-Dec-25	Current
Dr. Ify Okocha	Partner member, Community	1. Chief Executive (employee) of Oxleas NHS Foundation Trust	Financial interest	01-Nov-21	Current
		2. Director, Dr C I Okocha Ltd, providing specialist psychiatric consultation and care	Financial interest	01-Jan-96	Current
		3. Director, Sard JV Software Development	Financial interest	2011	Current
		4. Director, Oxleas Prison Services Ltd, providing pharmacy services to prisons and Kent and South East London	Financial interest	27-Sep-16	Current
		5. Holds admitting and practicing privileges for psychiatric cases to Nightingale Hospital	Financial interest	01-Jan-92	Current
		6. Fellow of the Royal College of Psychiatrists	Non-financial professional interest	01-Jan-92	Current
		7. Fellow of the Royal Society of Medicine	Non-financial professional interest	01-Jan-85	Current
		8. International Fellow of the American Psychiatric Association	Non-financial professional interest	01-Jan-85	Current
		9. Member of the British Association of Psychopharmacology	Non-financial professional interest	01-Jan-85	Current
		10. Member of the Faculty of Medical Leadership and Management	Non-financial professional interest	01-Jan-85	Current
		11. Advisor to several organisations including Care Quality Commission, Kings Fund, NHS Providers and NHS Confederation.	Non-financial professional interest	01-Jan-85	Current
Darren Summers	Director of Place, Southwark	1. Member of Council of Governors of Guys and St Thomas's NHS Foundation Trust as ICB representative	Non-financial professional interest	01-Jul-24	Current
Dr. George Verghese	Partner member, primary care	1. GP partner Waterloo Health Centre	Financial interest	01-Aug-13	Current
		2. Lambeth Together training and development hub director	Non-financial professional interest	01-Jan-22	Current
		3. Lambeth Healthcare GP Federation shareholder practice	Non-financial professional interest	01-Aug-19	Current

NHS South East London Integrated Care Board
ACTION LOG

REFERENCE	DATE ACTION AROSE	ACTION DESCRIPTION	STATUS	ACTION OWNER	DATE FOR COMPLETION	UPDATE/NOTES
ICB 005	08-Apr-26	ICB to consider how providers can learn from never events outside their own organisation	to be closed	Chief Nurse	22-Jul-25	Never events are listed and reviewed in ICB Quality Committee meetings.

DRAFT

Integrated Care Board meeting in Public

Minutes of the meeting on 8 April 2026

Brixton Tate Library

Present:

Name	Title and organisation
Richard Douglas [Chair]	ICB Chair
Andrew Bland	ICB Chief Executive Officer
Dr Angela Bhan	Bromley Place Executive Lead
Jessica Arnold	For Greenwich Place Executive Lead
Holly Eden	Director of Delivery for Neighbourhood and population health
Andrew Eyres	Lambeth Place Executive Lead
Michael Boyce	Deputy Chief of Staff
Georgina Fekete	Non Executive Member
Mike Fox	Chief Finance Officer
Dr Toby Garrood	ICB Joint Medical Director
Ceri Jacob	Lewisham Place Executive Lead
Prof Clive Kay	Partner Member Acute Care
Diane Jones	Chief Nurse
Peter Matthews	Non-Executive Member
Paul Najsarek	Non-Executive Member
Kate Lillywhite	For Partner Member Mental Health Services
Darren Summers	Southwark Place Executive Lead
Anu Singh	Non Executive Member
Dr Ify Okocha	Partner Member Community Services

In attendance:

Rupi Dev	Interim Director of Planning
Denis Lafitte	CDIO
Ranjeet Kaile	ICB Director of Communications and Engagement
Ruth Hutt	Director of Public Health Lambeth
Vanessa Burgess	ICB Chief Pharmacist
Dr ChiChi Ekhator	GP and lead for AT Beacon project
Lelly Oboh	Consultant Pharmacist
Raj Matharu	Community Pharmacy in south east London
Dominic Norton	Health Innovation Network
Keith Thompson	Chief Pharmacist & Medical Director Guys &St Thomas' NHS FT

1.	**Welcome and Apologies**
1.01	Apologies were noted from Dr George Verghese, Crystal Akass, Gabi Darby, Ade Odunlade, Amanda Pritchard, Ben Travis and Tosca Fairchild.
1.02	There were no conflicts advised with items on the agenda.
1.03	Richard Douglas noted that himself and Andrew Bland were now also Chair and Chief Executive of SWL ICB.

1.04	The notes of the previous meeting were approved as an accurate record
1.05	The action log was updated.
2	**Lambeth Showcase**
2.01	Andrew Eyres introduced a showcase on the Lambeth Care Together approach to prevention. Ruth Hutt outlined the approach which was based on a life course approach, both using evidence and creating evidence, and focusing on inequalities and measurable outcomes rather than process, and recognising that some interventions were better at a borough level and others with a hyper-local approach. Sharing some of the evidence from Lambeth over 15 years it was possible to see a decrease in all forms of mortality for those who had received health checks, with a modest improvement for those in less deprived areas but a significant improvement for those in the most deprived areas of the borough.
2.02	Dr ChiChi Ekhator described the Lambeth fruit and vegetables on prescription initiative which had run from 2022 in partnership with Lambeth Public Health and the Alexander Rose Charity and A.T Beacon project. This initiative reached people who rarely would come into general practice, but in providing support in supermarkets, barbershops, and local faith groups there was often feedback from local people about mistrust and broken promises. The vouchers cost just £8 per week for a voucher that could be spent on fruit and vegetables locally but with a cohort of 54 people there had been a statistical improvement of blood pressure, and benefits such as an increase in trust, as well as benefit to the local economy.
2.03	A local resident who had joined the fruit and veg on prescription scheme after it had been offered at a local church shared her experience of the scheme which had been helpful to support greater control over her own health.
2.04	Anu Singh welcomed the presentation and the example of very visible interventions in the community, asking whether there was also use of data to target those with long term conditions or frail elderly who may be less able to leave their homes.
2.05	Ceri Jacob expressed an interest in the potential of such schemes to address poor mental health and asked there were opportunities to measure this impact. Ruth Hutt noted neighbourhood working may benefit mental health although it would require resource and improvements in data. Dr ChiChi Ekhator added that Lambeth wellbeing plus was a dedicated services to support users with sources of help .
2.06	Dr Angela Bhan praised the work focused on empowering local community members and asked how it was being used to build trust. She asked how the mortality reductions could be linked to the health checks, either through vaccinations or blood pressure.
2.07	Dr Toby Garrood also asked whether there was evidence of causation as opposed to association with health checks, and whether there was a measurable return on investment with the scheme. Ruth Hutt noted that the there was an observational study rather than proving causation and that savings had been based on primary care use.
2.08	Dr ChiChi Ekhator explained that following the Covid-19 pandemic some deep

	dives on building trust and as part of this it was necessary to recognise that practices were not necessarily seen as the most trusted or safe places and so using social assets already in contact with people such as pharmacists, shopkeepers was an important way to slowly build trust.
2.09	Georgina Fekete asked if the scheme would have a generational impact and if there were any other schemes to embed healthy eating in early years.
2.10	Prof Clive Kay asked if the cost benefit analysis extended into secondary care appointments and costs being avoided and asked if patients presenting to providers could benefit from being referred into the programme. Ruth Hutt noted that the fruit and veg on prescription scheme was based on case finding rather than needing referral from secondary care.
2.11	Jessica Arnold asked if there was a related issue with people not having refrigerators or ovens and links to charities who could help. Dr ChiChi Ekhator confirmed that there were such cases as well as people faced with a choice between heating or food.
2.12	Sir Richard Douglas welcomed the update and suggested that the challenge was to scale up such work to other areas while keeping the local focus that was part of its success.
3	**ICB Change Update**
3.01	Ceri Jacob reminded the board that in March 2025 all ICBs had been asked to bring their running costs to within £19 per head of population. Subsequently south east London and South west London ICBs had taken the decision to form a cluster to improve resilience and ability to deliver the national ICB blueprint across both organisations. The ICBs had worked together to develop structures including several shared areas and had started a consultation with staff on 5 March which was still ongoing. A voluntary redundancy scheme had been conducted ahead of the consultation and 52 staff had agreed to leave under this scheme. A further scheme had been opened at the same time as the staff consultation. It was expected that compulsory redundance would be required in addition to voluntary redundancy to meet the reductions required. The ICB blueprint recommended that some functions be transferred from the ICB to providers to carry out. The ICB had identified Estates, HR, Medicines Management and Digital as areas for transfer and this would take place under the oversight of a Transition Committee chaired by a board non-executive member. Staff were able to submit responses to the consultation as well as other queries and regular all staff briefings and 'ask HR' sessions were being held. Following consultation a significant period of organisational development would be required to develop new ways of working in the new smaller ICb and this was being planned.
3.02	Andrew Bland added that alongside the process for the majority of staff, recruitment processes were underway for a new shared executive team.
3.03	The Board noted the update.
4	**Chief Executive Officers Report**
4.01	Andrew Bland referred to the report which outlined updates from each area as well

	as changes to leadership in the system, highlighting that further industrial action had been confirmed and although the system was accustomed to managing periods of industrial action it would likely have an impact on operations and cost. The executive and board recognised and were concerned by the impact on staff morale of the ICB changes.
4.02	Paul Najsarek welcomed the progress reported with integrated neighbourhood teams and asked how the boards ambitions in this area would be tracked.
4.03	Holly Eden clarified that the ICB had set a targets for boroughs to reach a minimum number of complex patients in the population through integrated neighbourhood teams. A set of metrics had recently been agreed which would enable progress to be measured more regularly but there was not enough data yet to report.
4.04	Georgina Fekete asked whether the activities in the report were sufficiently linked to the delivery of strategic commissioning.
4.05	Andrew Bland noted that the report was intended to appraise the board of the work taking place in each of the boroughs, and there would be a programme of work to discuss new governance and ways of working which would be reflected in the papers the board would receive.
4.06	The Board noted the chief executive officers report.
5	**Board Assurance Framework**
5.01	Michael Boyce introduced the Board Assurance Framework for approval. Changes had been highlighted.
5.02	The Board approved the Board Assurance Framework.
6	**Overall Report of ICB committees and provider collaboratives**
6.01	Michael Boyce referred to the ICB committees report there were no escalations for decision.
6.02	The Board noted the ICB committees report.
7	**Performance Report**
7.01	Rupi Dev presented the report which focused on performance against metrics agreed in planning for 2024/25. There had been some improvement in performance in relation to 12-hour delays, faster diagnosis standards, GP appointments and pharmacy first. There continued to be some variation in GP access across south east London. Continuing Healthcare Performance standards were being met but in the context of significant pressures on capacity. There remained pressure on referral to treatment times, and on performance targets for cancer, diagnostics, mental health and community waits. A strong focus on improving referral to treatment times nationally, regionally and locally had resulted in a fall in the number of long waits had fallen but particular specialities remained challenged as set out in the paper.
7.02	Prof Clive Kay noted some specialities continued to face pressure over several years and noted that the ICB should continue to support work across the system

	where this could help improve performance.
7.03	Georgina Fekete remarked that some areas of performance seemed to remain challenged at every board meeting. Taking cancer as an example, and noting the number of mitigations set out, she asked if these needed to be prioritised to focus on the action which would have the highest impact.
7.04	Rupi Dev responded that many of the performance challenges had multiple factors, not all of which were within the south east London system, and precisely quantifying the exact impact of each mitigating action would be difficult. There had been work to identify drivers of referral to treatment waits as well as and in addition to mitigations identified work to consider how the system development fund could be deployed to help the system address waits.
7.05	Prof Clive Kay noted that each individual provider would have their own actions for 2026/27 and in addition the SEL cancer alliance co-ordinated work across the system. Andrew Eyres reflected that challenges had persisted for many years; the cancer target applied to a large number of different pathways and improving performance would be the sum of many different actions.
7.06	The Board noted the performance report
8	**Quality and Nursing Report**
8.01	Diane Jones presented the quality and safety report. There had been 13 Never Events, with a theme around retained products, so quality improvement work was underway to improve the use of checklists. There was no serious harm as a result however impact on patient experience. Quality alerts submitted by clinicians on areas of concern in primary and secondary care had resulted in improvements for example in the way prescribing was carried out in the community. The ICB would be required required to co-ordinate with local authorities on their plans to reform SEND (special educational needs and disabilities) following the government white paper. The quality team working with providers in healthcare settings for quality oversight and assurance and seeing good work happening to improve care and experience for residents.
8.02	Prof Clive Kay asked how the ICB could ensure that individual providers learned from Never Events in other providers, through some mechanism to share with providers.
8.03	Diane Jones noted that the system quality group provided a forum to share this if providers were content to share, and the focus on how the trusts adopted recommendations.
8.04	Paul Najsarek referred to a succession of national reviews ongoing and asked if the right work was happening in each of the trust maternity services and ready to consider the recommendations.
8.05	Diane Jones noted that the Local maternity and neonatal system involved providers meeting together and with the ICB and had signed off a programme of improvement actions. Where there were recommendations these would also feed into the ICBs commissioning intentions.
8.06	Dr Ify Okocha asked as the role of the ICB changed how quality would change. Andrew Bland noted that a strategic commissioner would still wish to know that

	contracts were being delivered with the appropriate quality, access and performance. There would be a transition to the new arrangements, but it was likely that the quality was form part of the commissioning cycle.
8.07	The Board noted the quality and nursing report.
9	**Finance Report**
9.01	Mike Fox presented the finance report at Month 11. The ICB was reporting a break-even position. The ICS was reporting a deficit £14.1 but this is £5.7m better than planned for this time at the financial year. The ICS forecast had moved from break even to £1.3m surplus but the change was in effect due to an accounting adjustment rather than operational improvement. There were efforts to spend all the capital allocated to the system however there was currently a slight underspend.
9.02	Georgina Fekete noted that there continued to be financial challenge reported in relation to mental health and CHC (Continuing Healthcare) and asked if there was sufficient funding allocated or actions being taken.
9.03	Mike Fox noted that CHC was a significant area of spend for many ICBs and driven by demand. To allocate more resources however would require a reduction of spend elsewhere. Andrew Bland advised CHC decisions were governed by legal frameworks which limited the extent to which costs could be driven down, however the ICBs performance was good relative to other ICBs.
9.04	The Board noted the Finance report.
10	**Medicines Optimisation and Community Pharmacy**
10.01	Vanessa Burgess introduced the work of medicines optimisation and pharmacy highlighting its importance as one of the largest and most volatile areas of spending. Pharmacy worked every year to reduce cost and had identified £12m of savings in the next 12 months. There was however opportunity for the system to use well managed medicines to manage activity more generally, reduce hospital admissions and reduce inequalities of access and outcome. As new NICE approved medicines became available at increasing pace, the task of increasing value became more and more important.
10.02	Lelly Oboh explained that tackling overprescribing aimed to add value to patient safety, quality and inequalities. Overprescribing more often affected people in certain minority groups and people living with frailty or disabilities. Structured medication reviews involved patient and clinician working together to optimise medicines for a patient, not just focusing on stopping medicines. The overprescribing work involved collaboration across London as well as with our voluntary groups improve local access.
10.03	Raj Matharu outlined the role of community pharmacy, highlighting its role as a key place that was open for residents to access help, with a recent pharmacy needs assessment showing that on average access was available to patients within 20mins. The sector was under pressure and required support to realise the opportunity to help local people.
10.04	Keith Thompson outlined the focus of providers on achieving value by getting the

	best outcomes for patients from use of medicines as well as controlling finances. Successful examples of this included switching away from short-acting beta-agonists (SABA) which alleviated symptoms to more longer acting medicines which improved outcomes. Work to bring in electronic prescription and realise the potential of the Epic system helped to reduce fragmentation of the system and improve productivity and the shift from hospital to community. Expert pharmacists were working with neighbourhood teams to help identify patients who could benefit from medicines optimisation to improve their ongoing treatment and so reduce demand on other services.
10.05	Anu Singh welcomed the focus on engaging with patients on their own care areas such as structured medicines review, and asked about the opportunity for the ICB more generally to support and work with patients as part of the model of care to contribute to system sustainability.
10.06	Ranjeet Kaile praised the extensive work with communities and work with schools, faith groups, barber shops and so on and asked about opportunities to go further with engagement.
10.07	Vanessa Burgess agreed that pharmacy had improved its engagement with local people over the last couple of years and the help of the VCSE sector had been important. Lelly Oboh shared that the overprescribing work had involved in-depth patient engagement speaking to over 500 residents, developing resources for patient engagement and structured medicines review and trying to proactively incorporate the patient voice in design. Raj Matharu reflected on the importance of a clear message so that patients new what to expect and learning the lessons from pharmacy first.
10.08	Peter Matthew asked about the scaling up of community pharmacy as a whole, pointing out that a large high street chain providing pharmacy would experience different challenges to smaller local pharmacies.
10.09	Raj Matharu noted that community pharmacys is undergoing review of what pharmacy is and growing for example floor space with more consultation rooms. Have core service and extra commissioned services. Opportunity for prevention, ask patients from vital five.
10.10	Prof Clive Kay asked about the role of pharmacy in ongoing reviews, for example in cases where a patient's medicines were initially correctly prescribed, but became less appropriate over time, noting that identifying how this would be followed up would be important if there were significant changes to outpatient services.
10.11	Dr Angela Bhan asked whether in some cases not *enough* medicines were being prescribed for particular conditions rather than overprescribing and whether there was clear evidence for the link between short acting inhalers and hospital attendance and about the role of pharmacy in infection prevention and control and in trying to reduce antimicrobial resistance.
10.12	Dr Ify Okocha asked whether similar issues applied to mental health medicines.
10.13	Dr Toby Garrood asked where there might be other opportunities to look at optimising NICE approved medicines beyond the work on inhalers.
10.14	Keith Thompson highlighted agreed on the need to support patients with ongoing treatment, working with community pharmacy on reviews and measure the benefit

	for acute services, initiatives included the homecare transformation project and increasing the number of shared care arrangements.
10.15	Vanessa Burgess agreed that under-prescribing was an issue particularly in areas such as chronic kidney disease, work was underway to embed medicines in integrated neighbourhood teams model of care, and undertake case-finding for example for weight management drugs. In mental health, there was work on long term depot medicines, on those on antipsychotic medicines who may also have learning disabilities and need to help mental health patients manage long term physical conditions.
10.16	Darren Summers asked how the benefits and value of pharmacy interventions could be tracked, particularly where the benefits may be realised in other parts of the NHS rather than pharmacy services.
10.17	Sir Richard Douglas asked if the budget set for pharmacy was sufficient or too small to achieve the aims.
10.18	Georgina Fekete asked what measures could be used to judge success over the year.
10.19	Vanessa Burgess noted that there was large amounts of prescribing data which tended to be more accurate than other sources kinds of data but was not well integrated with them. With a strategic commissioning approach there was an opportunity to better integrate prescribing data into neighbourhood working to optimise patients care and avoid the need for expensive interventions. Similarly the budget tended to be relatively siloed and could benefit from integration with other elements of the hospital spend to allow flexible investment in medicines. Success would be the provision of fair access to the population within budget, increase value delivered through prescribing and by optimising medicines reduce the need for hospital attendance. Reducing overprescribing was key with data suggesting that where people over 75 were prescribed 15 or more medicines, 10% were likely to be inappropriate. Lelly Oboh emphasised the importance of patient involvement as medicines were also wasted if they were not taken as prescribed.
10.20	The Board noted the system position on medicines optimisation and community pharmacy and supported the continued shift towards value-based medicines use and scaling neighbourhood pharmacy models as part of integrated neighbourhood teams.
11	**Primary Care Interface**
11.01	Dr Toby Garrood updated on work which had taken place on the primary secondary care interface, where issues could cause frustration for patients, waste of resources and increased workload for primary care. NHSE had published suggested areas of focus including onward referral, call and recall, incomplete care, and providing a clear named contact. In south east London, interface groups had been established at all acute sites and were now maturing. A consensus document with rules of engagement had been presented to the board previously and had since been tested with community and with mental health colleagues and would be published. A patient-facing version of the document was also being explored and a document setting out alternatives to hospitals for blood tests had also been explored. Specific work on discharge

	summaries, system prescribing guidelines and outpatient letters was underway, and talks given to new doctors about the work. Quality alerts raised about issues by clinicians were used to identify areas for discussion, and although a direct link could not be proved the volume of these alerts had reduced since the work had started.
11.02	Dominic Norton noted that the Health Innovation Network had been working to operationalise the consensus document and organise the work into a programme of workstreams capturing what had already been done in south east London boroughs and trusts level, as well as similar national and international work which could provide useful examples. Several resources were not hosted along with the consensus document and a maturity matrix helped frame progress so far.
11.03	Jessica Arnold her perspective as chair of the Greenwich and Bexley Primary and Secondary care interface forum. Meeting every six weeks, there was good engagement at a senior level from trusts, GPs and clinical and care professionals. The membership had been expanded to include representatives of community representatives, and a permissive approach to areas for discussion and network events helped build commitment and positivity. Successes included developing an approach to inappropriate requests made to GPs which addressed concerns whilst preserving working relationships and feedback from hospital colleagues and a ward-by-ward examination of queen Elizabeth hospital had highlighted that the challenges with discharge could vary across wards in the same hospital. The group were prioritising urgent care interfaces, making elective recovery everyone's business and developing local incentives as well as making the most of national incentives. The work required robust discussion, high-trust integrated working and the commitment and positivity of all involved.
11.04	Dr Toby Garrood summed up by re-iterating the importance of relationships and ownership of the work by local Places being build by the work as a foundation of neighbourhood working in the future.
11.05	Holly Eden noted several 'digital pain points' with the ERS system which might hinder the smooth working of the interface and suggested an opportunity to use findings from this work to advocate for improvements to systems. A future phase of the work could explore the interface between hospital services and VCSE, pharmacy and local authority. Ceri Jacob added that interface between mental health services and acute and primary care was worth exploring. Andrew Eyres noted asked if there might evidence from MyChart which could help improve the interface with patients.
11.06	Paul Najsarek asked about the risk that the guidance would not be followed.
11.07	Georgina Fekete noted that trusts were 'exploring' discharge summaries and asked what was preventing trusts giving an undertaking to use them consistently. Jessica Arnold suggested that this was a wider piece of work with various issues reported about discharge notes, for example that they were two long or the action for GPs not clear.
11.08	Prof Clive Kay praised the work but observed that so far a 'coalition of the willing' were engaged in improving the interface, and reflected that the work would eventually need to extend to reach those who were so far unwilling to change or comply with the improvements.
11.09	Dr Toby Garrood reflected that although the work had started with GPs and acute

	trusts, it was not intended to stand still. There had already been conversations with south west London colleagues, where issues were similar. In general clinicians were motivated to improve things and so highlighting and discussing interface issues was often sufficient to prompt work towards their solution.
11.10	Sir Richard Douglas asked about the suggestion from the data that up to 20% of GP time was taken up with interface issues and asked how the ICB could demonstrate a reduction to this through interface work. Dr Toby Garrood noted that the simplest method was to survey GPs, however it would be important to do this carefully being mindful of survey fatigue.
11.11	The Board noted progress and supported the ongoing work.
12	**Any Other Business**
12.01	There was no other business.
13	**Public Questions**
13.01	A question was asked about the use of Palantir in south east London and referred to the more detailed question which had been submitted. The issues of data security, efficacy and public trust about the use of a company outside the UK, particularly ethical issues in using a company which in the words of amnesty international was complicit in serious human rights abuses. It was an issue for the local ICBs and trusts to consider in using such a software rather than to refer to national policy. National policy in relation to the use of this company was also under scrutiny. Break clause in February 2027 and asked what the intentions. There were alternatives for safe systems and no reason to pursue an ethically unsuitable option.
13.02	Denis Lafitte noted that the ICB and trusts took data protection seriously and made sure systems used were suitable. The ICB was not using this particular service, and providers were responsible for making their own decision. Providers were at different stages of maturity, and some had welcomed the potential of the Federated Data Platform to provided new functionality, whilst others had chosen not to pursue this due to already having comparable technologies. New systems and technology available for NHS in south east London was kept under constant review.
13.03	A comment was made highlighting the role and involvement of voluntary and community sector in particularly the Black VCSE to work with people to take control of their own health and represented as part of the work.
13.04	Andrew Bland welcomed the reminder of this work and noted the VCSE charter and the commitment of the board to work with local organisations.
	Close

South East London Integrated Care System

NHS South East London

SEL ICB Board

Title	**ICB Governance Framework**					
Meeting date	22 July 2026	Agenda item Number		Paper Enclosure Ref		
Author	Kieran Swann, Associate Director of Risk and Assurance Simon Beard, Associate Director of Corporate Operations					
Executive lead	Andrew Bland, Chief Executive Officer					
Paper is for:	Update		Discussion		Decision	x
Purpose of paper	To present the proposed South East London ICB Governance Framework and supporting governance documents for approval. The paper sets out the rationale for the revised governance model, describes the key components of the framework and seeks approval of the governance arrangements required to support the ICB's future operating model and strategic commissioning role.					
Summary	The proposed Governance Framework provides an integrated set of governance arrangements through which the ICB will exercise its statutory functions, delegate authority, provide assurance and maintain accountability. Developed alongside South West London ICB, the framework seeks to create a more strategic, streamlined and aligned approach to governance, supporting effective decision-making, clearer accountability, proportionate assurance and stronger delegation. The framework comprises amendments to the ICB Constitution, a revised Scheme of Reservation and Delegation and Schedule of Matters Delegated, updated Standing Financial Instructions, revised committee structures and terms of reference, and a Board Ways of Working statement. Subject to Board approval, the framework will be implemented from 1 October 2026, with constitutional amendments progressing through the required NHS England approval process.					
Potential conflicts of Interest	None					
Sharing and confidentiality	*Published.*					

Relevant to these boroughs	Bexley	x	Bromley	x	Lewisham	x
	Greenwich	x	Lambeth	x	Southwark	x
Equalities Impact	Not applicable at this stage.					
Financial Impact	None.					
Public Patient Engagement	Not applicable.					
Committee engagement	ICB Board seminar session, 18 February 2026 ICB Board seminar session, 18 March 2026 ICB Audit and Risk Committee, 9 July 2026 (SoRD and SFIs)					
Recommendation	The Board is asked to: a. Approve the proposed ICB Governance Framework and supporting governance documents. b. Approve the constitutional and governance amendments set out within the accompanying papers. c. Note that the constitutional amendments require subsequent approval by NHS England and agree that the necessary approval process should be initiated following Board approval. d. Agree that the ICB Governance Framework will take effect 1st October 2026 following Board approval and become the governance framework through which the ICB operates, subject to the constitutional approval requirements described above.					

NHS
South East London

NHS South East London Integrated Care Board Governance Framework

ICB Board
22 July 2026

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1. Purpose

1.1. This paper provides an overview of the proposed ICB Governance Framework for South East London Integrated Care Board (ICB) and the suite of governance documents that support its implementation.

1.2. The framework has been developed to support the ICB's continuing evolution as a strategic commissioning organisation, ensuring governance arrangements remain effective, proportionate and aligned to both statutory responsibilities and the organisation's future operating model.

1.3. The framework has been developed in the context of emerging South East London ICB and South West London ICB cluster arrangements, which are now operating in practice through a jointly appointed Chair and executive leadership and increasingly aligned ways of working. It provides the governance arrangements needed to support this operating model whilst maintaining the statutory responsibilities and accountabilities of each individual ICB.

1.4. The paper explains the rationale for the proposed framework, describes its principal components and seeks approval of the supporting governance documents.

2. The Governance Framework

2.1. The ICB Governance Framework provides the arrangements through which the organisation exercises its statutory functions, takes decisions, delegates authority, manages its activities and maintains accountability.

2.2. The framework is built as a hierarchy of interdependent governance documents. At its foundation are the legislative and regulatory requirements that establish the statutory responsibilities, powers and duties of Integrated Care Boards. These requirements are translated into the ICB Constitution, which establishes the formal governance arrangements of the organisation and the authority of the Board.

2.3. Supporting the Constitution are a set of additional documents that define how authority is exercised in practice. These include the Scheme of Reservation and Delegation and the Schedule of Matters Delegated, which sets out those matters reserved to the Board and those delegated to committees, members of the executive and other officers of the ICB. This document defines operational decision-making authority and financial delegations to postholders within the ICB. Related to this, the Standing Financial Instructions establish the organisation's financial control framework.

2.4. The Board's committees operate within this framework through formally approved Terms of Reference, which define their purpose, responsibilities, delegated authority and reporting arrangements.

2.5. The framework is further supported by a Board Ways of Working statement, which articulates the principles, behaviours and expectations that underpin effective governance and support the practical operation of the formal governance arrangements.

2.6. The purpose of this framework is not simply to define governance arrangements, but to support better decision-making, clearer accountability, effective delegation and stronger assurance across the organisation.

3. Alignment with South West London

3.1. The future governance framework has been developed collaboratively with South West London ICB as part of the wider development of a clustered governance function and aligned arrangements across the two organisations.

3.2. A key design principle has been to achieve alignment between South East London and South West London wherever reasonably practicable, creating a 'mirrored' governance architecture with consistent committee structures and delegation arrangements. This is intended to support the jointly appointed Chair and executive leadership team, reduce unnecessary variation, adopt best practice and to provide a stronger foundation for ongoing collaboration.

3.3. Whilst the intention has been to maximise alignment, limited areas of divergence remain where these are required to reflect local organisational arrangements or specific operational requirements. Any such differences have been introduced only where considered necessary and proportionate.

3.4. Additionally, SWL and SEL ICBs will establish a joint time-limited Transition Committee (ToRs attached as appendix 12) to provide assurance and oversight of the ICB SEL and SWL ICB organisational change programmes, reporting to SEL and SWL ICB Boards. The Committee's role is to ensure a safe and coherent transition, managing local risks, tracking progress and overseeing the development of organisational design and implementation of the change process, including the transfer of functions to providers over time.

4. Background and design approach

4.1. Over recent months, the ICB Board has considered how governance arrangements should evolve as the ICB increasingly focuses on its role as a strategic commissioning organisation with a remit to improve population health outcomes, reduce inequalities, deliver the strategic shifts towards prevention, community-based care and digital transformation, and exercising its wider system leadership and convening role.

4.2. At the same time, the organisation is operating with significantly reduced management capacity in response to national expectations regarding running costs,

and a new leadership model with a jointly appointed Chair, CEO and executive team. This requires governance arrangements that are efficient and capable of supporting effective decision-making in this context, and as new management structures are recruited to and postholders in place between July and December 2026.

4.3. Board discussions identified a number of consistent themes that have shaped the development of this framework. The framework is designed to support the ICB's evolving role as a strategic commissioner by:

- creating greater clarity regarding decision-making and accountability;
- strengthening delegation to committees, the Executive and officers;
- reducing duplication across governance arrangements;
- focusing Board and committee attention on strategic priorities, outcomes, risks and major trade-offs; and establishing committee structures and information flows that better integrate quality, performance, finance, workforce, transformation and risk oversight.

4.4. Together, these changes are intended to enable the Board to focus on strategic leadership and stewardship of the organisation whilst ensuring that appropriate scrutiny, assurance and decision-making arrangements remain in place throughout the ICB.

5. System and partnership governance

5.1. The formal governance arrangements described within this paper represent one component of the wider governance model through which the South East London system will operate.

5.2. The effectiveness of the proposed governance framework is dependent upon the effective functioning of partnership governance arrangements across the broader South East London health and care system. The ICB is only able to deliver on its statutory responsibilities and strategic ambitions through collaboration with NHS providers, local authorities, primary care, the voluntary, community and social enterprise sector and wider partners. As such, effective partnership governance is an essential enabler of the overall ICB governance model rather than a parallel or separate set of arrangements.

5.3. At a South East London level, the governance framework assumes the continued operation of system partnership arrangements that support the development and delivery of shared priorities and provide the collaborative leadership necessary to deliver system objectives. This includes the System Sustainability Group and wider partnership governance arrangements supporting key strategic enablers including workforce, digital and estates. These forums play a critical role in developing shared strategies, coordinating delivery, resolving cross-organisational issues and providing collective leadership across the system.

5.4. The ICB will continue to convene and support these and will be represented across them by the Chief Executive Officer and designated members of the ICB executive team.

5.5. The framework also recognises the critical role of place-based partnership arrangements in improving outcomes for local populations. Whilst place no longer forms part of the formal committee structure of the ICB, it remains a fundamental organising principle for delivery of integrated care, neighbourhood development and joint commissioning.

5.6. The ICB governance model therefore relies upon effective place-based partnership leadership across all six boroughs, bringing together NHS organisations, local authorities, primary care, voluntary sector partners and local communities to shape and deliver local priorities.

5.7. Health and Wellbeing Boards continue to play a particularly important role within this model. As statutory partnerships bringing together local government, the NHS and wider partners, Health and Wellbeing Boards provide an important forum for understanding local population need, shaping strategic priorities at place level, promoting integration and supporting delivery of improved outcomes for local residents. The ICB's governance framework assumes continued alignment between ICB governance arrangements and the strategic leadership exercised through Health and Wellbeing Boards at place level. This will include ICB support at place level for convening H&WBB chairs and Directors of Public Health and the ICB to coordinate the development of neighbourhood health plans

5.8. Future place-based responsibilities for neighbourhood development and joint commissioning will continue to be exercised through Place Executive Leads acting as officers of the ICB, operating within delegated authority and working closely with borough partners and place-based governance arrangements. This approach supports subsidiarity, strengthens local leadership and ensures that decisions are taken as close as possible to the populations and communities they affect whilst maintaining appropriate accountability to the ICB Board.

6. Components of the Governance Framework

Constitutional changes

6.1. The ICB Constitution establishes the formal governance architecture through which the Board discharges its responsibilities, delegates authority and exercises oversight. They provide the foundation upon which all other governance arrangements are based.

6.2. The framework includes a small number of proposed amendments to the Constitution to reflect the revised governance model and committee structure. These are included as appendix 1.

Scheme of Reservation and Delegation (SoRD) and Schedule of Matters Delegated (SoM)

6.3. The revised SoRD and SoM provides clarity regarding those decisions reserved to the Board and those delegated to committees, the Executive and officers of the ICB. It establishes clear accountability and decision-making routes and authorities, ensuring decisions are taken at the most appropriate level.

6.4. The ICB SoRD and SoM is included as appendix 2.

Standing Financial Instructions (SFIs)

6.5. The SFIs establish the organisation's financial governance framework, setting out financial responsibilities, controls and approval arrangements to ensure the proper stewardship of public resources and compliance with statutory and NHS requirements.

6.6. The SFIs have been reviewed and updated to align with the future governance model and revised committee structure, and are included as appendix 3.

Committee Terms of Reference

6.7. The Terms of Reference translate the governance framework into the practical operation of Board committees. Whilst the Constitution establishes the governance architecture, the SoRD and SoM define decision-making authority and the SFIs establish financial controls, the Terms of Reference define the specific purpose, responsibilities, membership, delegated authority and reporting arrangements for each committee. They provide clarity regarding the role each committee plays within the wider governance framework and how committees support the Board.

6.8. The proposed committee structure comprises:

- Strategic Commissioning Committee
- Neighbourhood Strategic Delivery Committee
- Patient Safety Committee
- Audit, Risk and Assurance Committee
- Remuneration Committee
- Charitable Funds Committee.
- ICB Transition Committee

6.9. Collectively, the committee structure is intended to provide a clear route through which strategy is translated into delivery, assurance is provided to the Board and delegated decision-making can be exercised at the appropriate level.

6.10. Committee terms of reference are included as appendices 5-11.

7. Board ways of working

7.1. The governance framework is underpinned by a Board Ways of Working statement which complements the formal governance documents by setting out the behaviours, expectations and operating principles that will support effective governance in practice.

7.2. Whilst the Constitution, delegations and committee Terms of Reference establish formal accountabilities and decision-making arrangements, the effectiveness of the governance framework will ultimately depend on how Board members, committee members, Executive leaders and officers work together to discharge their responsibilities.

7.3. The Ways of Working statement therefore reflects a collective commitment by Board members, committee members, Executive leaders and ICB officers to support effective governance.

7.4. In practice, this will be characterised by:

- A stronger focus on population outcomes, strategic priorities, major risks, delivery and significant trade-offs.
- Decisions being taken at the appropriate level, supported by clear accountability and effective delegation.
- Committees focusing on assurance, scrutiny, development of proposals and delegated decision-making, rather than duplicating Board or Executive responsibilities.
- Concise, evidence-based reporting that supports decision-making and assurance, with greater emphasis on insight, risks, opportunities and action.
- Constructive challenge and collective accountability, with Board members, committee members, Executive leaders and officers contributing actively to effective decision-making.
- Effective partnership working across South East London, recognising that delivery of the ICB's objectives depends upon collaboration with providers, local authorities, Health and Wellbeing Boards, primary care, the voluntary sector and wider partners.
- A culture that is strategic, outcome-focused, accountable and delivery-oriented, whilst maintaining appropriate assurance and oversight.

7.5. Together, these commitments are intended to support a governance culture that enables the organisation to make better decisions, provide stronger assurance and deliver improved outcomes for people in south east London.

7.6. The ICB Board ways of working statement is included as appendix 12.

8. Operationalising the ICB Governance Framework

8.1. Approval of the governance framework represents the beginning rather than the completion of the transition to the new governance model. Realising the intended benefits of the framework will require a coordinated programme of implementation across the ICB.

8.2. Following Board approval, work will be undertaken across the ICB's team to embed the new arrangements and support consistent operation of the framework.

Key implementation priorities will include:

8.3. Supporting Board members, committee members, Executive leaders and officers to understand their roles within the new governance framework and ways of working.

8.4. Establishing annual cycles of business and forward plans for the Board and committees, aligned to statutory requirements, strategic priorities and planning cycles.

8.5. Completing a set of approved guidance and related templates or agreed common formats to support the production of consistent information and reporting to committees and the Boards of both ICBs. This will be designed to align with the principles included in the ways of working document appended to this paper.

8.6. Designing and embedding a refreshed suite of integrated governance information products and assurance reports across both ICBs, reducing duplication and supporting a more strategic view of quality, performance, finance, workforce, transformation, delivery and risk across the organisation.

8.7. Alignment and mirroring of key ICB policies and procedures relating to corporate governance for SEL and SWL ICBs.

8.8. Utilising a part of initial meetings of committees to discuss and propose amendments to ToRs, which may include adjustments to both scope and membership, with proposed amendments being proposed to the ICB Board for review and approval. This will be supplemented by a six-month stocktake report to the Board on the implementation to date of this governance framework, with an opportunity to adjust and improve based on initial learnings.

8.9. Establishing mechanisms to monitor the effectiveness of the framework and identify opportunities for continuous improvement as the arrangements mature. This will be completed as an annual review of the governance framework, over and above the reviews of committee effectiveness.

9. Implementation of the framework

9.1. Subject to Board approval, the ICB Governance Framework and supporting governance documents will come into effect on 1st October 2026 following approval by the Board of both SEL and SWL ICBs. Elements of the implementation programme described in section 8 will continue during the initial period of operation

9.2. The revised committee structure, delegations, SFIs, and ways of working will collectively form the live governance framework through which the ICB operates from that point onwards.

9.3. The proposed constitutional amendments are subject to approval by NHS England in accordance with statutory requirements. Following Board approval, the necessary process for NHS England approval will be initiated. Pending formal approval by NHS England, the ICB will operate in accordance with the approved governance framework to the fullest extent permissible within the existing Constitution and applicable statutory requirements.

9.4. Any consequential updates to ICB corporate policies, governance handbook materials and supporting documentation will be completed following Board approval.

10. Documents presented for approval

10.1. The governance framework is supported by the following documents presented alongside this paper:

a. Proposed ICB constitutional amendments (appendix 1).

b. Scheme of Reservation and Delegation and Schedule of Matters Delegated (appendix 2b).

c. Standing Financial Instructions (appendix 3).

d. ICB headline governance structure (appendix 4)

e. Strategic Commissioning Committee Terms of Reference (appendix 5).

f. Neighbourhood Strategic Delivery Committee Terms of Reference (appendix 6).

g. Patient Safety Committee Terms of Reference (appendix 7).

h. Audit, Risk and Assurance Committee Terms of Reference (appendix 8).

i. Remuneration Committee Terms of Reference (appendix 9).

j. Charitable Funds Committee Terms of Reference (appendix 10).

k. Joint Transition Committee Terms of Reference (appendix 11)

l. ICB Board Ways of Working Statement (appendix 12).

11. Recommendation

11.1. The proposed ICB Governance Framework represents an integrated package of

governance arrangements designed to support the ICB's future operating model and strategic commissioning role.

11.2. The Board is asked to:

a. Approve the proposed ICB Governance Framework and supporting governance documents.

b. Approve the constitutional and governance amendments set out within the accompanying papers.

c. Note that the constitutional amendments require subsequent approval by NHS England and agree that the necessary approval process should be initiated following Board approval.

d. Agree that the ICB Governance Framework will take effect 1st October 2026 following Board approval and become the governance framework through which the ICB operates, subject to the constitutional approval requirements described above.

NHS South East London

Proposed changes to SEL ICB Constitution

NHS South East London ICB will retain its own Constitution to recognise its status as a statutory body in its own right. However, the Constitution will require amendment to reflect the changes in Board structure. At this point no other amendments are proposed.

Consequently, the Board is asked to consider an updated version of the Constitution, for submission to NHS England for approval, which has the following changes:

1.1. General changes throughout

- To update role name references for Chief Finance and Compliance Officer and Chief Nurse and Quality Officer.

1.2. Foreword section 1.1.

- Revised and simplified Foreword section drafted by ICB Chair.

1.3. Changes to section 2.2 Board membership

- Removing the six Place Executive Leads as Ordinary members
- Add a Chief Commissioning Officer as Ordinary member
- Confirm two additional non-executive members as Ordinary members (in addition to the minimum of two NEMs required by NHSE policy) – in other words a total of four NEMs

1.4. Changes to section 2.3. Regular Participants and Observers at Board Meetings

- Section 2.3. removed and replaced with a simplified addition to section 2.2. noting that in addition to Board members, 'The Chair may invite specified individuals to be Participants or Observers at its meetings in order to inform its decision-making and the discharge of its functions'.

1.5. Changes to Section 3 Appointments

- Update descriptions for role eligibility and appointment process to reflect the changes to Board membership detailed above

1.6. Changes to Section 4.1.

- Addition to specify a minimum of four Board meetings per year.

1.7. Changes to Section 4.7 Quorum

- Amend quoracy requirements to require at least three executive directors (from five previously) in attendance, update role names, and remove requirement to have at least three Place Executive Leads in attendance, with proportionate amendments for quoracy of last resort arrangements.

- Amend quoracy requirements to require at least one partner members (from three previously), with proportionate amendments for quoracy of last resort arrangements.

1.8. Changes to Section 4.9.

- Changes to the phraseology of the section on Board decision-making objectives to improve clarity.

NHS
South East London

NHS South East London Integrated Care Board

CONSTITUTION

Version	Date approved by the ICB	Effective date
V1.0	N/A	July 1st 2022
V1.1	Oct 2022	November 2nd 2022
	April 19th 2023	May 1st 2023
V1.3	April 17th 2024	May 14th 2024
V1.4	October 16th 2024	November 24th 2024
V1.5		

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1. Introduction

1.1 Foreword

South East London is one of the most diverse and dynamic places in the country. Its communities are rich in talent, resilience and ambition, yet too many people still experience avoidable inequalities in health and access to opportunity. The South East London Integrated Care Board is the steward of the NHS resources entrusted to us for our population. Through our responsibility for planning and commissioning NHS services across South East London, we work with our partners to ensure that resources are invested wisely, services meet the needs of our communities, and our decisions improve health, reduce inequalities and deliver value for the public. This Constitution and supporting documentation sets out the principles, responsibilities and governance through which we will fulfil that responsibility founded on accountability; inclusivity; collaboration and subsidiarity.

As a strategic commissioner, we will focus on the long term as well as the immediate. We will use evidence, insight and meaningful engagement with our communities to shape strategy, commission for outcomes, and invest where resources can make the greatest difference. We will champion prevention, tackle unwarranted variation, reduce health inequalities and ensure that every pound of public money delivers the greatest possible benefit. We will work through empowered places and neighbourhoods, recognising that decisions are often best made closest to the communities they affect, while providing the strategic leadership, assurance and stewardship needed to achieve the ambitions we share across South East London.

This Constitution and supporting documents reflects our belief that effective public service is founded on trust, partnership and responsible stewardship. It describes not only how we will govern ourselves, but how we will exercise leadership: by being clear about our purpose, transparent in our decisions, accountable for our performance and committed to continuous improvement. Working alongside patients, communities, staff, providers, local government and the voluntary, community and social enterprise sector, we will act with courage and humility to secure better health, better care and better value for current and future generations across South East London.

1.2 Name

1.2.1 The name of this Integrated Care Board is NHS South East London Integrated Care Board (“the ICB”).

1.3 Area Covered by the Integrated Care Board

1.3.1 The area covered by the ICB is south east London which includes the London Boroughs of Bexley, Bromley, Lambeth, Lewisham and Southwark and the Royal Borough of Greenwich.

1.4 Statutory Framework

1.4.1 The ICB is established by order made by NHS England under powers in the 2006 Act.

1.4.2 The ICB is a statutory body with the general function of arranging for the provision of services for the purposes of the health service in England and is an NHS body for the purposes of the 2006 Act.

1.4.3 The main powers and duties of the ICB to commission certain health services are set out in sections 3 and 3A of the 2006 Act. These provisions are supplemented by other statutory powers and duties that apply to ICBs, as well as by regulations and directions (including, but not limited to, those made under the 2006 Act).

1.4.4 In accordance with section 14Z25(5) of, and paragraph 1 of Schedule 1B to, the 2006 Act the ICB must have a Constitution, which must comply with the requirements set out in that Schedule. The ICB is required to publish its Constitution (section 14Z29). This Constitution is published at www.selondonics.org/icb

1.4.5 The ICB must act in a way that is consistent with its statutory functions, both powers and duties. Many of these statutory functions are set out in the 2006 Act but there are also other specific pieces of legislation that apply to ICBs. Examples include, but are not limited to, the Equality Act 2010 and the Children Acts. Some of the statutory functions that apply to ICBs take the form of general statutory duties, which the ICB must comply with when exercising its functions. These duties include but are not limited to:

a) Having regard to and acting in a way that promotes the NHS Constitution (section 2 of the Health Act 2009 and section 14Z32 of the 2006 Act)
b) Exercising its functions effectively, efficiently and economically (section 14Z33 of the 2006 Act)
c) Duties in relation children including safeguarding, promoting welfare etc (including the Children Acts 1989 and 2004, and the Children and Families Act 2014)
d) Adult safeguarding and carers (the Care Act 2014)
e) Equality, including the public-sector equality duty (under the Equality Act 2010) and the duty as to health inequalities (section 14Z35); and
f) Information law, (for instance, data protection laws, such as the UK General Data Protection Regulation 2016/679 and Data Protection Act 2018, and the Freedom of Information Act 2000)
g) Provisions of the Civil Contingencies Act 2004

1.4.6 The ICB is subject to an annual assessment of its performance by NHS England which is also required to publish a report containing a summary of the results of its assessment.

1.4.7 The performance assessment will assess how well the ICB has discharged its functions during that year and will, in particular, include an assessment of how well it has discharged its duties under:

a) section 14Z34 (improvement in quality of services)
b) section 14Z35 (reducing inequalities)
c) section 14Z38 (obtaining appropriate advice)
d) section 14Z40 (duty in respect of research)
e) section 14Z43 (duty to have regard to effect of decisions)
f) section 14Z45 (public involvement and consultation)
g) sections 223GB to 223N (financial duties) and
h) section 116B(1) of the Local Government and Public Involvement in Health Act 2007 (duty to have regard to assessments and strategies)

1.4.8 NHS England has powers to obtain information from the ICB (section 14Z60 of the 2006 Act) and to intervene where it is satisfied that the ICB is failing, or has failed, to discharge any of its functions or that there is a significant risk that it will fail to do so (section 14Z61).

1.5 Status of this Constitution

1.5.1 The ICB was established on 1 July 2022 by The Integrated Care Boards (Establishment) Order 2022, which made provision for its Constitution by reference to this document.

1.5.2 Changes to this Constitution will not be implemented until, and are only effective from, the date of approval by NHS England.

1.6 Variation of this Constitution

1.6.1 In accordance with paragraph 15 of Schedule 1B to the 2006 Act this Constitution may be varied in accordance with the procedure set out in this paragraph. The Constitution can only be varied in two circumstances:

a) where the ICB applies to NHS England in accordance with NHS England's published procedure and that application is approved; and
b) where NHS England varies the Constitution of its own initiative, (other than on application by the ICB).

1.6.2 The procedure for proposal and agreement of variations to the Constitution is as follows:

a) Any ICB board member can propose a change to the Constitution. This needs to be in writing to the ICB's chair or chief executive who will pass proposed changes to the ICB's governance team.
b) The ICB's governance team will advise the chair and chief executive on the reasonableness of proposed changes for consideration by the board for rejection or acceptance.
c) Following board acceptance of any changes the amended constitution will be presented to NHS England for approval.
d) Proposed amendments to this constitution will not be implemented until an application to NHS England for variation has been approved

1.7 Related Documents

1.7.1 This Constitution is also supported by a number of documents which provide further details on how governance arrangements in the ICB will operate.

1.7.2 The following are appended to the Constitution and form part of it for the purpose of clause 1.6 and the ICB's legal duty to have a Constitution:

a) Standing orders– which set out the arrangements and procedures to be used for meetings and the processes to appoint the ICB committees.

1.7.3 The following do not form part of the Constitution but are required to be published.

a) The Scheme of Reservation and Delegation (SoRD) sets out those decisions that are reserved to the board of the ICB and those decisions that have been delegated in accordance with the powers of the ICB and which must be agreed in accordance with and be consistent with the Constitution. The SoRD identifies where or to whom functions and decisions have been delegated to.

b) The Functions and Decision map is a high-level structural chart that sets out which key decisions are delegated and taken by which part or parts of the system. The Functions and Decision map also includes decision making responsibilities that are delegated to the ICB (for example, from NHS England).

c) The Standing Financial Instructions which set out the arrangements for managing the ICB's financial affairs.

d) The ICB Governance Handbook - This brings together all the ICB's governance documents, so it is easy for interested people to navigate. It includes:

- The above documents a) – c)

- Terms of reference for all committees and sub-committees of the board that exercise ICB functions
- Delegation arrangements for all instances where ICB functions are delegated, in accordance with section 65Z5 of the 2006 Act, to another ICB, NHS England, an NHS trust, NHS foundation trust, local authority, combined authority or any other prescribed body; or to a joint committee of the ICB and one of those organisations in accordance with section 65Z6 of the 2006 Act
- Terms of reference of any joint committee of the ICB and another ICB, NHS England, an NHS trust, NHS foundation trust, local authority, combined authority or any other prescribed body; or to a joint committee of the ICB and one or those organisations in accordance with section 65Z6 of the 2006 Act
- The up-to-date list of eligible providers of primary medical services under clause 3.7.2

e) **Key policy documents** which should also be included in the Governance Handbook or linked to it **–** including:
   - Standards of Business Conduct Policy (which includes the ICB's Conflicts of Interest Policy)
   - SEL ICS Working with People and Communities Strategic Framework Public Information Access Policy
   - Petitions Policy
   - Records Management Policy
   - Persistent and Unreasonable Contacts Policy
   - NHS Records Management Code of Practice 2021
   - Anti-fraud, Bribery and Corruption Policy

2 Composition of The Board of the ICB

2.1 Background

2.1.1 This part of the Constitution describes the membership of the Integrated Care Board. Further information about the criteria for the roles and how they are appointed is in section 3.

2.1.2 Further information about the individuals who fulfil these roles can be found on our website www.selondonics.org/icb.

2.1.3 In accordance with paragraph 3 of Schedule 1B to the 2006 Act, the membership of the ICB (referred to in this Constitution as "the board" and members of the ICB are referred to as "board Members") consists of:

a) a Chair
b) a Chief Executive
c) at least three Ordinary members.

2.1.4 The membership of the ICB (the board) shall meet as a unitary board and shall be collectively accountable for the performance of the ICB's functions.

2.1.5 NHS England Policy, requires the ICB to appoint the following additional Ordinary Members:

a) three executive members, namely:
   - Chief Financial Officer
   - Medical Director
   - Chief Nursing Officer
b) At least two non-executive members.

2.1.6 The Ordinary Members include at least three members who will bring knowledge and a perspective from their sectors. These members (known as Partner Members) are nominated by the following, and appointed in accordance with the procedures set out in Section 3 below**:**

- NHS trusts and foundation trusts who provide services within the ICB's area and are of a prescribed description
- the primary medical services (general practice) providers within the area of the ICB and are of a prescribed description
- the local authorities which are responsible for providing Social Care and whose area coincides with or includes the whole or any part of the ICB's area.

Whilst the Partner Members will bring knowledge and experience from their sector and will contribute the perspective of their sector to the decisions of the board, they are not to act as delegates of those sectors.

2.2 Board Membership

2.2.1 The ICB has five Partner Members as follows:

a) Partner Member NHS Trusts and Foundation Trusts X three
b) Partner Member Local Authorities
c) Partner Member Providers of Primary Medical Services

2.2.2 The ICB has also appointed the following additional Ordinary Members to the board:

a) Chief Commissioning Officer
b) Two non-executive members

2.2.3 The board is therefore composed of the following members:

a) Chair
b) Chief Executive
c) Three Partner members NHS and Foundation Trusts
d) One Partner member Primary medical services
e) One Partner member Local Authorities
f) FourNon-executive members (one of which, but not the Audit Committee Chair, will be appointed Deputy Chair; and one of which, who may be the Deputy Chair or the Audit Committee Chair, will be appointed the Senior Non-Executive Member)
g) Chief Finance & Compliance Officer
h) Medical Director
i) Chief Nurse & Quality Officer
j) Chief Commissioning Officer

2.2.4 The Chair will exercise their function to approve the appointment of the ordinary members with a view to ensuring that at least one of the Ordinary Members will have knowledge and experience in connection with services relating to the prevention, diagnosis and treatment of mental illness.

2.2.5 The board will keep under review the skills, knowledge, and experience that it considers necessary for members of the board to possess (when taken together) in order for the board to effectively carry out its functions and will take such steps as it considers necessary to address or mitigate any shortcoming.

2.2.6 The Chair may invite specified individuals to be Participants or Observers at its meetings in order to inform its decision-making and the discharge of its functions as it sees fit.

2.3 Regular Participants and Observers at Board Meetings

2.3.1 Participants will receive advanced copies of the notice, agenda and papers for board meetings. They may be invited to attend any or all of the board meetings, or part(s) of a meeting by the chair. Any such person may be invited, at the discretion of the chair to ask questions and address the meeting but may not vote.

2.3.2 Observers will receive advanced copies of the notice, agenda and papers for board meetings. They may be invited to attend any or all of the board meetings, or part(s) of a meeting by the chair. Any such person may not address the meeting and may not vote.

2.3.3 Participants and / or observers may be asked to leave the meeting by the Chair in the event that the board passes a resolution to exclude the public as per the Standing Orders.

3 Appointments Process for the Board

3.1 Eligibility Criteria for Board Membership:

3.1.1 Each member of the ICB must:

a) Comply with the criteria of the "fit and proper person test"
b) Be committed to upholding the Seven Principles of Public Life (known as the Nolan Principles)
c) Fulfil the requirements relating to relevant experience, knowledge, skills and attributes set out in a role specification.

3.2 Disqualification Criteria for Board Membership

3.2.1 A Member of Parliament.

3.2.2 A person whose appointment as a board member ("the candidate") is considered by the person making the appointment as one which could reasonably be regarded as undermining the independence of the health service because of the candidate's involvement with the private healthcare sector or otherwise.

3.2.3 A person who, within the period of five years immediately preceding the date of the proposed appointment, has been convicted

a) in the United Kingdom of any offence or
b) outside the United Kingdom of an offence which, if committed in any part of the United Kingdom, would constitute a criminal offence in that part, and, in either case, the final outcome of the proceedings was a sentence of imprisonment (whether suspended or not) for a period of not less than three months without the option of a fine.

3.2.4 A person who is subject to a bankruptcy restrictions order or an interim bankruptcy restrictions order under Schedule 4A to the Insolvency Act 1986, Part 13 of the Bankruptcy (Scotland) Act 2016 or Schedule 2A to the Insolvency (Northern Ireland) Order 1989 (which relate to bankruptcy restrictions orders and undertakings).

3.2.5 A person who, has been dismissed within the period of five years immediately preceding the date of the proposed appointment, otherwise than because of redundancy, from paid employment by any Health Service Body.

3.2.6 A person whose term of appointment as the chair, a member, a director or a governor of a health service body, has been terminated on the grounds:

a) that it was not in the interests of, or conducive to the good management of, the health service body or of the health service that the person should continue to hold that office
b) that the person failed, without reasonable cause, to attend any meeting of that health service body for three successive meetings

c) that the person failed to declare a pecuniary interest or withdraw from consideration of any matter in respect of which that person had a pecuniary interest or
d) of misbehaviour, misconduct or failure to carry out the person's duties

3.2.7 A Health Care Professional, meaning an individual who is a member of a profession regulated by a body mentioned in section 25(3) of the National Health Service Reform and Health Care Professions Act 2002, or other professional person who has at any time been subject to an investigation or proceedings, by any body which regulates or licenses the profession concerned ("the regulatory body"), in connection with the person's fitness to practise or any alleged fraud, the final outcome of which was:

a) the person's suspension from a register held by the regulatory body, where that suspension has not been terminated
b) the person's erasure from such a register, where the person has not been restored to the register
c) a decision by the regulatory body which had the effect of preventing the person from practising the profession in question, where that decision has not been superseded, or
d) a decision by the regulatory body which had the effect of imposing conditions on the person's practice of the profession in question, where those conditions have not been lifted.

3.2.8 A person who is subject to:

a) a disqualification order or disqualification undertaking under the Company Directors Disqualification Act 1986 or the Company Directors Disqualification (Northern Ireland) Order 2002, or
b) an order made under section 429(2) of the Insolvency Act 1986 (disabilities on revocation of administration order against an individual).

3.2.9 A person who has at any time been removed from the office of charity trustee or trustee for a charity by an order made by the Charity Commissioners for England and Wales, the Charity Commission, the Charity Commission for Northern Ireland or the High Court, on the grounds of misconduct or mismanagement in the administration of the charity for which the person was responsible, to which the person was privy, or which the person by their conduct contributed to or facilitated.

3.2.10 A person who has at any time been removed, or is suspended, from the management or control of any body under:

a) section 7 of the Law Reform (Miscellaneous Provisions) (Scotland) Act 1990(f) (powers of the Court of Session to deal with the management of charities), or

b) section 34(5) or of the Charities and Trustee Investment (Scotland) Act 2005 (powers of the Court of Session to deal with the management of charities)

3.3 Chair

3.3.1 The ICB Chair is to be appointed by NHS England, with the approval of the Secretary of State.

3.3.2 In addition to criteria specified at 3.1, this member must fulfil the following additional eligibility criteria

a) The Chair will be independent

3.3.3 Individuals will not be eligible if:

a) They hold a role in another health and care organisation within the ICB area
b) Any of the disqualification criteria set out in 3.2 apply

3.3.4 The term of office for the Chair will be a maximum of three years and the total number of terms a Chair may serve is two terms.

3.4 Deputy Chair and Senior Non-Executive Member

3.4.1 The Deputy Chair is to be appointed from amongst the non-executive members by the board subject to the approval of the Chair.

3.4.2 No individual shall hold the position of Chair of the Audit Committee and Deputy Chair at the same time.

3.4.3 The Senior Non-Executive Member is to be appointed from amongst the non-executive members by the board subject to the approval of the Chair.

3.5 Chief Executive

3.5.1 The Chief Executive will be appointed by the Chair of the ICB in accordance with any guidance issued by NHS England

3.5.2 The appointment will be subject to approval of NHS England in accordance with any procedure published by NHS England.

3.5.3 The Chief executive must fulfil the following additional eligibility criteria

a) Be an employee of the ICB or a person seconded to the ICB who is employed in the civil service of the State or by a body referred to in paragraph 19(4)(b) of Schedule 1B to the 2006 Act

3.5.4 Individuals will not be eligible if

a) Any of the disqualification criteria set out in 3.2 apply

b) Subject to clause 3.5.3(a), they hold any other employment or executive role

3.6 Three X Partner Members – NHS Trusts and Foundation trusts

3.6.1 These Partner Members are jointly nominated by the NHS Trusts and/or Foundation Trusts which provide services for the purposes of the health service within the ICB's area and meet the Forward Plan Condition or (if the Forward Plan Condition is not met) the Level of Services Provided Condition. Those organisations are:

a) Guy's & St Thomas' NHS Foundation Trust
b) King's College Hospital NHS Foundation Trust
c) Lewisham & Greenwich NHS Trust
d) South London & Maudsley NHS Foundation Trust
e) Oxleas NHS Foundation Trust
f) London Ambulance Service

3.6.2 These members must fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria:

a) Be a Chief Executive of one of the NHS Trusts or Foundation Trusts within the South East London ICB's area, listed at 3.5.1
b) They will bring perspectives across acute and emergency, mental health and community health services.
c) One of these partner members will have sufficient knowledge and experience in connection with services relating to the prevention, diagnosis and treatment of mental illness to meet the requirement of paragraph 2.2.4.

3.6.3 Individuals will not be eligible if

a) Any of the disqualification criteria set out in 3.2 apply

3.6.4 These members will be appointed by the ICB selection panel, subject to the approval of the Chair, following the appointment process detailed in section 3.6.5.

3.6.5 The appointment process will be as follows:

a) The ICB will create role descriptions for the Partner Members – NHS trusts / foundation trusts, which will set out the requirements associated with the roles, the expected skills, knowledge and expertise that is necessary, and the term of office.

Nomination process

b) The ICB will issue the role descriptions to the partners identified in section 3.6.1, together with a timeline for a nomination and selection process.

c) All eligible organisations, listed at 3.6.1, are invited to make their individual or shared nominations, for the partner members - NHS trusts and foundation trusts from the eligible partners listed at 3.6.1, to the ICB chair. The nominations should be accompanied by a brief statement setting out how the candidate meets the requirements of the job role.

d) Eligible organisations will be asked to confirm their joint agreement to the full list of nominees, to be formally proposed to the ICB; this constitutes jointly nominating all of the nominees for consideration for appointment by the ICB and chair. All nominating organisations will be contacted and asked whether they support the list of nominations to be submitted, with nil replies being counted as agreement; a simple majority of eligible partners supporting the list will be sufficient to submit it to the ICB for selection.

e) If agreement cannot be reached the nomination process will be re-run until majority acceptance is reached on the nominations put forward.

Selection process

f) Joint nominations will be considered by members of the selection panel to assess them against the criteria set out in the person specification and eligibility and disqualification criteria (clauses 3.6.2 and 3.6.3).

g) The ICB board will arrange a selection panel, who will assess the suitability against the criteria set, including the use of interviews as deemed appropriate, of each of the jointly nominated candidates for the partner member roles. This panel will be supported by a HR professional.

h) The ICB board selection panel will select the most suitable candidate for each role.

i) The chair of the ICB will determine whether to approve the appointment of the partner members.

j) The chair of the ICB will report the appointed partner members to the next meeting of the ICB board.

3.5.6 The term of office for these Partner Members will be three years. Individuals may serve a maximum of two terms. After a period of three years has passed since the end of their last term, an individual will become re-eligible for appointment. At the end of each term or should the selected member leave their Chief Executive role, or otherwise become ineligible for the partner member role, a new nomination and selection process will take place.

3.7 Partner Member - Providers of Primary Medical Services

3.7.1 This Partner Member is jointly nominated by providers of primary medical services for the purposes of the health service within the integrated care board's area, and that are primary medical services contract holders

responsible for the provision of essential services, within core hours to a list of registered persons for whom the ICB has core responsibility

3.7.2 The list of relevant providers of primary medical services for this purpose is published as part of the Governance Handbook. The list will be kept up to date but does not form part of this Constitution

3.7.3 This member must fulfil the eligibility criteria set out at 3.1 and will be selected from primary care providers within the South East London ICB area

3.7.4 Individuals will not be eligible if:

a) Any of the disqualification criteria set out in 3.2 apply

3.7.5 This member will be appointed by the ICB Board selection panel, subject to the approval of the Chair, following the appointment process detailed in section 3.7.6.

3.7.6 The appointment process will be as follows:

a) The ICB will create a role description for the Partner Member – Providers of primary medical services, which will set out the requirements associated with the roles, the expected skills, knowledge and expertise that is necessary, and the term of office.

Nomination process

b) The ICB will issue the role descriptions to all holders of a contract for core primary care services, who also hold a list of registered patients with a timeline for a nomination and selection process.

c) All eligible bodies are invited to make their individual or shared nominations to the ICB chair. This will be limited to one nomination per eligible body, which must be seconded by at least one other eligible body. Each nomination should be accompanied by a brief statement setting out how the candidate meets the requirements of the job role.

d) Eligible bodies will be asked to confirm their joint agreement to the full list of nominees, to be formally proposed to the ICB; this constitutes jointly nominating all nominees for consideration for appointment by the ICB and chair. All nominating bodies will be contacted and asked whether they support the list of nominations to be submitted, with nil replies being counted as agreement; a simple majority of eligible bodies supporting the list will be sufficient to submit it to the ICB for selection.

e) If agreement cannot be reached the nomination process will be re-run until majority acceptance is reached on the nominations put forward.

Selection process

f) Jointly nominated candidates will be shortlisted, by members of the selection panel, against the skills, knowledge and experience required to fulfil the role and eligibility and disqualification criteria (clauses 3.7.3 and 3.7.4).

g) The ICB board will arrange a selection panel, who will assess the suitability, including the use of interviews as deemed appropriate, of each of the shortlisted candidates for the partner member role. This panel will be supported by a HR professional.

h) The ICB board selection panel will select the most suitable candidate for the role.

i) The chair of the ICB will determine whether to approve the appointment of the partner member.

j) The chair of the ICB will report the appointed partner member(s) to the next meeting of the ICB board.

3.7.7 The term of office for this Partner Member will be three years. Individuals may serve a maximum of two terms. After a period of three years has passed since the end of their last term, an individual will become re-eligible for appointment. At the end of each term a new nomination and selection process will take place.

3.7.8 Should the selected member leave their role, within primary care, or step down prior to the completion of their term, or otherwise become ineligible for the role, a new nomination and selection process will take place.

3.8 Partner Member - Local Authorities

3.8.1 This partner member is jointly nominated by the local authorities whose areas coincide with, or include, the whole or any part of, the ICB's area. Those local authorities are:

a) London Borough of Bexley
b) London Borough of Bromley
c) Royal Borough of Greenwich
d) London Borough of Lambeth
e) London Borough of Lewisham
f) London Borough of Southwark

3.8.2 This member will fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria:

g) Be a Chief Executive, hold a relevant executive level role or be a cabinet member of one of the local authorities within the South East London ICB's area, listed at 3.8.1

3.8.3 Individuals will not be eligible if

a) Any of the disqualification criteria set out in 3.2 apply

3.8.4 This member will be appointed by the ICB Board selection panel, subject to the approval of the Chair, following the appointment process detailed in section 3.8.5.

3.8.5 The appointment process will be as follows:

a) The ICB will create a role description for the Partner Member – Local Authorities, which will set out the requirements associated with the role, the expected skills, knowledge and expertise that is necessary, and the term of office.

Nomination process

b) The ICB will issue the role description to the partners identified in section 3.8.1, together with a timeline for a nomination and selection process.

c) All eligible organisations, listed at 3.8.1, are invited to make their individual or shared nominations, for the partner member – Local Authorities from the eligible partners listed at 3.8.1, to the ICB chair. The nominations should be accompanied by a brief statement setting out how the candidate meets the requirements of the job role.

d) Eligible organisations will be asked to confirm their joint agreement to the full list of nominees, to be formally proposed to the ICB; this constitutes jointly nominating all of the nominees for consideration for appointment by the ICB and chair. All nominating organisations will be contacted and asked whether they support the list of nominations to be submitted, with nil replies being counted as agreement; a simple majority of eligible partners supporting the list will be sufficient to submit it to the ICB for selection.

e) If agreement cannot be reached the nomination process will be re-run until majority acceptance is reached on the nominations put forward.

Selection process

f) Joint nominations will be considered by a panel including the Chief Executive to assess the nominations against the criteria set out in the person specification and the eligibility and disqualification criteria (clauses 3.8.2 and 3.8.3).

g) The ICB board will arrange a selection panel, who will assess the suitability against the criteria set, including the use of interviews as deemed appropriate, of each of the jointly nominated candidates for the partner member role. This panel will be supported by a HR professional.

h) The ICB board selection panel will select the most suitable candidate for the role.

i) The chair of the ICB will determine whether to approve the appointment of the partner member.

j) The chair of the ICB will report the appointed partner member(s) to the next meeting of the ICB board.

3.8.6 The term of office for this Partner Member will be three years. Individuals may serve a maximum of two terms. After a period of three years has passed since the end of their last term, an individual will become re-eligible for appointment. At the end of each term or should the selected member leave their local authority role, or otherwise become ineligible, a new nomination and selection process will take place.

3.9 Medical Director

3.9.1 This member will fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria:

3.9.1.1 Be an employee of the ICB or a person seconded to the ICB who is employed in the civil service of the State or by a body referred to in paragraph 19(4)(b) of Schedule 1B to the 2006 Act

3.9.1.2 Be a registered Medical Practitioner

3.9.2 Individuals will not be eligible if:

3.9.2.1 Any of the disqualification criteria set out in 3.2 apply

3.9.3 This member will be appointed by the Chief Executive subject to the approval of the Chair.

3.10 Chief Nurse & Quality Officer

3.10.1 This member will fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria:

3.10.1.1 Be an employee of the ICB or a person seconded to the ICB who is employed in the civil service of the State or by a body referred to in paragraph 19(4)(b) of Schedule 1B to the 2006 Act

3.10.1.2 Be a registered Nurse

3.10.2 Individuals will not be eligible if:

3.10.2.1 Any of the disqualification criteria set out in 3.2 apply

3.10.3 This member will be appointed by the Chief Executive subject to the approval of the Chair.

3.11 Chief Finance & Compliance Officer

3.11.1 This member will fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria:

3.11.1.1 Be an employee of the ICB or a person seconded to the ICB who is employed in the civil service of the State or by a body referred to in paragraph 19(4)(b) of Schedule 1B to the 2006 Act

3.11.2 Individuals will not be eligible if:

3.11.2.1 Any of the disqualification criteria set out in 3.2 apply

3.11.3 This member will be appointed by the Chief Executive subject to the approval of the Chair

3.12 Chief Commissioning Officer

3.12.1 This member will fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria:

3.12.1.1 Be an employee of the ICB or a person seconded to the ICB who is employed in the civil service of the State or by a body referred to in paragraph 19(4)(b) of Schedule 1B to the 2006 Act

3.12.2 Individuals will not be eligible if:

3.12.2.1 Any of the disqualification criteria set out in 3.2 apply

3.12.3 This member will be appointed by the Chief Executive subject to the approval of the Chair

3.13 Four Non-Executive Members

3.13.1 The ICB will appoint four Non-Executive Members

3.13.2 These members will be appointed by a panel including the ICB Chair

3.13.3 These members will fulfil the eligibility criteria set out at 3.1 and also the following additional eligibility criteria

3.13.3.1 Not be an employee of the ICB or a person seconded to the ICB
3.13.3.2 Not hold a role in another health and care organisation in the ICS area
3.13.3.3 One shall have specific knowledge, skills and experience that makes them suitable for appointment to the Chair of the Audit Committee
3.13.3.4 Another should have specific knowledge, skills and experience that makes them suitable for appointment to the Chair of the Remuneration Committee

3.13.4 Individuals will not be eligible if

3.13.4.1 Any of the disqualification criteria set out in 3.2 apply

3.13.4.2 They hold a role in another health and care organisation within the ICB area

3.13.5 The term of office for a non-executive member will be three years and the total number of terms an individual may serve is two terms after which they will no longer be eligible for re-appointment.

3.13.6 Subject to satisfactory appraisal the Chair may approve the re-appointment of a non-executive member up to the maximum number of terms permitted for their role.

3.14 Board Members: Removal from Office

3.14.1 Arrangements for the removal from office of board members is subject to the term of appointment, and application of the relevant ICB policies and procedures.

3.14.2 With the exception of the Chair, board members shall be removed from office if any of the following occurs:

a) They no longer fulfil the requirements of their role or become ineligible for their role as set out in this Constitution, regulations or guidance

b) In the opinion of the Chair:

i. they fail to meet satisfactory performance standards in accordance with paragraph 3.1 of this Constitution

ii. they fail to attend a minimum of 75% of meetings to which they are invited unless agreed with the Chair in extenuating circumstances

iii. they are deemed to not meet the expected standards of performance at their annual appraisal

iv. they have behaved in a manner or exhibited conduct which has or is likely to be detrimental to the honour and interest of the ICB and is likely to bring the ICB into disrepute. This includes but it is not limited to dishonesty; misrepresentation (either knowingly or fraudulently); defamation of any member of the ICB (being slander or libel); abuse of position; non-declaration of a known conflict of interest; seeking to manipulate a decision of the ICB in a manner that would ultimately be in favour of that member whether financially or otherwise

v. they are subject to disciplinary proceedings by a regulator or professional body

3.14.3 Members may be suspended pending the outcome of an investigation into whether any of the matters in 3.14.2 apply.

3.14.4 Executive Directors (including the Chief Executive) will cease to be board members if their employment in their specified role ceases, regardless of the reason for termination of the employment.

3.14.5 The Chair of the ICB may be removed by NHS England, subject to the approval of the Secretary of State.

3.14.6 If NHS England is satisfied that the ICB is failing or has failed to discharge any of its functions or that there is a significant risk that the ICB will fail to do so, it may:

3.14.6.1 terminate the appointment of the ICB's Chief Executive; and
3.14.6.2 direct the Chair of the ICB as to which individual to appoint as a replacement and on what terms.

3.15 Terms of Appointment of Board Members

3.15.1 With the exception of the Chair and non-executive members arrangements for remuneration and any allowances will be agreed by the Remuneration Committee in line with the ICB remuneration policy and any other relevant policies published at www.selondonics.org/icb and any guidance issued by NHS England or other relevant body. Remuneration for Chairs will be set by NHS England. Remuneration for non-executive members will be set by a committee comprising the five partner members and approved by the Chair.

3.15.2 Other terms of appointment, outside of the Constitution, will be determined by the Remuneration Committee.

3.15.3 Terms of appointment of the Chair will be determined by NHS England.

4 Arrangements for the Exercise of our Functions

4.1 Good Governance

4.1.1 The ICB will, at all times, observe generally accepted principles of good governance. This includes the Nolan Principles of Public Life and any governance guidance issued by NHS England.

4.2 General

4.2.1 The ICB will:

a) comply with all relevant laws including but not limited to the 2006 Act and the duties prescribed within it and any relevant regulations
b) comply with directions issued by the Secretary of State for Health and Social Care
c) comply with directions issued by NHS England
d) have regard to statutory guidance including that issued by NHS England and
e) take account, as appropriate, of other documents, advice and guidance issued by relevant authorities, including that issued by NHS England
f) respond to reports and recommendations made by local Healthwatch organisations within the ICB area

4.2.2 The ICB will develop and implement the necessary systems and processes to comply with 4.2.1 (a)-(f), documenting them as necessary in this Constitution, its governance handbook and/or other relevant policies and procedures as appropriate.

4.3 Authority to Act

4.3.1 The ICB is accountable for exercising its statutory functions and may grant authority to act on its behalf to:

a) any of its members or employees
b) a committee or sub-committee of the ICB

4.3.2 Under section 65Z5 of the 2006 Act, the ICB may arrange with another ICB, an NHS trust, NHS foundation trust, NHS England, a local authority, combined authority or any other body prescribed in Regulations, for the ICB's functions to be exercised by or jointly with that other body or for the functions of that other body to be exercised by or jointly with the ICB. Where the ICB and other body enters such arrangements, they may also arrange for the functions in question to be exercised by a joint committee of theirs and/or for the establishment of a pooled fund to fund those functions (section 65Z6). In addition, under section 75 of the 2006 Act, the ICB may enter partnership arrangements with a local authority under which the local

authority exercises specified ICB functions or the ICB exercises specified local authority functions, or the ICB and local authority establish a pooled fund.

4.3.3 Where arrangements are made under section 65Z5 or section 75 of the 2006 Act the board must authorise the arrangement, which must be described as appropriate in the Scheme of Reservation and Delegation (SoRD).

4.4 Scheme of Reservation and Delegation

4.4.1 The ICB has agreed a scheme of reservation and delegation (SoRD) which is published in full at www.selondonics.org/icb

4.4.2 Only the board may agree the SoRD and amendments to the SoRD may only be approved by the board

4.4.3 The SoRD sets out:

a) those functions that are reserved to the Board
b) those functions that have been delegated to an individual or to committees and sub committees
c) those functions delegated to another body or to be exercised jointly with another body, under section 65Z5 and 65Z6 of the 2006 Act

4.4.4 The ICB remains accountable for all of its functions, including those that it has delegated. All those with delegated authority are accountable to the board for the exercise of their delegated functions.

4.5 Functions and Decision Map

4.5.1 The ICB has prepared a Functions and Decision Map which sets out at a high level its key functions and how it exercises them in accordance with the SoRD.

4.5.2 The Functions and Decision Map is published at www.selondonics.org/icb

4.5.3 The map includes:

a) Key functions reserved to the board of the ICB
b) Commissioning functions delegated to committees
c) Commissioning functions delegated under section 65Z5 and 65Z6 of the 2006 Act to be exercised by, or with, another ICB, an NHS trust, NHS foundation trust, local authority, combined authority or any other prescribed body
d) functions delegated to the ICB (for example, from NHS England)

4.6 Committees and Sub-Committees

4.6.1 The ICB may appoint committees and arrange for its functions to be exercised by such committees. Each committee may appoint sub-committees and arrange for the functions exercisable by the committee to be exercised by those sub-committees.

4.6.2 All committees and sub-committees are listed in the SoRD.

4.6.3 Each committee established by the ICB operates under terms of reference agreed by the board. Each sub-committee established by the ICB operates under terms of reference agreed by the committee to which it reports, as delegated by the board. All terms of reference are published in the Governance Handbook.

4.6.4 The board remains accountable for all functions, including those that it has delegated to committees and subcommittees and therefore, appropriate reporting and assurance arrangements are in place and documented in terms of reference. These include that the committee will report on its activities to the ICB Board via minutes. In addition, an accompanying report will summarise key points of discussion, items recommended for decisions, the key activities undertaken or coordinated by the committee and any actions agreed to be implemented. The minutes of meetings shall be formally recorded and reported to the ICB Board for the purposes of assurance and made publicly available as part of ICB meeting papers.

4.6.5 Any committee or sub-committee established in accordance with clause 4.6 may consist of or include persons who are not ICB Members or employees

4.6.6 All members of committees and sub-committees that exercise the ICB commissioning functions will be approved by the Chair. The Chair will not approve an individual to such a committee or sub-committee if they consider that the appointment could reasonably be regarded as undermining the independence of the health service because of the candidate's involvement with the private healthcare sector or otherwise.

4.6.7 All members of committees and sub-committees are required to act in accordance with this Constitution, including the standing orders as well as the Standing Financial Instructions (SFIs) and any other relevant ICB policy

4.6.8 The following committees will be maintained:

4.6.8.1 **Audit Committee:** This committee is accountable to the Board and provides an independent and objective view of the ICB's compliance with its statutory responsibilities. The committee is responsible for arranging appropriate internal and external audit.

The Audit Committee will be chaired by a Non-Executive member (other than the Chair and Deputy Chair of the ICB) who has the qualifications, expertise or experience to enable them to express credible opinions on finance and audit matters.

4.6.8.2 **Remuneration Committee:** This committee is accountable to the board for matters relating to remuneration, fees and other allowances (including pension schemes) for employees and other individuals who provide services to the ICB.

The Remuneration Committee will be chaired by a Non-Executive member other than the Chair or the Chair of the Audit Committee.

4.6.9 The terms of reference for each of the above committees are published in the governance handbook.

4.6.10 The board has also established a number of other committees to assist it with the discharge of its functions. These committees are set out in the SoRD and further information about these committees, including terms of reference, are published in the governance handbook.

4.7 Relationship between the Integrated Care Board, Integrated Care Partnership and Local Care Partnerships

4.7.1 Our Integrated care partnership will bring together leaders from across healthcare organisations, our local authorities and other partners to help set strategic direction and oversee the system, including developing an 'integrated care strategy' for health and care services. Our integrated care board will bring together senior leaders from health and care organisations and will have statutory responsibility for allocating resources for health and care within south east London and the planning and oversight of services. Groups of service providers in our system will work together in provider collaboratives to make better use of resources and improve how care is delivered. Local care partnerships within our six boroughs will be responsible for overseeing and improving out of hospital services.

4.8 Delegations made under section 65Z5 of the 2006 Act

4.8.1 As per 4.3.2 The ICB may arrange for any functions exercisable by it to be exercised by or jointly with any one or more other relevant bodies (another ICB, NHS England, an NHS trust, NHS foundation trust, local authority, combined authority or any other prescribed body).

4.8.2 All delegations made under these arrangements are set out in the ICB Scheme of Reservation and Delegation and included in the Functions and Decision Map.

4.8.3 Each delegation made under section 65Z5 of the Act will be set out in a delegation arrangement which sets out the terms of the delegation. This may, for joint arrangements, include establishing and maintaining a pooled fund. The power to approve delegation arrangements made under this provision will be reserved to the board.

4.8.4 The board remains accountable for all the ICB's functions, including those that it has delegated and therefore, appropriate reporting and assurance mechanisms are in place as part of agreeing terms of a delegation and these are detailed in the delegation arrangements, summaries of which will be published in the governance handbook.

4.8.5 In addition to any formal joint working mechanisms, the ICB may enter into strategic or other transformation discussions with its partner organisations on an informal basis.

5 Procedures for Making Decisions

5.1 Standing Orders

5.1.1 The ICB has agreed a set of standing orders which describe the processes that are employed to undertake its business. They include procedures for:

- conducting the business of the ICB
- the procedures to be followed during meetings and
- the process to delegate functions

5.1.2 The Standing Orders apply to all committees and sub-committees of the ICB unless specified otherwise in terms of reference which have been agreed by the board.

5.1.3 A full copy of the Standing Orders is included in Appendix 2 and form part of this Constitution.

5.2 Standing Financial Instructions (SFIs)

5.2.1 The ICB has agreed a set of SFIs which include the delegated limits of financial authority set out in the SoRD.

5.2.2 A copy of the SFIs is published at www.selondonics.org/icb

6 Arrangements for Conflict of Interest Management and Standards of Business Conduct

6.1 Conflicts of Interest

6.1.1 As required by section 14Z30 of the 2006 Act, the ICB has made arrangements to manage any actual and potential conflicts of interest to ensure that decisions made by the ICB will be taken and seen to be taken without being unduly influenced by external or private interest and do not, (and do not risk appearing to) affect the integrity of the ICB's decision-making processes.

6.1.2 The ICB has agreed policies and procedures for the identification and management of conflicts of interest, as part of the Standards of Business Conduct policy, which is published on the website, www.selondonics.org/icb.

6.1.3 All board, committee and sub-committee members and employees of the ICB will comply with the ICB policy on conflicts of interest, included within the Standards of Business Conduct policy, in line with their terms of office and / or employment. This will include but not be limited to declaring all interests on a register that will be maintained by the ICB.

6.1.4 All delegation arrangements made by the ICB under Section 65Z5 of the 2006 Act will include a requirement for transparent identification and management of interests and any potential conflicts in accordance with suitable policies and procedures comparable with those of the ICB.

6.1.5 Where an individual, including any individual directly involved with the business or decision-making of the ICB and not otherwise covered by one of the categories above, has an interest, or becomes aware of an interest which could lead to a conflict of interests in the event of the ICB considering an action or decision in relation to that interest, that must be considered as a potential conflict, and is subject to the provisions of this Constitution and the Standards of Business Conduct Policy, which includes the ICB's Conflicts Of Interest Policy.

6.1.6 The ICB has appointed the Audit Chair to be the Conflicts of Interest Guardian. In collaboration with the ICB's governance lead, their role is to:

- 6.1.6.1 Act as a conduit for members of the
- 6.1.6.2 and members of the partnership who have any concerns with regards to conflicts of interest
- 6.1.6.3 Be a safe point of contact for employees or workers to raise any concerns in relation to conflicts of interest
- 6.1.6.4 Support the rigorous application of conflict of interest principles and policies

6.1.6.5 Provide independent advice and judgment to staff and members where there is any doubt about how to apply conflicts of interest policies and principles in an individual situation

6.1.6.6 Provide advice on minimising the risks of conflicts of interest

6.2 Principles

6.2.1 In discharging its functions the ICB will abide by the principles included in the ICB's Standards of Business Conduct Policy as follows:

a) The 'Seven Principles of Public Life', also known as the Nolan principles

b) **Do business appropriately**: Conflicts of interest become much easier to identify, avoid and/or manage when the processes for needs assessments, consultation mechanisms, commissioning strategies and procurement procedures are right from the outset, because the rationale for all decision-making will be clear and transparent and should withstand scrutiny

c) **Be proactive, not reactive:** Everyone within the scope of this policy should seek to identify and minimise the risk of conflicts of interest at the earliest possible opportunity

d) **Be balanced, sensible and proportionate**: Rules should be clear and robust but not overly prescriptive or restrictive. They should ensure that decision-making is transparent and fair whilst not being overly constraining, complex or cumbersome

e) **Be transparent:** Document clearly the approach and decisions taken at every stage of NHS activity so that a clear audit trail is evident

f) Create an **environment and culture** where individuals feel supported and confident in declaring relevant information and raising any concerns

6.3 Declaring and Registering Interests

6.3.1 The ICB maintains registers of the interests of:

a) Members of the ICB
b) Members of the board's committees and sub-committees
c) Its employees

6.3.2 In accordance with section 14Z30(2) of the 2006 Act, registers of interest are published on the ICB website, www.selondonics.org/icb.

6.3.3 All relevant persons as per 6.1.3 and 6.1.5 must declare any conflict or potential conflict of interest relating to decisions to be made in the exercise of the ICB's commissioning functions.

6.3.4 Declarations should be made as soon as reasonably practicable after the person becomes aware of the conflict or potential conflict and in any event within 28 days. This could include interests an individual is pursuing.

Interests will also be declared on appointment and during relevant discussion in meetings.

6.3.5 All declarations will be entered in the registers as per 6.3.1

6.3.6 The ICB will ensure that, as a matter of course, declarations of interest are made and confirmed, or updated at least annually.

6.3.7 Interests (including gifts and hospitality) of decision-making staff will remain on the public register for a minimum of six months. In addition, the ICB will retain a record of historic interests and offers/receipt of gifts and hospitality for a minimum of six years after the date on which it expired. The ICB's published register of interests states that historic interests are retained by the ICB for the specified timeframe and details of whom to contact to submit a request for this information.

6.3.8 Activities funded in whole or in part by third parties who may have an interest in South East London ICB business such as sponsored events, posts and research will be managed in accordance with the ICB policy to ensure transparency and that any potential for conflicts of interest are well-managed.

6.4 Standards of Business Conduct

6.4.1 Board members, employees, committee and sub-committee members of the ICB will at all times comply with this Constitution and be aware of their responsibilities as outlined in it. They should:

a) act in good faith and in the interests of the ICB
b) follow the Seven Principles of Public Life; set out by the Committee on Standards in Public Life (the Nolan Principles)
c) comply with the ICB Standards of Business Conduct Policy which includes the requirements for managing conflicts of interest

6.4.2 Individuals contracted to work on behalf of the ICB or otherwise providing services or facilities to the ICB will be made aware of their obligation to declare conflicts or potential conflicts of interest. This requirement will be written into their contract for services and is also outlined in the ICB's Standards of Business Conduct policy.

7 Arrangements for ensuring Accountability and Transparency

7.1 The ICB will demonstrate its accountability to local people, stakeholders and NHS England in a number of ways, including by upholding the requirement for transparency in accordance with paragraph 12(2) of Schedule 1B to the 2006 Act.

7.2 Meetings and publications

7.2.1 Board and committee meetings composed entirely of board members, or which include all board members, will be held in public except where a resolution is agreed to exclude the public on the grounds that it is believed to not be in the public interest.

7.2.2 Papers and minutes of all meetings held in public will be published.

7.2.3 Annual accounts will be externally audited and published.

7.2.4 A clear complaints process will be published.

7.2.5 The ICB will comply with the Freedom of Information Act 2000 and with the Information Commissioner Office requirements regarding the publication of information relating to the ICB.

7.2.6 Information will be provided to NHS England as required.

7.2.7 The Constitution and governance handbook will be published as well as other key documents including but not limited to:

7.2.7.1 Standards of Business Conduct policy
7.2.7.2 Registers of interests
7.2.7.3 Key ICB policies

7.2.8 The ICB will publish, with our partner NHS trusts and NHS foundation trusts, a plan at the start of each financial year that sets out how the ICB proposes to exercise its functions during the next five years (the “Joint Forward Plan”). The plan will in particular:

a) describe the health services for which the ICB proposes to make arrangements in the exercise of its functions
b) explain how the ICB proposes to discharge its duties under:
    - sections 14Z34 to 14Z45 (general duties of integrated care boards), and
    - sections 223GB and 223N (financial duties).
c) set out any steps that the ICB proposes to take to implement the joint local health and wellbeing strategies for the following:
    - London Borough of Bexley
    - London Borough of Bromley

- Royal Borough of Greenwich
- London Borough of Lambeth
- London Borough of Lewisham
- London Borough of Southwark

d) set out any steps that the ICB proposes to take to address the particular needs of children and young persons under the age of 25.

e) set out any steps that the ICB proposes to take to address the particular needs of victims of abuse (including domestic abuse and sexual abuse, whether of children or adults).

7.3 Scrutiny and decision-making

7.3.1 At least four non-executive members will be appointed to the board including the Chair; and all of the board and committee members will comply with the Nolan Principles of Public Life and meet the criteria described in the Fit and Proper Person Test.

7.3.2 Healthcare services will be arranged in a transparent way, and decisions around who provides services will be made in the best interests of patients, taxpayers and the population, in line with the rules set out in the NHS Provider Selection Regime (PSR).

7.3.3 The ICB will comply with the requirements of the NHS Provider Selection Regime, including:

a) Ensuring that its planning and commissioning processes are compliant with the new PSR.

b) Ensuring that ICB policies reflect PSR requirements.

c) Providing training with planning, commissioning and finance teams to ensure a clear understanding of requirements.

d) Ensuring that governance is in place around the transparency of decision making, including the use of Local Care Partnerships for local commissioning decisions and the ICB Executive around south east London wide commissioning decisions

e) Ensuring that a contracts register is in place to enable oversight of the ICB's contract portfolio and the procurement pipeline, which will be overseen by the ICB's Planning and Delivery Group.

7.3.4 The ICB will comply with local authority health overview and scrutiny requirements.

7.4 Annual Report

7.4.1 The ICB will publish an annual report in accordance with any guidance published by NHS England and which sets out how it has discharged its functions and fulfilled its duties in the previous financial year. An annual report must in particular:

a) explain how the ICB has discharged its duties under section 14Z34 to 14Z45 and 14Z49 (general duties of integrated care boards)

b) review the extent to which the ICB has exercised its functions in accordance with the plans published under section 14Z52 (forward plan) and section 14Z56 (capital resource use plan)

c) review the extent to which the ICB has exercised its functions consistently with NHS England's views set out in the latest statement published under section 13SA(1) (views about how functions relating to inequalities information should be exercised), and

d) review any steps that the ICB has taken to implement any joint local health and wellbeing strategy to which it was required to have regard under section 116B(1) of the Local Government and Public Involvement in Health Act 2007.

8 Arrangements for Determining the Terms and Conditions of Employees

8.1 The ICB may appoint employees, pay them remuneration and allowances as it determines and appoint staff on such terms and conditions as it determines.

8.2 The board has established a Remuneration Committee which is chaired by a Non-Executive member other than the Chair or Audit Chair.

8.3 The membership of the Remuneration Committee is determined by the board. No employees may be a member of the Remuneration Committee, but the board ensures that the Remuneration Committee has access to appropriate advice by attendance of a senior HR representative from the ICB at meetings.

8.4 The board may appoint independent members or advisers to the Remuneration Committee who are not members of the Board.

8.5 The main purpose of the Remuneration Committee is to exercise the functions of the ICB regarding remuneration included in paragraphs 18 to 20 of Schedule 1B to the 2006 Act. The terms of reference agreed by the board are published in the Governance Handbook.

8.6 The duties of the Remuneration Committee are detailed in its terms of reference, which can be found in the Governance Handbook, and include:

- Confirming the ICB Pay Policy including adoption of any pay frameworks, including Agenda for Change, for all employees including senior managers/directors (including board members)

8.7 The ICB may make arrangements for a person to be seconded to serve as a member of the ICB's staff.

9 Arrangements for Public Involvement

9.1 In line with section 14Z45 of the 2006 Act the ICB has made arrangements to ensure that individuals to whom services which are, or are to be, provided pursuant to arrangements made by the ICB in the exercise of its functions, and their carers and representatives, are involved (whether by being consulted or provided with information or in other ways) in:

a) the planning of the commissioning arrangements by the Integrated Care Board

b) the development and consideration of proposals by the ICB for changes in the commissioning arrangements where the implementation of the proposals would have an impact on the manner in which the services are delivered to the individuals (at the point when the service is received by them), or the range of health services available to them, and

c) decisions of the ICB affecting the operation of the commissioning arrangements where the implementation of the decisions would (if made) have such an impact

9.2 In line with section 14Z54 of the 2006 Act the ICB has made arrangements to consult and engage its population on its system plan as outlined in the SEL ICS working with people and communities strategy available on the ICS website.

9.3 The ICB has adopted the ten principles set out by NHS England for working with people and communities:

a) Put the voices of people and communities at the centre of decision-making and governance, at every level of the ICS.

b) Start engagement early when developing plans and feed back to people and communities how it has influenced activities and decisions.

c) Understand your community's needs, experience and aspirations for health and care, using engagement to find out if change is having the desired effect.

d) Build relationships with excluded groups – especially those affected by inequalities.

e) Work with Healthwatch and the voluntary, community and social enterprise sector as key partners.

f) Provide clear and accessible public information about vision, plans and progress to build understanding and trust.

g) Use community development approaches that empower people and communities, making connections to social action.

h) Use co-production, insight and engagement to achieve accountable health and care services.

i) Co-produce and redesign services and tackle system priorities in partnership with people and communities.

j) Learn from what works and build on the assets of all partners in the ICS – networks, relationships, activity in local places.

9.4 These principles will be used when developing and maintaining arrangements for engaging with people and communities.

9.5 These arrangements, include:

a) Outreach
b) Working with trusted voices in VCSE and faith organisations
c) Community champions
d) Citizens' Panel
e) Let's talk health and care in south east London engagement platform
f) Working with people with lived experience
g) Co-production

Appendix 1: Definitions of Terms Used in This Constitution

2006 Act	National Health Service Act 2006, as amended by the Health and Social Care Act 2012 and the Health and Care Act 2022
ICB Board	Members of the ICB
Area	The geographical area that the ICB has responsibility for, as defined in part 1.3 of this Constitution
Committee	A committee created and appointed by the ICB Board.
Sub-Committee	A committee created and appointed by and reporting to a committee.
Forward Plan Condition	The 'Forward Plan Condition' as described in the Integrated Care Boards (Nomination of Ordinary Members) Regulations 2022 and any associated statutory guidance.
Level of Services Provided Condition	The 'Level of Services Provided Condition' as described in the Integrated Care Boards (Nomination of Ordinary Members) Regulations 2022 and any associated statutory guidance.
Integrated Care Partnership	The joint committee for the ICB's area established by the ICB and each responsible local authority whose area coincides with or falls wholly or partly within the ICB's area.
Local Care Partnership	Local Care Partnerships are collaborative arrangements responsible for arranging and delivering health and care services in a locality or community. They involve the Integrated Care Board, local government and providers of health and care services, including the voluntary, community and social enterprise sector, people and communities, as well as primary care provider leadership, represented by Primary Care Network clinical directors or other relevant primary care leaders
Provider Collaborative	Provider Collaboratives are collaborative arrangements that take responsibility for the planning, delivery and improvement of services on an area basis where there is benefit in adopting a collaborative rather than a by provider approach. SEL has a provider collaborative for acute services (the Acute Provider Collaborative) and for mental health (the South London Partnership). Provider

	Collaboratives form part of the formal system architecture and governance of the Integrated Care Body and Integrated Care System
Primary Care Leadership Group	The primary care leadership group is a networked primary care collaborative which encourages sharing and learning across south east London to secure improved primary care services to meet the needs of its local community within a principle of equality for all. It provides a forum for engagement with a wide-ranging group of primary care leaders who are able to work with conflict and difference and provide clear direction to the system.
Ordinary Member	The Board of the ICB will have a Chair and a Chief Executive plus other members. All other members of the Board are referred to as Ordinary Members.
Partner Members	Some of the Ordinary Members will also be Partner Members. Partner Members bring knowledge and a perspective from their sectors and are and appointed in accordance with the procedures set out in Section 3 having been nominated by the following: • NHS trusts and foundation trusts who provide services within the ICB's area and are of a prescribed description • the primary medical services (general practice) providers within the area of the ICB and are of a prescribed description • the local authorities which are responsible for providing Social Care and whose area coincides with or includes the whole or any part of the ICB's area.
Health Service Body	Health service body as defined by section 9(4) of the NHS Act 2006 or (b) NHS Foundation Trusts.
Chief Nurse & Quality Officer	This is NHS South East London ICB's name for the Director of Nursing. The Director of Nursing is a required additional ordinary member according to NHS England Policy

Chief Finance & ComplianceOfficer	This is NHS South East London ICB's name for the Director of Finance. The Director of Finance is a required additional ordinary member according to NHS England Policy
Health Care Professional	An individual who is a member of a profession regulated by a body mentioned in section 25(3) of the National Health Service Reform and Health Care Professions Act 2002

Appendix 2: Standing Orders

1. Introduction

1.1. These Standing Orders have been drawn up to regulate the proceedings of NHS South East London Integrated Care Board so that the ICB can fulfil its obligations as set out largely in the 2006 Act (as amended). They form part of the ICB's Constitution.

2. Amendment and review

2.1. Standing Orders will be reviewed on an annual basis or sooner if required.

2.2. Amendments to these Standing Orders will be made as per clause 1.6 of the Constitution.

2.3. All changes to these Standing Orders will require an application to NHS England for variation to the ICB Constitution and will not be implemented until the Constitution has been approved.

3. Interpretation, application and compliance

3.1. Except as otherwise provided, words and expressions used in these Standing Orders shall have the same meaning as those in the main body of the ICB Constitution and as per the definitions in Appendix 1.

3.2. These standing orders apply to all meetings of the Board, including its committees and sub-committees unless otherwise stated. All references to Board are inclusive of committees and sub-committees unless otherwise stated.

3.3. All members of the Board, members of committees and sub-committees and all employees, should be aware of the Standing Orders and comply with them. Failure to comply may be regarded as a disciplinary matter.

3.4. In the case of conflicting interpretation of the Standing Orders, the Chair, supported with advice from the ICB's governance department will provide a settled view which shall be final.

3.5. All members of the Board, its committees and sub-committees and all employees have a duty to disclose any non-compliance with these Standing Orders to the Chief Executive as soon as possible.

3.6. If, for any reason, these Standing Orders are not complied with, full details of the non-compliance and any justification for non-compliance and the circumstances around the non-compliance, shall be reported to the next formal meeting of the Board for action or ratification and the Audit Committee for review.

4. Meetings of the Integrated Care Board

4.1. Calling Board Meetings

4.1.1. Meetings of the Board of the ICB shall be held at regular intervals a minimum of 4 times per year and at such times and places as the ICB may determine.

4.1.2. In normal circumstances, each member of the Board will be given not less than one month's notice in writing of any meeting to be held. However:

a) The Chair may call a meeting at any time by giving not less than 14 calendar days' notice in writing.

b) One third of the members of the Board may request the Chair to convene a meeting by notice in writing, specifying the matters which they wish to be considered at the meeting. If the Chair refuses, or fails, to call a meeting within seven calendar days of such a request being presented, the Board members signing the requisition may call a meeting by giving not less than 14 calendar days' notice in writing to all members of the Board specifying the matters to be considered at the meeting.

c) In emergency situations the Chair may call a meeting with two days' notice by setting out the reason for the urgency and the decision to be taken.

4.1.3. A public notice of the time and place of meetings to be held in public and how to access the meeting shall be given by posting it at the offices of the ICB body and electronically at least three clear days before the meeting or, if the meeting is convened at shorter notice, then at the time it is convened.

4.1.4. The agenda and papers for meetings to be held in public will be published electronically in advance of the meeting excluding, if thought fit, any item likely to be addressed in part of a meeting not likely to be open to the public.

4.2. Chair of a meeting

4.2.1. The Chair of the ICB shall preside over meetings of the Board.

4.2.2. If the Chair is absent or is disqualified from participating by a conflict of interest, the Deputy Chair shall preside over meetings in the Chair's stead.

4.2.3. If both the Chair and Deputy Chair are absent or disqualified from participating by a conflict of interest, the assembled members shall appoint a temporary Deputy, from the remaining non-conflicted Non-Executive Members, for the purpose of chairing the meeting.

4.2.4. The Board shall appoint a Chair to all committees and sub-committees that it has established. The appointed committee or sub-committee Chair will preside over the relevant meeting. Terms of reference for committees and sub-committees will specify arrangements for occasions when the appointed Chair is absent.

4.3. Agenda, supporting papers and business to be transacted

4.3.1. The agenda for each meeting will be drawn up and agreed by the Chair of the meeting.

4.3.2. Except where the emergency provisions apply, supporting papers for all items must be submitted at least seven calendar days before the meeting takes place. The agenda and supporting papers will be circulated to all members of the Board at least five calendar days before the meeting.

4.3.3. Agendas and papers for meetings open to the public, including details about meeting dates, times and venues, will be published on the ICB's website at www.selondonics.org/icb.

4.4. Petitions

4.4.1. Where a valid petition has been received by the ICB it shall be included as an item for the agenda of the next meeting of the Board in accordance with the ICB policy as published in the Governance Handbook.

4.5. Nominated Deputies

4.5.1. Board Members are expected to attend all Board meetings and as such deputies will not be permitted to attend on their behalf.

4.6. Virtual attendance at meetings

4.6.1. The Board of the ICB and its committees and sub-committees may meet virtually using telephone, video and other electronic means when necessary, unless the terms of reference prohibit this.

4.7. Quorum

4.7.1. The quorum for meetings of the Board will be at least 50% of members, rounded up to the next whole number member and must also include:

a) At least 3 executive members including the Medical Director or Chief Nurse & Quality Officer and Chief Executive or Chief Finance & Compliance Officer
b) At least 3 non-executive members
c) At least 1 partner member

4.7.2. Where a quorum cannot be convened from the membership of the meeting, including owing to the arrangements for managing conflicts of interest or potential conflicts of interest, the Chair of the meeting shall consult with the Board on the action to be taken. This may include (such a position shall be recorded in the minutes of the meeting):

a) deferring the discussion and/or the passing of a resolution. The meeting must then proceed to the next business item.
b) requiring another of the ICB's committees or sub-committees (as appropriate) to progress the item of business, or if this is not possible)
c) as a last resort, reducing the quorum to 40% of members but must include:
   i) At least 2 Executive members including the Medical Director or Chief Nurse & Quality Officer and the Chief Executive or Chief Finance & Compliance Officer ii) At least 2 Non-Executive members
   iii) At least 1 partner members

4.7.3. For the sake of clarity:

a) No person can act in more than one capacity when determining the quorum.
b) An individual who has been disqualified from participating in a discussion on any matter and/or from voting on any motion by reason of a declaration of a conflict of interest, shall no longer count towards the quorum.

4.7.4. For all committees and sub-committees, the details of the quorum for these meetings and status of deputies are set out in the appropriate terms of reference.

4.8. Vacancies and defects in appointments

4.8.1. The validity of any act of the ICB is not affected by any vacancy among members or by any defect in the appointment of any member.

4.8.2. In the event of a vacancy or defect in appointment the quorum will remain as outlined in section 4.7 Quorum (i.e., there will be no reduction in the quoracy outlined in these standing orders).

4.9. Decision making

4.9.1. The ICB has agreed to use a collective model of decision-making that seeks to find consensus and make decisions based on unanimity as the norm..

4.9.2. Generally it is expected that decisions of the ICB will be reached by consensus. Should this not be possible then a vote will be required. The process for voting, which should be considered a last resort, is set out below:

a) All members of the Board who are present at the meeting will be eligible to cast one vote each.

b) In no circumstances may an absent member vote by proxy. Absence is defined as being absent at the time of the vote, but this does not preclude anyone attending by teleconference or other virtual mechanism from participating in the meeting, including exercising their right to vote if eligible to do so.

c) For the sake of clarity, any additional participants and observers will not have voting rights.

d) A resolution will be passed if more votes are cast for the resolution than against it, on condition that the votes cast for / against are not entirely executive votes.

e) If an equal number of votes are cast for and against a resolution, then the Chair (or in their absence, the person presiding over the meeting) will have a second and casting vote.

f) Should a vote be taken, the outcome of the vote, and any dissenting views, must be recorded in the minutes of the meeting.

<u>Disputes</u>

4.9.3. Where helpful, the board may draw on third party support to assist them in resolving any disputes, such as peer review or support from NHS England.

Urgent decisions

4.9.4. In the case of urgent decisions and extraordinary circumstances, every attempt will be made for the Board to meet virtually. Where this is not possible the following will apply:

a) The powers which are reserved or delegated to the Board, may for an urgent decision be exercised by the ICB Chair or Chief Executive (or relevant Chair in the case of committees) subject to every effort having been made to consult with as many members as possible in the circumstances. This would usually include at least two other Board representatives, including at least one non-executive. In the case of committee Chairs this consultation should also include the Chair and / or Chief Executive.

b) These emergency action functions may be exercised by such other persons as the Chair and Chief Executive may respectively nominate in writing.

c) The exercise of such powers shall be reported to the next formal meeting of the Board for formal ratification and a log maintained for inspection by the audit committee.

4.10. Minutes

4.10.1. The names and roles of all members present shall be recorded in the minutes of the meetings.

4.10.2. The minutes of a meeting shall be drawn up and submitted for agreement at the next meeting where they shall be signed by the person presiding at it.

4.10.3. No discussion shall take place upon the minutes except upon their accuracy or where the person presiding over the meeting considers discussion appropriate.

4.10.4. Where providing a record of a meeting held in public, the minutes shall be made available to the public.

4.11. Admission of the public and the press

4.11.1. In accordance with Public Bodies (Admission to Meetings) Act 1960 All meetings of the Board and all meetings of committees which are comprised of entirely board members or all board members, at which public functions are exercised, will be open to the public.

4.11.2. The Board may resolve to exclude the public from a meeting or part of a meeting where it would be prejudicial to the public interest by reason of the confidential nature of the business to be transacted or for other special reasons stated in the resolution and arising from the nature of that business or of the proceedings or for any other reason permitted by the Public Bodies (Admission to Meetings) Act 1960 as amended or succeeded from time to time.

4.11.3. The person presiding over the meeting shall give such directions as he/she thinks fit with regards to the arrangements for meetings and accommodation of the public and representatives of the press such as to ensure that the ICB's business shall be conducted without interruption and disruption.

4.11.4. As permitted by Section 1(8) Public Bodies (Admissions to Meetings) Act 1960 as amended from time to time) the public may be excluded from a meeting to suppress or prevent disorderly conduct or behaviour.

4.11.5. Matters to be dealt with by a meeting following the exclusion of representatives of the press, and other members of the public shall be confidential to the members of the Board.

5. Suspension of Standing Orders

5.1. In exceptional circumstances, except where it would contravene any statutory provision or any direction made by the Secretary of State for Health and Social Care or NHS England, any part of these Standing Orders may be suspended by the Chair in agreement with at least two other members.

5.2. A decision to suspend Standing Orders together with the reasons for doing so shall be recorded in the minutes of the meeting.

5.3. A separate record of matters discussed during the suspension shall be kept. These records shall be made available to the Audit Committee for review of the reasonableness of the decision to suspend Standing Orders.

6. Use of Seal and Authorisation of Documents

6.1 The ICB has a seal for executing documents where necessary. The seal shall be kept in safe custody by the Chief Executive Officer or a person appointed by the Chief Executive Officer.

6.2 The following individuals or officers can authenticate the seal's use by their signature. Two of the following are required to seal documents:

- The Chief Executive Officer
- The Chief Finance & Compliance Officer
- The Medical Director
- The Chief Nurse & Quality Officer

NHS South East London ICB & NHS South West London ICB Integrated Scheme of Reservation & Delegation

The arrangements made by NHS South East London Integrated Care Board (SEL) and NHS South West London Integrated Care Board (SWL) for the reservation and delegation of decisions are set out in this integrated Scheme of Reservation and Delegation (SoRD).

Each ICB remains accountable for all its functions, including those that it has delegated, however acknowledging that the current clustering arrangements in place between SEL and SWL, as the consequent Board roles held by individuals across both ICB Boards, the delegations are combined within a single document for ease of reference.

Given the individual statutory status of each ICB, delegations may differ between ICBs. These are clearly noted in the document and must be adhered to.

This SoRD may be amended from time to time to reflect changes in legislation, ICB policy or operational requirements.

The ICB will coordinate regular reviews of financial, procurement and contracts signing financial delegations for positions and limits.

This document should be read in conjunction with Board committee terms of reference, which detail the responsibilities within the remit of each committee. Specific delegations from the Board to committee are shown in the tables below.

A revised version is submitted to the Senior Management Team and Audit, Risk and Assurance Committees for endorsement before submission to the respective ICB Boards for approval.

Accountability

ICB employees are reminded of their accountability under this Scheme of Reservation and Delegation, and the associated obligations.

As an officer of the ICB with delegated authorities, as detailed in this document, you have personal accountability for:

- Complying with all the authority limits set out in this document, noting that variations exist between SEL ICB and SWL ICB delegated limits, and applying these appropriately.
- Delivering your service within the budgetary limits set, unless express agreement to a variance to budget is reached with the respective delegated officers in advance
- Regularly monitoring income and expenditure against agreed budgets, including meeting your management accountant on a monthly basis.
- Highlighting any significant actual or forecast variances to your executive director (or the CFCO if you are an executive director yourself) and finance business partner as early as possible in order that mitigating actions can be developed and implemented.
- Ensuring awareness and compliance to the ICBs counter fraud, bribery and corruption policies within your teams at all times.
- Ensuring adherence to the respective ICBs Standard of Business Conduct Policy.
- Complying with all organisational policies and procedures.
- Ensuring appropriate local systems are in place to ensure these delegations are operated effectively.
- Providing the necessary reports to respective ICB committees, to enable those committees to conduct their assurance duties as required.

Failure to adhere to organisational policies and procedures, including adherence with the delegated authority limits within the Scheme of Reservation and Delegation and Operational Scheme of Delegation may result in disciplinary action in line with the organisations Policy.

Section A:

NHS South East London ICB & NHS South West London ICB Integrated Scheme of Reservation & Delegation

V1

Decision	Reserved or delegated to Board	Chief Executive	Chief Finance and Compliance Officer	Committees and Sub-committees
STRATEGY				
Agree the vision and values and overall strategic direction of the ICB	✓			
Agree the overall South East London and South West London Neighbourhood strategy	✓			
Agree joint working on estates, procurement, supply chain and commercial strategies to maximise value for money across the system and support wider goals of development and sustainability	✓			
GOVERNANCE ARRANGEMENTS				
Prepare the ICB's overarching Scheme of Reservation and Delegation			✓	
Approval of the group's overarching Scheme of Reservation and Delegation.	✓			
Prepare the ICB's operational scheme of delegation.			✓	
Approval of the ICB's operational scheme of delegation.	✓ (as part of SoRD)			
Consideration and approval of applications to NHS England on any matter concerning changes to the ICB's constitution or standing orders.	✓			

Decision	Reserved or delegated to Board	Chief Executive	Chief Finance and Compliance Officer	Committees and Sub-committees
Prepare detailed financial policies that underpin the ICB's standing financial instructions			✓	
Approve policies required to ensure ICB meets statutory or legislative guidance	✓			
Approve operational policies required to provide internal direction (which do not relate to specific statutory or legislative guidance)				Senior Management Team
Approve any changes to the ICB's committee structure	✓			
Approval of the arrangements for discharging the ICB's statutory financial duties.	✓			In consultation with Audit, Risk and Assurance Committee
Exercise or delegation of those functions of the ICB which have not been reserved to the board or other committee or sub-committee or [specified] employee		✓		
Authority to make decisions relating to operational matters, within the financial limits specified in the Operational Scheme of Delegation, where not explicitly delegated elsewhere or defined elsewhere in the Scheme of Delegation				Senior Management Team
APPROVAL OF PLANS				
Approval of the NHS Joint Forward plan	✓			
APPROVAL OF BUDGETS AND FINANCE MATTERS				
Approval of the ICB's annual corporate budgets	✓			

Decision	Reserved or delegated to Board	Chief Executive	Chief Finance and Compliance Officer	Committees and Sub-committees
Approval of a comprehensive system of internal control, including budgetary control, that underpins the effective, efficient and economic operation of the ICB				Audit, Risk and Assurance Committee
Approval of the ICB's annual report and annual accounts				Audit, Risk and Assurance Committee
OPERATING STRUCTURES AND HR MATTERS				
Approval of the ICB's operating structure (in relation to organisational structures within the ICB)	✓			
Approval of terms and conditions, pensions, remuneration, fees and allowances payable to board members, employees and to other persons providing services to the ICB outside of agenda for change				Remuneration Committee
Approval of the use of any Redundancy Scheme (RS)				Remuneration Committee
Approval of TUPE or other staff transfer schemes				Remuneration Committee
Approval of the arrangements for discharging the ICB's statutory duties as an employer	✓			
Leading system implementation of people priorities including delivery of the People Plan and People Promise by aligning partners across the ICS to develop and support 'one workforce', including through closer collaboration across the health and care sector, with local government, the voluntary and community sector and volunteers	✓			

Decision	Reserved or delegated to Board	Chief Executive	Chief Finance and Compliance Officer	Committees and Sub-committees
CLINICAL QUALITY AND PATIENT SAFETY				
Approve arrangements to minimise clinical risk and maximise patient safety				Patient Safety Committee
Approve arrangements to secure continuous improvement in quality and patient outcomes				Strategic Commissioning Committee
Approve arrangements for supporting NHS England in discharging its responsibilities in relation to securing continuous improvement in the quality of general medical services.				Patient Safety Committee
RISK & ASSURANCE MATTERS				
Approval of the ICB's counter fraud and security management arrangements				Audit, Risk and Assurance Committee
Approval of the ICB's risk management arrangements.				Audit, Risk and Assurance Committee
Approval of changes to the provision or delivery of assurance services to the ICB including internal audit and counter fraud				Audit, Risk and Assurance Committee
Approve the appointment (and where necessary dismissal) of external auditors (and where necessary change/removal) of external audit				Audit, Risk and Assurance Committee acting as Auditor Panel

Decision	Reserved or delegated to Board	Chief Executive	Chief Finance and Compliance Officer	Committees and Sub-committees
CORPORATE MATTERS				
Approve ICBs publication scheme		✓		
Approval of the ICB's contracts for corporate support (for example finance provision)		✓		
COMMISSIONING, CONTRACTING AND RISK POOLING				
Approve arrangements for risk sharing and or risk pooling with other organisations (for example arrangements for pooled funds with other Integrated Care Boards or pooled budget arrangements under section 75 of the NHS Act 2006).	✓			
Approval of the ICB's contracts for any contracting / commissioning support including in respect of any commissioning functions delegated by NHSE		✓		
Approval of a new pooled budget, with a South East or South West London local authority	✓			
Approve decisions delegated to joint committees established under section 75 of the 2006 Act.	✓			
Approve primary care commissioning arrangements in South East and South West London (Bexley, Bromley, Greenwich, Lambeth, Southwark, Lewisham and Kingston, Richmond, Croydon, Wandsworth, Merton, Sutton)				SEL: Strategic Commissioning Committee
Approval of the arrangements for promoting integration and co-ordinating the commissioning of services with other integrated care boards, provider collaboratives, place and/or with the local authority/ies, where appropriate	✓			

Decision	Reserved or delegated to Board	Chief Executive	Chief Finance and Compliance Officer	Committees and Sub-committees
Approval of the arrangements for discharging the ICB's statutory duties associated with its commissioning functions for promoting improvement in the quality of services and promoting the involvement of each patient, patient choice, public engagement and consultation				Strategic Commissioning Committee
Approval of the arrangements for discharging the ICB's statutory duties associated with its commissioning functions to promote reductions in inequalities	✓			

Section B:

NHS South East London ICB & NHS South West London ICB
Integrated Operational Scheme of Delegation (schedule of matters delegated to officers and committees)

1. Introduction

1.1 This Operational Scheme of Delegation has been developed in conjunction with the organisation's Standing Financial Instructions and will provide guidance for the ICB.

1.2 The delegation shown below is the lowest level to which authority is delegated. Authority can be delegated upwards with no further action being required. However, delegation to lower levels is only permitted with written approval of the Chief Executive Officer. Decision making with a financial impact must be carried out in accordance with the ICB's Standing Orders, Prime Financial Policies and detailed financial procedures. All financial limits in this schedule of matters delegated to Officers are subject to sufficient budget being available.

1.3 The Standing Financial Instructions set out the financial responsibilities of the various Committees, Groups and individuals within the ICB.

2. Levels of Delegation

2.1 The Operational Scheme of Delegation covers only matters delegated by NHSE to the ICB and those matters reserved for the ICB Board its committees, Executive Officers and/or Partners.

2.2 Further delegation may be approved:

a. by the ICB Board in approving specific management policies

b. by the ICB Chief Executive

c. as part of Financial Procedures approved by the Chief Finance and Compliance Officer (CFCO)

2.3 Each Executive Director will need to consider the arrangements for authorisation of expenditure against delegated budgets and further delegation of management/professional responsibilities.

2.4 In accordance with the Scheme of Delegation the ICB Board exercises financial supervision and control by:

a. Authorising the operational plan;
b. Requiring the submission and approval of budgets within approved allocations / overall income;
c. Defining and approving essential features in respect of important procedures and financial systems (including the need to obtain value for money); and
d. Defining specific responsibilities placed on members of the ICB Board, its Committees, Partners and employees as indicated in the Scheme of Delegation.

2.5 Once the ICB Board, has reviewed and approved the Operating Plan and any supporting financial plan / budget, the Board, will delegate approval to the Chief Executive; the Chief Finance and Compliance Officer and other Executive Directors and employees to commit these resources for the purpose set out in the plan subject to the financial thresholds set out in this document.

2.6 Levels of Sub-Delegation
The Delegation of financial limits are also linked to the position/role of the staff member and the Levels outlined in Schedule 1 will be those set on the financial system.

2.7 Types of Delegation Authority

The types of financial delegation outlined in this document include:

a. Expenditure approval delegations
b. Invoices and credit note requests
c. Business case approval delegations

d. Procurement delegations
e. Contracts signing delegations
f. Other non-financial delegations.

3. Principles

3.1 General Delegation Principles

3.2 Delegates Must:

a. Act within your authority by ensuring you hold the relevant delegation
b. Understand your authority by referring to relevant guidance, limitations and directions
c. Act with the ICB's values in mind
d. Avoid conflicts of interest
e. Consider the ICB's business needs
f. Seek expert advice when making a decision
g. Make decisions objectively, reasonably and fairly.

3.3 Delegates Must Not:

a. Exercise delegations in respect of someone outside of your immediate line of control
b. Exercise powers in respect of a position higher than your own
c. Exercise a delegation in respect of yourself (i.e. confer a personal benefit)
d. Exercise a delegation on behalf of an absent employee unless it is within the scope of your delegated authority, or you are officially acting in the position.

3.4 Compliance

a. All delegates are required to comply with manuals and directives issued by the ICB

b. Delegated authority is subject to internal controls and to any overriding National laws

3.5 Responsibility

a. Delegations are made to positions, not to persons, and are specific to the position's work unit and/or role. Ultimate responsibility for performance of the functions or exercise of the authority or power rests with the authority holder.

b. Where an authority holder delegates an authority to an individual position, the person occupying that position becomes personally accountable for the delivery of that authority.

c. The delegation to a position is unique and is not transferable by the delegate.

d. Delegations extend to the Officer substantively appointed to that position and any person acting in that position for a specified period unless otherwise excluded in the terms of the temporary appointment. Delegations do not extend to volunteers.

e. Where the Scheme of Delegation specifies a delegate, the position to which the delegate reports is also deemed to have the delegated authority except where otherwise determined by legislation, policy or a Chief Executive instruction.

f. Where the permanent Officer takes leave, it is their responsibility to instruct the relieving Officer (and the finance department) of the level of delegation that is attached to the position and the responsibilities associated with the delegation for the agreed period. In case of the Chief Executive Officer their responsibility would be delegated to the Deputy Chief Executive Officer in the first instance for any leave.

3.6 Application

a. Delegates are expected to exercise their powers, authorities, duties or functions delegated to them in a responsible, efficient, consistent and cost-effective manner.

b. Discretion is to be utilised by the delegate in determining whether to exercise a delegation or refer the matter to a higher authority.

c. When an Officer is exercising their financial delegation, they are required to clearly provide their name, position and date when signing.

3.7 Financial Delegation Principles

3.8 Delegates Must:

a. Only approve expenditure in cost centres under the delegate's authority
b. Only approve expenditure where there is sufficient budget to cover the cost
c. Only approve expenditure on goods and services related to official work and business use
d. Only approve expenditure where all relevant ICB's procedures and policies have been followed
e. Only approve expenditure to the financial limit of the delegation
f. Only approve expenditure where evidence exists that goods have been received and/or services have been performed in accordance with and at the rate/s of an agreed contract or arrangement
g. Employees are to note that an expenditure approval is to be given prior to any commitment being made, contract signed, or purchase order raised.

3.9 Delegates Must Not:

a. approve a gift or settlement of any legal claim unless specifically delegated this authority
b. transfer the financial delegation granted by the ICB Chief Executive to another employee
c. break one purchase down into several smaller items to avoid breaching the financial limit of the delegation
d. approve expenditure on capital works, contracts or special payments unless specifically delegated this authority
e. exceed their delegation limits even if automated systems permit this to occur
f. Approve any expenditure incurred by the delegate on travel, meals, conferences and other similar expenditure
g. Assume the financial delegation of an absent delegate if you are not authorised to do so.
h.

3.10 Suspension, Revocations and Reductions in Financial Delegations

a. The terms of any financial delegation cannot be exceeded under any circumstances.

b. Financial delegations cannot be sub-delegated once granted by the ICB Chief Executive.

c. Improper performance of responsibilities may result in disciplinary action being taken against the employee concerned.

d. The power to revoke, suspend or reduce financial delegations granted to positions within the ICB rests with the Chief Executive in respect of delegations made.

e. If circumstances arise which warrant the suspension, revocation or reduction of a financial delegation, full details must be forwarded to the ICB's Chief Finance and Compliance Officer. The ICB's Chief Finance and Compliance Officer will submit an appropriate recommendation to the Chief Executive for consideration.

f. If the recommendation is approved, the delegation will be amended to reflect that reduction, suspension or revocation.

3.11 Reviewing and Maintaining the Scheme of Delegations

a. This Scheme of Delegation may be amended from time to time to reflect changes in legislation, ICB policy or operational requirements.

b. The ICB will coordinate regular reviews of financial, procurement and contracts signing financial delegations for positions and limits. A revised version is submitted to the Senior Management Team (SMT) and Audit, Risk and Assurance Committee for endorsement before submitting it to the ICB Board for approval.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
1	**Bank Accounts** Maintenance and operation in accordance with banking mandates.	All banking must be managed in accordance with organisational Standing Financial Instructions (SFIs) and Standing Orders (SOs).
2	**ICB Budgets** All ICB staff have the responsibility of keeping expenditure to within agreed budgets. The following delegated authority to spend is only extended to where the approved budget is available: a) At individual budget level (Pay and non-Pay) b) At a service level c) For the totality of service covered by directorate d) For the totality of the budget delegated to Place e) The totality of the ICB budget **In the event of an urgent unpredicted need for expenditure which does not have allocated budget, no expenditure must be committed to without prior discussion with the Chief Finance and Compliance Officer or Chief Executive.** **Management of budgets – variations to agreed budgets:** a) Approving expenditure where there is a variation to budget, or in the tender price up to 10% or £100,000 whichever is the higher. b) Approving expenditure where there is a variation to budget or in the tender price greater than 10% or £100,000 and less than 20% or £250,000, whichever is the higher. c) Approving expenditure where there is a variation to budget or in the tender price greater than 20% or £250,000, whichever is the higher. **Budget Virements:** a) At individual budget level within a service up to £10,000 b) At individual budget level within a service over £10,000 and < £100,000	a) Budget Holders b) Head of Service/Departmental Manager c) Executive Director d) Place Executive Director e) Chief Executive a) Chief Finance and Compliance Officer or Chief Executive Officer b) Chief Finance and Compliance Officer and Chief Executive Officer c) ICB Board a) Budget Holders b) Executive Directors

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	c) Between services up to £500,000 d) Services greater than £500,000 Virements should not be used to create new budgets. Virements are not permitted between pay and non-pay budgets. The virement must be signed by both the budget holder from whom the budget is transferring and the budget holder to whom the budget is transferring.	c) Chief Finance and Compliance Officer or Chief Executive Officer d) ICB Board
3	**Business Case and Investment Approvals** Business cases for investments must be prepared for changes to services and/or expenditure including capital or revenue, procurement of services and pathway redesigns. Business cases seeking external funding must be approved by the relevant body prior to making the external request for funds. The following delegated authority is only extended to where the approved budget is available: Capital: a) Estimated cost up to £2.50m b) Estimated cost up to £5.00m c) Estimated cost up to £10.00m d) Estimated cost up to £24.99m e) Above £25.00m The ICB is required to have oversight of significant investment (no matter whether they are revenue or capital or a combination) made by partners in the system to ensure they align with the ICS and ICP strategy and provide value for money. These business cases typically relate to additional funds made available by NHSE for specific projects and the strategic investments as part of the system-wide capital plan. On agreeing they align to the system priorities, are affordable and value for money the ICB will be expected to write a letter of support for the scheme. Where the NHS makes allocations to individual organisations within the ICS/allocation values are updated in year e.g. pay awards. These would be excluded from requiring a business case.	a) Chief Finance and Compliance Officer b) Chief Executive Officer c) CFO and CEO/SMT d) SMT e) ICB Board

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	Prior to the ICB reviewing and agreeing its support to any cases it is expected these will have been through individual partner organisations governance (in line with their SFIs and Scheme of Delegation). The appropriate approval process for a business case is determined by the value of the business case. Business cases should follow national guidance in their construct: **Letters of Support** *a) Estimated cost up to £5m* *b) Estimated cost up to £10m* *c) Estimated cost up to £20m* *d) Estimated cost up to £50m* *e) Above £50m* Revenue: Annual values L1 - <£50k L2 - £50k-£250k L3 - £250k-£2.5m L4 - £2.5m - £10m L5 - £10m - £25m L6 - >£25m Where expenditure is not fully budgeted for, the capital limits will apply – for example the CFCO will have delegated authority up to £2.50m.	a) CFCO b) CFCO and CEO c) CFCO, CEO and SMT d) SMT and SCC e) SMT, SCC and ICB Board Decision Owner / Committee Involvement Budget Holder – None Director – Exec visibility Exec Director – SMT reporting SMT – Notify SCC SCC – SMT (prior to SCC) Board – Final Approval

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
4	**Capital Schemes** The financial limits for quotations, tendering and contract procedures are set out below in section 30 and apply to capital expenditure. In addition, the following delegations apply to capital schemes: a) Selection of architects, quantity surveyors, consultant engineer and other professional advisers within EU regulations b) Financial monitoring and reporting on all capital scheme expenditure c) Granting and termination of leases with annual rent <£100,000 d) Granting and termination of leases with annual rent >£100,000 The ICB capital plan is developed and then reviewed by Senior Management Team before being approved by the ICB Board prior to publication. ICB capital includes: a) Primary care improvement grants b) Primary care ICT grants c) Buildings and equipment d) Leases (under IFRS16) – including the authority to extend or exit tenancy agreements for ICB offices It excludes tenancy agreements and void costs that should be managed in line with non-pay revenue investments/spends unless specifically listed below: a) preparation and signature of all tenancy agreements/ licences b) Extensions to existing leases c) Letting of premises to/from outside organisations d) Approval of rent based on professional assessment ICB capital projects are subject to the DHSC and Cabinet office rules for approval as in the Procurement Section. Authority to approve capital expenditure in excess of the total capital budget	a) Chief Executive or Chief Finance and Compliance Officer on recommendation of estates director b) Chief Finance and Compliance Officer c) Chief Finance and Compliance Officer d) SMT a) Chief Finance and Compliance Officer b) Chief Finance and Compliance Officer c) Chief Finance and Compliance Officer and Chief Executive d) Chief Finance and Compliance Officer Approval of an increase in the total capital budget above the value signed off by ICB Board would breach CDEL and is not permitted.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	Charitable and Endowment Funds NHS Greenwich Charitable Fund Approval of Charity purpose, vision and strategic objectives Approval of annual charitable funds budget Approval of annual report and assuring compliance Day to day control and management of charitable fund Approval of grants: a) Up to £10k b) £10k to £200k c) £200k+	 NHS South East London ICB (SEL ICB) is the Corporate Trustee for NHS Greenwich Charitable Funds. SEL ICB Board Chief Finance and Compliance Officer Chief Finance and Compliance Officer Place Executive Lead (Greenwich) a) Associate Director of Finance (Greenwich & Lewisham) b) Place Executive Lead (Greenwich) c) Chief Finance and Compliance Officer (on recommendation from Charitable Funds Committee)
5	**Clinical Trials** Authorisation of Clinical Trials	 Chief Executive Officer and Medical Director.
6	**Discretionary Grants to Local Authorities and Voluntary Bodies** Discretionary grants to local authorities and voluntary bodies should only be awarded in exceptional circumstances. The expectation of the ICB is that all contractual commitments are made through the normal procurement process for contracts and in line with the approval limits set out in section 30 below.	 For all agreements to provide grants/ MOUs to third parties including local authorities and voluntary bodies: Chief Finance and Compliance officer or Chief Executive or Chief Commissioning Officer

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
7	**Commissioning Expenditure - purchase of Healthcare from all bodies** i) Service Level Agreements (SLAs) SLAs within the annual budget: a) Signing of annual local contracts/voluntary sector contracts and SLAs up to £5m b) Signing of annual local contracts/voluntary sector contracts and minor SLAs over £5m Authorisation of monthly invoices in excess of agreed SLA value. ii) Contract Exclusions (NHS and Non-NHS) Individual Funding Requests (IFRs) a) Approval of Requisitions in line with approved IFR b) Approval of Purchase Order iii) All other contract exclusions iv) For any other funded commissioning arrangements under this category not included above: Agreement of Named Placements (not Continuing Healthcare Placements) a) Up to £50,000 and within approved budget b) Above £50,000 or in excess of available resources **Continuing Healthcare Packages** To recognise the current differences between the processes in SEL ICB and SWL ICB, different authority limits shown below for each ICB:	a) Executive Director b) Chief Executive and Chief Finance and Compliance Officer Deputy Chief Executive/ Chief Finance and Compliance Officer and reported to SMT for information. a) IFR Lead after IFR panel approval of expenditure b) Procurement Operations Manager Executive Director Chief Finance and Compliance Officer, or Chief Executive Officer, or Chief Commissioning Officer a) Head of service b) Executive Director

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	SWL ICB Delegation for all placements, value relates to the period of award up to the next review point. a) Up to £1,500 per week or £75,000 per annum b) Up to £4,000 per week or £200,000 per annum c) Up to £10,000 per week or £500,000 per annum d) Above £10,000 or over £500,000 per annum SEL ICB a) Agreement of named placements up to £1,650 per week b) between £1,651 and £3,450 per week c) for values exceeding those above d) Signing of Contract for placements e) Approval of invoices within package contract value as per ISFE limits f) Authorisation of monthly invoices in excess of contract value **Joint commissioning with Local Authorities** Where services are being commissioned jointly with local authorities, delegated responsibility for the arrangements will by held by the Place Executive Leads, with assurance and co-ordination delivered by the Neighbourhood Delivery Committee.	a) Deputy director of CHC *b)* Operational Place lead *On recommendation of the Deputy Director of CHC* *c)* Chief Nursing and Quality Officer *On recommendation of the Operational Place lead* *d)* Senior Management Team *On recommendation of the Chief Nursing and Quality Officer* a) CHC teams within boroughs b) Head of service c) Executive Director d) Head of service up to value of £100,000 and Executive Director for over £100,000 e) As per ISFE limits set per user f) Executive Director Place Executive lead holds delegated responsibility, with assurance and oversight through Neighbourhood Delivery Committees.
8	**Complaints (Patients and Relatives)** a) Overall responsibility for ensuring that all complaints are dealt with effectively b) Responsibility for ensuring that complaints are investigated thoroughly c) Medico – Legal Complaints - Co-ordination of their management	a) Chief Executive b) Relevant Executive Director c) Director of Corporate Services

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
9	**Condemning & Disposal of capitalised equipment** Items obsolete, redundant, stolen, and irreparable or cannot be repaired cost effectively. *Note: these write-offs, once agreed, will impact on individual budgets – there is no central provision. A bad debt write-off for these purposes is the writing off of any income due to the ICB whether or not invoiced – it does not include adjustments relating to invoices raised in error.*	Approval for disposal to be made by two officers - the Chief Finance and Compliance Officer plus either the Chief Executive or Director of Corporate Services. All disposals to be reported to the next meeting of the Audit Risk and Assurance Committee.
10(a)	**Payments to Independent Contractors - GP contract payments including pensions SEL ICB ONLY** Notifying GP practices of approved annual allocation Approval of regular monthly instalments schedules of approved reimbursements: a) <£20,000,000 as per SE London schedule b) >£20,000,001 as per SE London schedule Further reimbursement of expenditure within approved allocation, including Out of Hours (OOH) Community Schemes and Local Incentive Schemes expenditure: a) Up to £10,000 b) £10,001 to £50,000 c) Over £50,000	Executive Director a) Head of service and Associate Director of Corporate Finance b) As above plus Director of Operational Finance a) Borough Head of Primary Care (or equivalent) b) Borough Director of Primary Care (or equivalent) c) Executive Director
10(b)	**Payments to Independent Contractors – Primary Care Dental, Ophthalmology and Community Pharmacy** Approval of regular monthly instalments schedules of approved reimbursements: a) less than £20,000,000 as per SE London schedule b) greater than £20,000,001 as per SE London schedule	a) Director or Chief Commissioning Officer and Associate Director of Corporate Finance b) Director or Executive Director and Director of Corporate Finance

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	Approval of contract changes on dental system	Director or Executive Director
11	**Medicines Optimisation** Authorisation of New Drugs: a) Estimated total yearly cost per drug of up to £25,000 per 100,000 population (£500,000 for South East London based on 2 million population) b) Estimated total yearly cost per drug above £25,000 per 100,000 population (£500,000 for South East London based on 2 million population)	a) Chief Pharmacist b) SCC on the recommendation of medicines optimisation team Where NICE guidance is not followed, this should be reported to the Board at the next opportunity.
12	**Legal matters** Engagement of legal advice **Where costs are estimated to exceed £100k, SMT to be alerted.** Legal settlements	Approval by an Executive Director and Director of Corporate Services. Chief Executive and Chief Finance and Compliance Officer approval
13	**Engagement of Agency Staff** All agency Costs of £600 or more per day (excluding VAT) or over 6 months in duration (regardless of cost) will require approval by NHSE via submission of a business case.	<u>For SEL ICB</u> Prior to engaging any agency/interim staff, staff must follow the Hiring of Interim Resources Policy and comply with the NHS England and ICB guidance. All requests for agency staff (regardless of the daily rate and length of contract) require the approval of the VR panel before any appointment can be made. Prior endorsement is required from finance and HR. **For booking of agency/interim staff and approval of invoices, within budget:** a) Up to £10,000 – budget holder b) £10,001 to £50,000 – head of service c) Over £50,000 – Executive director The sums above exclude VAT, agency fees and expenses. **For booking of Bank or Agency Staff – in excess of budget:**

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
		Employment of fixed term contractors is approved by Executive Director – following approval by the VR panel Fixed Term contractors should be considered where the requirement is time limited. In all cases HR advice should be taken prior to recruitment to understand any ICB liability that may be incurred at the end of the assignment. Approval is via the VR panel.
14	**Extended Role Activities** Approval of Nurses to undertake duties/procedures which can properly be described as beyond the normal scope of Nursing Practice.	Chief Nursing and Quality Officer
15	**Facilities for staff not employed by the ICB to gain practical experience** a) Professional Recognition, Honorary Contracts and other Memorandums of Understanding b) Non-Medical Work experience students	a) Chief Nurse or Medical Director (as appropriate) - HR to be advised b) An ICB Director - HR to be advised
16	**Review of fire precautions at ICB sites**	Director of Corporate Services in conjunction with local authority/landlord as appropriate
17	**Hospitality and Gifts** Responsibility for implementing arrangements to ensure Conflicts of Interest and Gifts and Hospitality policy is adhered to. Limits as determined in the ICBs Conflicts of Interest/ Standards of Business Conduct Policy.	Director of Corporate Services
18	**Arrangements for Internal and External Audit – including the implementation of Audit Recommendations**	All audit plans will be approved by the ICB's Audit, Risk and Assurance Committee on the recommendation of the Chief Finance and Compliance Officer or Director of Operational Finance.
19	**Approval of any proposed non-audit based professional services to be delivered by the ICB's external auditor**	Chief Finance and Compliance Officer AND the Chair of the Audit, Risk and Assurance Committee *Prior endorsement required by:* Senior Management Team Approval noted by the Audit, Risk and Assurance Committee

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
20	**Insurance Policies including NHSLA** Negotiation and agreement of premiums Ensuring the ICB has arrangements in place for the provision of adequate insurance cover for the ICB that are not indemnified through the NHS Resolution	 Director of Corporate Services Chief Executive and Chief Finance and Compliance Officer, via Director of Corporate Services
21	**Certification of invoices where no Purchase Order or contractual arrangement is in place** a) Up to £10,000 b) £10,001 to £50,000 c) £50,001 to £250,000 d) £250,001 to £1,000,000 e) £1,000,001 to £5,000,000 f) £5,000,001 to £10,000,000 g) Up to £500.0m h) Up to £1.0bn	 a) Budget managers at Bands 6 and 7 b) Budget managers at Band 8a and 8b c) Assistant/Associate Directors at Band 8c and 8d d) Directors at Band 9 e) ICB Operational Director of Finance f) Borough, Place Directors or equivalent (VSM) g) Deputy Chief Executive Officer h) Chief Executive Officer and Chief Finance and Compliance Officer
22	**Losses, Write-off & Compensation** a) Losses and Cash due to theft, fraud, overpayment etc. b) Fruitless Payments (including abandoned Capital Schemes) c) Claims Abandoned d) Damage to buildings, fittings, furniture and equipment and loss of equipment and property in stores and in use due to culpable causes (e.g. fraud, theft, arson) or other e) Compensation payments made under legal obligation f) Extra Contractual payments to contractors g) Ex-gratia Payments to Patients and staff for loss of personal effects h) Ex-gratia payments for Clinical negligence (negotiated settlements following legal advice) – up to £250,000 including claimant's legal costs i) Ex-gratia payments for personal injury claims involving negligence where legal advice obtained and followed j) Other ex-gratia payments except cases of maladministration where there is no financial loss by claimant - up to £50,000 k) Write back of Non-NHS Debtors < £250,000 l) Write back of Non-NHS Debtors > £250,000	 a) Chief Finance and Compliance Officer b) Chief Finance and Compliance Officer c) Chief Finance and Compliance Officer d) Chief Finance and Compliance Officer e) Chief Finance and Compliance Officer and Chief Executive f) Chief Finance and Compliance Officer g) Chief Finance and Compliance Officer h) Chief Finance and Compliance Officer i) Chief Finance and Compliance Officer j) Chief Finance and Compliance Officer k) Chief Finance and Compliance Officer l) Audit, Risk and Assurance Committee
23	**Maintenance & Update of ICB Financial Procedures**	Chief Finance and Compliance Officer

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
24	**Management Consultants** All contracts for management consultants will require agreement in line with any ICB process and reporting to NHS England in addition to the process below. All contracts for management consultants over £50,000 require a business case to be submitted to NHS England for approval prior to any appointment being made. All requests for management consultancy must be assessed as being consistent with IR35 compliance before any appointment can be made. a) Obtaining at least 3 quotations / carrying out competitive interviews, **within budget**, where aggregate commitment in any one year or total commitment is to £24,999. b) Obtaining at least 3 competitive tenders / quotes or undertaking competitive interviews **within budget**, where aggregate commitment in any one year is £25,000 or more c) Authorising contracts of engagement following the above. d) Authorisation of consultants **in excess of budget.**	a) Executive Director b) Executive Director c) Executive Director d) Chief Finance and Compliance Officer or Chief Executive
25	**Personnel & Pay** This is subject to compliance with all ICB recruitment processes, e.g. submission to VR/recruitment panel. a) Authority to fill funded post on the establishment with permanent staff b) Authority to appoint staff to post not on the agreed establishment c) All requests for upgrading/re-grading to be dealt with in accordance with ICB procedures and recruitment panel approval.	a) Pre-approval by Director, Finance lead and HR. Once confirmation/approval received from VR panel/RSC, JD and advert to be sent to the Resources team by recruiting manager. HR to be informed. b) As above for a). Chief Executive approval will be required should there be no recruitment panel in place. c) Once approval received, finance department and workforce department to be informed of outcome for budgetary and ESR purposes.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	d) Approval of the extension of staff on fixed term contracts within budget.	d) Fixed term contracts should only be extended in specific circumstances. In all cases this should be discussed with HR and the relevant financial officer prior to submission of a request to the VR/recruitment panel.
	e) Pay	e)
	i) Staff appointments – Executive directors and other directors referenced on the ICB Board	i) Chief Executive and Chair
	ii) Approval of remuneration arrangements (including additional allowances above basic salary) – all staff levels excluding Executive Directors and Directors referenced in the Board	ii) Human Resources and finance leads and relevant executive director
	iii) Approval of remuneration arrangements for Executive Directors and other Directors referenced on the ICB Board	iii) Remuneration Committee
	iv) Authority to complete standing data forms affecting pay, new starters, variations and leavers	iv) Budget holder and Associate Director of Corporate Finance. HR to be informed. All staff change forms must be approved by a Director and either the Chief Financial Officer or Director of Corporate Finance.
	v) Authority to authorise overtime/flexible working	v) Line Manager (with guidance from a Director)
	vi) Authority to authorise travel & Subsistence expenses on Workforce	vi) Line Manager
	vii) Approval of staff timesheets – both substantive and temporary staff	vii) Line Manager
	f) Leave	f)
	i) Approval of Annual Leave	i) Line Manager
	ii) Approval to carry forward annual leave	ii) Annual leave carry over will be approved in line with the annual leave policy for each respective ICB.
	iii) Payment of Annual Leave in exceptional circumstances	iii) Chief Executive
	iv) Special leave for bereavement up to 5 days	iv) Head of Service
	v) Extended Special Leave for bereavement	v) Executive Director
	vi) Special leave arrangements • carers leave - up to 3 days in any six-month period • carers leave - up to 5 days in any six-month period	vi) • Head of Service. HR to be informed • Executive Director. HR to be informed
	vii) Leave without pay	vii) Executive Director
	viii) Time off in lieu	viii) Line manager
	ix) Maternity and Paternity Leave – paid and unpaid	ix) Automatic approval as per guidance and ICB Policy; HR to be informed.
	g) Sick Leave (in exceptional circumstances)	g)
	i) Extension of sick leave – in line with contract of employment	i) To seek advice from the Director of HR before any extension is agreed
	ii) Extension of sick leave – any non-contractual payments	ii) Remuneration Committee
	iii) Return to work part-time on full pay to assist recovery	iii) Executive Director with advice with advice from occupational health and HR.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	h) Study Leave i) Study leave outside the UK ii) All other study leave (UK)	h) i) Chief Executive in line with ICB training policy ii) Executive Director in line with ICB training policy
	i) Removal Expenses, Excess Rent and House Purchase (to be agreed prior to advertising posts). Authorisation of payment of removal expenses incurred by officers taking up new appointments (providing consideration was promised at interview): i) Up to £2,000 ii) Over £2,000	i) Executive Director, and notification to HR ii) Chief Executive and notification to HR
	j) Grievance Procedure All grievance cases must be dealt with strictly in accordance with the applicable ICB Grievance Procedure and the advice of HR	As per applicable (i.e. SEL or SWL) ICB procedure
	k) Authorised Car & Mobile Phone Users i) Requests for new posts to be authorised as car users ii) Requests for new posts to be authorised as mobile telephone users	k) i) As per applicable ICB policy ii) As per applicable ICB policy
	l) Redundancy Authorisation to agree voluntary redundancy and determine compulsory redundancies in accordance with policy	l) Remuneration Committee with support from HR and appropriate permission/approvals from NHS England. Chief Finance and Compliance Officer to be consulted at all times.
	m) Ill Health Retirement Decision to pursue retirement on the grounds of ill-health.	m) Chief Executive or Executive Director following advice from HR **and** occupational health.
	n) Employment of voluntary workers / work experience.	n) An ICB Director with advice from HR
	o) Approval of expense claim within assigned delegation limit (refer schedule 1) and claim is allowable per the NHS Terms and Conditions of Service handbook	o) Employees line manager
	p) Approval of expense claim above assigned delegation limit (refer schedule 1) and claim is allowable per the NHS Terms and Conditions of Service handbook	p) Executive Director
26	**Petty Cash Disbursements**	The ICB has no petty cash function.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
27	**Quotation, Tendering & Contract Procedures** For Health Care services of any Contract Value, The Health Care Services (Provider Selection Regime) Regulations 2023 (PSR) and supporting Statutory Guidance, must be followed. PSR Provider Representations: Decisions following escalation of Representations made against a Provider Selection Regime decision, to the NHSE Independent Patient Choice and Procurement Panel PSR Urgent Awards or Urgent Contract Modifications: Where: a) a new service needs to be arranged rapidly in an unforeseen emergency or local, regional or national crisis, e.g., to deal with a pandemic. b) urgent quality/safety concerns pose risks to patients or the public and necessitate rapid changes. c) an existing provider is suddenly unable to provide services under an existing contract (for example, a provider becomes insolvent or experiences a sudden lack of critical workforce) and a new provider needs to be found. An urgent award or modification must only be made when all the below apply: a) the award or modification must be made urgently the reason for the urgency was not foreseeable by and is not attributable to the relevant authority. b) delaying the award of the contract to conduct a full application of the regime would be likely to pose a risk to patient or public safety. Urgent Awards or Urgent Contract Modifications must not be used if the urgency is attributable to the ICB not leaving sufficient time to make	SMT Approved by the Chief Finance and Compliance Officer (up to £100,000) and Chief Finance and Compliance Officer and Chief Executive (£100,000 and over); reported to the Audit, Risk and Assurance Committee. Where the Chief Executive is requesting an Urgent Award or Urgent Contract Modification, approval is by the Chair and the Chief Finance and Compliance Officer.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	procurement decisions and run a provider selection process– poor planning is not an acceptable reason to use these provisions. With the exception of Section (i) below, relating to Contract Variations, the remainder of this section (30) only applies to non-healthcare services. For non-health care services the following applies: This section does not apply to Management Consultants (see section 26) (or other services included elsewhere within this schedule) a) Goods/services up to £24,999 - (Minimum of 1 written quote required) b) Goods/services from £25,000 - £75,000 (Minimum of 3 written quotations required) c) Goods/services from £75,000 - £100,000 (Minimum of 3 competitive tenders required) d) Goods/services from £100,000 - £200,000 (Minimum of 3 competitive tenders required) e) Goods/services over £200,000 - (Minimum of 4 competitive tenders required) f) Waiving of quotations and tenders subject to Standing Financial Instructions g) Opening Tenders and Quotations h) Authorisation of payments to public partnership schemes under existing contracts i) Contract variations i) Variation of +/- 20% of contract value and less than £100,000 ii) Variation of over 20% of contract value and/or more than £100,000 Exceptions and instances where formal tendering/quotes need not be applied: Where: a) the estimated expenditure or income does not, or is not reasonably expected to, exceed £24,999 or	a) Head of Service b) Associate Director or Director c) Executive Director d) Chief Finance and Compliance Officer e) Chief Executive f) Approved by the Chief Finance and Compliance Officer (up to £100,000) and Chief Finance and Compliance Officer and Chief Executive (£100,000 and over); reported to the Audit, Risk and Assurance Committee. Where the Chief Executive is requesting the tender waiver, approval is by the Chair and the Chief Finance and Compliance Officer. g) Two senior officers/managers designated by the CEO/CFCO and not from the originating department. h) Chief Finance and Compliance Officer i) Executive Director ii) Chief Finance and Compliance Officer

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	b) where the supply is proposed under special arrangements negotiated by the DH in which event the said special arrangements must be complied with. Formal tendering procedures may be waived by the Chief Executive and Chief Financial Officer in the following circumstances: c) in very exceptional circumstances where formal tendering/quoting procedures would not be practicable or the estimated expenditure would not warrant formal tendering procedures, and the circumstances are detailed in an appropriate record; d) where the requirement is covered by an existing contract; e) where PASA agreements or Public Sector Framework Agreements are in place; f) where a consortium arrangement is in place and a lead organisation has been appointed to carry out tendering activity on behalf of the consortium members; g) where the timescale genuinely precludes competitive tendering but failure to plan the work properly would not be regarded as a justification for a single tender; h) where specialist expertise is required and is available from only one source; i) when the task is essential to complete the project, and arises as a consequence of a recently completed assignment and engaging different consultants for the new task would be inappropriate; j) there is a clear benefit to be gained from maintaining continuity with an earlier project. However in such cases the benefits of such continuity must outweigh any potential financial advantage to be gained by competitive tendering; k) for the provision of legal advice and services providing that any legal firm or partnership commissioned is regulated by the Law Society for England and Wales for the conduct of their business (or by the Bar Council for England and Wales in relation to the obtaining of Counsel's opinion) and are generally recognised as having sufficient expertise in the area of work for which they are commissioned. The Chief Executive, Chief Financial Officer, Chief of Staff will ensure that any fees paid are reasonable and within commonly accepted rates for the costing of such work. l) where allowed and provided for in the Capital Investment Manual.	

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	The waiving of competitive tendering procedures should not be used to avoid competition or for administrative convenience or to award further work to a consultant originally appointed through a competitive procedure, except in exceptional circumstances. The Chief Executive & Chief Financial Officer will abide by the anti-bribery policy for the ICB at all times when approving tender waivers. Where it is decided that competitive tendering is not applicable and should be waived, the fact of the waiver and the reasons should be documented and reported to the Audit Committee. **Selection of preferred tenderer(s)/compliant direct contract award under either the *Public Procurement Regulations 2015* or the *Provider lection Regime Regulations 2023*** This delegation has application when a formal competitive tender process is conducted. At the conclusion of the tender evaluation stage, the evaluation team will make a decision on the award of contracts and will prepare a recommendation report that recommends the preferred tenderer(s). The report will detail the factors (including price, quality, and timing) that define the tender that provides the best overall value for money, and provide a comparison with the details of the nearest competing bids, where appropriate, with reasons for their rejection. The Delegated Officers have authority to approve the recommendation report. Following approval award, post-tender negotiations can be initiated with the successful tenderer to improve the successful offer, where appropriate, and the formal contract should be prepared. Where a purchase exceeds the limit set in the Public Contracts Regulations 2015, but only a single provider is identified having advertised our requirements, approval must be sought from the Chief Finance and Compliance Officer together with one other executive prior to award of the contract.	

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	Capital <u>Revenue</u> Up to £2,500,000 (if within delegated budget) Over £2,500,000 to £5,000,000 (if within delegated budget) Over £5,000,000 up to £25,000,000 Over £25,000,000 <u>Authority to approve acceptance of late tenders</u> Tender received within two hours after the specified tender closing time Tender received more than two hours after the specified tender closing time **Commercial contracts**	 Chief Finance and Compliance Officer Contract and Procurement group *Prior endorsement required by* Place, Primary Care Executive or Department as appropriate Senior Management Team Prior endorsement required by: Contract and Procurement group Place, Primary Care Executive or Department as appropriate Strategic Commissioning Committee *Prior endorsement required by:* Senior Management Team, Contract and Procurement group Place, Primary Care Executive or Department as appropriate ICB Board *Prior endorsement required by:* Contract and Procurement group, Senior Management Team AND Strategic Commissioning Committee Place, Primary Care Executive or Department as appropriate Executive director or manager Chief Finance and Compliance Officer

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
	A commercial contract could take the form of a deed, contract, agreement, release, discharge, indemnity, guarantee, consent, instrument, and any other documents which binds the ICB legally to another party by imposing an obligation on each party a) Less than £2,500,000 b) Over £2,500,000 up to £7,500,000 *c)* Over £7,500,000 and Better Care Fund investment/ Section 75 agreement	a) Chief Executive OR Chief Finance and Compliance Officer OR Chief Commissioning Officer *Prior approval required by:* Contracting and Procurement Group b) Chief Executive OR Chief Finance and Compliance Officer OR Chief Commissioning Officer *Prior approval required by:* Contracting and Procurement Group c) ICB Chair or Chief Executive or Chief Finance and Compliance Officer or Chief Commissioning Officer *Prior approval required by:* Strategic Commissioning Committee
28	**Research Projects** Authorisation of Research Projects	SMT in conjunction with the Chief Nursing and Quality Officer
29	**Register of Interests** The keeping of a Declaration of Interests Register	Director of Corporate Services.
30	**Commercial Sponsorship** Sponsorship of events, including courses, conferences and meetings by external bodies should only be approved if it can be demonstrated that the event will result in clear benefits for SEL ICB and the wider NHS.	Approval by the Chief Executive or Chair only and entry in declarations of interest register required.

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
31	**Extension of Contract** Where an extension to an existing contract (both NHS and non-NHS) is required and is **allowable in the contract and is within budget**, details should be provided of the term of extension and the monetary value of the extension. Before any contract extension is agreed, a review will be required that demonstrates positive assurance in respect of value for money, performance and quality. Set up of new suppliers	Approval of the contract extension (including the review of the contract) is as per the delegated limits as set out in section 8 above. There is no automatic delegated approval for contracts to be extended. Instead, a contract tender waiver (for non-healthcare services) or Urgent Award/Urgent Contract Modification (for healthcare services) would need to be produced and approved in accordance with the limits set out in section 30 above. Requests need to be submitted to the Financial Controller, with supporting evidence. Once approved the request will be forwarded to the national SBS team for actioning.
32	**Raising of sales orders** Authorisation of sales orders for goods or services provided by the ICB: a) Up to £10,000 (credit memo up to £10,000) b) £10,001 to £100,000 (credit memo £10,001 up to £50,000) c) £100,001 to £250,000 (credit memo £50,001 to £250,000) d) £250,001 to £500,000 (credit memo £250,001 to £1,000,000) e) £500,001 to £1,000,000 f) £1,000,001 to £2,500,000 (credit memo up to £5,000,000) g) £500,001 to £2,500,000 (credit memo up to £10,000,000) h) Up to £5,000,000 (credit memo up to £500.0m) i) Up to £1.0bn (credit memo up to £1.0bn) j) No delegated financial responsibility	a) Budget managers at Bands 6 and 7 b) Budget managers at Band 8a and 8b c) Assistant/Associate Directors at Band 8c and 8d d) Directors at Band 9 e) ICB Finance Directors f) ICB Finance Directors g) Borough Directors or equivalent (VSM) h) Deputy Chief Executive i) Chief Executive and Chief Finance and Compliance Officer j) System Administration
33	**Sealing of Documents**	Two of the following are required to seal documents: Chief Executive Officer Chief Finance and Compliance Officer Chief Medical Officer Chief Nursing Officer

REF	DELEGATED MATTERS	AUTHORITY DELEGATED TO
34	**Disposal of Records** Physical records, either on site or held in archive by a third party, should be disposed of in accordance with the ICBs approval Records Retention Schedule.	Director of Corporate Services
35	**Authority to sign non legally binding administrative arrangements** *This delegation has application when the ICB is entering into non-legally binding administrative arrangement with one or more other parties. The non-legally binding administrative arrangements could relate to one of the following:* • *Memoranda of Understanding (either intra-ICB, with other NHS organisations, or with a commercial third party)* • *Service level agreements (intra/Inter ICB, other NHS Organisations)* • *Operating level agreements (intra/Inter ICB, other NHS Organisations)*	Chief Executive or Chief Finance and Compliance Officer or Chief Commissioning Officer

Section C:

Delegation levels

Revenue

Level	Value	Decision Owner	Committee Involvement
L1	<£50k	Budget Holder	None
L2	£50k-£250k	Director	Exec visibility
L3	£250k-£2.5m	Exec Director	SMT reporting
L4	£2.5m-£10m	SMT	Notify SCC
L5	£10m-£25m	SCC	SMT prior review and recommendation
L6	>£25m	Board	Final Approval

Capital

Value	
<£2.5m	Chief Finance and Compliance Officer
£2.5m - £5m	CEO
£5m - £10m	CFCO and CEO / SMT
£10m - £25m	SMT
>£25m	Board

NHS
South East London

SFI and Scheme of Reservation and Delegation review

1. What does our governance model require?

1.1. In redesigning the governance structure, feedback was sought from Board members on the current governance function. Governance was seen as, “duplicative”, “lack of clarity”, “requires simplification”. There was a view that we should, “shift to strategy-led agendas”, and, “avoid operational detail”.

1.2. The two ICB’s new draft governance model reflects this feedback and is very clear structurally;

- Board = strategic, non-operational authority, final approval and oversight
- Committees = scrutiny, assurance, selective approvals.
- SMT = principal forum for operational management of the ICB's business activities.

1.3. In relation to what this means for our Schemes of Reservation and Delegation (SoRD) and Standing Financial Instructions (SFIs), it forces three design decisions;

- Committees must not be approval bottlenecks. They must focus on strategy, challenge and assurance, but with suitable delegation of decision-making to Strategic Commissioning Committee.
- SMT must become the primary decision engine, moving out of committees and into SMT.
- Delegation must remove ambiguity. There are currently unclear boundaries and overlapping committees and therefore our SoRD must remove all duplication and create clean decision routes.

2. Design Principles

2.1. ✅ Principle 1: Strategic Board – only deciding strategy, major investments and risk appetite.

2.2. ✅ Principle 2: Executive-led decision system. SMT becomes the primary decision maker, committees scrutinise.

2.3. ✅ Principle 3: Single route per decision – with one owner, no parallel approvals and no duplication.

2.4. ✅ Principle 4: Assurance, not re-decision. Committees will challenge, recommend, and escalate, not re-run decisions already made.

2.5. ✅ Principle 5: Visibility replaces control layering – instead of extra approvals, use structure reporting, transparency, and escalation triggers.

3. Harmonised delegation model

Level	Value	Decision Owner	Committee Involvement
L1	<£50k	Budget Holder	None
L2	£50k-£250k	Director	Exec visibility
L3	£250k-£2.5m	Exec Director	SMT reporting
L4	£2.5m-£10m	SMT	Notify Strategic Commissioning Committee
L5	£10m-£25m	SCC	SMT prior review and recommendation
L6	>£25m	Board	Final Approval

3.1. In the above model, Board only handles L6, this removes committee duplication and lack of decision space, and L1-L4 do not go to committees.

3.2. Board has no operational approvals below L6.

3.3. Strategic Commissioning Committee has a strategic challenge role, provides commissioning direction and major service change advice and is delegated approvals at L5 for items reviewed and recommended by SMT.

3.4. Neighbourhood Delivery Committee – no financial delegation

3.5. Audit, Risk and Assurance Committee – assurance only, no decisions

3.6. SEL ICB Charitable Funds Committee - assurance only, no decisions. Financial delegation via CCFO.

3.7. SMT – the decision-making body for operational, financial and commissioning decisions within delegated limits.

3.8. Specialised Commissioning Alignment – given large spend, high risk, and governance complexity, propose a trigger escalation model based on above limits but noting that complexity may require consultation at committee level or consideration at the Board as per section 3.9, below.

3.9. The risk, complexity and sensitivity of individual matters will also be considered when determining the appropriate level of decision-making and may act as a trigger for escalation within the governance framework where appropriate.

Trigger	Action
>£10m specialised investment	SMT and SCC visibility
Service Reconfiguration	SCC
Financial risk	CFO sign off and SMT
National service constraint	NHSE alignment

4. Capital

Value	
<£2.5m	Chief Finance and Compliance Officer
£2.5m - £5m	CEO
£5m - £10m	CFCO and CEO / SMT
£10m - £25m	SMT
>£25m	Board

4.1. ICB letters of support for system business cases e.g. Where NHS England require ICB letters of support for capital business cases and sensitive, novel and contentious business cases.

a. Estimated cost up to £5m – CFCO
b. Estimated cost up to £10m – CFCO and CEO
c. Estimated cost up to £20m – CFCO and CEO/SMT
d. Estimated cost up to £50m – SMT and Strategic Commissioning Committee
e. Above £50m – SMT, Strategic Commissioning Committee and Board

5. SFI Alignment

5.1. The SFIs already give statutory controls, CFO responsibility and procurement compliance and audit requirements and these thresholds do not change. What does change is decision routing clarity, reduction in committee approvals and stronger executive control. “Operational decisions will be taken at the lowest appropriate level in accordance with the Scheme of Delegation. Committees shall not duplicate operational decision-making but provide assurance, scrutiny, and escalation.

6. Summary

6.1. Decisions sit with the executive (SMT), scrutiny sits with committees and a limited authority for SCC to take significant commissioning decisions as determined at L5 in the above table, and the Board focuses on strategy.

NHS
South East London

NHS South East London Integrated Care Board Standing Financial Instructions

July 2026 revision

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1. Purpose and statutory framework

1.1.1 These Standing Financial Instructions (SFIs) shall have effect as if incorporated into the NHS South East London Integrated Care Board's (ICB) constitution. In accordance with the National Health Service Act 2006, as amended by the Health and Care Act 2022, the ICB must publish its constitution.

1.1.2 In accordance with the Act as amended, NHS England is mandated to publish guidance for ICBs, to which each ICB must have regard, in order to discharge their duties.

1.1.3 The purpose of this governance document is to ensure that the ICB fulfils its statutory duty to carry out its functions effectively, efficiently and economically. The SFIs are part of the ICB's control environment for managing the organisation's financial affairs as they are designed to ensure regularity and propriety of financial transactions.

1.1.4 SFIs define the purpose, responsibilities, legal framework and operating environment of the ICB. They enable sound administration, lessen the risk of irregularities and support commissioning and delivery of effective, efficient and economical services.

1.1.5 The ICB is established under Chapter A3 of Part 2 of the National Health Service Act 2006, as inserted by the Health and Care Act 2022 and has the general function of arranging for the provision of services for the purposes of the health services in England in accordance with the Act.

1.1.6 Each ICB is to be established by order made by NHS England for an area within England, the order establishing an ICB makes provision for the constitution of the ICB.

1.1.7 All members of the ICB (its board) and all other Officers should be aware of the existence of these documents and be familiar with their detailed provisions. The ICB SFIs will be made available to all Officers on the intranet and internet website for each statutory body.

1.1.8 Should any difficulties arise regarding the interpretation or application of any of these SFIs, the advice of the Chief Executive or the Chief Finance and Compliance Officer must be sought before acting.

1.1.9 Failure to comply with the SFIs may result in disciplinary action in accordance with the ICB's applicable disciplinary policy and procedure in operation at that time.

2. Scope

2.1.1 All members of staff and officers of the ICB, without exception, are within the scope of the SFIs without limitation. The term officer includes, permanent employees, secondees and contract workers.

2.1.2 Within this document, words imparting any gender include any other gender. Words in the singular include the plural and words in the plural include the singular.

2.1.3 Any reference to an enactment is a reference to that enactment as amended.

2.1.4 Unless a contrary intention is evident, or the context requires otherwise, words or expressions contained in this document, will have the same meaning as set out in the applicable Act.

3. Roles and Responsibilities

3.1 Staff

3.1.1 All ICB Officers are severally and collectively, responsible to their respective employer(s) for:

- abiding by all conditions of any delegated authority
- the security of the statutory organisation's property and avoiding all forms of loss
- ensuring integrity, accuracy, probity and value for money in the use of resources and
- conforming to the requirements of these SFIs

3.2 Chief Executive Officer

3.2.1 The ICB constitution provides for the appointment of the chief executive officer by the ICB chair. The chief executive is the accountable officer for the ICB and is personally accountable to NHS England for the stewardship of the ICB's allocated resources.

3.2.2 The Chief Finance and Compliance officer reports directly to the ICB chief executive officer and is professionally accountable to the NHS England regional finance director.

3.2.3 The chief executive will delegate to the Chief Finance and Compliance officer the following responsibilities in relation to the ICB:

- preparation and audit of the annual accounts
- adherence to the directions from NHS England in relation to accounts preparation
- ensuring that the allocated annual revenue and capital resource limits are not exceeded, jointly, with system partners
- ensuring that there is an effective financial control framework in place to support accurate financial reporting, safeguard assets and minimise risk of financial loss
- meeting statutory requirements relating to taxation
- ensuring that there are suitable financial systems in place (see Section 6)
- meeting the financial targets set for it by NHS England
- use of incidental powers such as management of ICB assets, entering commercial agreements
- ensuring that the Governance statement and annual accounts & reports are signed
- planned budgets are approved by the relevant Board; developing the funding strategy for the ICB to support the board in achieving ICB objectives, including consideration of place-based budgets
- making use of benchmarking to make sure that funds are deployed as effectively as possible
- executive members (partner members, ordinary members and non-executive members) and other officers are notified of and understand their responsibilities within the SFIs
- specific responsibilities and delegation of authority to specific job titles are confirmed

- financial leadership and financial performance of the ICB
- identification of key financial risks and issues relating to robust financial performance and leadership and working with relevant providers and partners to enable solutions and
- the Chief Finance & Compliance Officer will support a strong culture of public accountability, probity, and governance, ensuring that appropriate and compliant structures, systems, and processes are in place to minimise risk

3.3 Audit, Risk and Assurance Committee

3.3.1 The Board and Chief Executive Officer should be supported by an Audit, Risk and Assurance Committee, which should provide proactive support to the board in advising on:

- the management of key risks
- the strategic processes for risk
- the operation of internal controls
- control and governance and the governance statement
- the accounting policies, the accounts, and the annual report of the ICB
- the process for review of the accounts prior to submission for audit, management's letter of representation to the external auditors and the planned activity and results of both internal and external audit
- Seeking assurance that the ICB has adequate arrangements in place for countering fraud and bribery, and compliance with NHS Counter Fraud Authority (NHS CFA) requirements. This will include but is not limited to reports from the Local Counter Fraud Specialist (LCFS), the annual self-assessment submission to NHS CFA Counter Fraud Functional Standard Return, NHS CFA inspection reports and actions resulting from counter fraud activity including NHS CFA quality assessment reports will be monitored.
- The Audit, Risk and Assurance Committee is also responsible for approving the annual counter fraud work plan and the outcomes of all anti-fraud and bribery work within the ICB.The ICB will be liable to prosecution if a person associated with it bribes another person intending to obtain or retain business or an advantage in the conduct of business for the ICB, unless it can show that despite a particular case of bribery it nevertheless had adequate procedures in place to prevent persons associated with it from bribing.

4. Management accounting and business management

4.1.1 The Chief Finance and Compliance Officer is responsible for maintaining policies and processes relating to the control, management and use of resources across the ICB.

4.1.2 The Chief Finance and Compliance Officer will delegate the budgetary control responsibilities to budget holders through a formal documented process.

4.1.3 The Chief Finance and Compliance Officer will ensure:

- the promotion of compliance to the SFIs through an assurance certification process
- the promotion of long-term financial heath for the NHS system (including ICS)
- budget holders are accountable for obtaining the necessary approvals and oversight of all expenditure incurred on the cost centres they are responsible for
- the improvement of financial literacy of budget holders with the appropriate level of expertise and systems training
- that the budget holders are supported in proportion to the operational risk and
- the implementation of financial and resources plans that support the NHS Long term plan objectives

4.1.4 In addition, the Chief Finance and Compliance Officer should have financial leadership responsibility for the following statutory duties:

- the ICB , in conjunction with its partner NHS trusts and NHS foundation trusts, should exercise its functions with a view to ensuring that, in respect of each financial year:
  - local capital resource use does not exceed the limit specified in a direction by NHS England
  - local revenue resource use does not exceed the limit specified in a direction by NHS England
  - the duty of the ICB to perform its functions as to secure that its expenditure does not exceed the aggregate of its allotment from NHS England and its other income and
  -

4.1.5 The Chief Finance and Compliance Officer and any senior officer responsible for finance within the ICB should also promote a culture where budget holders and decision makers consult their finance business partners in key strategic decisions that carry a financial impact.

5. Income, banking arrangements and debt recovery

5.1 Income

5.1.1 An ICB has power to do anything specified in section 7(2)(a), (b) and (e) to (h) of the Health and Medicines Act 1988 for the purpose of making additional income available for improving the health service.

5.1.2 The Chief Finance and Compliance Officer is responsible for:

- ensuring order to cash practices are designed and operated to support, efficient, accurate and timely invoicing and receipting of cash. The processes and procedures should be standardised and harmonised across the NHS System by working co-operatively
- ensuring the debt management strategy reflects the debt management objectives of the ICB and the prevailing risks

5.2 Banking

5.2.1 The Chief Finance and Compliance Officer is responsible for ensuring the ICB complies with any directions issued by the Secretary of State with regards to the use of specified banking facilities for any specified purposes.

5.2.2 The Chief Finance and Compliance Officer will ensure that:

- the ICB holds the minimum number of bank accounts required to run the organisation effectively. These should be raised through the government banking services contract and
- the ICB has effective cash management policies and procedures in place

5.3 Debt management

5.3.1 The Chief Finance and Compliance Officer is responsible for the ICB debt management strategy.

5.3.2 This includes:

- a debt management strategy that covers end-to-end debt management from debt creation to collection or write-off in accordance with the losses and special payment procedures;
- ensuring the debt management strategy covers a minimum period of 3 years and must be reviewed and endorsed by the Audit,Risk and Assurance Committee every 12 months to ensure relevance and provide assurance
- accountability to the SMT that debt is being managed effectively
- accountabilities and responsibilities are defined with regards to debt management to budget holders and
- responsibility to appoint an officer responsible for day-to-day management of debt.

6. Financial systems and processes

6.1 Provision of finance systems

6.1.1 The Chief Finance and Compliance Officer is responsible for ensuring systems and processes are designed and maintained for the recording and verification of finance transactions such as payments and receivables for the ICB.

6.1.2 The systems and processes will ensure, inter alia, that payment for goods and services is made in accordance with the provisions of these SFIs, related procurement guidance and prompt payment practice.

6.1.3 As part of the contractual arrangements for ICBs officers will be granted access where appropriate to the Integrated Single Financial Environment ("ISFE"). This is the required accounting system for use by ICBs, access is based on single access log on to enable users to perform core accounting functions such as to transacting and coding of expenditure/income in fulfilment of their roles.

6.1.4 The Chief Finance and Compliance Officer will, in relation to financial systems:

- promote awareness and understanding of financial systems, value for money and commercial issues
- ensure that transacting is carried out efficiently in line with current best practice – e.g. e-invoicing
- ensure that the ICB meets the required financial and governance reporting requirements as a statutory body by the effective use of finance systems
- enable the prevention and the detection of inaccuracies and fraud, and the reconstitution of any lost records
- ensure that the financial transactions of the authority are recorded as soon as, and as accurately as, reasonably practicable
- ensure publication and implementation of all ICB business rules and ensure that the internal finance team is appropriately resourced to deliver all statutory functions of the ICB
- ensure that risk is appropriately managed
- ensure identification of the duties of officers dealing with financial transactions and division of responsibilities of those officers
- ensure the ICB has suitable financial and other software to enable it to comply with these policies and any consolidation requirements of the ICB
- ensure that contracts for computer services for financial applications with another health organisation or any other agency shall clearly define the responsibility of all parties for the security, privacy, accuracy, completeness, and timeliness of data during processing, transmission and storage. The contract should also ensure rights of access for audit purposes and
- where another health organisation or any other agency provides a computer service for financial applications, the Chief Finance and Compliance Officer shall periodically seek assurances that adequate controls are in operation.

7. Procurement and purchasing

7.1 Principles

7.1.1 The Chief Finance and Compliance Officer will take a lead role on behalf of the ICB to ensure that there are appropriate and effective financial, contracting, monitoring and performance arrangements in place to ensure the delivery of effective health services.

7.1.2 The ICB must ensure that procurement activity is in accordance with the Public Contracts Regulations 2015 (PCR) and Healthcare Service Provider Selection Regime Regulations 2023 (PSR), as well as any associated statutory requirements whilst securing value for money and sustainability.

7.1.3 The ICB must consider, as appropriate, any applicable NHS England guidance that does not conflict with the above.

7.1.4 The ICB will have a procurement policy which sets out all of the legislative requirements.

7.1.5 All revenue and non-pay expenditure must be approved, in accordance with the ICB Scheme of Reservation and Delegation , prior to an agreement being made with a third party that enters a commitment to future expenditure.

7.1.6 All officers must ensure that any conflicts of interest are identified, declared and appropriately mitigated or resolved in accordance with the ICB standards of business conduct policy.

7.1.7 Budget holders are accountable for obtaining the necessary approvals and oversight of all expenditure incurred on the cost centres they are responsible for. This includes obtaining the necessary internal and external approvals which vary based on the type of spend, prior to procuring the goods, services or works.

7.1.8 Undertake any contract variations or extensions in accordance with PCR 2015, Provider Selection Regime 2023, and the ICB procurement policy.

7.1.9 Retrospective expenditure approval should not be permitted. Any such retrospective breaches require approval from any committee responsible for approvals before the liability is settled. Such breaches must be reported to the Audit,Risk and Assurance Committee.

8. Staff costs and staff related non pay expenditure

8.1 HR service

8.1.1 The HR service, outsourced to a partner provider, will lead the development and delivery of the long-term people strategy of the ICB ensuring this reflects and integrates the strategies of all relevant partner organisations within the ICS.

8.1.2 Operationally the HR function will be responsible for:

- defining and delivering the organisation's overall human resources strategy and objectives and
- overseeing delivery of human resource services to ICB employees

8.1.3 The HR function will ensure that the payroll system has adequate internal controls and suitable arrangements for processing deductions and exceptional payments.

8.1.4 Where a third-party payroll provider is engaged, the HR function shall proactively manage this supplier through effective contract management.

8.1.5 The HR function is responsible for management and governance frameworks that support the ICB employees' life cycle.

9. Annual reporting and Accounts

9.1.1 The Chief Finance and Compliance Officer (responsibility for the annual accounts) and Director of Communications (responsibility for the annual report) will ensure, on behalf of the Chief Executive Officer and ICB board, that:

- the ICB is in a position to produce its required monthly reporting, annual report, and accounts, as part of the setup of the new organisation and
- the ICB, in each financial year, prepares a report on how it has discharged its functions in the previous financial year

An annual report must, in particular, explain how the ICB has:

- discharged its duties in relation to improving quality of services, reducing inequalities, the triple aim and public involvement
- review the extent to which the board has exercised its functions in accordance with its published forward plans and capital resource use plan and
- review any steps that the board has taken to implement any joint local health and wellbeing strategy

9.1.2 NHS England may give directions to the ICB as to the form and content of an annual report.

9.1.3 The ICB must give a copy of its annual report to NHS England by the date specified by NHS England in a direction and publish the report.

9.2 Internal audit

9.2.1 The Chief Executive, as the accountable officer, is responsible for ensuring there is appropriate internal audit provision in the ICB. For operational purposes, this responsibility is delegated to the Chief Finance and Compliance Officer to ensure that:

- all internal audit services provided under arrangements proposed by the Chief Finance and Compliance Officer are approved by the Auditor Panel, on behalf of the ICB board
- the ICB must have an internal audit charter. The internal audit charter must be prepared in accordance with the Public Sector Internal Audit Standards (PSIAS)
- the annual ICB internal audit plan must be endorsed by the Audit, Risk & Assurance Committee
- the Head of Internal Audit must provide an annual opinion on the overall adequacy and effectiveness of the ICB Board's framework of governance, risk management and internal control as they operated during the year, based on a systematic review and evaluation
- the Head of Internal Audit should attend Audit, Risk and Assurance Committee meetings and have a right of access to all Audit,Risk and Assurance Committee members, the Chair and Chief Executive of the ICB
- the appropriate and effective financial control arrangements are in place for the ICB and that accepted internal and external audit recommendations are actioned in a timely manner.

9.3 External Audit

9.3.1 The Chief Finance and Compliance Officer is responsible for:

- liaising with external audit colleagues to ensure timely delivery of financial statements for audit and publication in accordance with statutory, regulatory requirements
- ensuring that the ICB appoints an auditor in accordance with the Local Audit and Accountability Act 2014; in particular, the ICB must appoint a local auditor to audit its accounts for a financial year not later than 31 December in the preceding financial year. The ICB must appoint a local auditor at least once every 5 years, and
- ensuring that the appropriate and effective financial control arrangements are in place for the ICB and that accepted external audit recommendations are actioned in a timely manner
- should any suspicion of fraud, bribery or corruption be identified by External Audit, these must be referred to the LCFS immediately.

10. Losses and special payments

10.1.1 HM Treasury approval is required if a transaction exceeds the delegated authority, or if transactions will set a precedent, are novel, contentious or could cause repercussions elsewhere in the public sector.

10.1.2 The Chief Finance and Compliance Officer will support a strong culture of public accountability, probity, and governance, ensuring that appropriate and compliant structures, systems, and process are in place to minimise risks from losses and special payments.

10.1.3 NHS England has the statutory power to require an integrated care board to provide NHS England with information. The information, is not limited to losses and special payments, must be provided in such form, and at such time or within such period, as NHS England may require.

10.1.4 As part of the new compliance and control procedures, ICBs must submit an annual assurance statement confirming the following:

- details of all exit packages (including special severance payments) that have been agreed and/or made during the year
- that NHS England and HMT approvals have been obtained before any offers, whether verbally or in writing, are made and
- adherence to the special severance payments guidance as published by NHS England

10.1.5 All losses and special payments (including special severance payments) must be reported to the ICB Audit, Risk and Assurance Committee and NHS England noting that ICBs do not have a delegated limit to approve losses or special payments.

10.1.6 For detailed operational guidance on losses and special payments, please refer to the ICB losses and special payment guide which is appended to this document.

11. Fraud, bribery and corruption (Economic crime)

11.1.1 The ICB is committed to identifying, investigating and preventing economic crime.

11.1.2 The ICB Chief Finance and Compliance Officer is responsible for ensuring appropriate arrangements are in place to provide adequate counter fraud provision which should include reporting requirements to the Board and Audit,Risk and Assurance Committee, and defined roles and accountabilities for those involved as part of the process of providing assurance to the Board.

11.1.3 The ICB Chief Finance and Compliance Officer should provide clear and demonstrable support and strategic direction for counter fraud, bribery and corruption work, and review the proactive management, control and the evaluation of counter fraud, bribery and corruption work.

11.1.4 The ICB has an Anti-Fraud and Bribery Policy in place, agreed by the Audit, Risk and Assurance Committee. The ICB is required to complete an annual Counter Fraud Functional Standard Return (CFFSR), a self-review tool developed by the NHS CFA, to measure the effectiveness of counter fraud processes and demonstrate the level of compliance with the Government Functional Standard for Counter Fraud - www.gov.uk/government/publications/government-functional-standard-govs-013-counter-fraud, as issued by NHS CFA and any guidance issued by NHS England.. The ICB will ensure that policies and procedures for all work related to fraud and bribery are implemented. The ICB will consider the major findings of investigations and proactive work and respond accordingly.

11.1.5 The LCFS will provide written reports to the Audit, Risk and Assurance Committee on counter fraud work within the ICB.

11.1.6 Further information on fraud, bribery and corruption can be found in the ICB's Anti-Fraud and Bribery Policy.

12. Capital Investments & security of assets and Grants

12.1.1 The Chief Finance and Compliance Officer is responsible for:

- ensuring that at the commencement of each financial year, the ICB and its partner NHS trusts and NHS foundation trusts prepare a plan setting out their planned capital resource use
- ensuring that the ICB, in respect of each financial year, local capital resource use does not exceed the limit specified in a direction by NHS England
- ensuring that there is an effective appraisal and approval process in place for determining capital expenditure priorities and the effect of each proposal upon business plans
- ensuring that there are processes in place for the management of all stages of capital schemes, which will ensure that schemes are delivered on time and to cost
- ensuring that capital investment is not authorised without evidence of availability of resources to finance all revenue consequences and
- for every capital expenditure proposal, the Chief Finance and Compliance Officer is responsible for ensuring there are processes in place to ensure that a business case is produced

12.1.2 Capital commitments typically cover land, buildings, equipment, capital grants to third parties and IT, including:

- authority to spend capital or make a capital grant
- authority to enter into leasing arrangements

12.1.3 Advice should be sought from the Chief Finance and Compliance Officer or nominated officer if there is any doubt as to whether any proposal is a capital commitment requiring formal approval.

12.1.4 For operational purposes, the ICB shall have nominated senior officers accountable for ICB property assets and for managing property.

12.1.5 ICBs shall have a defined and established property governance and management framework, which should:

- ensure the ICB asset portfolio supports its business objectives and
- comply with NHS England policies and directives and with this standard

12.1.6 Disposals of surplus assets should be made in accordance with published guidance and should be supported by a business case which should contain an appraisal of the options and benefits of the disposal in the context of the wider public sector and to secure value for money.

12.2 Grants

12.2.1 The Chief Finance and Compliance Officer is responsible for providing robust management, governance and assurance to the ICB with regards to the use of specific powers under which it can make capital or revenue grants available to:

- any of its partner NHS trusts or NHS foundation trusts, and
- to a voluntary organisation, by way of a grant or loan

13. Legal and insurance

13.1.1 This section applies to any legal cases threatened or instituted by or against the ICB. The ICB should have policies and procedures detailing:

- engagement of solicitors / legal advisors
- approval and signing of documents which will be necessary in legal proceedings and
- Officers who can commit or spend ICB revenue resources in relation to settling legal matters

13.1.2 ICBs are advised not to buy commercial insurance to protect against risk unless it is part of a risk management strategy that is approved by the chief executive officer.

14. Review

14.1 To ensure that these SFIs remain up-to-date and relevant, the Chief Finance and Compliance Officer will review them at least annually. Following consultation with the Chief Executive for the ICB and scrutiny by the ICB's Audit,Risk and Assurance Committee, the Chief Finance and Compliance Officer will recommend amendments, as fitting, to the ICB Board for approval.

Appendix 1 - ICB Losses and Special Payment Guidance

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1. Introduction and guidance statement

1.1.1 The Losses and Special Payments guidance is prepared as procedural guidance for integrated Care Boards (ICBs).

1.1.2 The purpose of this document is to establish best practice that can be incorporated into the ICB's Standing Financial Instructions.

1.1.3 It should be noted that the user of this procedural guidance should be compliant with the respective ICB's SFIs. If there is a need to interpret or difficulty in application of this guidance, please send an email to the NHS England Head of Assurance and Counter Fraud: england.assurance@nhs.net

1.1.4 HM Treasury retains the authority to approve losses and special payments which are classified as being either:

- novel or contentious
- contains lesson that could be of interest to the wider community
- involves important questions of principle
- might create a precedent and/or
- highlights the ineffectiveness of the existing control systems

1.1.5 Losses and special payments are therefore subject to special control procedures compared to the generality of payments, and special notation in the accounts to bring them to the attention of parliament. The annual accounts reporting requirements are detailed herein.

1.1.6 For the avoidance of doubt, as NHS England, has not delegated authority to ICBs for the approval of losses and special payments, all cases relating to ICB losses and special payments must be submitted to NHS England for approval.

1.1.7 NHS England cannot delegate any authority to ICBs as this would result in "double delegation".

1.1.8 This should be clearly reflected in the detailed financial policies for all ICBs.

1.1.9 For losses that indicate or give rise to suspicion of fraud or corruption, please follow the guidance as provided by your local counter fraud specialist.

1.1.10 In dealing with individual cases, ICBs must consider the soundness of their internal control systems, the efficiency with which they have been operated, and take any necessary steps to put failings right.

1.1.11 The outcome of the review of the case under consideration (1.1.10) must be clearly indicated when submitting cases to NHS England as part of the account's consolidation process at year end or as part of the approval process.

2. Scope

2.1.1 This procedural document is applicable to the following NHS bodies:

- Integrated Care Boards

3. Definitions

3.1.1 Unless a contrary intention is evident or the context requires otherwise, words or expressions contained in this document will have the same meaning as set out in HMT managing public money. As per the DHSC Group Accounting Manual, Managing Public Money applies to all DHSC group bodies.

3.2 Losses

3.2.1 A loss refers to any case where full value has not been obtained for money spent or committed.

3.2.2 Examples of types of losses which cannot be treated as business as usual are cash losses, bookkeeping losses, fruitless payments and claims waived or abandoned.

3.3 Special Payment

3.3.1 Special Payments relate to the following:

- any compensation payments
- extra-contractual or ex-gratia payments and
- any payment made without specific identifiable legal power In accordance with the National Health Service Act 2006, as amended by the Health and Care Act 2022.

3.4 Special Severance and retention payments

3.4.1 ICBs have not been delegated a limit to approve special severance or retention payments. For detailed guidance, please refer to the ICB special severance payments document as published on the NHSEI SharePoint finance library.

3.4.2 For clarity, any special severance payments that are being considered for approval must be submitted to NHS England (england.assurance@nhs.net) prior to settlement.

3.4.3 The table below lists all the various expenditure classifications for losses and special payments.

Payment Type	Classification	Definition
Fruitless Payment	Loss	A fruitless payment is a payment which cannot be avoided because the recipient is entitled to it even though nothing of use to the department will be received in return Fruitless payments include payments for rail fares and hotels that are not required but could not be cancelled without a partial or full charge being incurred
Bookkeeping Losses	Loss	Bookkeeping losses (un-vouched or incompletely vouched payments) including missing items or inexplicable or erroneous debit balances
Constructive loss	Loss	A constructive loss is a similar form of payment to stores losses and fruitless payments, but one where procurement action itself caused the loss. For example, stores or services might be correctly ordered, delivered or provided, then paid for as correct, but later, perhaps because of a change of policy, they might prove not to be needed or to be less useful than when the order was placed
Administrative costs	Loss	An expense incurred in controlling and directing an organisation
Claims Waived or Abandoned	Loss	Losses may arise if claims are waived or abandoned because, though properly made, it is decided not to present or pursue them
Extra-contractual payments	Special Payment	Payments which, though not legally due under contract, appear to place an obligation on a public sector organisation which the courts might uphold. Typically, these arise from the organisation's action or inaction in relation to a contract. Payments may be extra-contractual even where there is some doubt about the organisation's liability to pay, e.g. where the contract provides for arbitration, but a settlement is reached without it. A payment made as a result of an arbitration award is contractual
Extra-statutory	Special Payment	Payments which are within the broad intention of the statute or regulation but go beyond a strict interpretation of its terms
Extra-regulatory payments	Special Payment	Payments which are within the broad intention of the statute or regulation but go beyond a strict interpretation of its terms
Compensation payments	Special Payment	Payments made to provide redress for personal injuries (except for payments under the Civil Service Injury Benefits Scheme), traffic accidents, and damage to property etc., suffered by civil servants or others. They include other payments to those in the public service outside statutory schemes or outside contracts
Special severance payments	Special Payment	Payments made to employees, contractors and others beyond above normal statutory or contractual requirements when leaving employment in public service whether they resign, are dismissed or reach an agreed termination of contract

Ex gratia payments	Special Payment	Go beyond statutory cover, legal liability, or administrative rules, including payments: • made to meet hardship caused by official failure or delay • out of court settlements to avoid legal action on grounds of official inadequacy and • payments to contractors outside a binding contract, e.g. on grounds of hardship
Retention payments	Special Payment	Payments, designed to encourage staff to delay their departures, particularly where transformations of ALBs are being negotiated, are also classified as novel and contentious. Such payments always require explicit Treasury approval, whether proposed in individual cases or in groups. Treasury approval must be obtained before any commitment, whether oral or in writing, is made.

3.5 Annual assurance statements

3.5.1 As part of the new compliance and control procedures, ICBs must submit an annual assurance statement confirming the following:

- details of all exit packages (including special severance payments) that have been agreed and/or made during the year
- that NHS England and HMT approvals have been obtained (in relation to non-contractual pay elements or amounts that exceed the ICB delegated limits) before any offers, whether verbally or in writing, are made, and
- adherence to the special severance payments guidance as published by NHS England

3.6 Interpretation

3.6.1 Should any difficulties arise regarding the interpretation or application of any part of this losses and special payments guidance, the advice of the Head of Assurance and Counter Fraud (england.assurance@nhs.net) must be sought before acting.

3.7 Delegation of Function, Duties and Powers

3.7.1 The ICB Constitution must have a governing body that makes provision for the appointment of the Audit & Risk Committee.

3.7.2 The ICB Standing Financial Instructions should clearly indicate the role that the Audit & Risk Committee has in reviewing and approving losses and special payments.

3.7.3 The ICB Schedule of Matters Delegated to Officers should indicate the delegated limits that have been agreed by the Board for operational purposes, however, it should be noted that they are localised limits and not delegated from NHS England or the Department of Health and Social Care.

4. Integrated care board reporting requirements

4.1 Capturing of losses and special payments

4.1.1 The ICB chief financial officer is responsible for ensuring that processes and procedures that facilitate the capturing and reporting of losses and special payments are in place and ensure that a losses and special payments register is maintained.

4.1.2 All losses and special payments for ICBs must be recorded in the register and reviewed as part of the internal controls process.

4.2 Parliamentary accountability and audit report

4.2.1 The ICB must maintain a losses and special payments register that provides the requested information to complete the NHS England group account.

4.2.2 It should be noted that ICBs do not have a mandatory requirement to produce a Parliamentary accountability and audit report as other entities that report directly to Parliament do. However, it is a mandatory requirement that ICBs produce an audit certificate and report.

There will be a need to collect data for the NHS England consolidated account. NHS England will also use this information to complete the DHSC summarisation schedule for the DHSC consolidated account. Therefore, regardless of applicability of this report, all ICBs must ensure the summarisation schedule is completed.

4.2.3 If there are any individual cases or a group of losses or special payments that exceed the aggregate value of £100,000, the related payment should be noted separately on the ICB year-end template completed for the NHS England group account.

5. Roles and responsibilities

5.1 Financial Control

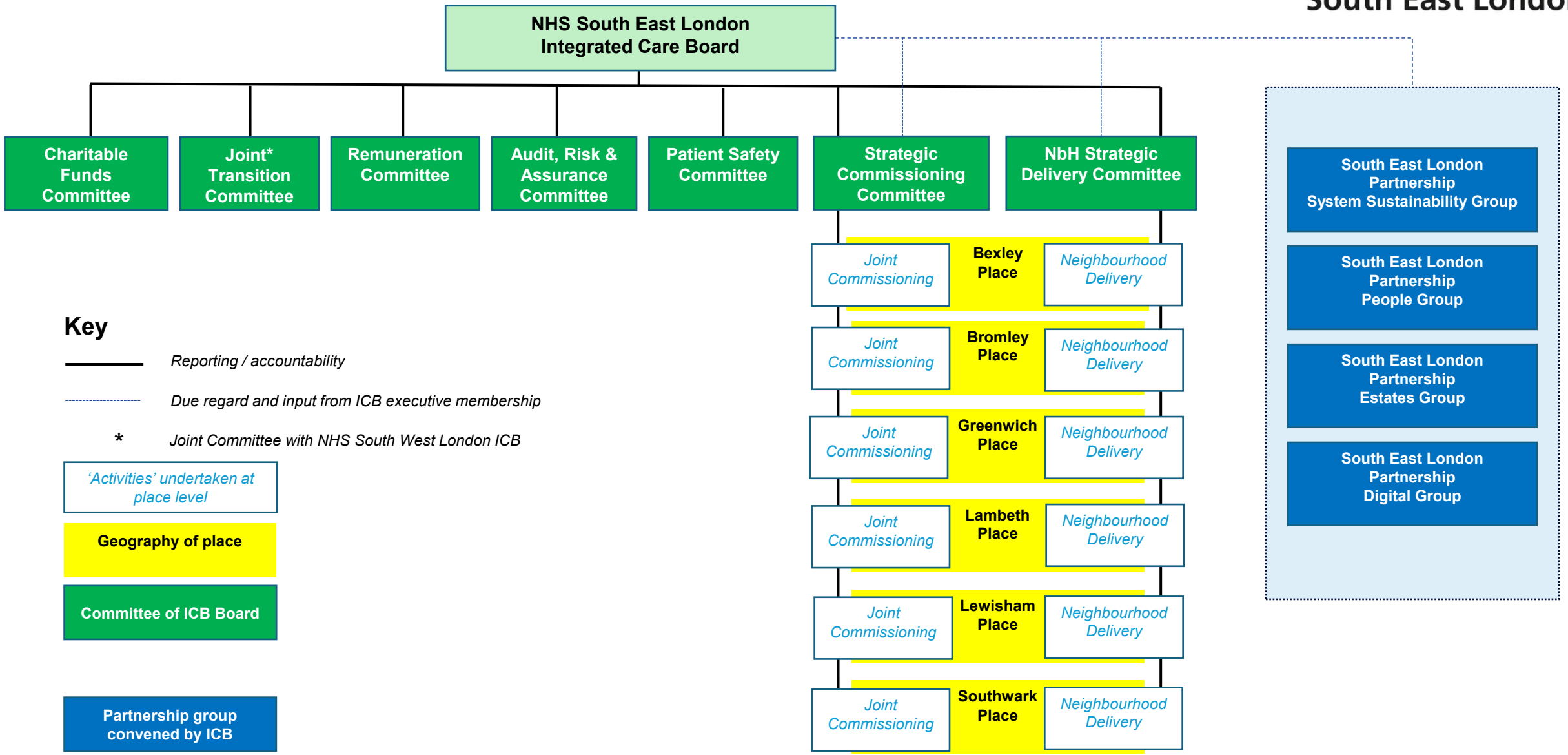
5.1.1 Chief Financial Officer

5.1.2 It is noted and acknowledged that the roles and responsibilities for the chief financial officer vary in all the ICBs. The chief financial officer should implement a system of internal control that details the process for reporting losses, recording losses, monitoring and reporting the losses and special payments to the ICB's Audit & Risk Committee based on existing reporting cycles.

5.1.3 The reporting cycle should also clarify the delegated sum that the chief financial officer can authorise as a loss or special payment. The delegated sum should be in line with the ICB escalation process for losses and special payments.

SEL ICB top-level governance structure

NHS South East London



Key

Reporting / accountability

Due regard and input from ICB executive membership

* *Joint Committee with NHS South West London ICB*

'Activities' undertaken at place level

Geography of place

Committee of ICB Board

Partnership group convened by ICB

NHS South East London Integrated Care Board
Strategic Commissioning Committee
Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	April 2026	Initial core parts of ToRs developed by Directors of Commissioning in SEL ICB
2	15 May 2026	Initial draft in ToRs template by SEL governance team for engagement
3	28 May 2026	To reflect SWL F&P Committee inputs on future committee functions
4	3 June 2026	Updated to incorporate amendments proposed by SWL Director of Corporate and SEL NEM member for governance
5	10 June 2026	Updated to incorporate amendments proposed by SWL and SEL NEMs
6	28 June 2026	CEO review and comments
7	7 July 2026	Added 2 x PELs as committee members

Approved by

Version	Date	Committee / Group name

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1. Constitution

1.1 The Strategic Commissioning Committee is established by the Integrated Care Board (the Board or ICB) as a committee of the Board. in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board. They will be published as part of the Governance Handbook on the ICB's website.

2. Authority

2.1 The Strategic Commissioning Committee is authorised by the Board to:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Strategic Commissioning Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so, the Committee must follow any procedures put in place by the ICB for obtaining legal or professional advice;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Strategic Commissioning Committee members. The Strategic Commissioning Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Strategic Commissioning Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Strategic Commissioning Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1 The Strategic Commissioning Committee is established to act as the principal forum for strategic commissioning development and oversight on behalf of the Board.

3.2 The ICB's core purpose is strategic commissioning, inclusive of the development of enhanced strategic commissioning capabilities and approaches, with an increasing focus on improving population health outcomes, reducing inequalities, delivering its strategic ambitions (including the 'left shift' towards prevention, community-based care and digital), commissioning high quality, effective and efficient services and exercising its system convening role working with partners.

3.3 The Strategic Commissioning Committee is a formal committee of the ICB Board and will be authorised by the Board to put forward proposals which meet the objectives of the ICB's strategic plan, including proposals for service change, procurement, outcomes and key enablers for strategic commissioning.

3.4 The Committee has a role in ensuring alignment of borough based joint commissioning with the ICB strategy and the effective delivery of agreed ICB priorities and outcomes where relevant. Joint commissioning will be led at place (with defined budgets and commissioning responsibility) to enable close alignment with local authority commissioning and delivery responsibilities, including public health and social care and with a focus on ensuring that joint commissioning arrangements support integration of services and care for local populations.

3.5 The Committee will have a close working relationship with the Neighbourhood Based Care Committee. The Neighbourhood Based Care Committee will be responsible for overseeing the delivery of the ICB's neighbourhood based care agenda which includes ensuring the delivery of commissioning priorities and actions relating to the development and delivery of neighbourhood care. The Strategic Commissioning Committee will have a role in ensuring that the ICB's strategic commissioning, operational plans and use of payor/contracting functions support the effective delivery of the agreed neighbourhood care objectives and outcomes.

3.6 An annual programme of business will be agreed before the start of the financial year and will include strategic commissioning, integrated planning, affordability, delivery oversight and performance and assurance matters. The programme will remain flexible to new and emerging priorities, risks and system pressures.

4. Responsibilities of the Strategic Commissioning Committee

4.1 The committee will operate across the commissioning cycle in an advisory and scrutinising capacity, providing targeted assurance to support the Board rather than replicating an oversight role.

4.2 The committee will take decisions on matters delegated to it as determined by the ICB's SoRD.

4.3 A key role of the committee's is to advise and provide recommendations to the Board to ensure commissioning strategy is data and insights driven and reflects population need, inequalities, and the Board's agreed strategic intent, ensuring consideration of the evidence supporting health outcomes clinical outcomes and cost-effectiveness. clinical and cost-effectiveness evidence base.

4.4 Specific responsibilities of the Committee are as follows. In discharging these responsibilities the Committee will:

Delivery of the ICB strategy

4.5 Provide assurance to the Board on the delivery of the ICB's strategic ambitions, including the 'three shifts' and impact on outcomes and value.

4.6 Assure that there is alignment between commissioning priorities, operational planning, activity assumptions, workforce implications and financial frameworks to support delivery of agreed outcomes and system sustainability.

4.7 Advise the Board on major service change, reconfiguration, investment prioritisation, affordability considerations and strategic trade-offs to support Board decision making.

4.8 Provide oversight and scrutiny of strategic commissioning and procurement approaches relating to major service transformation, contracting arrangements and commercial decisions within the scope of the SoRD.

4.9 Provide strategic oversight of how the ICB uses its commissioning, contracting and 'payor function' responsibilities to shape the future health and care market to help contribute to delivery of its longer-term strategic objectives.

4.10 Provide advice, support and oversight of the development of key strategic commissioning capabilities and outputs e.g. frameworks and decision-making tools to support decision making and prioritisation, outcomes and evaluation.

4.11 Approve arrangements to secure continuous improvement in quality and patient outcomes

4.12 Ensure the ICB's underpinning care pathway and borough based joint commissioning integrated arrangements are aligned to the ICB's commissioning strategy and financial frameworks.

In-year contract delivery and performance

4.13 Review integrated system performance across commissioned services including financial, activity. quality[1], and delivery performance, together with associated risk, prioritising material variance, affordability and strategic concern.

4.14 Review delivery of savings, efficiency and productivity programmes linked to the ICB's commissioning, revenue and capital plans, including associated risks, mitigations and delivery confidence.

[1] The SCC will assume primary responsibility for the oversight of the quality of ICB-commissioned services as is required by current NHSE / NQB guidance. The ICB Patient Safety Committee will focus on assurance related to the safety and quality of care for the ICB's statutory functions, such as CHC, LeDeR. See Patient Safety Committee ToRs.

4.15 Review forecast financial and operational performance, including material risks, delivery assumptions and mitigating actions relating to commissioned services and system plans.

4.16 Escalate material financial, operational and commissioning or delivery concerns to the Board where risks, performance or affordability issues exceed agreed tolerances or require strategic decision-making.

4.17 Assure the evaluation of the impact and outcomes of services commissioned by the ICB including consideration of patients' views / voice, to ensure that lessons learned are identified and used to inform future commissioning decisions.

Other responsibilities

4.18 Approval of contracts at a level specified in the SoRD.

4.19 During the transition to new legislation, new Board membership and future ICB governance arrangements, the Committee may provide an important forum for review and discussion of key commissioning matters, supporting the ICB to conduct its commissioning business.

5. Membership and attendance

Membership

5.1 The Strategic Commissioning Committee membership is as follows:

- 5.1.1 Non Executive Lead – Chair
- 5.1.2 Non Executive Director – Deputy Chair
- 5.1.3 ICB Chief Executive Officer
- 5.1.4 Chief Commissioning Officer
- 5.1.5 Directors of Strategic Commissioning
- 5.1.6 Chief Financial & Compliance Officer
- 5.1.7 Place Executive Leads x 2 (representing all place PELs)
- 5.1.8 Chief Nurse
- 5.1.9 Medical Director.

5.2 The role of Chair will be undertaken by Non-Executive Lead.

5.3 Members are required to attend a minimum of 75% of meetings, other than absence due to sickness.

5.4 Members may nominate deputies to represent them in their absence and make decisions on their behalf, subject to the approval of the Chair.

5.5 Members will possess between them knowledge, skills and experience in the issues pertinent to the Strategic Commissioning Committee 's business. When determining the membership of the Strategic Commissioning Committee, active consideration will be made to diversity and equality.

Chair and vice chair

5.6 The Strategic Commissioning Committee will be chaired by a member appointed on account of their specific knowledge skills and experience making them suitable to chair the Strategic Commissioning Committee.

5.7 Strategic Commissioning Committee members may appoint a Vice Chair who will be nominated by the Chair of the Strategic Commissioning Committee. If the Committee Chair is absent or is disqualified from participating by a conflict of interest, the Vice Chair, if present, shall preside. If the Chair or Vice Chair are absent from any meeting, a Chair shall be nominated by other members attending that meeting.

5.8 The Chair will be responsible for agreeing the agenda and ensuring matters discussed meet the objectives as set out in these ToR.

Attendees

5.9 The Strategic Commissioning Committee shall have the following non-voting attendees (as and when required), noting this list is not intended to be exhaustive and may also change over time as agreed with the committee Chair:

- Chief Pharmacist for the ICB,
- Director of Delivery for Neighbourhoods and Population Health
- Director of Insights
- Additional Place Executive Leads
- Director of Public Health.

5.10 Attendees may present at meetings and contribute to the relevant discussions but are not allowed to participate in any formal vote.

5.11 Attendees may nominate deputies to represent them in their absence, with agreement of the Chair.

5.12 The Strategic Commissioning Committee may call additional experts to attend meetings on a case-by-case basis to inform discussion.

5.13 The Strategic Commissioning Committee may invite or allow people to attend meetings as observers. Observers may not present at meetings, contribute to any discussion or participate in any formal vote.

5.14 The Chair of the ICB will be invited to attend one meeting each year in order to gain an understanding of the Committee's operations.

5.15 The Strategic Commissioning Committee Chair may ask any or all of those who normally attend, but who are not members, to withdraw to facilitate open and frank discussion of particular matters.

6. Meeting Frequency, Quoracy and Decisions

6.1 The Strategic Commissioning Committee will usually meet every two months. Arrangements and notice for calling meetings are set out in the Standing Orders. Additional meetings may take place as required.

6.2 The Board, Chair or Chief Executive may ask the Strategic Commissioning Committee to convene further meetings to discuss particular issues on which they want the committee's advice.

6.3 Meetings will be held in-person, unless previously agreed by the Chair. The Strategic Commissioning Committee may choose to meet online, via MS Teams or suitable alternative platform.

Quorum

6.4 For a meeting to be quorate the following members will be required:

6.4.1 Chair or Deputy Chair;

6.4.2 One Director of Strategic Commissioning;

6.4.3 Finance Representative;

6.4.4 Place Executive Lead

6.4.5 Medical Director or Chief Nurse

6.5 If any member of the Strategic Commissioning Committee has been disqualified from participating in an item on the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

6.6 If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Decision making and voting

6.7 Decisions will be taken in accordance with the Standing Orders. The Strategic Commissioning Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

6.8 Only members of the Strategic Commissioning Committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.9 Where there is a split vote, with no clear majority, the Chair of the Strategic Commissioning Committee will hold the casting vote.

6.10 If a decision is needed which cannot wait for the next scheduled meeting, the Chair may conduct business on a 'virtual' basis through email or other electronic communication.

6.11 Any decisions taken virtually must be recorded at the next scheduled meeting.

7. Accountability and reporting

7.1 The Strategic Commissioning Committee is accountable to the ICB Board and shall provide reports to the Board on how it discharges its responsibilities and shall draw to the attention to any issues that require disclosure or require action.

7.2 The Strategic Commissioning Committee shall make any such recommendations to the Board it deems appropriate on any area within its remit where action or improvement is needed.

7.3 The minutes of the meetings shall be formally recorded by the secretary and approved at the next scheduled meeting.

7.4 The Strategic Commissioning Committee will report to the Board at least annually to describe how it has fulfilled its ToR, give details of any significant issues that it has considered, and how they were addressed.

8. Conflicts of Interest

8.1 Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2 The Strategic Commissioning Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3 All members and attendees of the Strategic Commissioning Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

ICB values

9.1 Members will be expected to conduct business in line with the ICB values and objectives.

9.2 Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3 Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1 The Strategic Commissioning Committee shall be supported with a secretariat function which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the Board; and
- The Strategic Commissioning Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1 The Strategic Commissioning Committee will review its effectiveness at least annually and recommend any changes it considers necessary to the Board.

11.2 These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to the Board for approval.

Date of approval: TBC [to be added]

Date of next review: TBC [to be added]

NHS South East London
Integrated Care Board

Neighbourhood Strategic Delivery Committee

Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	April 2026	Initial core parts of ToRs developed by directors of Neighbourhood Delivery in SEL and SWL
2	15 May 2026	Initial draft in ToRs template by SEL governance team for engagement
3	3 June 2026	Updated to incorporate amendments proposed by SWL Director of Corporate and SEL NEM member for governance
4	12 June 2026	Updated to incorporate suggestions from ICB NEMs
5	28 June 2026	CEO review and comments.

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1. Constitution

1.1 The Neighbourhood Strategic Delivery Committee (the Committee) is established by the Integrated Care Board (the Board or ICB) as a committee of the Board. in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board.

2. Authority

2.1 The Neighbourhood Strategic Delivery Committee is authorised by the ICB Board:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Neighbourhood Strategic Delivery Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so, the Committee must follow any procedures put in place by the ICB for obtaining legal or professional advice;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Neighbourhood Strategic Delivery Committee members. The Neighbourhood Strategic Delivery Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Neighbourhood Strategic Delivery Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Neighbourhood Strategic Delivery Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1 The purpose of the Neighbourhood Strategic Delivery Committee will be to act as the principal forum for agreeing the overall SEL ICB approach to development of the

Neighbourhood Health Service and assuring progress in delivering the Neighbourhood Health Service on behalf of the Board.

3.2 The committee will develop proposals for care models, strategic plans, outcomes and key enablers for integrated neighbourhood health and care.

3.3 The Committee has a role in ensuring alignment of other system plans with the Neighbourhood Delivery Plan and the effective delivery of agreed ICB priorities and outcomes where relevant. Whilst those other plans (e.g. digital, estates and people) may be led by other parts of the system, the Neighbourhood Delivery Committee will work with the relevant accountable bodies across the system with the aim of ensuring that relevant transformation and delivery plans support the delivery of integrated neighbourhood health and care.

3.4 The Committee will be responsible for ensuring the delivery of commissioning priorities and actions relating to the development and delivery of neighbourhood care as set by the Strategic Commissioning Committee. The Neighbourhood Strategic Delivery Committee will not make decisions on any commissioning activities (i.e. the award of new contracts).

3.5 The committee will also operate as a key component of the wider system partnership arrangements, working in close alignment with system groups responsible for system sustainability, workforce, digital and estates. Executive members of the committee will provide the primary link between these forums, ensuring strategic alignment and coordinated delivery across the system.

4. Responsibilities of the Neighbourhood Strategic Delivery Committee

4.1 The committee will undertake the following responsibilities:

4.2 Advise and provide recommendations to the Board on the strategic direction of the integrated neighbourhood health and care, the overall delivery roadmap and milestones and setting of population health outcomes.

4.3 Provide oversight of the development of key neighbourhood strategies and resources including the model of care.

4.4 Approve plans with integrator partnerships for the development of integrated neighbourhood health and care.

4.5 Monitor delivery progress, ensuring plans are adjusted as required to secure delivery of agreed outcome and milestones. This will include driving improvement further and faster where it is feasible to do so.

4.6 Provide assurance to the Board on the delivery of commissioning priorities and actions relating to the development and delivery of neighbourhood care as set by the Strategic Commissioning Committee, managing unwarranted variation in approach.

4.7 Provides assurance to the Board on the ICB's response to NHS England and The Department of Health and Social Care's neighbourhood health requirements.

4.8 Assures the work of ICB executives as part of other groups across London and within South East London to facilitate the development to the tools and infrastructure required to enable integrated neighbourhood health and care.

4.9 Through the committee's executive membership links to system partnership groups in SEL on system sustainability, workforce, digital and estates, and also the London Neighbourhood Board.

5. Membership and attendance

Membership

5.1 The Neighbourhood Strategic Delivery Committee membership is as follows:

- Non-Executive Director – Co-Chair
- Local Authority representative (executive member representing the local authority sector in SEL) – Co-Chair
- ICB Partner Member (Primary Care)
- Place representatives (including representatives from ICB place leadership and provider integrators).
- SEL Director of Delivery for Neighbourhoods and Population Health for SEL
- Director of Strategic Commissioning
- Director of Public Health
- SEL VCSE Alliance Representative
- ICB Medical Director

5.2 The role of Chair will be undertaken by the ICB Non-Executive Director and the Local Authority Member, who will act as co-Chair.

5.3 Members are required to attend a minimum of 75% of meetings, other than absence due to sickness.

5.4 Members may nominate deputies to represent them in their absence and make decisions on their behalf, subject to the approval of the Chair.

5.5 Members will possess between them knowledge, skills and experience in the issues pertinent to the Neighbourhood Strategic Delivery Committee's business. When

determining the membership of the Neighbourhood Strategic Delivery Committee, active consideration will be made to diversity and equality.

Chair and vice chair

5.6 The Neighbourhood Strategic Delivery Committee will be chaired by a member appointed on account of their specific knowledge skills and experience making them suitable to chair the Neighbourhood Strategic Delivery Committee.

5.7 Neighbourhood Strategic Delivery Committee members may appoint a Vice Chair who will be nominated by the Chair of the Neighbourhood Strategic Delivery Committee. If the Committee Chair is absent or is disqualified from participating by a conflict of interest, the Vice Chair, if present, shall preside. If the Chair or Vice Chair are absent from any meeting, a Chair shall be nominated by other members attending that meeting.

5.8 The Chair will be responsible for agreeing the agenda and ensuring matters discussed meet the objectives as set out in these ToR.

Attendees

5.9 The Neighbourhood Strategic Delivery Committee shall have the following non-voting attendees (as and when required):

- Other Directors and/or Managers as appropriate;

5.10 Attendees may present at meetings and contribute to the relevant discussions but are not allowed to participate in any formal vote.

5.11 Attendees may nominate deputies to represent them in their absence, with agreement of the Chair.

5.12 The Neighbourhood Strategic Delivery Committee may call additional experts to attend meetings on a case by case basis to inform discussion.

5.13 The Neighbourhood Strategic Delivery Committee may invite or allow people to attend meetings as observers. Observers may not present at meetings, contribute to any discussion or participate in any formal vote.

5.14 The Chair of the ICB will be invited to attend one meeting each year in order to gain an understanding of the Committee's operations.

5.15 The Neighbourhood Strategic Delivery Committee Chair may ask any or all of those who normally attend, but who are not members, to withdraw to facilitate discussion of particular matters.

6. Meeting Frequency, Quoracy and Decisions

6.1 The Neighbourhood Strategic Delivery Committee will usually meet once f each quarter. Arrangements and notice for calling meetings are set out in the Standing Orders. Additional meetings may take place as required.

6.2 The Board, Chair or Chief Executive may ask the Neighbourhood Strategic Delivery Committee to convene further meetings to discuss particular issues on which they want the committee's advice.

6.3 Meetings will be held in-person, unless previously agreed by the Chair. The Neighbourhood Strategic Delivery Committee may choose to meet online, via MS Teams or suitable alternative platform.

Quorum

6.4 For a meeting to be quorate the following members will be required:

- One of Non-Executive Director – Co-Chair or local authority representative – Co-Chair (proposed)
- Place representatives (including representatives from ICB place leadership and provider integrators)
- SEL Director of Delivery for Neighbourhoods and Population Health.

6.5 If any member of the Neighbourhood Strategic Delivery Committee has been disqualified from participating in an item on the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

6.6 If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Decision making and voting

6.7 Decisions will be taken in accordance with the Standing Orders. The Neighbourhood Strategic Delivery Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

6.8 Only members of the Neighbourhood Strategic Delivery Committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.9 Where there is a split vote, with no clear majority, the Chair of the Neighbourhood Strategic Delivery Committee will hold the casting vote.

6.10 If a decision is needed which cannot wait for the next scheduled meeting, the Chair may conduct business on a 'virtual' basis through email or other electronic communication.

6.11 Any decisions taken virtually must be recorded at the next scheduled meeting.

7. Accountability and reporting

7.1 The Neighbourhood Strategic Delivery Committee is accountable to the ICB Board and shall provide reports to the ICB Board on how it discharges its responsibilities and shall draw to the attention to any issues that require disclosure or require action.

7.2 The Neighbourhood Strategic Delivery Committee shall make any such recommendations to the Board as it deems appropriate on any area within its remit where action or improvement is needed.

7.3 The minutes of the meetings shall be formally recorded by the secretary and approved at the next scheduled meeting.

7.4 The Neighbourhood Strategic Delivery Committee will report to the ICB Board at least annually to describe how it has fulfilled its ToR, give details of any significant issues that it has considered, and how they were addressed.

7.5 The committee will, as required and through its executive members, act to ensure there is alignment between its work and that of system partnership Boards for sustainability, workforce, estates and digital.

8. Conflicts of Interest

8.1 Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2 The Neighbourhood Strategic Delivery Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3 All members and attendees of the Neighbourhood Strategic Delivery Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

ICB values

9.1 Members will be expected to conduct business in line with the ICB values and objectives.

9.2 Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3 Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1 The Neighbourhood Strategic Delivery Committee shall be supported with a secretariat function which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the ICB Board and;
- The Neighbourhood Strategic Delivery Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1 The Neighbourhood Strategic Delivery Committee will review its effectiveness at least annually and recommend any changes it considers necessary to the ICB Board.

11.2 These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to the ICB Board for approval.

Date of approval: TBC [to be updated]

Date of next review: TBC [to be updated]

NHS South East London Integrated Care Board
Patient Safety Committee

Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	8 May 2026	Core components drafted by SWL governance team, SWL CNO and CMO, based on previous RemCo ToRs and aligned with NHS England guidance.
2	1 June 2026	Updated from earlier draft developed with executive leads to full ToRs template
3	15 June 2026	Updated by SWL and SEL Governance teams to reflect split of responsibilities with SCC
4	28 June 2026	CEO review and comments.

Approved by

Version	Date	Committee / Group name

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1. Constitution

1.1 The Patient Safety Committee (the Committee) is established by the Integrated Care Board (the Board or ICB) as a committee of the Board. in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board

2. Authority

2.1 The Patient Safety Committee is authorised by ICB Board to:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Patient Safety Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so, the Committee must follow any procedures put in place by the ICB for obtaining legal or professional advice;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Patient Safety Committee members. The Patient Safety Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Patient Safety Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Patient Safety Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1 Under the current NHS Operating Model and Strategic Commissioning Framework the key requirements for quality oversight in an ICS published by the National Quality Board remain. All health and care organisations remain accountable for quality and have a responsibility to work together to proactively share intelligence, drive improvement and mitigate and escalate quality risks.

3.2 The ICB Patient Safety Committee will undertake assurance and oversight on behalf of the ICB for the identification, monitoring and escalation of quality, safeguarding and ICB operational performance issues in relation to ICB statutory responsibilities and duties. The Commissioning Strategy Committee will consider the quality of commissioned services as part of its role in assuring the delivery by providers of services commissioned by the ICB.

3.3 The ICB Patient Safety Committee will provide oversight and assurance to the Board that the ICB continues to meet its responsibilities for system quality as defined by the National Quality Board. The Committee is established to ensure that the ICB is delivering its functions in a way that secures continuous improvement in the quality of services, against each of the dimensions of quality set out in the Shared Commitment to Quality and enshrined in the Health and Care Act 2022.

3.4 The committee will provide oversight and assurance to the Board and that where NHSE have requested targeted measures for improvement through contractual levers these are being monitored and actioned.

3.5 The Committee exists to scrutinise the robustness of, and gain and provide assurance to the ICB, that there is an effective system of quality governance and internal control that supports it to effectively deliver its strategic objectives and provide sustainable, high-quality care.

3.6 The duties of the Patient Safety Committee will be driven by the organisation's objectives and the associated risks. An annual programme of business will be agreed before the start of the financial year; however this will be flexible to new and emerging priorities and risks.

4. Responsibilities of the Patient Safety Committee

4.1 Specific responsibilities of the Committee are as follows:

4.2 The committee is responsible for ensuring the robustness of the systems in place across the ICB to secure effective quality and safeguarding governance and assurance. As such the committee will approve arrangements to minimise clinical risk and maximise patient safety.

4.3 The committee will ensure that these systems and processes allow the ICB to comply with all relevant legislation and national guidance, to effectively deliver its strategic objectives and commission sustainable, high-quality care whilst also ensuring appropriate safeguards are in place to protect the most vulnerable.

4.4 The committee will approve arrangements for supporting NHS England in discharging its responsibilities in relation to securing continuous improvement in the quality of general medical services.

4.5 The Committee will provide assurance that the ICB is meeting its system level responsibilities as defined by National Quality Board (NQB) in that the ICB is expected to:

- maintain System Quality Groups (SQG) as a forum for intelligence sharing and partnership working to improve quality.
- ensure the SQG has representation from all system partners as per the NQB guidance, including Integrated Health Organisations (IHO) as appropriate
- review SQG footprints, aligning with cluster/ merged footprints as appropriate
- be represented at the Regional Quality Group, ensuring effective escalation.

4.6 The Committee will receive reports from the System Quality Group (SQG) to review and identified themes and shared learning from Serious Case Reviews, Adult Learning Reviews and Domestic Homicide reviews drawing on intelligence and collaboration with place based Local Safeguarding Partnerships, Safeguarding Boards and Safer Community Partnerships, working collaboratively with ICB partners to do so.

4.7 Oversee and scrutinise the ICB's response to all relevant directives, regulations, national standard, policies, reports, reviews inspections, and best practice as issued by the Department for Health and Social Services (DHSC), NHS England and other regulatory bodies / external agencies (e.g. the Care Quality Commission (CQC), National Institute for Health and Care Excellence (NICE).

4.8 Maintain an overview of changes in the methodology employed by regulators and changes in legislation/regulation and assure the ICB that these are disseminated and implemented across all sites.

4.9 Receive assurance that the ICB has effective and transparent mechanisms in place to monitor mortality and that it learns from deaths (including coronial inquests and Prevention of Future Death reports).

4.10 Provide the ICB with assurance that it is delivering its statutory duties for safeguarding adults, children, children looked after and Special Educational Needs and Disability as laid out in Section 11 - The Children Act, 2004, Working Together to Safeguard Children, 2018, The Care Act, 2014, Promoting the Health and Wellbeing of Looked After Children 2015, SEND code of practice 0- 25yrs, 2015.

4.11 Comprehensively scrutinise the robustness of the arrangements for, and assure compliance with, the ICB's statutory responsibilities for:

- infection prevention and control
- medicines optimisation and safety
- equality and diversity where these relate to specific performance standards or matters of care quality.

4.12 To arrange a rolling programme of deep-dive reviews across both the committee and SQG with the aim of understanding in detail key areas of ICB performance and quality and contributing through this process to improvement activities and the promotion of shared learning.

4.13 Ensure that the SQG maintains effective processes for system-wide learning from significant events including themes and trends from incidents and safeguarding reviews. This assurance will be provided via SQG reports and supplementary papers. The committee's role is to ensure that lessons learned are implemented and make a difference.

4.14 Follows National Quality Board guidance to maintain its system level responsibilities for quality on behalf of the ICB. This means the Patient Safety Committee will work in collaboration with other ICB committees (especially Commissioning Committee, the System Quality Group, regional groups for example Regional Quality Group and Joint Strategic Oversight Groups).

4.15 The Committee will receive and review a risk report to agree on the main risks related to quality. Whilst responsibility for detailed review and remedial action on risks rests with the ICB executive leadership, the Committee is expected to maintain an awareness of quality and performance related risks and assure itself that the proposed actions are adequate.

4.16 Work in support of the ICB Strategic Commissioning Committee to provide assurance that the ICBs responsibilities for contractual oversight of quality are being met.

5. Membership and attendance

Membership

5.1 The Patient Safety Committee membership is as follows:

- Non-Executive Member (Chair)
- Non-Executive Member
- ICB Chief Nursing Officer
- ICB Medical Director

5.2 The role of Committee Chair will be undertaken by a Non-Executive Member (who cannot be the Chair of the ICB or the Audit and Risk Committee Chair).

5.3 Members are required to attend a minimum of 75% of meetings, other than absence due to sickness.

5.4 Members may nominate deputies to represent them in their absence and make decisions on their behalf, subject to the approval of the Chair.

5.5 Members will possess between them knowledge, skills and experience in the issues pertinent to the Patient Safety Committee's business. When determining the membership of the Patient Safety Committee, active consideration will be made to diversity and equality.

Chair and vice chair

5.6 The Patient Safety Committee will be chaired by a member appointed on account of their specific knowledge skills and experience making them suitable to chair the Patient Safety Committee.

5.7 Patient Safety Committee members may appoint a Vice Chair who will be nominated by the Chair of the Patient Safety Committee. If the Committee Chair is absent or is disqualified from participating by a conflict of interest, the Vice Chair, if present, shall preside. If the Chair or Vice Chair are absent from any meeting, a Chair shall be nominated by other members attending that meeting.

5.8 The Chair will be responsible for agreeing the agenda and ensuring matters discussed meet the objectives as set out in these ToR.

Attendees

5.9 The Patient Safety Committee shall have the following non-voting attendees (as and when required):

- ICB Director of Quality (SRO for Quality)
- Other Directors and/or managers as appropriate;
- Other partners, as required for specific agenda items;
- Representatives from other organisations, as required.

5.10 Attendees may present at meetings and contribute to the relevant discussions but are not allowed to participate in any formal vote.

5.11 Attendees may nominate deputies to represent them in their absence, with agreement of the Chair.

5.12 The Patient Safety Committee may call additional experts to attend meetings on a case-by-case basis to inform discussion.

5.13 The Patient Safety Committee may invite or allow people to attend meetings as observers. Observers may not present at meetings, contribute to any discussion or participate in any formal vote.

5.14 The Chair of the ICB will be invited to attend one meeting each year in order to gain an understanding of the Committee's operations.

5.15 The Patient Safety Committee Chair may ask any or all of those who normally attend, but who are not members, to withdraw to facilitate open and frank discussion of particular matters.

6. Meeting Frequency, Quoracy and Decisions

6.1 The Patient Safety Committee will usually meet once every two months. Arrangements and notice for calling meetings are set out in the Standing Orders. Additional meetings may take place as required.

6.2 The Board, Chair or Chief Executive may ask the Patient Safety Committee to convene further meetings to discuss particular issues on which they want the Patient Safety Committee's advice.

6.3 Meetings will be held in-person, unless previously agreed by the Chair. The Patient Safety Committee may choose to meet online, via MS Teams or suitable alternative platform.

Quorum

6.4 For a meeting to be quorate the following members will be required:

- Non-Executive Member (Chair)

And two of the following:

- Non-Executive Member
- Chief Nursing Officer
- Chief Medical Officer

6.5 If any member of the Patient Safety Committee has been disqualified from participating in an item on the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

6.6 If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Decision making and voting

6.7 Decisions will be taken in accordance with the Standing Orders. The Patient Safety Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

6.8 Only members of the Patient Safety Committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.9 Where there is a split vote, with no clear majority, the Chair of the Patient Safety Committee will hold the casting vote.

6.10 If a decision is needed which cannot wait for the next scheduled meeting, the Chair may conduct business on a 'virtual' basis through email or other electronic communication.

6.11 Any decisions taken virtually must be recorded at the next scheduled meeting.

7. Accountability and reporting

7.1 The Patient Safety Committee is accountable to the ICB Board and shall provide reports to the Board on how it discharges its responsibilities and shall draw to the attention to any issues that require disclosure or require action.

7.2 The Patient Safety Committee shall make any such recommendations to the ICB Board it deems appropriate on any area within its remit where action or improvement is needed.

7.3 The minutes of the meetings shall be formally recorded by the secretary and approved at the next scheduled meeting.

7.4 The Patient Safety Committee will report to the Board at least annually to describe how it has fulfilled its ToR, give details of any significant issues that it has considered, and how they were addressed.

8. Conflicts of Interest

8.1 Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2 The Patient Safety Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3 All members and attendees of the Patient Safety Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

ICB values

9.1 Members will be expected to conduct business in line with the ICB values and objectives.

9.2 Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3 Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1 The Patient Safety Committee shall be supported with a secretariat function which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the ICB Board]; and
- The Patient Safety Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1 The Patient Safety Committee will review its effectiveness at least annually and recommend any changes it considers necessary to the ICB Board.

11.2 These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to the ICB Board for approval.

Date of approval: TBC [to add]

Date of next review: TBC [to add]

NHS South East London Integrated Care Board

Audit, Risk and Assurance Committee

Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	May 2026	Initial draft developed by ICB governance teams in SWL and SEL
2	13 May 2026	Draft for engagement
3	3 June 2026	Updated to incorporate amendments proposed by SWL Director of Corporate and SEL NEM member for governance
4	10 June 2026	Updated to include amendments suggested by NEMs

Approved by

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1. Constitution

1.1 The Audit, Risk and Assurance Committee is established by the Integrated Care Board (the Board) as a committee of the Board in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board. They will be published as part of the Governance Handbook on the ICB's website.

1.3 The Committee is a non-executive committee of the Board and its members, including those who are not members of the Board, are bound by the Standing Orders and other policies of the
ICB.

2. Authority

2.1 The Audit, Risk and Assurance Committee is authorised by Board to:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Audit, Risk and Assurance Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so, the Committee must follow any procedures put in place by the ICB for obtaining legal or professional advice;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Audit, Risk and Assurance Committee members. The Audit, Risk and Assurance Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Audit, Risk and Assurance Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Audit, Risk and Assurance Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1 To contribute to the overall delivery of the ICB's strategic and statutory objectives by providing oversight and assurance to the Board on the adequacy of governance, risk management and internal control processes within the ICB, and providing independent scrutiny that enhances decision-making, strengthens accountability, and supports continuous improvement.

3.2 The duties of the committee will be driven by the organisation's objectives and the associated risks. An annual programme of business will be agreed before the start of the financial year; however, this will be flexible to new and emerging priorities and risks.

4. Responsibilities of the Audit, Risk and Assurance Committee

4.1. The committee will undertake the following principal duties:

4.2. Governance and internal control

- To review the adequacy and effectiveness of governance, risk management, assurance and internal control systems across the ICB that support the achievement of its objectives, and to highlight any areas of weakness to the Board.
- To ensure that financial systems and governance are established which facilitate compliance with DHSC's Group Accounting Manual.
- To ensure that the ICB acts consistently with the principles and guidance established in HMT's Managing Public Money.
- To seek reports and assurance from directors and managers as appropriate, concentrating on the systems of governance and internal control, together with indicators of their effectiveness.
- To identify opportunities to improve governance and internal control processes across the ICB.
- To have oversight of urgent decisions exercised by the Board.

4.3. Internal audit

- To ensure that there is an effective internal audit function that meets the Public Sector Internal Audit Standards and provides appropriate independent assurance to the Board. This will be achieved by:
  - Considering the provision of the internal audit service and the costs involved
  - Approving the appointment of the ICB's internal auditor service

- Reviewing and approving the annual internal audit plan and more detailed programme of work, ensuring that this is consistent with the audit needs of the organisation as identified in the assurance framework
- Considering the findings of all internal audit reports, including the Head of Internal Audit Opinion (and management's response) and ensure coordination between the internal and external auditors to optimise the use of audit resources
- Ensuring that the internal audit function is adequately resourced and has appropriate standing within the organisation; and
- Monitoring the effectiveness of internal audit and carrying out an annual review.

4.4. External audit

- To review and monitor the external auditor's independence and objectivity and the effectiveness of the audit process. In particular, the committee will review the work and findings of the external auditors and review and assess the implications and management's responses to their work. This will be achieved by:
  - Considering the performance of the external auditors, as far as the rules governing the appointment permit;.
  - Discussing and agreeing with the external auditors, before the audit commences, the nature and scope of the audit as set out in the annual plan.
  - Discussing with the external auditors their evaluation of audit risks and assessment of the organisation and the impact on the audit fees.
  - Reviewing all external audit reports, including to those charged with governance (before its submission to the Board) and any work undertaken outside the annual audit plan, together with the appropriateness of management responses.
- The Audit, Risk & Assurance Committee shall not have responsibility for appointment or selection of the external auditors.

4.5. Risk Management

- Report to the Board with a clear assessment of whether principal risks to the delivery of the organisation's corporate objectives are controlled within appetite (where appropriate) and supported by sufficient, reliable assurance.
- To review the adequacy and effectiveness of the organisations risk management framework and its implementation across the ICB.
- To receive regular updates on the ICB's Board Assurance Framework.
- Undertake periodic structured deep-dives into principal risks to assess control effectiveness, test assurance robustness, and identify required management actions.

4.6. Assurance

- To review and triangulate assurance from internal audit, external audit, regulatory bodies, and other ICB sources of information and business intelligence to form an independent, evidence-based view of the ICB's control environment.

- To maintain and scrutinise the Assurance Map aligned to strategic objectives, assessing the adequacy, balance and interdependency of assurance across the three lines model and identifying gaps, duplication or over-reliance on single sources.

- Monitor and independently test the implementation and effectiveness of agreed actions arising from audit and other reviews, ensuring that issues are genuinely resolved in practice and that underlying risks have been demonstrably reduced.

- Review and assess intelligence on organisational culture including speaking up and escalation mechanisms, to assess whether risks and issues surface within the organization are followed up and resolved.

- To receive regular reports on tender waivers approved within the ICB.

- To review the findings of assurance functions in the ICB, and to consider the implications for the governance of the ICB.

- Working with chairs of ICB committees, to review the work of other committees in the ICB with the aim of providing relevant assurance to the Audit and Risk Committee's own areas of responsibility.

- To review the assurance processes in place in relation to financial performance across the ICB including the completeness and accuracy of information provided.

- To review the findings of external bodies including the ICB's external regulators, and consider the implications for governance of the ICB. These will include, but will not be limited to:

- Reviews and reports issued by arm's length bodies or regulators and inspectors: e.g. National Audit Office, Select Committees, NHS Resolution, CQC; and reviews and reports issued by professional bodies with responsibility for the performance of staff or functions (e.g. Royal Colleges and accreditation bodies).

- Pay due regard to risk and assurance activities of providers with a view to sharing understanding and learning across the health and care system in circumstances where that may offer mutual benefit to all partners.

4.7. Counter fraud

- To assure itself that the ICB has adequate arrangements in place for counter fraud,

bribery and corruption (including cyber security) that meet the NHS Counter Fraud Authority's (NHSCFA) Requirements for the Counter Fraud Functional Standard and review the outcome of work in these areas. This should include assurance that the ICB has the appropriate "reasonable procedures to prevent the occurrence of fraud" to mitigate the exposure of the ICB to the Failure to Prevent Fraud offence governed by the Economic Crime and Corporate Transparency Act 2023.

- To review, approve and monitor counter fraud work plans, receiving regular updates on counter fraud activity, monitor the implementation of action plans, provide direct access and liaison with those responsible for counter fraud, review annual reports on counter fraud, and discuss the outcomes of the annual Counter Fraud functional Standard Return (CFFSR).

- To ensure that the counter fraud service submits an Annual Report and CFFSR, outlining work undertaken during each financial year to meet the Requirements of the NHSCFA Counter Fraud Functional Standard.

- To ensure that the counter fraud service submits an Annual Report and Self-Review Assessment, outlining work undertaken during each financial year to meet the NHS Standards for Commissioners, Fraud, Bribery and Corruption.

4.8. Freedom to Speak Up

- To review the adequacy and security of the ICB's arrangements for its employees, contractors and external parties to raise concerns, in confidence, in relation to financial, clinical management, or other matters. The Committee shall ensure that these arrangements allow proportionate and independent investigation of such matters and appropriate follow up action.

4.9. Information Governance (IG)

- To receive regular updates on IG compliance (including uptake & completion of data security training), data breaches and any related issues and risks.

- To review the annual Senior Information Risk Owner (SIRO) report, the submission for the Data Security & Protection Toolkit and relevant reports and action plans.

- To receive reports on audits to assess information and IT security arrangements, including the annual Data Security & Protection Toolkit audit.

- To provide assurance to the Board that there is an effective framework in place for the management of risks associated with information governance

4.10. Financial Reporting

- To monitor the integrity of the annual financial statements of the ICB and any formal announcements relating to its financial performance.

- To ensure that the systems for financial reporting to the Board, including those of

budgetary control, are subject to review as to the completeness and accuracy of the information provided.

- To review and recommend to the Board the annual report and annual financial statements (including accounting policies) for submission, and reporting to the Board, focusing particularly on:
  - The wording in the Governance Statement and other disclosures relevant to the Terms of Reference of the committee
  - Changes in accounting policies, practices and estimation techniques
  - Unadjusted mis-statements in the Financial Statements
  - Significant judgements and estimates made in the preparation of the Financial Statements
  - Significant adjustments resulting from the audit
  - Letter of representation
  - Qualitative aspects of financial reporting.

4.11. Conflicts of Interest

- The chair of the committee will be the nominated Conflicts of Interest Guardian.
- The committee shall satisfy itself that the ICB's policy, systems and processes for the management of conflicts, (including gifts and hospitality and bribery) are effective, including receiving reports relating to non-compliance with the ICB's policy and procedures relating to conflicts of interest.

5. Membership and attendance

Membership

5.1. The Committee members shall be appointed by the Board in accordance with the ICB Constitution.

5.2. The Board will appoint no fewer than three members of the Committee comprising three Non-Executive Members of the Board. Other members of the Committee need not be members of the Board, but they may be.

5.1 The role of Chair will be undertaken by ICB Non-Executive Member.

5.2 Members are required to attend a minimum of 75% of meetings, other than absence due to sickness.

5.3 Members may nominate deputies to represent them in their absence and make decisions on their behalf, subject to the approval of the Chair.

5.4 Members will possess between them knowledge, skills and experience in the issues pertinent to the Audit, Risk and Assurance Committee's business. When determining the membership of the Audit, Risk and Assurance Committee, active consideration will be made to diversity and equality.

Chair and vice chair

5.5 The Audit, Risk and Assurance Committee will be chaired by a member appointed on account of their specific knowledge skills and experience making them suitable to chair the Audit, Risk and Assurance Committee.

5.6 Audit, Risk and Assurance Committee members may appoint a Vice Chair who will be nominated by the Chair of the Audit, Risk and Assurance Committee. If the Committee Chair is absent or is disqualified from participating by a conflict of interest, the Vice Chair, if present, shall preside. If the Chair or Vice Chair are absent from any meeting, a Chair shall be nominated by other members attending that meeting.

Attendees

5.7 The Audit, Risk and Assurance Committee shall have the following non-voting attendees:

- Chief Finance and Compliance Officer or their nominated deputy
- Representatives of internal and external audit, and the local counter fraud service provider
- ICB Director of Corporate Services
- Other attendees as appropriate, agreed in advance with the committee chair
- Representatives from other organisations, as required.

5.8 Attendees may present at meetings and contribute to the relevant discussions but are not allowed to participate in any formal vote.

5.9 Attendees may nominate deputies to represent them in their absence, with agreement of the Chair.

5.10 The Audit, Risk and Assurance Committee may call additional experts to attend meetings on a case by case basis to inform discussion.

5.11 The Audit, Risk and Assurance Committee may invite or allow people to attend meetings as observers. Observers may not present at meetings, contribute to any discussion or participate in any formal vote.

5.12 The Audit, Risk and Assurance Committee Chair may ask any or all of those who normally attend, but who are not members, to withdraw to facilitate private discussion of particular matters.

6. Meeting Frequency, Quoracy and Decisions

6.1 The Audit, Risk and Assurance Committee will usually meet quarterly. Arrangements and notice for calling meetings are set out in the Standing Orders. Additional meetings may take place as required.

6.2 The Board, Chair or Chief Executive may ask the Audit, Risk and Assurance Committee to convene further meetings to discuss particular issues on which they want the Audit, Risk and Assurance Committee advice.

6.3 Meetings will be held in-person, unless previously agreed by the Chair. The Audit, Risk and Assurance Committee may choose to meet online, via MS Teams or suitable alternative platform.

Quorum

6.4 For a meeting to be quorate the following members will be required:

- 2 x ICB Non-Executive Directors

6.5 If any member of the Audit, Risk and Assurance Committee has been disqualified from participating in an item on the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

6.6 If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Decision making and voting

6.7 Decisions will be taken in accordance with the Standing Orders. The Audit, Risk and Assurance Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

6.8 Only members of the Audit, Risk and Assurance Committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.9 Where there is a split vote, with no clear majority, the Chair of the Audit, Risk and Assurance Committee will hold the casting vote.

6.10 If a decision is needed which cannot wait for the next scheduled meeting, the Chair may conduct business on a 'virtual' basis through email or other electronic communication.

6.11 Any decisions taken virtually must be recorded at the next scheduled meeting.

7. Accountability and reporting

7.1 The Audit, Risk and Assurance Committee is accountable to the ICB Board and shall provide reports to the ICB Board on how it discharges its responsibilities and shall draw to the attention to any issues that require disclosure or require action.

7.2 The Audit, Risk and Assurance Committee shall make any such recommendations to the ICB Board it deems appropriate on any area within its remit where action or improvement is needed.

7.3 The minutes of the meetings shall be formally recorded by the secretary and approved at the next scheduled meeting.

7.4 The Audit, Risk and Assurance Committee will report to ICB Board at least annually to describe how it has fulfilled its ToR, give details of any significant issues that it has considered, and how they were addressed.

8. Conflicts of Interest

8.1 Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2 The Audit, Risk and Assurance Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3 All members and attendees of the Audit, Risk and Assurance Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

Commented [GF1]: Link to ways of working document.

ICB values

9.1 Members will be expected to conduct business in line with the ICB values and objectives.

9.2 Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3 Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1 The Audit, Risk and Assurance Committee shall be supported with a secretariat function which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the ICB Board and
- The Audit, Risk and Assurance Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1 The Audit, Risk and Assurance Committee will review its effectiveness at least annually and recommend any changes it considers necessary to ICB Board.

11.2 These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to ICB Board for approval.

Date of approval: [TBC]

Date of next review: [TBC]

NHS South East London Integrated Care Board

Remuneration Committee

Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	8 May 2026	Core components drafted by SWL and SEL governance teams based on previous RemCo ToRs and aligned with NHS England guidance.
2	1 June 2026	Added initial draft to ToRs template

Approved by

Version	Date	Committee / Group name

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1. Constitution

1.1 The Remuneration Committee (the Committee) is established by the Integrated Care Board (the Board or ICB) as a committee of the Board. in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board.

2. Authority

2.1 The Remuneration Committee is authorised by the ICB Board to:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Remuneration Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Remuneration Committee members. The Remuneration Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Remuneration Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Remuneration Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1. The Committee's main purpose is to exercise the functions of the ICB relating to paragraphs 17 to 19 of Schedule 1B to the NHS Act 2006. In summary: Confirm the ICB Pay Policy including adoption of any pay frameworks for all employees including very senior managers/directors (including board members) and Non-Executive Members excluding the Chair.

3.2. The Board has also delegated the following functions to the Committee:

3.3. Ensuring the ICB follows national pay and terms and condition frameworks to set the pay policy for ICB employees.

3.4. Setting remuneration, allowances, performance related pay incentives and terms and conditions for the Chief Executive and Very Senior Managers (VSMs) in line with national guidance.

3.5. Setting remuneration, allowances and terms and conditions for Integrated Care Board members, where required.

3.6. Agreeing any discretionary payments or terms and conditions for staff employed by the ICB.

3.7. Approving any termination or redundancy payments.

3.8. Approving TUPE or other staff transfers into or out of the ICB.

3.9. Setting the ICB pay policy and standard terms and conditions of employment for all individuals appointed by the ICB as clinical leads, workers, office holders (this may include pensions, remuneration, fees, travelling or other allowances payable), and any pay awards for these individuals.

4. Responsibilities of the Remuneration Committee

Specific responsibilities of the Committee include:

For the Chief Executive, Directors and other Very Senior Managers:

4.1 Through the Chief Executive, Directors and other Very Senior Managers to: determine all aspects of remuneration including but not limited to salary, (including any performance-related elements) bonuses, and other additional benefits.

4.2 Through the Chair: provide feedback on the Chief Executive's performance so as to support the monitoring and evaluation of their performance.

4.3 To consider and approve proposals to establish any new management posts at Very Senior Manager grade.

4.4 To oversee and advise the Board on arrangements for redundancy, termination payments, the use of Pay in Lieu of Notice proposals or any special payments following scrutiny of their proper calculation and taking account of such national guidance as appropriate.

4.5 As permitted, to approve the use of the model voluntary redundancy scheme (VRS) and seek NHS England regional approval.

For ICB Board Members:

4.6 Determine remuneration, allowances and terms and conditions for Integrated Care Board members[1].

For all staff:

4.7 Determine the ICB pay policy (including the adoption of pay frameworks such as Agenda for Change).

4.8 Determine the arrangements for redundancy, termination payments, the use of Pay in Lieu of Notice proposals or any special payments following scrutiny of their proper calculation and taking account of such national guidance as appropriate.

[1] When determining Non-Executive Board member remuneration the ICB Chair, Chief Executive and either a NHSE or partner representative will meet. Non-Executive Members will not be involved in discussion about their own pay.

4.9 Agree any discretionary payments or terms and conditions for staff employed by the ICB.

Additional functions included in the scope of the committee include:

4.10 Setting the ICB pay policy and standard terms and conditions of employment for all individuals appointed by the ICB as clinical leads, workers, office holders (this will include pensions, remuneration, fees, travelling or other allowances payable), and any pay awards for these individuals.

4.11 Assurance in relation to ICB statutory duties relating to people such as compliance with employment legislation including such as Fit and Proper Person Test (FPPT).

4.12 Approving TUPE or other staff transfers into or out of the ICB.

5. Membership and attendance

Membership

5.1 The Committee members shall be appointed by the Board in accordance with the ICB Constitution.

5.2 The Remuneration Committee membership is as follows:

- Non-Executive Member (Chair)
- Non-Executive Member
- Non-Executive Member

5.3 The role of Chair will be undertaken by Non-Executive Member

5.4 Members are required to attend a minimum of 75% of meetings, other than absence due to sickness.

5.5 Members may nominate deputies to represent them in their absence and make decisions on their behalf, subject to the approval of the Chair.

5.6 Members will possess between them knowledge, skills and experience in the issues pertinent to the Remuneration Committee's business. When determining the membership of the Remuneration Committee, active consideration will be made to diversity and equality.

Chair and vice chair

5.7 The Committee will be chaired by an independent member of the Board which could either be the Chair or a Non-Executive Member, appointed on account of their specific knowledge skills and experience making them suitable to chair the Remuneration Committee.

5.8 Remuneration Committee members may appoint a Vice Chair who will be nominated by the Chair of the Remuneration Committee. If the Committee Chair is absent or is disqualified from participating by a conflict of interest, the Vice Chair, if present, shall preside. If the Chair or Vice Chair are absent from any meeting, a Chair shall be nominated by other members attending that meeting.

5.9 The Chair will be responsible for agreeing the agenda and ensuring matters discussed meet the objectives as set out in these ToR.

Attendees

5.10 Only members of the Committee have the right to attend Committee meetings, but the Chair may invite relevant staff to the meeting as necessary in accordance with the business of the Committee.

5.11 Meetings of the Committee may also be attended by the following individuals who are not members of the Committee for all or part of a meeting as and when appropriate. Such attendees will not be eligible to vote:

- The ICB's most senior HR Advisor or their nominated deputy;
- Chief Finance Officer or their nominated deputy;
- Chief Executive or their nominated deputy;
- The Director of Corporate Services.

5.12 The Board may appoint independent members or advisers to the Remuneration and Nominations Committee who are not members of the Board.

5.13 The Committee Chair may ask any or all of those who normally attend, but who are not members, to withdraw to facilitate open and frank discussion of particular matters.

5.14 No individual should be present during any discussion relating to:

- Any aspect of their own pay;
- Any aspect of the pay of others when it has an impact on them.

5.15 Attendees may present at meetings and contribute to the relevant discussions but are not allowed to participate in any formal vote.

5.16 Attendees may nominate deputies to represent them in their absence, with agreement of the Chair.

5.17 The Remuneration Committee may call additional experts to attend meetings on a case-by-case basis to inform discussion.

6. Meeting Frequency, Quoracy and Decisions

6.1 The Remuneration Committee will usually meet a minimum of twice annually. Arrangements and notice for calling meetings are set out in the Standing Orders. Additional meetings may take place as required.

6.2 The Board, Chair or Chief Executive may ask the Remuneration Committee to convene further meetings to discuss particular issues on which they want the advice.

6.3 Meetings will be held in-person, unless previously agreed by the Chair. The Remuneration Committee may choose to meet online, via MS Teams or suitable alternative platform.

Quorum

6.4 For a meeting to be quorate the following members will be required:

- Non-Executive Member
- Non-Executive Member

6.5 If any member of the Remuneration Committee has been disqualified from participating in an item on the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

6.6 If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Decision making and voting

6.7 Decisions will be taken in accordance with the Standing Orders. The Remuneration Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

6.8 Only members of the Remuneration Committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.9 Where there is a split vote, with no clear majority, the Chair of the Remuneration Committee will hold the casting vote.

6.10 If a decision is needed which cannot wait for the next scheduled meeting, the Chair may conduct business on a 'virtual' basis through email or other electronic communication.

6.11 Any decisions taken virtually must be recorded at the next scheduled meeting.

7. Accountability and reporting

7.1 The Remuneration Committee is accountable to the ICB Board and shall provide reports to the Board on how it discharges its responsibilities and shall draw to the attention to any issues that require disclosure or require action.

7.2 The Remuneration Committee shall make any such recommendations to the ICB Board it deems appropriate on any area within its remit where action or improvement is needed.

7.3 The minutes of the meetings shall be formally recorded by the secretary and approved at the next scheduled meeting.

7.4 The Remuneration Committee will report to the ICB Board at least annually to describe how it has fulfilled its ToR, give details of any significant issues that it has considered, and how they were addressed.

8. Conflicts of Interest

8.1 Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2 The Remuneration Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3 All members and attendees of the Remuneration Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

ICB values

9.1 Members will be expected to conduct business in line with the ICB values and objectives.

9.2 Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3 Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1 The Remuneration Committee shall be supported with a secretariat function which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the ICB Board and
- The Remuneration Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1 The Remuneration Committee will review its effectiveness at least annually and recommend any changes it considers necessary to the ICB Board.

11.2 These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to the ICB Board for approval.

Date of approval: TBC [to add]

Date of next review: TBC [to add]

NHS South East London Integrated Care Board
Charitable Funds Committee
Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	10 June 2026	Updated to reflect future governance arrangements and new SoRD by Programme Director, System Sustainability, and approved by Charitable Funds Committee on 10 June 2026

Approved by

Version	Date	Committee / Group name

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1. Constitution

1.1 The Charitable Funds Committee is established by the Integrated Care Board (the Board or ICB) as a committee of the Board. in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board. They will be published as part of the Governance Handbook on the ICB's website.

2. Authority

2.1 The Charitable Funds Committee is authorised by the Board to:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Charitable Funds Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so, the Committee must follow any procedures put in place by the ICB for obtaining legal or professional advice;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by the Charitable Funds Committee members. The Charitable Funds Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Charitable Funds Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Charitable Funds Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1 NHS South East London Integrated Care Board (ICB) is the Corporate Trustee of NHS Greenwich Charitable Funds, Charity number: 1097722. The ICB Board serves as its agent in the administration of the charitable funds. The funds are known in Greenwich as 'Greenwich Healthier Communities Fund'

3.2 Declaration of Trust, dated 25 March 2003, as affected by Statutory Instrument 2004 No. 1643 as amended on 17 Apr 2020.

3.3 The charitable objects are 'For any charitable purpose or purposes relating to the National Health Service, for the Health & Wellbeing of the residents and staff in the population served by the Royal Borough of Greenwich.'

3.4 The Charitable Funds Committee has been established by the Board to provide oversight of the annual plan, and delivery or the grant making strategy.

3.5 The Scheme of Reservation and Delegation sets out that role of the Corporate Trustee enacted through the Chief Finance & Compliance Officer, with oversight of the day to day control and management of the charitable funds enacted through the Place Executive Lead (Greenwich).

3.6 The key purpose of the Committee is to support these two key roles in helping to govern, manage, regulate and plan the finances, accounts, investments, assets, business and all affairs of the charity, including the authorisation of expenditure.

3.7 The scope of the Committee will be the responsibility for charitable funds, where the NHS South East London Integrated Care Board (ICB) is the Corporate Trustee.

4. Responsibilities of the Charitable Funds Committee

4.1. Within the budget, priorities and spending criteria determined by the ICB as Trustee and consistent with the requirements of the Charities Act 2016 (or any modification of that Act) to apply the charitable funds in accordance with their respective governing documents.

4.2. To ensure that the ICB policies and procedures for charitable funds investments are followed. To make decisions involving the sound investment of charitable funds in a way which both preserves their capital value and produces proper return consistent with prudent investment and ensuring compliance with:

- Trustee Act 2000
- The Charities Act 2016
- Terms of the Funds' Governing documents

4.3. To ensure the approval and submission of the annual accounts and Trustees' report in accordance with the Charities Act.

4.4. To monitor the ICB's scheme of delegation for expenditure for the levels in accordance with policy and delegated limits.

4.5. To monitor income, expenditure and investments in relation to charitable funds.

4.6. To receive proposals for major expenditure and to approve charitable fund bids in accordance with the schedule of matters delegated to officers

4.7. To consider strategy in relation to the charity and charitable funds, with a view to making recommendations to the ICB Board as Corporate Trustee, ensuring that the outcomes are delivered against the charitable objects.

4.8. To ensure appropriate advice is sought in relation to the health & wellbeing needs of the population of Greenwich, so that this informs the strategy and evaluation of impact, being mindful of the inequalities within the borough.

4.9. To ensure appropriate evaluation of the Greenwich Healthier Communities Fund grant giving, through independent evaluation partners, and that learning is built into future grant giving approach, as well as shared across SEL ICB to inform the partnership working with Voluntary Sector and Faith Communities across the ICS.

4.10. The Committee will determine the strategy and policies for fundraising, including whether the ICB should undertake major fundraising appeals, establishing the appropriate framework to ensure that any appeal is properly managed. To monitor fundraising performance and compliance with fundraising regulations. This may include fundraising partnership opportunities with Royal Borough of Greenwich, and opportunity to formally collaborate and pool funding using established Section 75 agreement, and develop sustainable funding flows.

4.11. To identify and monitor risks in relation to the charity and charitable funds, including investments and grants made.

4.12. To ensure that training needs of Committee members are identified and met.

5. Membership and attendance

Membership

5.1. The Charitable Funds Committee membership is as follows:

- A Non-Executive Member, ICB
- Chief Financial & Compliance Officer, ICB
- Greenwich Place Executive Director, ICB
- Director of Public Health, Royal Borough of Greenwich

5.2. Members are required to attend a minimum of 75% of meetings, other than absence due to sickness.

5.3. Members may nominate deputies to represent them in their absence and make decisions on their behalf, subject to the approval of the Chair.

5.4. Members will possess between them knowledge, skills and experience in the issues pertinent to the Charitable Funds Committee ’s business. When determining the

membership of the Charitable Funds Committee, active consideration will be made to diversity and equality.

Chair and vice chair

5.5. The Committee will be chaired by the Non-Executive Member. In the event that the Non- Executive Member is unable to attend, the Committee will be chaired by one of the members.

5.6. The Charitable Funds Committee will be chaired by a member appointed on account of their specific knowledge skills and experience making them suitable to chair the Charitable Funds Committee.

5.7. Charitable Funds Committee members may appoint a Vice Chair who will be nominated by the Chair of the Charitable Funds Committee. If the Committee Chair is absent or is disqualified from participating by a conflict of interest, the Vice Chair, if present, shall preside. If the Chair or Vice Chair are absent from any meeting, a Chair shall be nominated by other members attending that meeting.

5.8. The Chair will be responsible for agreeing the agenda and ensuring matters discussed meet the objectives as set out in these ToR.

Attendees

5.9. The Charitable Funds Committee shall have the following non-voting attendees (as and when required):

- Charity Manager (nominated Greenwich representative to work with Charity Advisor on regular coordination)
- Charity Advisor
- Associate Director of Finance, Greenwich & Lewisham
- Director of Neighbourhood Development, Greenwich, ICB
- Director of Voluntary Sector Collaboration & Partnerships (SE London ICS)
- Subject matter leads will be invited to the Committee as required

5.10. Attendees may present at meetings and contribute to the relevant discussions but are not allowed to participate in any formal vote.

5.11. Attendees may nominate deputies to represent them in their absence, with agreement of the Chair.

5.12. The Charitable Funds Committee may call additional experts to attend meetings on a case-by-case basis to inform discussion.

5.13. The Charitable Funds Committee may invite or allow people to attend meetings as observers. Observers may not present at meetings, contribute to any discussion or participate in any formal vote.

5.14. The Charitable Funds Committee Chair may ask any or all of those who normally attend, but who are not members, to withdraw to facilitate discussion of particular matters.

6. Meeting Frequency, Quoracy and Decisions

6.1. The Charitable Funds Committee will usually meet two times each year. Arrangements and notice for calling meetings are set out in the Standing Orders. Additional meetings may take place as required.

6.2. The Board, Chair or Chief Executive may ask the Charitable Funds Committee to convene further meetings to discuss particular issues on which they want the committee's advice.

6.3. Meetings will be held in-person, unless previously agreed by the Chair. The Charitable Funds Committee may choose to meet online, via MS Teams or suitable alternative platform.

Quorum

6.4. For a meeting to be quorate the committee will require a minimum of 50% of its members.

6.5. If any member of the Charitable Funds Committee has been disqualified from participating in an item on the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

6.6. If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Decision making and voting

6.7. Decisions will be taken in accordance with the Standing Orders. The Charitable Funds Committee will ordinarily reach conclusions by consensus. When this is not possible the Chair may call a vote.

6.8. Only members of the Charitable Funds Committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.9. Where there is a split vote, with no clear majority, the Chair of the Charitable Funds Committee will hold the casting vote.

6.10. If a decision is needed which cannot wait for the next scheduled meeting, the Chair may conduct business on a 'virtual' basis through email or other electronic communication.

6.11. Any decisions taken virtually must be recorded at the next scheduled meeting.

7. Accountability and reporting

7.1. The Committee is a committee of the ICB Board. The Committee will provide a report to the ICB Board a minimum of once each year.

7.2. Updates (at least annually) from the Committee will be provided to the Healthier Greenwich Partnership, so wider system partners are able to contribute to and be informed on the charity's activity and plans.

7.3. The Committee shall have the authority to appoint an investment manager to advise it on investment matters and may delegate day-to-day management of some of all of the investments to that investment manager. In exercising this power the Committee must ensure that:

- The scope of the power delegated is clearly set out in writing and communicated with the person or persons who will exercise it.
- There are in place adequate internal controls and procedures which ensure that the power is being exercised properly and prudently.
- It regularly reviews the performance of the person or person's exercising the delegated power.
- Where an investment manager is appointed, that the person is regulated under the Financial Services and Markets Act 2000.
- It establishes and maintains an approved list of counter parties for investment activities.
- It will obtain appropriate professional advice to support its investment activities.
- It will regularly review investments to see if other opportunities or investment managers offer a better return.

7.4. The Committee shall also have the authority to appoint a partner organisation that can support the Charity in the formation and implementation of a strategy to distribute funds held. In exercising this power the Committee must ensure that:

- The scope of the power delegated is clearly set out in writing and communicated with the person or persons who will exercise it.
- There are in place adequate internal controls and procedures which ensure that the power is being exercised properly and prudently.
- It regularly reviews the performance of the person or person's exercising the delegated power.
- It will obtain appropriate professional advice to support.

8. Conflicts of Interest

8.1. Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2. The Charitable Funds Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3. All members and attendees of the Charitable Funds Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

ICB values

9.1. Members will be expected to conduct business in line with the ICB values and objectives.

9.2. Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3. Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1. The Charitable Funds Committee shall be supported by the Greenwich ICB place team, which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the Board; and

- The Charitable Funds Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1. The Charitable Funds Committee will review its effectiveness at least annually and recommend any changes it considers necessary to the Board.

11.2. These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to the Board for approval.

Date of approval: TBC [to be added]

Date of next review: TBC [to be added]

NHS South East London & NHS South West London Integrated Care Boards

Joint Transition Committee

Terms of Reference

Document Management

Revision history

Version	Date	Summary of changes
1	9 July 2026	Update of existing ToRs and integration of SWL and SEL ToRs into single ToRs for joint committee.

Approved by

Version	Date	Committee / Group name

1. Constitution

1.1 The Transition Committee (the Committee) is established by the Integrated Care Board (the Board or ICB) as a committee of the Board. in accordance with its Constitution, Standing Financial Instructions, Standing Orders and Scheme of Reservation and Delegation (SoRD).

The Transition Committee is established as a joint Committee of both NHS South East London Integrated Care Board ICB and South West London ICB as of 1st August 2026.

1.2 These Terms of Reference (ToR) set out the membership, remit, responsibilities, and reporting arrangements of the Committee and may only be changed with the approval of the Board.

2. Authority

2.1 The Transition Committee is authorised by the ICB Board:

- Undertake any activity within its ToR;
- Seek any information it requires within its remit, from any employee or member of the ICB (who are directed to co-operate with any request made by the Transition Committee) within its remit as outlined in these ToR;
- Commission any reports it deems necessary to help fulfil its obligations;
- Obtain legal or other independent professional advice and secure the attendance of advisors with relevant expertise if it considers this is necessary to fulfil its functions. In doing so, the Committee must follow any procedures put in place by the ICB for obtaining legal or professional advice;
- Create task and finish sub-groups in order to take forward specific programmes of work as considered necessary by Committee members. The Transition Committee shall determine the membership and ToR of any such task and finish sub-groups in accordance with the ICB's Constitution, Standing Orders and SoRD but may not delegate any decisions to such groups.

2.2 For the avoidance of doubt, the Transition Committee will comply with, the ICB Standing Orders, Standing Financial Instructions and the SoRD, except as outlined in these ToR.

2.3 The Transition Committee has no executive powers, other than those delegated in the SoRD and specified in these ToR.

3. Purpose

3.1 The purpose of the Committee is to provide assurance and oversight of the SEL and SWL ICB Change Programmes for the ICB Boards. As set out in the Model ICB

Blueprint v.1.0, the Committee will ensure a safe and coherent transition, managing local risks, tracking progress and overseeing the development of organisational design and implementation of the change process, including the transfer of functions to providers over time.

3.2 The Committee is time-limited until the end of the 2026/2027 financial year, unless otherwise decided. A review will take place in January 2027 to take stock and determine whether it has completed its duties.

4. Responsibilities of the Joint Transition Committee

4.1 The Committee will undertake the following responsibilities:

4.2 Provide assurance to the Board that the SEL and SWL Change Programmes are being delivered according to the structure set out in the consultation and to provide assurance that the structures, process and ways of working being developed by both organisations meet the ambitions and requirements of ICBs as set out in the ICB model Blueprint and 10-year plan.

4.3 Provide oversight of the ICB's implementation of the Change Programmes to ensure that the executive leadership of the ICB acts to:

a. meet financial requirements of the change (costs savings of £19/head; redundancy budgets, in-year financial requirements and future recurrent requirements).

b. adhere to local policies (HR and other related policies), regional and national requirements.

c. Implement arrangements as fairly, transparently and effectively as possible. Equality Impact Assessments (EIA) will be carried out before and after the Change Programme, and following Redundancy rounds.

d. identify, assess and mitigate risks. Risks will be categorised as shared risks or risks pertaining to either SEL or SWL, with clear ownership and oversight.

e. implement arrangements according to the timetable set out in the consultation and management response documents.

4.4. Consider, assess and recommend the preferred option regarding the transfer of functions to other NHS providers to the Board on the safe transfer of functions to providers in a way that meets national rules and guidelines for transferring services or functions, including 'sender and receiver' arrangements.

4.5. Provide assurance that the transition plan for the organisations is being delivered in line with agreed implementation timetables. The responsibility for the future operating model of the organisation / organisation development remains the responsibility of both ICB Boards.

4.6. Provide input to the learning that is captured throughout the Change Programmes.

4.7. The Committee will follow the following ways of working:

- Offer constructive challenge and support to the Executive Team regarding implementation/transition plans and longer-term plans to embed the restructure

- Operate as an oversight body and delivery will be undertaken by the Executive Teams.
- Ensure decisions are accountable and aligned with the public interest.
- Report regularly to the Board with insights, concerns, and recommendations.
- Operate in a collegial and transparent manner.

5. Membership and attendance

5.1. The Transition Committee membership is as follows:

- Non-Executive Member 1 SEL ICB Board (Chair)
- Non-Executive Member 2 SEL ICB Board
- Non-Executive Member 3 SWL ICB Board
- ICB Chief People Officer
- Chief Finance and Compliance Officer for SEL and SWL ICBs
- The Joint ICB Change Programme SROs ((SEL and SWL) in attendance)).

5.2. Additional individuals will attend as required to support the agenda.

5.3. The committee will be chaired by the Non Executive Member of the Board.

5.4. The vice chair will be one of the other Non Executive Members.

6. Meeting Frequency, Quoracy and Decisions

6.1. A quorum is formed by 50% of the membership including two of the following four members:

- 2 x Non-Executive Member ICB Board
- Chief People Officer or Chief Finance and Compliance Officer
- 1 x ICB SROs

6.2. If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

6.3. Decisions will be taken in accordance with the Standing Orders. The committee will ordinarily reach conclusions by consensus. When this is not possible the chair may call a vote.

6.4. Only members of the committee may vote. Each member is allowed one vote and a majority will be conclusive on any matter.

6.5. Where there is a split vote, with no clear majority, the chair of the committee will hold the casting vote.

6.6. If a decision is needed which cannot wait for the next scheduled meeting, the chair may conduct business on a 'virtual' basis through the use of telephone, email or other electronic communication.

6.7. The Committee will meet once per month unless there is a need for ad-hoc additional meetings driven by the agenda of the Change Programmes.

6.8. In accordance with the Standing Orders, the committee may meet virtually when necessary and members attending using electronic means will be counted towards the quorum.

7. Accountability and Reporting

7.1. The Transition Committee is accountable to SEL and SWL ICB Boards and shall provide reports to the ICB Boards on how it discharges its responsibilities and shall draw the Boards' attention to any issues that require disclosure or require action.

7.2. The minutes of the meetings shall be formally recorded by the secretary and approved at the next scheduled meeting.

8. Conflicts of Interest

8.1 Conflicts of Interest shall be dealt with in accordance with the ICB Conflicts of Interest Policy.

8.2 The Transition Committee will have a Conflicts of Interest Register that will be presented as a standing item on the agenda.

8.3 All members and attendees of the Transition Committee must declare any relevant personal, non-personal, pecuniary or potential interests at the commencement of any meeting. The Chair will determine if there is a conflict of interest such that the member and/or attendee will be required not to participate in a discussion.

9. Behaviours and Conduct

ICB values

9.1 Members will be expected to conduct business in line with the ICB values and objectives.

9.2 Members of, and those attending, the Committee shall behave in accordance with the ICB's Constitution, Standing Orders, and Standards of Business Conduct Policy.

Equality and diversity

9.3 Members must demonstrably consider the equality and diversity implications of decisions they make.

10. Secretariat and Administration

10.1 The Transition Committee shall be supported with a secretariat function which will include ensuring that:

- The agenda and papers are prepared and distributed in accordance with the Standing Orders having been agreed by the Chair with the support of the relevant executive lead;
- Attendance of those invited to each meeting is monitored and highlighting to the Chair those that do not meet the minimum requirements;
- Records of members' appointments and renewal dates and the Board is prompted to renew membership and identify new members where necessary;
- Good quality minutes are taken, agreed with the Chair, and a record of matters arising. Action points and issues, with progress updates, will be carried forward between meetings;
- The Chair is supported to prepare and deliver reports to the ICB Board and;
- The Transition Committee is updated on pertinent issues / areas of interest / policy developments.

11. Review

11.1 The Transition Committee will review its effectiveness at least annually and recommend any changes it considers necessary to the ICB Board.

11.2 These ToR will be reviewed at least annually and more frequently if required. Any proposed amendments to the ToR will be submitted to the ICB Board for approval.

Date of approval: TBC [to be updated]

Date of next review: TBC [to be updated]

NHS
South East London

SEL ICB Board ways of working: position statement

July 2026

1. Our role and purpose as a Board

1.1. The Board of the Integrated Care Board (ICB) for South-East London (SEL) is a unitary board (of Executives and Non-Executives) that sets the overall strategic direction and provides oversight of the ICB as a strategic commissioning organisation.

1.2. At present in South East London, the ICB's Strategic Commissioning Plan has four broad aims: i) to improve population health outcomes; ii) to ensure the system is sustainable; iii) to improve patients' experience of healthcare services and iv) secure access to high quality and safe services.

1.3. The Board is collectively accountable for assuring that the organisation delivers on its full range of statutory duties.

1.4. Executive members are responsible for operational delivery, within the risk appetite and strategic parameters set by the Board, as well as holding specified statutory functions on behalf of the Board. Their role is to provide operational insight and delivery accountability,

1.5. Non-executives (and other partner members) provide independent oversight and external perspective.

2. How we make decisions

2.1. As a unitary board, we take decisions, based on 4 principles:

2.1.1. **Accountability**: We are accountable for understanding and taking action that secure the best possible health outcomes equitably for south east Londoners now and in the medium and long term.

2.1.2. **Inclusivity**: We value, hear and respond to the voices of our residents/ patients our staff and our wider partners and actively seek out diverse perspectives

2.1.3. **Collaborative**: We identify, convene and cooperate with those partners and communities that contribute, through their services, resources and authority to the improved health and wellbeing of our population, building on a strength-based approach. We hold both ourselves to account for the agreements we make and expect the same from those we collaborate with. This includes working in close collaboration with partner providers, VCSE and local authorities at the ICB and place/ neighbourhood levels.

2.1.4. **Subsidiarity**: This means issues and decisions should be dealt with at the most local level consistent with their effective resolution. In this, the Board recognises that delivery occurs through executive leadership, system partners and place-based partnerships.

2.2. We take timely decisions, at the right level, and hold ourselves accountable for performance, based on the lifecycle of strategic commissioning (understanding need, defining long-term strategy, delivering through resource allocation and evaluating impact).

2.3. We consider options and trade-offs, underpinned by evidence (including clinical and cost-effectiveness), rather than narrative updates.

2.4. The Board is responsible for setting the risk appetite and tolerance for the organisation and, through this, we balance risk-taking with governance, and aim to innovate how healthcare is delivered, fostering a culture of experimentation and learning.

2.5. We constructively challenge issues (not people), to test assumptions, avoid groupthink and ensure alternative perspectives have been considered.

2.6. We are an outward-facing Board, owning and explaining major system decisions publicly, adhering to our statutory obligations regarding transparency and decision making.

2.7. Committee chairs are responsible for approving and presenting reports to the Board (in conjunction with the lead Executive member for the Committee) and will consider matters of business to 'alert, 'advise' or 'assure' the Board.

3. How we behave with each other

3.1. We treat each other with respect and listen to views and perspectives different to our own; we encourage open and honest dialogue.

3.2. We come prepared to take decisions and once a decision is made, we speak with one voice.

3.3. We hold ourselves accountable, taking feedback from each other and striving to improve ourselves. We reflect informally on our effectiveness at the end of each meeting and formally on an annual basis.

3.4. In Board meetings we give the papers and each other full attention.

4. Information, agendas and documents

4.1 There should be no surprises. The Board expects documents that are concise and transparent, highlighting the purpose of the paper, the recommendation, main risks and issues. The Executive poses specific questions for the Board to consider.

4.2 Information presented to the Board should support effective assurance: integrating quality, finance, performance and risk to provide a clear view of delivery. Reporting should be insight-led, highlighting key risks, trade-offs and areas requiring Board attention, rather than detailed descriptive updates.

4.3 The 'pipeline' of future work including prospective items for decision are planned in advance and clearly understood, leaving room for flexibility to allow for changes in context or urgent decisions that are required.

4.4 Board agendas should be explicitly designed to reflect the Board's strategic role, with a clear distinction between items for decision, strategic discussion and assurance.

4.5 Board agendas will contain standing items (e.g. regular reports), and other items on the agenda will be items requiring decision, assurance or issues posing a major risk to the organisation. Sufficient time should be allocated for in-depth conversations.

4.6 Follow-up to Board decisions and actions is systematically tracked, with clear ownership (to either executive members or Committees depending on the item) and reporting back to the Board to confirm delivery and impact.

5. Dependencies

5.1. Achievement of the ICB's objectives, delivery of its duties and adherence to its agreed ways of working only work if the broader partnerships within which the ICB plays a core role also function effectively.

5.2. There are therefore critical dependencies in the ICB undertaking the above ways of working which rely on a successful partnerships which are not directly governed by the ICB governance structure. These include, but are not limited to the SEL System Sustainability Group, People Board, Estates Board, and Digital Board, and at place level, Health and Wellbeing Boards and neighbourhood place partnerships in each borough.

South East London Integrated Care System

NHS South East London

Board Meeting in Public

Title	**Bromley Community Services Contract Award (Commencement December 2026)**						
Meeting date	22 July 2026		Agenda item Number	4	Paper Enclosure Ref	C	
Author	Sean Rafferty – Director of Integrated Commissioning, SEL ICB, Bromley / LBB						
Executive lead	Dr Angela Bhan – Place Executive Lead – Bromley, SEL ICB						
Paper is for:	Update	x	Discussion		Decision		
Purpose of paper	To advise the board on mobilisation progress for the Bromley Community Services contract commencing 1st December 2026 (following completion of Provider Selection Regime (PSR) Direct Award C regulations process).						
Summary of main points	This paper confirms that the Bromley Community Services Contract mobilisation plan is meeting milestones for the period up to June 2026. This is the one of conditions precedent for the PSR Direct Award C to the incumbent Provider (Bromley Healthcare CIC (BHC)) with service to commence of 1st December 2026, for a period of 5 years and with the option for a 2-year contract extension. The paper confirms that progress is being made for service readiness, and that processes are in place to reach the subsequent milestones as set out in the Conditions Precedent.						
Potential conflicts of Interest	Conflicts of interest will be managed in line with SEL ICB policy.						
Relevant to these boroughs	Bexley		Bromley	x	Lewisham		
	Greenwich		Lambeth		Southwark		
Equalities Impact	EOI has been undertaken as part of the process and is being updated as part of the contract award.						
Financial Impact	Included in recurrent Community Budget – delegate to Bromley Borough.						
Public Patient Engagement	No specific engagement has taken place regarding Direct Award; however, engagement has been sought in relation to Community Services as part of 5-year plan in Bromley.						
Committee engagement	Bromley Procurement & Contracts Committee						
Recommendation	The Board is asked to: - • **Note that the June 2026 milestones for the Mobilisation Plan have been met and there is good confidence that subsequent milestones will be met.**						

Board Report for July 2026
SEL ICB Board & Bromley LCP

1. Summary of Progress

The SEL ICB Board on 21st May 2025 agreed to proceed with undertaking an evaluation against Provider Selection Regime (PSR) Direct Award C. This decision followed a review of the PSR options with advice from NEL Commercial Services (Procurement Hub) and legal advice from Capsticks. The evaluation process concluded that there is more than sufficient assurance to recommend the award of the contract to the Provider under Direct Award C. An Intention to Award Notice was published on 17th October 2025. Within the standstill period no representations were received.

The contract was signed by all parties and Conditions Precedent (appendix) and detailed mobilisation plan were included to provide assurance of readiness for contract commencement date.

As of 3rd June, 51% of mobilisation actions have been completed. There have been delays to some mobilisation actions – these have been discussed with commissioners in contract sub-groups and are escalated to Contract Management Board as appropriate. Causes for delays are for various reasons including agreeing to take more time on joint development projects or waiting for national guidance. These delays are deemed to be low risk.

2. Next Steps

The next milestone is 30th September 2026. A further review of the progress will be made at this time.

The contract commencement date is 1st December 2026. It is proposed a progress report will be shared with Board in advance of this date (October/ November 2026).

Appendix 1 – Conditions Precedent

The following schedule is included in the BHC contract.

1. Evidence of appropriate Indemnity Arrangements

2. Evidence of CQC registration in respect of Provider and Material Sub-Contractors

3. Evidence of the Provider Licence in respect of Provider and Material Sub-Contractors

4. Evidence of local plans for integration and joint working with Kings College Hospital NHS Foundation Trust and relevant partners. These will demonstrate active involvement with the 'Bromley Integrator' programme. To include (but not limited to):

   a. Admissions avoidance
   b. Frailty
   c. Hospital at Home/ Virtual Wards
   d. Supported discharge

5. Evidence of progress against the Provider Quality and Improvement Plan which evidence improvements during the mobilisation period for the ambitions as set out in the Commissioning Community Services Strategy including:
   - Digital approaches and use of technology
   - Prevention and self-management support
   - Upskilling generalist staff with the knowledge and confidence to avoid unnecessary referrals to specialist services
   - Implementing personalised care approaches
   - Identifying and addressing avoidable health inequalities
   - Involving people with lived experience in the design and evaluation of services.

   Evidence will demonstrate implemented operational change for each of the above.

1. The Provider will have completed:
    a. By **30th June 2026**, a minimum of 50% of the Mobilisation Plan (~53 of the 106 mobilisation actions) and evidence of achievement will have been accepted at Contract Management Board.
    b. By **30th September 2026**, a minimum of 75% of the Mobilisation Plan (~79 of the 106 mobilisation actions) and evidence of achievement will have been accepted at Contract Management Board.
    c. By **1st December 2026**, a minimum 90% of the Mobilisation Plan (~95 of the 106 mobilisation actions) and evidence of achievement will have been accepted at Contract Management Board (those not achieved will be included in the Service Development and Improvement Plan).

2. The Provider will be fully **reporting** against 95% of the contract KPIs (shadow monitoring will have taken place during the mobilisation period).

A Longstop Date of 31st January 2027 will be included in the contract which is only applicable where external unforeseen events may cause slippage against the timetable.

Appendix 1 – Mobilisation Plan

Activity Type	Overall Objective	No.	Key Activities	Expected Completion Date	
Current Contract SDIP	1.CYP asthma pathway	1.1	Design service outline (in line with partnership approach and national guidance)	Feb-26	**COMPLETE**
		1.2	Business case prepared and shared with ICB	Feb-26	**COMPLETE**
		1.3	ICB to review Business Case and respond	Apr-26	**COMPLETE**
		1.4	Finalise service specification and assoc. KPIs	May-26	In Progress
		1.5	Mobilisation plan for implementation (subject to business case)	May-26	In Progress
		1.6	Service ready to launch for December	Dec-26	Not started
	2.Implementing 'Universal, Targeted and Specialist' model for CYP SaLT	2.1	Roll-out to all mainstream schools	2025	**COMPLETE**
		2.2	Review service specification and assoc. KPIs	Mar-26	**COMPLETE**
		2.3	End of year review	Sep-26	Not started
	3. ASD assessment service	3.1	Develop service outline in line with service specification	Mar-26	SUPERCEDED
		3.2	Update service specification and assoc. KPIs	Mar-26	SUPERCEDED
		3.3	Mobilisation plan for implementation	Mar-26	SUPERCEDED
		3.4	Recruitment of key staff	Apr-26	SUPERCEDED
		3.5	Service launched	Jun-26	SUPERCEDED
	4.Care home support	4.1	Confirm current service provision for care home residents	Jan-26	**COMPLETE**
		4.2	Develop options paper	Feb-26	**COMPLETE**
		4.3	Share proposal with ICB	Feb-26	**COMPLETE**
		4.3	Document for Contract (brief specification) and confirm reporting requirements	Mar-26	In Progress
		4.4	Service ready to launch	Jun-26	Not started
	5.Digitalisation	5.1	Development of 25_26 Wound App initiative	Apr-26	Minor delays
		5.2	Development of 25_26 Windows11 (IT Infrastructure) initiative	Apr-26	**COMPLETE**

		5.3	Development of 25_26 Cybersecurity improvement initiative	Dec-25	**COMPLETE**
		5.4	Development of 25_26 DN Tech Improvement (Ambient Voice Technology) initiative	Apr-26	Minor delays
		5.5	Development of 25_26 RPA/Robotics for referrals initiative	Apr-26	**COMPLETE**
		5.6	Development of 25_26 Hybrid Mail, Digital Letters & Online Forms initiative	Apr-26	**COMPLETE**
		5.7	Development of 26_27 Digitalisation initiatives	May-26	**COMPLETE**
		5.8	26_27 Digital Initiatives launched	Jun-26	**COMPLETE**
Services for Joint Development	6a. Community Health Locality Teams and neighbourhood working	6.1	Establishment of management structure for CHLTs, including interface with INTs	Apr-26	**COMPLETE**
		6.2	Incorporating Health Inequalities focus for CHLTs working within BHC	Apr-26	In Progress
		6.3	Mobilisation plan for CHLT service specification implementation	Jan-26	In Progress
		6.4	CHLT support INT phase 1 go-live – 1 INT to launch pilot model (SW) with mLTCs and frailty	Feb-26	In Progress
		6.5	CHLT support INT phase 2 go-live - Phase 2 – pilot model rolled out to all INTs	Jun-26	Minor delays
		6.6	Update service specification and assoc. KPIs	Sep-26	Not started
	6b. Hospital Discharge (as part of CHLTs)	6.7	Development of Hospital Discharge model as part of CHLT	Mar-26	In Progress
		6.8	Mobilisation plan for Hospital Discharge model	Apr-26	Minor delays
		6.9	Update service specification and assoc. KPIs	Apr-26	Minor delays
		6.10	Service launched for Hospital Discharge	Dec-26	Not started
	6c. Community Falls and Frailty Service (as part of CHLTs)	6.11	Service review and scoping of options	Apr-26	In Progress
		6.12	Develop service outline for **generalised integrated** falls and frailty pathway (identification, assessment, falls rehabilitation)	May-26	In Progress
		6.13	Develop **specialist** falls and frailty model	Jun-26	Minor delays
		6.14	Mobilisation plan for implementation	Sep-26	Not started
		6.15	Develop service specification and assoc. KPIs	Sep-26	Not started
		6.16	Service ready to launch	Dec-26	Not started
	7.Intermediate and Rehabilitation Care Beds	7.1	Service review and scoping of options	Feb-26	**COMPLETE**
		7.2	BHC to produce options appraisal and submit to ICB	Mar-26	**COMPLETE**
		7.3	Develop service outline for bed-based activity and services for step-down, step-up and rehabilitation	Apr-26	In Progress
		7.4	Mobilisation plan for implementation	May-26	Minor delays

		7.5	Develop service specification and assoc. KPIs	Sep-26	Not started
		7.6	Service ready to launch	Dec-26	Not started
	8.Same Day Urgent Care	8.1	Developing the in-reach role to the acute setting	2025	**COMPLETE**
		8.2	Launch the Single Point of Access (SPoA) pilot	Jan-26	**COMPLETE**
		8.3	Developing the UCR and H@H model	Jan-26	**COMPLETE**
		8.4	Mobilisation plan for UCR and H@H implementation	Feb-26	**COMPLETE**
		8.5	Develop UCR H@H service specification and assoc. KPIs	Jul-26	Minor delays
		8.6	SPoA pilot review	Mar-26	Minor delays
		8.7	Full SPoA service launched	Apr-26	Minor delays
	9.Children and Young People's Occupational Therapy	9.1	Develop service outline for an integrated CYP OT service	May-26	NO LONGER A REQUIREMENT
		9.2	Joint options appraisal prepared and submitted to ICB/ LBB	Apr-26	NO LONGER A REQUIREMENT
		9.3	ICB to review options appraisal and respond	Jun-26	NO LONGER A REQUIREMENT
		9.4	Mobilisation plan for implementation	Jul-26	NO LONGER A REQUIREMENT
		9.5	Develop service specification and assoc. KPIs	Jul-26	NO LONGER A REQUIREMENT
		9.6	Service launched	Dec-26	NO LONGER A REQUIREMENT
	10.Integration with Secondary Care	10.1	Integration plans agreed with secondary care	Dec-26	Not started
Confirmed Service Specifications	11.Adult Speech and Language Therapy Service	11.1	Mobilisation plan for service specification implementation	Jan-26	**COMPLETE**
		11.2	Service ready to launch	Dec-26	**COMPLETE**
	12.All-age Podiatry	12.1	Mobilisation plan for service specification implementation	Mar-26	**COMPLETE**
		12.2	Service ready to launch	Dec-26	**COMPLETE**
	13.Integrated Respiratory Service & Home Oxygen	13.1	Mobilisation plan for service specification implementation	Jan-26	**COMPLETE**
		13.2	Service ready to launch	Dec-26	**COMPLETE**
	14.Bladder and Bowel Service	14.1	Scope options for establishment of ambulatory catheter care service	Mar-26	**COMPLETE**
		14.2	Prepare options appraisal and share with ICB	Apr-26	**COMPLETE**
		14.3	ICB to review options appraisal and respond	May-26	In Progress
		14.4	Mobilisation plan for service specification implementation	Jun-26	Not started

		14.5	Service ready to launch	Dec-26	Not started
	15.Neuro rehabilitation	15.1	Prepare options appraisal and share with ICB re MND OT	Feb-26	**COMPLETE**
		15.2	ICB to review options appraisal and respond	Mar-26	**COMPLETE**
		15.3	Mobilisation plan for service specification implementation	Apr-26	**COMPLETE**
		15.4	Service ready to launch	Dec-26	In Progress
	16.Children's Community Nursing Team & Hospital at Home	16.1	Mobilisation plan for service specification implementation	Jan-26	In Progress
		16.2	Service ready to launch	Dec-26	Not started
	17.Children's Community Dietetics Team	17.1	Options paper regarding HEF contract implementation requirements developed and shared with ICB	Feb-26	**COMPLETE**
		17.2	ICB to review options appraisal and respond	Apr-26	**COMPLETE**
		17.3	Mobilisation plan for service specification implementation	May-26	In Progress
		17.4	Service ready to launch	Dec-26	Not started
	18.Community Paediatrics (ADHD & ASD Pathway Redesign)	18.1	Scope provision of ADHD / ASD service	Apr-26	**COMPLETE**
		18.2	Identify areas of efficiency	Apr-26	**COMPLETE**
		18.3	Develop service outline for ADHD & ASD redesign	Apr-26	**COMPLETE**
		18.4	Recruitment of key staff	Jun-26	**COMPLETE**
		18.5	Develop service specification and assoc. KPIs	May-26	**COMPLETE**
		18.6	Mobilisation plan for ADHD & ASD pathway	Jun-26	In Progress
		18.7	Service ready to launch	Jul-26	Not started
Data Requirements	19.Community Currencies (current SDIP)	19.1	Scope existing data collection and systems against community currency guidance	Feb-26	Minor delays
		19.2	Full data collection launch as per current guidance	Dec-26	Not started
	20.Medicines, device, and products data	20.1	Agreement on dataset	Jan-26	In Progress
		20.2	Data collection launch	Dec-26	Not started
	21.Key performance indicators and outcomes	21.1	Scope gaps in data collection	Feb-26	**COMPLETE**
		21.2	Design of data collection systems	Apr-26	Minor delays
		21.3	Implementation of data collection processes	Aug-26	Not started
		21.4	Shadow data collection period launched	Sep-26	Not started
		21.5	Full KPIs reported (excluding ones that have been TBC'd)	Dec-26	Not started
Transfer of Services	22.Wheelchair services	22	Actions will be defined once confirmation of processes are detailed	Dec-26	**COMPLETE**

	23.Diabetes	23	Actions will be defined once confirmation of processes are detailed	Dec-26	**COMPLETE**
	24.GP OOHs	24	Actions will be defined once confirmation of processes are detailed	Dec-26	In Progress
	25.CYP Audiology	25.1	Agree exit plan for BHC provision ending on 31st March 26	Jan-26	In Progress
		25.2	Finalise TUPE arrangements for 2 staff with GSTT for staff to transfer on 1st April 26	Jan-26	In Progress
	26.Hollybank	26	Actions will be defined once confirmation of processes are detailed	Dec-26	Not started
	27. Primary Care IT	27.1	MOU signature	Feb-26	**COMPLETE**
		27.2	Financial baseline and variation agreed	Feb-26	In Progress
		27.3	Delivery in accordance with MOU	Dec-26	Not started

South East London Integrated Care System

NHS South East London

Board Meeting in Public

Title	**2025/26 ICB Annual Report and Accounts**						
Meeting date	22 July 2022		Agenda item Number	**5**	Paper Enclosure Ref		**D**
Author	David Maloney, Director of Corporate Finance Julie Witherall, Director of Financial Management						
Executive lead	Dinah McLannahan, Chief Finance & Compliance Officer Tosca Fairchild, Chief of Staff						
Paper is for:	Update	**x**	Discussion	**x**	Decision		
Purpose of paper	The purpose of the paper is to present to the Board at a meeting held in public the ICBs audited annual report and accounts for 2025/26.						
Summary of main points	The South East London ICB annual report and accounts for 2025/26 is presented to the Board in line with NHS England guidance, which requires presentation at a meeting held in public by 30 September 2026. The audited annual report and accounts were submitted to NHS England on Thursday 18 June 2026, ahead of the deadline of Friday 19 June 2026, following the issue of an unqualified opinion by the ICBs external auditors. The annual accounts reported achievement of all the ICBs financial duties and targets for 2025/26 and were commended by the external auditors for their high quality. The annual report has been produced in compliance with NHS guidance and the report is used by NHS England as a principal source of assurance for the ICBs annual assessment. The ICB is required to publish the annual report and accounts on its website by 30 September 2026. A summary annual report for ease of reading will also be produced and published.						
Potential conflicts of Interest	None advised						
Relevant to these boroughs	Bexley	**x**	Bromley	**x**	Lewisham		**x**
	Greenwich	**x**	Lambeth	**x**	Southwark		**x**
Equalities Impact	Not applicable						
Financial Impact	The accounts report the financial performance of the ICB for the 2025/26 financial year.						
Public Patient Engagement	The annual report and accounts are required to be presented at a meeting held in public by the end of September 2026. This report presented at the Board meeting held in public meets this obligation.						
Committee engagement	The draft and audited annual report and accounts were reviewed and approved for submission by the ICB Audit & Risk Committee, in line with the ICB's scheme of reservation and delegation.						

Recommendation	Board members are asked to **note** the ICB 2025/26 annual report and accounts.

2025/26 Annual Accounts and Annual Report
NHS South East London Integrated Care Board (ICB) 22 July 2026

Introduction

The purpose of this paper is to provide an update for the Board on the ICB annual accounts and annual report for the year ending 31 March 2026.

Submission of Annual Accounts and Annual Report

The final, audited ICB annual accounts and annual report, together with all required supporting documentation, was submitted to NHS England in advance of the national deadline of 19 June 2026.

Governance Arrangements (including the external audit) of the Annual Accounts and Annual Report

The annual accounts and annual report were presented to the ICB Audit & Risk Committee on 16 June 2026, for approval under delegation from the Board. The ICB's external auditors presented their Audit Findings Report and Annual Audit Report to the committee members at the same meeting, noting an exceptionally effective audit and high quality of accounts. The auditors noted the high levels of engagement and professionalism from the finance team in dealing with their queries, and the quality of the work carried out in the context of all the current ICB changes. An unmodified opinion to the accounts was proposed. In addition, the ICBs internal auditors delivered their annual report and the committee determined the outcomes of this report were consistent with the contents of the annual report. On this basis, the Audit & Risk Committee approved the annual accounts and annual report.

The ICB received an unqualified opinion on the annual accounts from its external auditors. Specifically:

- An unqualified opinion on the financial statements.
- Regularity - the expenditure and income recorded in the financial statements had been applied to the purposes intended by Parliament and the financial transactions conform to the authorities which govern them.
- Nothing to report in respect of the ICB's arrangements for securing economy, efficiency and effectiveness in its use of resources.

There was one unadjusted misstatement in the ICB's annual accounts. Due to the usual time delay, the ICB is required to estimate the costs of prescribing for the final 2 months of the year and include this as an accrual in the year-end accounts. The actuals when received differed by £2.1m (an overstatement). Management agreed with the audit team not to adjust as this was an estimation error which was not material given total ICB expenditure was circa £5.9bn. (This is set out in more detail in the Audit Findings (ISA 260) Report).

Aside from the usual presentational changes agreed with our external auditors, the final audited annual accounts were consistent with the draft accounts submitted at the end of April 2026.

Financial Performance of the ICB

The ICB met all of its financial duties for 2025/26, with the performance as detailed below:

- a surplus of £117k against its overall Revenue Resource Limit.
- an underspend against its Running Cost allocation (RCA) of £9.1m. This is primarily due to the full allocation (£12.6m) received in respect of redundancy costs being badged as RCA whereas some of the expenditure has been classified as programme costs.
- compliance with the Mental Health Investment Standard (£12.8m above requirement).
- all targets under the Better Payment Practice code.
- a year-end cash balance of £1.6m, well within the stipulated target.

Annual report

The annual report has been produced in accordance with NHS England and Treasury guidance. The contents have been sourced from departments across the ICB in order to ensure the contents provide commentary and analysis of the critical areas required by the guidance. This includes statutory reporting around Task Force on Climate-related Financial Disclosures (TFCD), and governance and internal control reporting.

The report has been reviewed as part of the external audit process with no material matters of concern noted. The relevant information included in the remuneration and staff report sections has been fully audited, and this has been noted in the report.

In addition to the full annual report and accounts, published in line with guidance, a summary of highlights will be published for ease of reference. This document is currently in production and will be shared on the ICBs website alongside the full annual report and accounts.

Conclusion

The final, audited versions of the annual accounts and annual report will be made available to view on the ICB's website in line with the 30 September 2026 deadline set by NHS England. The report will be available here: https://www.selondonics.org/icb/meetings-board-papers-reports/reports/

The requirement to also present the annual report and accounts at a meeting of the ICB held in public by this date is being met through delivery of this report to the ICB Board.

South East London Integrated Care System

NHS South East London

Board meeting in Public

Title	**Chief Executive Officer's Report**						
Meeting date	22 July 2026		Agenda item Number	6	Paper Enclosure Ref	E	
Author	Andrew Bland, ICB Chief Executive Officer						
Executive lead	Andrew Bland, ICB Chief Executive Officer						
Paper is for:	Update	x	Discussion		Decision		
Purpose of paper	To receive the report from the Chief Executive Officer						
Summary of main points	This report updates the Board on matters of interest across NHS South East London since the last Board meeting on 15 October 2025						
Potential conflicts of Interest	None						
Relevant to these boroughs	Bexley	x	Bromley	x	Lewisham	x	
	Greenwich	x	Lambeth	x	Southwark	x	
Equalities Impact	Equality Impact Assessments are considered where applicable						
Financial Impact	N/A						
Public Patient Engagement	Public engagement takes place where appropriate and this report is presented to the Board meeting in public and published on the ICB website						
Committee engagement	N/A						
Recommendation	That the Board receive the Chief Executive Officer's Report						

Chief Executive Officer's Report

NHS South East London Integrated Care Board (ICB) 22 July 2026

The report that follows provides an overview of the activities of the ICB and its partners across the system seeking to highlight those issues that the Executive Directors and their teams have been addressing over the last period and to record those developments of note in our work

Since our last Board meeting the system has continued to manage considerable demand and other pressure on services whilst continuing to provide services for residents. In the ICB, the change programme reached an important milestone with the publication of the management response to the consultation and the process of filling posts has begun. It remains a very worrying and stressful time for ICB colleagues, who continue to show resilience, care for each other and professionalism as part of our work for residents. I am therefore particularly pleased to be able to share in this report some of excellent and often award winning work associated with the ICB as we continue to play our part with partners in delivering the ten year health plan.

In our meeting today we start to lay foundations for the future ICB by discussing a refreshed governance structure better suited to help us fulfil our strategic commissioning purpose, working with partners and across our cluster to make the most efficient use of our streamlined resource. Across our own system, we consider some of the ongoing challenges facing our own system such as diagnostics following previous board discussions. Looking across the NHS nationally we note reports in relation to maternity services and Antisemitism and other forms of racism; these highlight some difficult and important lessons for us all in the NHS and we will need to consider their recommendations carefully with our local residents and colleagues in mind.

1. Maternity reports

1.1. The Ockenden (Nottingham) Report (24 June 2026) and the Amos Report (30 June 2026) together provide a strong, system-wide assessment of maternity and neonatal services in England. Both reports are considered on our agenda today.

Ockenden (Nottingham) Report

1.2. The Ockenden Review is the largest maternity inquiry in NHS history, examined over 2,500 cases at Nottingham University Hospitals and identified longstanding, systemic failures in leadership, staffing, culture, and clinical practice. It found hundreds of cases of potentially avoidable harm, driven by poor risk recognition, inadequate training, failure to listen to women and families, and weak governance, with repeated missed opportunities to learn and improve.

Amos Report

1.3. The Amos Report builds on these findings at a national level, it provides a comprehensive national assessment of maternity and neonatal services, drawing on evidence from families, staff and 12 NHS trusts. It identifies persistent, system-wide challenges including women not being listened to, workforce pressures, fragmented care pathways, and inequalities affecting outcomes and experience.

1.4. The report sets out eight strategic recommendations to deliver long-term transformation. These include establishing a statutory Maternity and Neonatal Commissioner to strengthen leadership and accountability; embedding systematic listening to women and families as a core safety requirement; and improving responses when harm occurs through better investigation, transparency, and learning.

1.5. It also calls for a modern national service framework, action to address racism and inequality, strengthened governance and team culture, and investment in workforce, estates, and digital infrastructure.

1.6. Overall, the report emphasises urgent, coordinated action to improve safety, consistency, and compassion in care, supported by clear national leadership and sustained system reform.

1.7. The ICB will ensure a robust maternity and neonatal governance framework with strong leadership, working in partnership with all providers to deliver safe, high-quality care in which every woman's voice is consistently heard, respected, and responded to.

2. Leadership changes

2.1. Since our last Board meeting we have appointed Mike Parkinson as our new non-executive member who will take over from Peter Matthew at the end of this month. Mike is a chartered accountant with over 40 years' senior experience in finance, including as Chief Financial Officer and Board director. We extend our thanks to Peter for his contribution to the ICB since its inception in 2022.

2.2. We also said goodbye to executive colleagues Diane Jones our interim Chief Nurse and Mike Fox our Chief Finance Officer and I would thank them for both for their contribution to the executive team and the work of the ICB.

2.3. Dinah McLannahan has been appointed as Chief Finance and Compliance Officer across both South East London and South West London ICBs. Dinah joined South West London ICB in December 2025 on a 12 month secondment, bringing more than 25 years' experience in NHS finance across acute, mental health, community and system settings.

2.4. Ed Waller has been appointed as Chief Commissioning Officer and Deputy Chief Executive appointed across South East and South West London ICBs. Ed joins the ICB from Kent and Medway ICB where he is currently the Deputy Chief Executive and Chief Commissioning Officer. Ed will join the ICB in early September.

2.5. In April we announced the appointment of Elaine Clancy as Chief Nurse and Quality Officer across both SEL and SWL. We can now confirm that Elaine will also commence in post on 7 September 2026.

3. Equalities Update

Lord Mann Report:

3.1. Lord Mann's rapid review of Antisemitism and other forms of racism in the NHS and healthcare regulatory system was published in June 2026. The review found that racism, Antisemitism and Anti-Muslim Hostility remain prevalent across the NHS, with significant implications for staff wellbeing, psychological safety, engagement, retention and career progression. It also highlighted the impact discrimination can have on patient and community trust, potentially affecting access, experience and health outcomes. NHS England has accepted the review's 36 recommendations and issued eight immediate actions for NHS organisations.

3.2. For the ICB, the review provides an important opportunity to strengthen assurance on how we prevent, identify and respond to racism and discrimination across our workforce and services. An initial review has identified that many of the recommended areas are already reflected in existing arrangements, including workforce equality reporting, PSED reporting, EDI governance, Board oversight through the Equalities Senior Responsible Officer, Freedom to Speak Up arrangements and champions, executive leadership on domestic abuse and sexual harassment, staff engagement forums, and bullying, harassment and discrimination policies and processes. A gap analysis against each of the requirements for NHS organisations is set out below:-

Recommendation	Progress / Action required
To join NHS England in signing up to the NHS Race and Health Observatory Seven Anti-Racism Principles.	**The Board is asked to endorse** the signing up to the NHS Race and Health Observatory Seven Anti-Racism Principles. This commitment will be communicated through an article published both internally and externally. It will also be referenced in the introduction to the 2026 WRES report.
Ensure implementation of the Violence Prevention and Reduction Standard, including data capture and use to target improvement with affected groups, this will be monitored via the NHS Oversight Framework.	SEL ICB has a range of policies and mechanisms in place to support a safe, respectful and inclusive working environment, including bullying and harassment procedures, a zero-tolerance approach to violence and aggression, Freedom to Speak Up arrangements, and executive oversight in relation to domestic violence and sexual harassment. Relevant policies and arrangements include: - Bullying and Harassment Policy - Disciplinary Policy and Procedure - Freedom to Speak Up arrangements - Investigation Guidelines - Grievance Policy - Subscription to the Sexual Safety Charter The Violence Prevention and Reduction Standard will be implemented to align with the transition into a strategic commissioning organisation.
Prepare to implement the forthcoming NHS Staff Standards.	The actions in the NHS Staff Standards link closely with EDI, OD and HR priorities. Further work will align with the transition to the new strategic commissioning organisation.

Adopt the new government definition of anti-Muslim hostility.	The **Board is asked to endorse** the adopting the definition of Anti-Muslim Hostility and its commitment to the NHS Race and Health Observatory principles and the adoption of the IHRA definition of Antisemitism.
Ensure all staff have completed the NHS Core Skills Framework module on Equality, Diversity and Human Rights (EDHR), which includes content on antisemitism and Islamophobia.	SEL ICB staff are required to complete the e-LfH Equality, Diversity and Human Rights training every three years. Contact has been made with NHS England to understand forthcoming changes, including any London-specific developments with new training expected to come into effect in 2026.
Ensure all staff complete the new bitesize module co-created with faith leaders in the NHS when it becomes available (with the exception of those who have completed EDHR mandatory training in the last 6 months).	Further information is awaited from NHS England regarding the availability and implementation of these modules.
Ensure Board agreement to undertake new anti-racism training when it becomes available	**The Board is asked to agree and commit to undertaking the new when it becomes available.** Further information is awaited from NHS England regarding the new anti-racism training and associated implementation requirements.
Ensure colleagues, staff representatives, patients and communities are aware of your actions through your internal and external communications channels and are appropriately engaged in further developments to address antisemitism and all forms of racism locally.	The ICB already publishes its equality reports internally and externally, including: - WRES - WDES - WSOES - Gender Pay Gap Report - PSED report The Anti-Racism Strategy and EDI Strategy are also already published. The ICB will also publish its commitment to the NHS Race and Health Observatory principles, the IHRA definition of Antisemitism, and the Government definition of Anti-Muslim Hostility following board endorsement and support.
Ensure your Board and its relevant committees fully understand your staff survey data on the experience of racism in your organisation and are taking appropriate action on key problem areas related to this issue and monitoring progress.	Although ICBs are not required to national staff survey, SE ICB has participated, pausing participation during the management programme, replacing it with quarterly pulse surveys to pick up feedback quickly and address themes in a timely manner.
Can you confirm whether your organisation has adopted the IHRA definition of antisemitism and the Government definition of anti-Muslim hostility.	**The Board is asked to endorse adoption of** the IHRA definition of antisemitism and the Government definition of anti-Muslim hostility.

3.3. It is recommended that the Board

- Adopts the NHS Race and Health Observatory's Seven Principles for Anti-Racism, the IHRA definition of Antisemitism, and the Anti-Muslim Hostility definition. These commitments will be communicated to staff and the communities in the EDI section of the intranet and SEL ICB website respectively by 31 July 2026.
- Agree and commit to undertaking the new when it becomes available.

Equality Delivery Plan:

3.4. The Equality Delivery Plan (EDP) is an important monitoring tool within the ICB used to track progress against equality, diversity and inclusion priorities, including actions arising from statutory and mandatory reporting requirements such as the Equality Objectives, Workforce Equality Standards, and relevant organisational strategies. The plan has recently been reviewed and strengthened to make it easier to record, monitor and update actions in a consistent manner. This will support clearer oversight of progress, help identify areas where further action or support may be needed, and provide a more structured approach to evidencing delivery and impact. The refreshed EDP will continue to play a key role in supporting accountability and ensuring that equality-related actions remain visible, measurable and aligned with wider organisational priorities.

Equality Impact Assessments/Accreditations:

3.5. **Equality Impact Assessment:** Equality Impact Assessments (EIAs) support the ICB to identify potential equality impacts when services, policies or processes are developed or changed, helping to meet Equality Act duties and strengthen inclusive decision-making. Following the redesign of the EIA process, the quality and timeliness of submissions has continued to improve, with EIAs now providing more detailed evidence and being completed at an earlier stage. Between June 2025 and June 2026, 59 EIAs were submitted, compared with 22 during the same period in 2023 before the redesigned process was introduced. This represents an increase of over 160%.

3.6. **Accreditations:** Under the patient choice framework, independent providers are able to apply to deliver services for patients. The EDI Team reviews these applications to ensure equality, diversity and inclusion considerations are embedded from the outset and that providers can demonstrate how they will meet the needs of diverse communities. This includes reviewing evidence around accessibility, inclusive service delivery, workforce considerations and how potential equality impacts will be identified and addressed. To date, the EDI Team has continued to provide input into each application reviewed, helping to strengthen assurance and ensure that EDI remains a core part of the accreditation process.

4. Planning Directorate Update

2026/27 Strategic Investment Fund

4.1. The Strategic Investment Fund (SIF) is the South East London ICB's primary mechanism for delivering system-wide transformation in 2026/27, aligning investment to the priorities set out in the Five Year Strategic Commissioning Plan. A significant

proportion of the 26/27 SIF was allocated to Integrator Partnerships at place to invest in Neighbourhood Care and Prevention supporting the shift towards proactive, preventative, and community-based models of care. Following a considerable co-production process across each Integrator Partnership and a structured assurance process involving system-wide strategic commissioners, the Executive has approved all plans to move into delivery, recognising that they are sufficiently developed to support mobilisation and utilisation of funding within 2026/27.

4.2. While the overall quality and ambition of plans is strong, plans will continue to be strengthened to ensure maximum deliverability and impact. This includes further work on measurable outcomes and in-year trajectories, strengthening the coherence of prevention models, and addressing risks around workforce duplication and substitution. This ongoing work will be managed through SEL Neighbourhood Governance.

4.3. The plans will meet national expectations of supporting the 1.5% of our population identified as having the highest need through Integrated Neighbourhood Teams by March 2027. Our plans go beyond this by ensuring that mental health is integrated into neighbourhood health models and ensuring that neighbourhoods with the highest health inequalities as well as the populations at future risk of ill-health are being supported through preventative neighbourhood models.

Patient Involvement and Choice Award

4.4. We are proud to announce South East London ICB won the national Patient Involvement and Choice Award at the very first NHS Excellence Awards. This award recognised the depth of engagement and involvement of women and girls in the development and delivery of Women's and Girls' Health Hub pilots across Lambeth and Greenwich and Bexley.

4.5. The hubs bring GPs, nurses and specialists together in one place, ensuring women and girls can get help with more than one concern in a single appointment and work in partnership with public health and local voluntary and community sector organisations to outreach into communities. The model was developed with input from over 1,400 women and girls via survey campaign alongside targeted community-based events and focus groups, ensuring lived experience shaped the services offered.

4.6. Since launching in 2025, more than 700 women and girls have received care, with 860 referrals overall. This sits alongside investment in women's health development workers and ambassadors and VCSE micro-grants enabling health promotion and targeted outreach to address inequalities in access. The average wait for an appointment is around four weeks, compared with 14 to 17 weeks through hospital gynaecology services. Work is underway to expand the hub models, including moving to increase capacity and ensure more timely referrals into community-based care through single points of access. An independent evaluation is underway to inform opportunities to improve, spread and scale the model across south east London.

5. Bexley Borough Update

From Analogue to Digital

Local context and digital exclusion

5.1. Digital access brings clear benefits for residents, but it is important to recognise that digital exclusion remains a significant issue. Nationally, around 1.6 million people are estimated to be living largely offline, around a quarter of the population may struggle to use online services, and almost a third of people do not access services online at all.

5.2. For those affected, digital exclusion can be associated with poorer access to employment, higher costs, reduced ability to manage finances and worse health outcomes. In Bexley, support is available through a range of community and voluntary sector initiatives, including library-based access to hardware and digital support, the Bexley Voluntary Service Council Digital Champions programme, the Mind in Bexley Digital Hub, Age UK Bexley digital support sessions, and the Live Well, Age Well programme. This local infrastructure is important in ensuring that digital transformation is inclusive as well as efficient.

getUbetter App

5.3. The getUbetter App was commissioned by the Bexley Wellbeing Partnership and launched in Bexley in July 2025. It is a proactive and preventative digital tool designed to help residents self-manage simple musculoskeletal conditions and pain. The app has been promoted through GP practices in Bexley, Oxleas NHS Foundation Trust and a wider Bexley Wellbeing Partnership communications campaign, reflecting a whole-system approach to adoption.

5.4. Reported impact to date is positive. A total of 7,460 Bexley residents registered with the app. There has been a reduction in waiting times for routine physiotherapy appointments from 25 weeks in September 2025 to approximately 6 weeks in March 2026, which suggests that the app may be helping to manage demand and support earlier self-care. Most residents heard about the app from their GP, indicating the importance of primary care endorsement.

NHS App

5.5. The NHS App is the NHS's digital front door and provides residents with a secure way to access a range of services through a smartphone or tablet. These include ordering prescriptions, managing appointments, viewing parts of the GP health record, contacting a GP practice, viewing an NHS number, recording organ donation preferences and accessing NHS 111 online. The app is therefore a key enabler of the wider move from analogue to digital and supports residents to take a more active role in managing their health and care.

5.6. Bexley has made significant progress over the past four years in promoting the NHS App and increasing resident uptake. Bexley has the second highest uptake in south east London at 71.4%, exceeding the south east London target of 65%. Over the previous five months, just under 500,000 messaging fragments were sent to and read on the NHS App by Bexley residents, with an associated saving to the NHS of just under £10,000 in text messaging costs alone.

5.7. Local promotion has contributed to this progress. The Bexley Voluntary Service Council Digital Champions programme has supported NHS App promotion through community support and GP practice engagement, helping residents to sign up and reducing digital exclusion. The Better Access Bexley campaign increased local awareness through advertising, which coincided with a stronger than usual rise in registrations.

Universal Care Plan

5.8. The Universal Care Plan is a shared digital care plan that records what matters to a person, together with relevant health information and support needs, so that health and care staff across London can see updates in real time. It is available to all residents and is particularly valuable for people with long-term conditions or complex needs, because it helps the different professionals involved in a person's care to work from the same information. Residents can view, create and update parts of their plan within the NHS App, and healthcare organisations, including general practice, can also create or update a plan with the resident.

5.9. The NHS App now enables residents to develop and review their own Universal Care Plan. Whilst national promotion is being introduced on a phased basis, Bexley is seeking to support local uptake, for example through the Bexley Live Well Age Well Hub, with local Digital Champions providing practical on-site support to help residents create and maintain their plans and strengthen personalised, coordinated care.

Delivering Neighbourhood Health

Integrated Neighbourhood Teams

5.10. The Clocktower Neighbourhood 3+ Long Term Conditions Integrated Neighbourhood Health and Care Pilot has provided strong local evidence for the direction set out in the national 10 Year Health Plan: shifting care from hospital to community, from reactive treatment to prevention, and from fragmented provision to more coordinated neighbourhood-based support. The pilot has demonstrated how Integrated Neighbourhood Teams can work with general practice, community, acute, voluntary sector and non-medical partners to identify residents with multiple long-term conditions earlier, understand their wider health and social needs, and coordinate more personalised support closer to home.

Carers Week

A Day for Carers in Bexley

5.11. Carers Week in Bexley commenced with A Day for Carers, held at the Holiday Inn, Bexley, on 8 June 2026. The event was attended by more than 80 unpaid carers and provided a dedicated opportunity to recognise their contribution and connect them with support. Attendees received a gift bag on arrival and were able to access a marketplace offering information, advice and support from a range of local services. The programme also included wellbeing activities such as massage, reiki, sound baths, art and singing sessions, gentle exercise, and talks on stress management and direct payments.

5.12. A second carers event was held on 30 June 2026 at the Guru Nanak Darbar Gurdwara in Belvedere. Unpaid carers from across Bexley were invited to attend this community event, which celebrated the vital contribution unpaid carers make to families and communities across the borough.

5.13. Taking place at the start of South Asian Heritage Month, the event provided an opportunity to recognise the diversity of Bexley's communities and to acknowledge the important role unpaid carers play across all cultures and communities.

5.14. Approximately 70 unpaid carers attended the Belvedere event. Feedback indicated that more than half of attendees from the South Asian community had not previously attended an event of this nature to support carers. The event provided an important opportunity to hear directly from carers about their experiences, including the barriers they face when navigating support services.

6. Bromley Borough Update

Improving equity and reducing variation

6.1. One Bromley partners have agreed to strengthen the delivery of the health equity agenda by establishing a system wide Equity in Health and Wellbeing Forum and agreeing a shared vision for health equity. The vision is to improve equity in health and wellbeing by recognising the wider determinants of health and strengthening prevention, early intervention, and population health management.

6.2. A new operational group has been established to help to oversee delivery of health equity initiatives across Bromley and ensure that health equity insights shape commissioning, neighbourhood development, and pathway redesign. The group also seeks to coordinate activity across partners, resolving operational barriers. Delivery will be through neighbourhood-based approaches, working with Integrated Neighbourhood Teams to target interventions at population segments with the greatest need, and will include working closely with VCSE partners and community groups to co-design interventions and improve reach into underserved communities.

6.3. Initial work has commenced on identifying specific populations and work is underway in the following areas:

- Improving access to primary care and improving the health of veterans. Bromley has no military locations in its borough boundaries but has the largest population of veterans (6700) in London. An assessment of some of the key issues has shown that although many practices are designated as veteran friendly, this is not always clear and some veterans are reporting that they are not signposted to appropriate services and support. Interventions being undertaken include educational events for primary care (direct training and webinar), reviewing the referral optimisation pathway and developing infographics for practice staff.
- Identification and support of carers. Since the introduction of the One Bromley Carers Charter (co-ordinated by Bromley Third Sector Enterprise), annual surveys of Bromley GPs have revealed only modest increases in carer identification and there continue to be inequities in identification by location within the borough, age, index of deprivation and ethnicity. Current measures are likely not reaching carers from these underrepresented groups.
- Improving access to primary care services for those who are housebound. There are over 5,000 housebound residents in Bromley, 50% of whom are over 85 years and 50% have a frailty score. Stakeholder engagement has shown that this population feels forgotten about, are less likely to have preventive care and often

have multiple long-term conditions and complex needs. A new proactive care for the housebound programme is being launched which includes elements such health and wellbeing checks, long term condition reviews and vaccinations.

Winter Evaluation 2025-26 and Recommendations

6.4. The collective management of winter pressures in 2025/26 was evaluated and key recommendations have been developed to improve system performance and resilience for the winter ahead. The review process was developed using a structured 5 step evaluation process using:

- Quantitative performance review
- Review of winter-funded / commissioned services
- Provider feedback including clinical reflection
- A&E Delivery Board Winter Wash-Up Workshop
- Synthesis and recommendation development.

6.5. Ten recommendations emerged:

- Improve and sustain high vaccination uptake in Bromley, with targeted action to increase coverage among care home residents and vulnerable groups. This will include staff training and awareness programmes and improved access through dedicated vaccination clinics for immunocompromised and housebound patients.
- Continue to promote and embed Pharmacy First across lower utilisation pathways, including low utilisation GP practices, 111 and UTC.
- Developing a sustainable model for primary care to address same day care demand with consistent and equitable use of wider UEC pathways.
- Significantly improve consistency of acute Consultant Connect pathways, including improvement in call pick-up rate for surgical SDEC.
- Improve paediatric urgent care capacity and flow through development of a paediatric SDEC model, improved UTC to ED triage and referral pathways, and increased promotion of alternative pathways such as Hospital at Home.
- Undertake a deep dive into mental health UEC activity between Oxleas and PRUH and King's to reduce Urgent and Emergency Care activity through improved early interventions and other pathways. Reduction in mental health UEC activity through system wide approach to preventing admission/effective discharge, including work with housing and care homes.
- Develop an integrated, clinically led triage pathway for unscheduled patients attending hospital, ensuring they are directed to the right place first time, significantly reducing Emergency Department congestion and eliminating corridor care. The pathway will align with primary care services and make best use of local community, mental health, voluntary sector provision and SDEC provision, ensuring patients receive care in the most appropriate setting.
- Achieve a mature Frailty SPOA accessed consistently by LAS and newly offered to care homes supported by access to a range of community based alternative care

pathways, including a community frailty hub, UCR and a H@H service, all of which have clearly defined thresholds for admission.

- Agree a consistent way across provider organisations (including care homes) to differentiate acuity from complexity in patients so that the correct clinical pathway is accessed without an immediate default to treatment in hospital.
- Strengthen dementia and Delirium Identification and Management with a focused pieced of work across care homes and the redesign of the hospital discharge pathway for those with the most complex needs.

This evaluation provides a clear set of recommendations to inform improvement and development areas for 2026/27 across the same day urgent and emergency care system as well as priority areas for 2026/27 Winter Planning.

7. Greenwich Borough Update

7.1. Greenwich has celebrated a number of successes over the last quarter including winning the Local Government Chronicle Award for the Digital Health and Care Technology programme and the Greenwich Women and Girl's hub forming one of the hubs which won the wider NHS Excellence Award in the ICB.

7.2. In addition, we have undertaken extensive engagement and design work to inform the deployment of Strategic Investment Fund in the borough and are pleased to have ICB approval to these plans and to be moving into implementation.

7.3. We have continued our series of Healthier Greenwich Partnership design labs, with excellent sessions on Housing, Employment and VCSE collaboration opportunities in Neighbourhoods.

Primary Care

Neighbourhood Development Progress for 2026

7.4. Greenwich has expanded Frailty into a borough-wide Proactive Care Pathway, targeting a 50% Year 1 caseload increase and stronger focus on moderate frailty and rising risk. Funding is supporting a Local Incentive Scheme to improve frailty coding and Universal Care Plan completion.

7.5. The LTCs pathway launches in July, focusing on proactive identification, prevention and wellbeing support for hypertension, CVD and diabetes. It builds on the Heritage PCN CKD pilot, with Health Coach funding secured across all four Neighbourhoods.

7.6. Following the Local Child Health Team pilot, Greenwich is planning community-based child health clinics aligned to SEL strategies. An East INT Family Hub, immunisation and paediatric appointment pilot has shown early success since April.

7.7. Local services are being restructured to align District Nursing, Adult Social Care and wider services to Neighbourhoods.

7.8. Neighbourhood Health Centre planning is progressing through the Estates and Spaces workstream. Sites were shortlisted by HGP Board in February 2026, with further resource needed to develop hub-and-spoke plans and maximise regeneration, Section 106 and CIL opportunities.

General Practice

7.9. Six of 29 Greenwich practices are CQC-rated Requires Improvement. One practice is under caretaker management following GP partnership registration cancellation in March 2026. Wider pressures include closures, retirements, contract changes, investigations, estates issues and two Serious Incidents.

7.10. The Greenwich Interface programme continues to improve cross-sector referral, prescribing, discharge, urgent care and pathway processes. The Greenwich and Bexley programme has been recognised nationally by NHS Confederation and presented at INT Community of Practice and HSJ Care Closer to Home events in 2026.

Integrated Commissioning – Children

Local Child Health Teams

7.11. Greenwich continues to pilot Local Child Health Teams in two Primary Care Networks. Feedback from families on the impact has been positive in terms of providing care closer to home and improving the timeliness of support. In addition, there has been positive feedback from clinicians involved in the work. The evidence of impact in respect of reductions in Primary Care and Secondary Care presentations is still building. However, early reporting indicates similar reductions seen elsewhere in Southwark, Lambeth and Bromley. The lessons from the work will inform wider discussion on the development of CYP INTs locally.

SEND Reforms

7.12. Work has continued to progress on the SEND Reforms with the local Plan submitted to the Department for Education on 19 June. The plan has been shaped together with families locally and in discussion with NHS Providers, this includes proposals for reshaping and strengthening local governance and oversight on the work. Early feedback from the Department for Education advisors has been positive with limited changes required on the plan prior to final submission.

Integrated Commissioning – Adults

Home First, Urgent and Emergency Care, and Winter Resilience

7.13. The Adults team continues to strengthen community provision, urgent care and admission avoidance, with the Transfer of Care Hub, ASC operating model and intermediate care work progressing in parallel.

- Eltham Community Hospital remains on track for D2A and step up/down provision from November 2026.
- Virtual Ward recommendations are progressing, including remote health monitoring linked to DHACT.
- Oxleas is reviewing Meadowview intermediate care beds alongside wider demand, capacity and model requirements.
- Reablement work continues to support short-term care aligned to local need and INT priorities.

ASC Operating Model Changes

7.14. The new ASC operating model is moving into implementation, with focus on INT alignment, local ways of working, culture change and digital enablers to improve earlier access to support, advice and guidance.

Strategic Commissioning and Digital Transformation

7.15. The Digital Health and Care Technology service won the Health and Social Care category at the Local Government Chronicle Awards, recognising its role in supporting residents at home and strengthening integrated care. The service is generating evidence on length of stay and proactive NHS pathway support, and remains a key enabler for Integrated Neighbourhood Teams

INTs - Digital and Data Enablers

7.16. Digital and data enabler work is progressing through recruitment, funding opportunities and INT prioritisation. The immediate focus is on operational gaps, system self-assessments and a steering group aligned to SEL and local transformation needs.

District Nursing

7.17. Oxleas has shared proposals to embed district nursing improvements and move towards INT footprints, aligned with Regain, homecare redesign and delegated health task testing.

Homecare Transformation and Community-Based Support

7.18. Resident and care worker engagement has started, with design workshops informing options for the new INT-aligned model. Community micro-enterprise development continues to support local, personalised provision and address gaps in INT-aligned community options.

Mental Health

7.19. Community mental health transformation is progressing across the system, aligned with INT development, SEL and national requirements, resident engagement and SIF proposals. Key areas include:

- S117 alignment with national requirements, SEL funding principles and local policy improvements.
- Supported accommodation work to help residents access the right support and move on when ready.
- MH Alliance development to broaden VCS, NHS, local authority and partner involvement.
- Continued alignment with SEL-wide mental health priorities and new ICB ways of working.

8. Lambeth Borough Update

Our Health, Our Lambeth 2023-28

8.1. At the June Lambeth Together Care Partnership Board meeting, members formally approved the third annual refresh of 'Our Health, Our Lambeth', the Lambeth Together

Health and Care Plan 2023-28. The refreshed plan reflects the significant national and system-level developments over the past year, including the publication of the national 10-Year Health Plan in July 2025, the Neighbourhood Health Framework issued in March 2026, the South East London Strategic Commissioning Plan and the refresh of the Lambeth Health and Wellbeing Strategy. These plans have shaped the priorities and actions set out for 2026/27, aligned to Lambeth's own local intelligence and ambitions, and Lambeth will continue to ensure plans remain consistent in direction and effectively reinforce one another.

8.2. Lambeth partners seek to continuously reflect, learn from and refine the Health and Care Plan as it progresses throughout the five years. Local data indicates where good progress has been made against the 15 agreed Our Health Our Lambeth outcomes over the last year. As a partnership, Lambeth have improved support to people with long term physical health conditions, exceeding national standards on Bowel Cancer Screening, achieving the annual target on delivering the Type 2 Diabetes 8 Care Processes, and providing an increased number of NHS Health Checks and Vital 5 Checks.

8.3. Partners also continue to work collaboratively to identify emotional and mental health issues and offer earlier support, with an increase in the number of mental health A&E attendances avoided through the Evening Sanctuary and an improvement in black service users' access to short term mental health support throughout the year. However, there will still be more work to do over the coming years to improve outcomes across a range of other areas such as increasing uptake of vaccinations and immunisations and delivering more sustained mental health recovery rates.

Leadership and Governance

8.4. Following the local government elections in May, the Council is in no overall control (Green 29 councillors, Labour 26 and Liberal Democrat's 8). Two further by-elections will now take place in Clapham Park and Streatham St Leonards wards on 9 July after two elected Green councillors did not take their seats.

8.5. Lambeth Council has appointed Richard Sparkes as Director of Adult Social Care following a competitive recruitment process. Richard will join the Lambeth Together Board in his role as the Council's statutory Director of Adult Social Services.

8.6. In addition to this, June was the final Board meeting of the current term for the Patient and Public Voice (PPV) Members. PPVs support the Board to make better decisions and to be more connected with communities, ensuring that significant issues for Lambeth people are identified and heard. Sarah Flannagan and Eugenie Dadie were appointed in 2024 for a two-year term which will end in August 2026. Recruitment for the PPV positions is now underway and open to Lambeth residents and service users who have an interest in health and care. The closing date for applications is 26 July and for more information visit Become a voice to shape health and care in Lambeth!

Working in the Community

8.7. **Neighbourhood and Wellbeing Delivery Alliance (NWDA)** – Lambeth have now finished the first round of planning for Strategic Investment Funding (SIF), aligned to SEL's strategic priorities to help Lambeth deliver on ambitions for the neighbourhood health service. The first three workstreams (covering Integrator Development, INT

Delivery, and Prevention) are signed off and Lambeth submitted proposals for Mental Health and Pushing Boundaries funds at the end of June.

8.8. **Multiple Long-Term Conditions (mLTC)** Integrated Neighbourhood Teams went live in April 2026, supporting residents living with cardiovascular disease, chronic kidney disease and diabetes. The testing phase involved eight participating practices across three Lambeth neighbourhoods: Clapham, Streatham, and North Lambeth & Stockwell. These vanguard practices are focusing on testing and refining the mLTC neighbourhood model before a wider scale-up across all of Lambeth. Early learning from the testing phase has been encouraging. People have responded positively to the holistic “one-stop” review model, where medical, social and wellbeing needs can be addressed within a single appointment.

8.9. **Living Well Network Delivery Alliance (LWNA)** – Changes to the way community mental health services are delivered are now underway. Lambeth will now start to reorganise its three Living Well Centres (LWCs) into five neighbourhood teams over the summer, with the changes expected to come into effect in September. There will be no change to the locations of the three LWCs, which will remain as: North at 332 Brixton Road, South East at 312 Brixton Road and South West at Gracefield Gardens, Streatham. Most people will continue to receive support in the same place with the same staff. These changes will be well communicated to both those people who Lambeth support and staff, with anyone affected supported by any reasonable means to minimise impact on their treatment and recovery.

8.10. **Children and Young Person Delivery Alliance (CYPA)** – The Alliance has continued to develop its neighbourhood working plan for children and young people including partners from across children’s social care, primary care, secondary care, Family Hubs, Special Educational Needs and Disability, education and the Families First Partnership to support a more coordinated approach to neighbourhood delivery. In May, the Alliance launched the Child Health, Education and Social Support meeting (CHESS) in the Norwood neighbourhood. CHESS is a multi-agency forum which supports children and young people where there is emerging risk, or where needs may not meet the threshold for statutory care or support. The pilot is testing how partners can come together around individual children and families, agree a shared view of need, and identify coordinated next steps across health, education, social care and wider support services.

8.11. **Equality, Diversity, and Inclusion** –The third London Inspire Black Health Inequalities Summit supported by Caribbean and African Health Network (CAHN) was held on the 20th of April and saw over 700 people attend. The Summit offered ten workshops with a wide range of themes from dismantling structural racism and driving equity in digital healthcare, to maternity and neonatal outcomes, perinatal infant mental health, cardiovascular disease, Mental Health Act reform, ageing and social care, and climate justice. The impact and any learnings will be brought back to the Lambeth Together partnership and EDI Group. At the South East London Integrated Care System EDI Awards, Charlotte Afuape, from the HDRC (HEART) team was the winner in the South East London Rising Star in EDI Award. Congratulations Charlotte, a great achievement! In addition, the Lambeth Women and Girls’ Health Hub team were the winners in the Community Impact in EDI Award.

Outcome of Inspections

8.12. On 15th May, the Care Quality Commission (CQC) published its report outlining Lambeth Adult Social Care's rating and performance. Lambeth has achieved an overall rating of "Good" and achieving a "Good" rating in eight of the nine quality statements within the framework, and an "Outstanding" rating for the "Equity in Experience and Outcomes" quality statement. With around three quarters of local authority reports now published, this overall score places Lambeth in the top 10% nationally for CQC assessment performance. This is an excellent achievement for the service and reflects the hard work and high performance of colleagues from across adult social care, commissioning and Lambeth partnerships. A key feature of the CQC report is the strong performance in relation to championing and delivering equity and inclusion in services.

9. Lewisham Borough Update

Prevention

9.1. A Prevention Workshop brought together partners across health, the council and the voluntary sector to begin co-designing a neighbourhood-based prevention model for Lewisham. The session focused on developing a shared vision for prevention aligned to the Integrated Neighbourhood Team (INT) model, with an emphasis on early intervention, reducing inequalities and improving outcomes for residents at rising risk. Discussions highlighted strong consensus that Lewisham does not require additional services, but instead needs better coordination, integration and community-led delivery. Over 60 people from the sector attend the session.

9.2. To date, there is broad agreement on the direction of travel for a prevention offer centred on integrated neighbourhood hubs, with strong outreach into communities and a co-produced approach with residents and the VCSE sector. The emerging model will include proactive identification of residents, navigation support, and delivery across community settings through a wide partnership of providers.

9.3. Initial implementation will focus on East and South neighbourhoods, with the prevention offer embedded within hubs and funded through the Strategic Investment Fund. The idea is to have a strong health check/ meds management approach with wraparound support via the VCS and other services address social issues. Work is now progressing to develop a draft prevention offer, alongside further engagement, co-design and refinement of outcomes and measures.

Downham Leisure Centre Selected as Neighbourhood Health Hub

9.4. Downham Health & Leisure Centre has been identified as one of the new Neighbourhood Health Hub sites within Lewisham. This forms part of the broader shift towards integrated, place-based care, bringing together health, care and community services within accessible neighbourhood settings. The selection reflects both the strategic location of Downham and the opportunity to build on an existing, well-used community asset to support prevention, early intervention and population health management.

9.5. The site has been awarded £500k capital funding to support internal reconfiguration of the health space, with the aim of improving utilisation and enabling better connectivity with other services within the centre.

Mental Health Strategic Investment Fund

9.6. Lewisham has developed a proposed approach to allocating its £710k Mental Health Strategic Investment Fund for 2026/27 to support integration of mental health into Integrated Neighbourhood Teams for long-term conditions and ageing well. The model has been co-developed with SLaM, commissioners, primary care, public health and VCSE partners, with further scrutiny through the Lewisham mental health alliance and stakeholder forums.

9.7. The proposal focuses on a multidisciplinary offer across working age and older adults, including embedded clinical and wellbeing roles in INTs for LTC already set-up, specialist support for older adults, and a VCSE-led outreach model to improve engagement and address inequalities. A small community chest will support emerging priorities, particularly around inclusion and trust building. The approach is explicitly test-and-learn, with monitoring and a six-month review to assess impact, equity and deliverability, incorporating ongoing community and VCSE feedback.

Investing in Primary Care - The Lewisham GP Premium

9.8. The Lewisham GP PMS Premium is a locally commissioned service designed to fund general practice activity that goes beyond core contractual requirements.

9.9. It directly supports system priorities, addresses local health needs, reduces health inequalities, and improves outcomes for residents. The service has been in existence for nine years.

9.10. In 2025/26, there was excellent service delivery in all areas of the PMS Premium. Most practices met, while some exceeded, the minimum requirements of the Premium with great improvements seen in all areas including the cancer screening (bowel and breast) areas.

9.11. Furthermore, Multidisciplinary Team Meetings (MDMs) have been aligned to the Lewisham Integrated Neighbourhood Teams model aimed at delivering proactive care to the most complex and vulnerable patients to reduce avoidable exacerbations of ill-health and improve quality of care.

9.12. Lewisham has successfully launched a refreshed PMS Premium for 26/27 with the aim of building on the recent success. Two new areas, Physical Health Checks for People with Serious Mental Illness and Fractional Exhaled Nitric Oxide testing in General Practice, were also included in the revised version.

Physical Health Checks for People with Serious Mental Illness.

9.13. This service has been introduced to reduce health inequalities for this patient cohort and in doing so progress general practice towards meeting an important national and local target.

9.14. The service offers a complete physical health check including:

a) a measurement of weight (BMI or BMI + waist circumference).

b) a blood pressure and pulse check (diastolic and systolic blood pressure recording or diastolic and systolic blood pressure + pulse rate);

c) a blood lipid including cholesterol test (cholesterol measurement or QRISK® measurement).

d) a blood glucose test (blood glucose or HbA1C measurement).

e) an assessment of alcohol consumption.

f) an assessment of smoking status.

9.15. Practices are also expected to perform the following additional assessments:

a) an assessment of nutritional status, diet and level of physical activity (nutrition/diet status + physical activity/exercise) status.

b) an assessment of use of illicit substance/non prescribed drugs;

c) medicines reconciliation or review

Fractional Exhaled Nitric Oxide testing in General Practice

9.16. This service has been introduced to support the use of Fractional Exhaled Nitric Oxide (FeNO) testing in general practice as part of evidence-based asthma diagnosis and management. It will provide equitable and timely access to high quality respiratory diagnostic services by providing FeNO testing for patients (both adults and children) presenting in primary care where there is a clinical suspicion of either Asthma or where a diagnosis of Asthma needs to be excluded or confirmed.

10. Southwark Borough Update

Partnership Southwark Strategic Board

The Partnership Southwark Strategic Board held public meetings in March and May.

10.1. In March, the Board discussed the emerging King's College Hospital Five-Year Strategy (2026–2031), welcoming its direction while highlighting the need to demonstrate clear impact, strengthen partnership working and integration, and support the development of neighbourhood health models.

10.2. The Board also welcomed a refreshed Children and Young People's Mental Health Action Plan, developed following a multi-agency workshop. Members supported the focus on streamlining pathways and improving access, while emphasising the need for clear accountability, robust governance, and measurable reductions in waiting times.

10.3. Community Southwark and participating organisations presented the positive impact of the Funding Differently pilot, which is helping grassroots VCSE organisations access funding through simpler processes and greater community involvement in decision-making. The Board discussed the importance of strengthening impact evaluation and exploring how the model could inform more inclusive and sustainable funding approaches across the system.

10.4. In May, the Board received an update from the Southwark Integrator on progress in establishing the infrastructure and operating model for Integrated Neighbourhood Teams (INTs). Members welcomed the progress made and stressed the importance of clearly demonstrating and communicating the benefits for residents as the model is implemented.

10.5. The Board also received a presentation on progress in developing an Ageing Well neighbourhood partnership model to provide more proactive and preventative support for people living with frailty. Members supported the focus on prevention, reducing social isolation and strengthening VCSE involvement, while highlighting the need for sustainable funding, stronger support for people with complex needs, including dementia, and clearer evidence of impact to inform future commissioning decisions.

CYP INT Workshop

10.6. A multi-agency co-design workshop was held on 15 June 2026 at King's College London, bringing together over 50 stakeholders across Southwark including parents, schools, NHS, local authority, VCSE and research partners to inform the development of the Southwark Children's Emotional Wellbeing Integrated Neighbourhood Team (INT) model. The workshop focused on children aged 9–14 with mental health, neurodevelopmental and school attendance challenges, using journey mapping and shared system insights to explore lived experiences and system barriers.

Neighbourhood Programme and INT Mobilisation

INT Mobilisation

10.7. The INT programme has transitioned from planning into delivery, with INTs supporting people with frailty and multiple long-term conditions (mLTC) going live in in April. Initial feedback from patients has been positive and there is work underway to develop a structured report on activity, staff and resident feedback. This represents a starting point and the service model will continue to evolve to include wider teams across health, care and VCSE partners.

10.8. As previously advised, Southwark Place is pump-priming INT infrastructure, including clinical and management roles at neighbourhood level. All roles have now been recruited, with a number already in post, and a full complement in place by the end of June.

Voluntary & Community Sector Engagement

10.9. The ICB is investing to support VCSE engagement in the development of Neighbourhood Health and Integrated Neighbourhood Teams (INTs) across Southwark during 2026/27. This investment will fund a VCSE Neighbourhood Engagement Lead in each of Southwark's five neighbourhoods, alongside a coordinating organisation. The Coordinator will support collaboration across the five Neighbourhood Engagement Leads and produce a final report setting out key insights and recommendations for a sustainable approach to ongoing VCSE involvement in the development of Neighbourhood Health in Southwark.

10.10. To date, four of the five Neighbourhood Engagement Lead roles have been appointed, each from a local organisation with connections to the neighbourhood in which they will be based, with the remaining position expected to be confirmed shortly.

CHILDs win at NHSE excellence awards

10.11. The neighbourhood-based CHILDs health programme in Lambeth and Southwark has been named London regional winner in the NHS Excellence Awards 2026, recognising its success in improving care for children and young people while reducing health inequalities. Further details can be found here: Lambeth and Southwark children's health programme wins NHS Excellence Awards - South East London ICS

Improving the Primary-Secondary Care Interface

10.12. The interface between primary and secondary care remains a critical determinant of patient experience and clinical outcomes. However, it does not always function as effectively as required. To address this, the SEL ICB Medical Directorate, supported by the Health Innovation Network South London, has led a programme of work focused on improving quality across this interface. As the programme concludes, there is now a clear emphasis on supporting providers and ICB colleagues to embed and apply the outputs, which are available via the Connected website.

Key outputs include:

- A co-designed consensus document setting out shared principles to guide effective working across primary, secondary, community and mental health services
- A patient charter outlining what individuals can expect when moving between services
- Agreed standards for discharge summaries and outpatient letters
- An updated SEL interface prescribing policy
- Best practice guidance on clear and accessible points of contact for patients
- A training package to support uptake and implementation of the interface principles

Engagement and local implementation

10.13. The programme has brought colleagues together through a SEL-wide group and four local interface forums to coordinate improvement activity and share best practice. Locally, the Lambeth and Southwark forum has focused on practical improvements, including:

- Improved processes for issuing Fit Notes
- Strengthening onward referral pathways from secondary care
- Enhanced use of Trust EPIC functionality to support better patient experience

10.14. Future sessions will focus on:

- Development of Trust single point of access pathways
- Improvement work led by Neighbourhood Nursing Teams

Shaping Neighbourhood Care Together in General Practice

10.15. Throughout July, engagement events have been held in each of Southwark's five neighbourhoods, for staff working in GP practices. Facilitated by the Health Innovation Network, the sessions have aimed to raise awareness of the work underway to develop a Neighbourhood Health Service and support practices to understand their role within this emerging model.

10.16. The events have introduced practice staff to key members of their local Integrated Neighbourhood Team (INT) and the Integrator leadership team, while also providing an opportunity to network with colleagues from neighbouring practices. Through informal discussion and shared learning over lunch, participants have been able to build relationships with practices they may not previously have worked closely with.

Southwark GP Premium Refresh

10.17. The Southwark GP Premium has been refreshed as a key mechanism for driving quality improvement and transformation in general practice, supporting delivery of the Southwark Neighbourhood Health Model. Following a comprehensive review, the scheme has been aligned with borough, SEL and national priorities while maintaining existing investment.

10.18. The revised specification strengthens the contribution of general practice to proactive care coordination, prevention, reducing health inequalities and improving outcomes for people with long-term conditions. It places greater emphasis on collaborative working through Integrated Neighbourhood Teams, supporting multidisciplinary approaches to care and more joined-up support for residents with complex needs.

10.19. The refreshed scheme also incorporates medicines optimisation within a single integrated framework, aligning incentives across key areas of quality improvement and population health management. Alongside a continued focus on cancer and long-term condition outcomes, the scheme streamlines existing requirements and targets investment towards interventions that deliver measurable improvements in outcomes, experience and equity for Southwark residents.

South East London Integrated Care System

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Board Meeting in Public

Title	**BAF review**						
Meeting date	22 July 2026		Agenda item Number	**7**	Paper Enclosure Ref	**F**	
Author	Kieran Swann, Associate Director of Risk and Assurance						
Executive lead	Tosca Fairchild Chief of Staff						
Paper is for:	Update		Discussion		Decision	**x**	
Purpose of paper	To present the updated Board Assurance Framework (BAF) to the ICB Board for approval, following endorsement by SEL SMT on 11 June 2026. The paper provides an overview of all risks currently exceeding ICB risk appetite thresholds, together with key changes arising from the annual review and refresh of the ICB's risk registers.						
Summary	A comprehensive review of the ICB's risk registers has been undertaken for 2026/27, resulting in the closure of 41 risks, the addition of 13 new risks and a total of 83 risks remaining across the ICB's risk registers. Of these a total of 14 risks exceed agreed risk tolerance thresholds and as such are included on the latest BAF. This includes 8 risks at SEL level and 6 further risks (all related to ADHD/autism services waiting times) from place risk registers. Key changes include the addition of risks relating to the delivery of strategic commitments, which consolidates three specific performance related risks which were included on the previous BAF, and a de-escalated BAF risk relating to commissioned paediatric audiology services.						
Potential conflicts of Interest	None						
Sharing and confidentiality	*Onward sharing permitted at members' discretion*						
Relevant to these boroughs	Bexley	**x**	Bromley	**x**	Lewisham	**x**	
	Greenwich	**x**	Lambeth	**x**	Southwark	**x**	
Equalities Impact	Not applicable at this stage.						
Financial Impact	None.						
Public Patient Engagement	Not applicable.						

Committee engagement	SEL ICB Risk Forum, 12 May 2026 SEL SMT, 11 June 2026 Audit and Risk Commmittee, 9 July 2026
Recommendation	The Board is asked to review and approve the ICB's Board Assurance Framework, following endorsement by the SMT on 11 June 2026.

South East London Integrated Care System

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SEL ICB Board Assurance Framework

Prepared for SEL ICB Board, 22 July 2026

Context and latest updates

A reminder on ICB risk governance

- The ICB's risk appetite matrix is used by the ICB Board to set risk tolerance levels for various categories of risk across the organisation. This approach is designed to promote and support local ownership of risk across the ICB's governance and delegation arrangements. It also means that the Board will receive a view on those risks that have been assessed as exceeding the tolerance levels set.

- The ICB's Audit and Risk Committee is responsible for review and approval of the ICB's risk management arrangements on behalf of the Board. ICB SMT has assumed responsibility from the ICB's Executive Committee for review of the draft BAF and its recommendation for Board approval (see next page).

- The ICB also operates with a corporate risk register, which includes risks which are material and are assessed as having some likelihood of impacting objectives or the ability of the ICB and/or wider ICS system to deliver business objectives. The Board Assurance Framework (BAF) document represents the full range of ICB risks from the risk register that sit above the permitted level of risk tolerance.

Latest updates including risk stock-take and refresh 2025/26 > 2026/27

- Beginning in April 2026, the ICB risk and assurance team have worked with directorate SMTs, risk leads, place executive leaders and colleagues in central ICB teams to undertake a full stock-take of existing BAF and corporate risk register risks. This process has involved a requirement that all risks are fully reviewed and updated to reflect the new operational context of the ICB and are relevant to the new financial year. The ICB team also ran a check-in session for all ICB risk leads on 12 May to review updates.

- In total 111 existing risks across BAF and corporate risk registers were reviewed, with 41 of these closed and 13 opened, to leave a total set of risks on all registers of 83.

- BAF risk reflect the latest risk entries as of 31 May 2026, and as a reminder include only those risks identified on risk registers as scoring above agreed risk tolerance thresholds.

- In addition to work within the ICB, the team also continue to chair the system risk leaders meeting, which brings together all risk leads from NHS trusts in the local system to discuss common issues related to risk, to collectively review the current and future risk environment and to discuss new and potential areas of risk and plans for mitigations. The April 2026 meeting focused on approaches to 'horizon-scanning' and digital risk management systems.

Structure of the BAF

- All risks on the SEL and place risk registers have been updated by designated risk owners working with their teams.
- **Appendix A**: includes the full risk register entries for all ICB risks which are above the tolerance levels (summarised on slides 7 - 9) and all place risks which are above tolerance levels (summarised on slide 10).
- The detailed descriptions of risks in the appendices, include the following information:
  - risk owners and sponsors
  - the risk category that the risk falls into
  - the risk appetite for that category of risk
  - a description of the risk
  - controls that are in place to mitigate the risk
  - assurances
  - initial and residual risk scores.

Role of the Board and recommendation

The ICB Board:

- Is responsible for **setting the strategic direction for risk management and overseeing the arrangements for identifying and managing risk** across the organisation (including those exercised by joint committees or committees-in-common).
- Has a role in **agreeing the scope of delegated activity** to be undertaken by the SEL Senior Management Team (SMT) on its behalf in relation to risk.
- The Board has delegated the detailed oversight of risks to SMT and is kept appraised of risk-related activity undertaken by relevant Board committees via committee reporting arrangements. The ICB **Board retains overall responsibility for formal approval of the ICB's BAF**.

Recommendation to the Board

- **Approve** the ICB BAF endorsed by the SEL SMT on 11 June 2026.

Key points and summary of changes (1 of 2)

Key points to note

- Risks reflect the assessed position as recorded on the ICB's Datix system for risk management on 31 May 2026.
- The current version of the BAF includes 7 SEL risks above threshold and 6 place risks (Bexley, Bromley, Greenwich, Lambeth, Lewisham, Southwark).
- Following review by SEL SMT on 11 June:
    - **SEL 657**, which relates to the risk of ICB information governance delivery failure and risks of IG incidents was proposed for inclusion on the BAF (score = 16). However, review by SMT confirmed that this risk should remain on the corporate risk register for close monitoring. The risk category for this risk has therefore been amended to "workforce" and the score reduced to 15.
    - **SEL 601**, relating to workforce capacity risks (ICB change programme) - an additional control has been added around the development of future ICB operating model and supporting OD programme.

Escalation of risks

- **No risk** with a score greater than the risk appetite threshold are **proposed to be escalated** onto the BAF:

BAF risks proposed for inclusion of the ICB BAF

- **1 new risk** with a score greater than the risk appetite thresholds is **proposed to be added** to the BAF:
    - **SEL 663,** which relates to strategic commitments and delivery priorities: implementation of ICB strategic commitments, approved plans, and delivery priorities. This has been added following the consolidation and closure of three previous BAF risks, which established separate performance delivery risks for specific areas of risk within the previous year's ICB operating plan (SEL 384, 386, 504).

Key points and summary of changes (2 of 2)

Closed BAF risks

- **3 previous** BAF risks are proposed for **closure**
    - **SEL 384, 386, 504,** related to elective waiting times, urgent and emergency care pressures and cancer performance standards of commissioned services respectively, and are proposed for closure as they have been incorporated into a single new risk (risk 663, see previous page) covering the delivery of the ICB's strategic commitments and priorities in aggregate. The rationale for this was to include a single strategic risks relating to achievement of delivery commitments, rather than to select three of the full range for inclusion within the BAF.

De-escalation of risks

- **1 risk** is proposed to **de-escalate** off the BAF:
    - **SEL 630,** relating to the quality of commissioned paediatric audiology services following a national programme of clinical review. This has been de-escalated from the BAF following the lookback review being completed in SEL and capacity in place for patients to be seen, including arrangements for those with high/moderate clinical needs being prioritised. The existing risk was reviewed by the Paediatric Audiology Oversight Group in May 2026 with assurances and gaps updated, and the group advising that the risk be downgraded to a 2x3, so residual score of 6, which sits beneath the threshold score. The risk remains on the ICB corporate risk register.

Summary of SEL risks exceeding tolerance levels (1 of 3)

Risk Category	Risk ID	Risk title / summary of risk	Key controls include	Max tolerance score	Residual risk score
Data and Information Management	**651**	Significant disruptions to the ICT and digital systems across out provider and/or supply settings.	Up-to-date incident response plan in place. DSPT compliance indicates that monitoring is in place to detect potential security problems and to track the ongoing effectiveness of protective security measures. SEL ICB has various mechanisms in place to ensure security defences remain effective and to detect cyber security events and incidents which may potentially impact essential functions.	9	12
Operational: relating to the effective day to day running of the ICB organisation (MCR)	**601**	ICB Change Programme – workforce capacity risks	Agreement to cease some non-priority work to focus on must do areas, reappraisal of consultation timeline aligned to national guideline, and programme governance structure in place. Development of future ICB operating model and supporting OD programme.	15	20
	602	ICB Change Programme – impact on staff	Weekly CEO-led all staff briefings in place, dedicated intranet and MS Teams space for transparent communication, regular “ask HR” sessions in place.		20

Key: Primarily system risk Primarily ICB risk

Summary of SEL risks exceeding tolerance levels (2 of 3)

Risk Category	Risk ID	Risk title / summary of risk	Key controls include	Max tolerance score	Residual risk score
Clinical, Quality and Safety	404	New and emerging High Consequence Infections Diseases (HCID) & pandemics.	Staff offered vaccines, HCID and pandemic plans in place, collaboration with system to minimised impact on the workforce, hybrid working in place, process for re-deployment established.	9	12
	468	Risk of variation in performance across SEL with FNC (funded nursing care) reviews.	Monthly assurance pack reviewed at CHC meeting, tracking FNC reviews, and individual borough plans set out how boroughs will clear overdue reviews.		12
	391	Increased waiting times for autism diagnostics assessments.	Backlog clearance to reduce waiting times includes development of services to meet demand, clinical leads recruited to focus on autism across all ages. Autism strategy approved and launched, and core offer for CYP autism assessment developed and agreed.		16

Key: Primarily system risk Primarily ICB risk

Summary of SEL risks exceeding tolerance levels (3 of 3)

Risk Category	Risk ID	Risk title / summary of risk	Key controls include	Max tolerance score	Residual risk score
Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	**633**	Implementation of ICB strategic commitments, approved plans, and delivery priorities	Mitigating controls include delivery of agreed operational plan trajectories for cancer performance, supported by system wide oversight through the SEL Cancer Alliance, Cancer Executive and regional performance review arrangements. Targeted support and assurance are provided through NHSE oversight, diagnostics and UEC programmes, quality surveillance and harms reviews, alongside investment through Strategic Investment Fund to support recovery and operational plan delivery. These arrangements are complimented by the ICB's five-year strategic commissioning plan and ongoing partnership working with providers to monitor progress, address delivery issues and support the shift from hospital to community care.	12	16

Key: Primarily system risk Primarily ICB risk

South East London Integrated Care System

Summary of place risks exceeding tolerance levels

NHS South East London

Risk Category	Risk ID	Risk title / summary of risk	Key controls include	Max tolerance score	Residual risk score
Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	**Bex 642**	Residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments.	SEL-wide neurodevelopmental improvement programme established under the CYP MH and Wellbeing Partnership Board to oversee ASD and ADHD diagnostic pathways, waiting times, and consistency of the core offer across SEL boroughs / places. New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays. Targeted capacity investment including non-recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. Exploring the opportunity to join arrangements with other boroughs to ensure residents have equity of access to medication and review pathways	12	16
	Bro 509	Residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments.			16
	Gre 635	Residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments.			16
	Lam 129	Waiting time targets for children and young people waiting for an autism or ADHD assessment is unacceptably long.			16
	Lew 644	Residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments.			16
	Sou 520	Residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments.			16

Key: Primarily system risk Primarily ICB risk

‘Heat Map’ of BAF risks

The heatmap below shows the likelihood and impact scores of the current BAF risks. They have also been differentiated by primarily ICB risks and primarily system risks.



ID	Summary risk descriptions
602	ICB Change Programme – impact on staff
633	Implementation of ICB strategic commitments and priorities
391	Increased waiting times for autism diagnostics assessments
601	ICB Change Programme – workforce capacity risks
642	Residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments.
509	
635	
129	
644	
520	
404	ICB oversight of new & emerging HCID & pandemics
468	Variation in performance with funded nursing care
651	Significant disruptions to the ICT and digital systems across out provider and/or supply settings.

Appendices: risk scoring matrices

Risk scoring matrices (1 of 3)

The matrices below are taken from the ICB's Risk Management Framework and represent the possible combined risk scores based on a measurement of both the likelihood (probability) and severity (impact) of risk issues. A combination of likelihood and severity score provides the combine risk score.

Likelihood x Severity = Risk Score

			Likelihood				
			1	2	3	4	5
			Rare	Unlikely	Possible	Likely	Almost certain
Severity	5	Catastrophic	5	10	15	20	25
	4	Major	4	8	12	16	20
	3	Moderate	3	6	9	12	15
	2	Minor	2	4	6	8	10
	1	Negligible	1	2	3	4	5

Likelihood Matrix:

Likelihood (Probability) Score	1	2	3	4	5
Descriptor	**Rare**	**Unlikely**	**Possible**	**Likely**	**Almost certain**
Frequency How often might it/does it happen	This will probably never happen/recur	Do not expect it to happen/recur but it is possible it may do so	Might happen or recur occasionally	Will probably happen/recur but it is not a persisting issue	Will undoubtedly happen/recur, possibly frequently
Frequency Time-frame	Not expected to occur for years	Expected to occur at least annually	Expected to occur at least monthly	Expected to occur at least weekly	Expected to occur at least daily
Frequency Will it happen or not?	<0.1%	0.1 to 1%	1 to 10%	10 to 50%	>50%

Risk scoring matrices (2 of 3)

Severity matrix

Severity (Impact) Score	1	2	3	4	5
Descriptor	**Negligible**	**Minor**	**Moderate**	**Major**	**Catastrophic**
Impact on the safety of patients, staff or public (physical / psychological harm)	Minimal injury requiring no/minimal intervention or treatment. No time off work	Minor injury or illness, requiring minor intervention Requiring time off work for >3 days Increase in length of hospital stay by 1-3 days	Moderate injury requiring professional intervention Requiring time off work for 4-14 days Increase in length of hospital stay by 4-15 days RIDDOR/agency reportable incident An event which impacts on a small number of patients	Major injury leading to long-term incapacity/disability Requiring time off work for >14 days Increase in length of hospital stay by >15 days Mismanagement of patient care with long-term effects	Incident leading to death Multiple permanent injuries or irreversible health effects An event which impacts on a large number of patients
Adverse publicity/ reputation	Rumours Potential for public concern	Local media coverage – short-term reduction in public confidence Elements of public expectation not being met	Local media coverage – long-term reduction in public confidence	National media coverage with <3 days service well below reasonable public expectation	National media coverage with >3 days service well below reasonable public expectation. MP concerned (questions in the House) Total loss of public confidence
Business objectives/ projects	Insignificant cost increase/ schedule slippage	<5 per cent over project budget Schedule slippage	5–10 per cent over project budget Schedule slippage	Non-compliance with national 10–25 per cent over project budget Schedule slippage Key objectives not met	Incident leading >25 per cent over project budget Schedule slippage Key objectives not met
Service Business Interruption	Loss interruption of 1-8 hours Minimal or no impact on the environment /ability to continue to provide service	Loss interruption of 8-24 hours Minor impact on environment / ability to continue to provide service	Loss of interruption 1-7 days Moderate impact on the environment / some disruption in service provision	Loss interruption of >1 week (not permanent) Major impact on environment / sustained loss of service which has serious impact on delivery of patient care resulting in major contingency plans being invoked	Permanent loss of service or facility Catastrophic impact on environment / disruption to service / facility leading to significant “knock on effect”
Personal Identifiable Data [Information Management Risks]	Damage to an individual’s reputation. Possible media interest e.g. celebrity involved Potentially serious breach Less than 5 people affected or risk assessed as low e.g. files were encrypted	Damage to a team’s reputation. Some local media interest that may not go public. Serious potential breach and risk assessed high e.g. unencrypted clinical records lost. Up to 20 people affected.	Damage to a service reputation. Low key local media coverage. Serious breach of confidentiality e.g. up to 100 people affected.	Damage to an organisations reputation. Local media coverage. Serious breach with either particular sensitivity e.g. sexual health details or up to 1000 people affected.	Damage to NHS reputation. National media coverage. Serious breach with potential for ID theft or over 1000 people affected.

Risk scoring matrices (3 of 3)

Severity matrix (contd.)

Severity (Impact) Score	1	2	3	4	5
Descriptor	**Negligible**	**Minor**	**Moderate**	**Major**	**Catastrophic**
Complaints / Claims	Locally resolved complaint Risk of claim remote	Justified complaint peripheral to clinical care e.g. civil action with or without defence. Claim(s) less than £10k	Below excess claim. Justified complaint involving lack of appropriate care. Claim(s) between £10k and £100k	Claim above excess level. Claim(s) between £100k and £1 million. Multiple justified complaints	Multiple claims or single major claim >£1 million. Significant financial loss >£1 million
HR / Organisational Development **Staffing and Competence**	Short term low staffing level temporarily reduces service quality (< 1 day)	Ongoing low staffing level that reduces service quality.	Late delivery of key objectives/service due to lack of staff. Unsafe staffing level or competence (>1 day) Low staff morale Poor staff attendance for mandatory / key training.	Uncertain delivery of key objective / service due to lack of staff Unsafe staffing level or competence (>5 days) Loss of key staff Very low staff morale No staff attending mandatory / key training	Non-delivery of key objectives / service due to lack of staff Ongoing unsafe staffing levels or incompetence Loss of several key staff No staff attending mandatory training / key training on an ongoing basis
Financial (damage / loss / fraud) **[Financial Risks]**	Negligible organisational / financial loss (£< 1000	Negligible organisational / financial loss (£1000- £10000)	Organisational / financial loss (£10000 -100000)	Organisational / financial loss (£100000 - £1m)	Organisational / financial loss (£>1million)
Inspection / Audit	Minor recommendations Minor non-compliance with standards	Recommendations given Non-compliance with standards Reduced performance rating if unresolved	Reduced rating Challenging recommendations Non-compliance with core standards Prohibition notice served.	Enforcement action Low rating Critical report. Major non-compliance with core standards. Improvement notice	Prosecution. Zero rating. Severely critical report. Complete systems change required.

Risk ID	Risk Owner	Risk Sponsor	Risk Title	Risk Type	Risk Appetite	Risk Description	Initial Likelihood	Initial Consequence	Initial Rating	Current Likelihood	Current Consequence	Current Rating	Target Likelihood	Target Consequence	Target Rating	Control Summary	Gaps in Control Summary	Assurance in Place	Gaps in Assurance
391	Carol-Ann Murray	Chief Nurse	Increased waiting times for Autism diagnostic assessments	Clinical, Quality and Safety	7 - 9	There is a risk of increased waiting times for a diagnostic assessment for Autistic Spectrum Disorder (ASD) for adults and children and resulting non-contracted activity costs due to patient choice referrals to private providers. This is caused by increased demand for assessments combined with historical waiting lists. The impact on the ICB will be on its ability to meet statutory obligations and increased spend due to non-contracted activity. Achieving timely access to assessment will reduce diagnosis waiting times and ensure support can be put in place earlier and help improve patient outcomes.	3	4	12	4	4	16	4	4	16	Backlog clearance achieved with waiting times reduced in boroughs served by Oxleas, service funded to meet the demand and maintain waiting times within 6 - 8 months. All age autism strategies in all six boroughs and recurrent funding of £80k provided to each to support non- medical needs and provide support in the community. Core offer for CYP Autism assessment developed and agreed with stakeholder. Set up of Community of practice to share best practice and find solutions to ongoing issues. Exploring options for assessment of 16,17 to 18 year olds before adulthood to prevent longer waits in adult services. - Piloting SEL CYP Neurodiverity Hub for diagnostic assessments and medication pathways. Waiting list validation undertaken in SLaM for Lambeth and Lewisham. Providers funded to undertake assessments 17 to 18 year olds before adulthood to prevent longer waits in adult services. Piloting SEL CYP Neurodiversity Hub for diagnostic assessments and medication pathways.	Demand for assessments increasing and difficult to predict if and when it will level out.	SEL LDA Strategic Executive Group Agenda and Minutes List the assurance evidence. SEL LDA Operational Board Agenda and Minutes. to be relaunched. Minutes from the quarterly Joint Region and System LDA Heath Partnership meeting.	No gaps on assurances
404	Simon Beard - Associate Director Corporate Governance	Tosca Fairchild - Chief of Staff	New and emerging High Consequence Infections Diseases (HCID) & pandemics	Clinical, Quality and Safety	7 - 9	There is a risk that new and emerging HCID & pandemics could occur at any time and are likely to occur in one or more waves. This could cause disruption to the operation of the ICB with staff illnesses/absence and reprioritisation of workload which could lead to a detrimental effect of communities and staff within SE London.	4	4	16	4	3	12	4	2	8	Staff are offered flu and covid-19 vaccines to mitigate as far as possible the impact on the workforce. HCID & pandemic plan is in place. Antiviral plan in place for SEL system. Collaboration with organisations across the system through forums such as Borough Resilience Forums enables the ICB to horizon scan for potential emerging HCID issues and put mitigating actions in place early to minimise impact to the workforce and ICB operations. Hybrid working arrangements are in place, supported by cloud-based access to IT systems, which enables the ICB to reduce face to face interactions between staff should this be necessary as a measure to reduce spread of infections. The ICB has an established process for considering staff redeployment to focus on business critical services. Employee assistance is available - e.g. mental health first aiders; occupational health and employee assistance programme. During the 2024-25 year there are plan to run tabletop and workbook exercises with the primary care teams and GPs to test and exercise the ICB plans for HCIDs. A national exercise - Op Pegasus - is planned to be run to test plans.	UK HSA have published updated communicable disease outbreak management guidance which will be used in London to develop a pan London MoU for managing complex infectious disease outbreaks. Once completed, the ICB HCID response plan will need to be reviewed for alignment.	SEL ICB - System approach utilised and implemented for HCIDs. EPRR Practitioners network is in place enabling early sharing of information/horizon scanning in relation to HCIDs, which will ensure organisations can take early mitigating actions. HCID plan reviewed and updated in 2024. Refreshed plan has been endorsed by ICB AEO and approved for publication by ICB Executive Committee. SEL ICB Head of EPRR and the Bromley PEL Angela Bhan, have been involved in the initial scoping discussion for the pan London MoU and are engaged in the ongoing development work.	No gaps in assurance
468	Jane Waite - Head of CHC/CYPCC Governance Assurance and QIPP	Diane Jones - Chief Nursing Officer	There is a risk of variation in performance across SEL with the FNC (Funded Nursing Care) reviews.	Clinical, Quality and Safety	7 - 9	There is a risk of variation in performance across SEL with the FNC (Funded Nursing Care) reviews. This is due to a significant number of reviews over the required time frames (National Standard). This is impacting on the ICB's ability to meet statutory requirements. This is a clinical risk which impacts on financial control across the system and patient experience.	4	4	16	3	4	12	2	4	8	This risk is monitored at the NHSE assurance meeting monthly. The SEL Head of CHC/CYPCC governance assurance and QIPP has oversight of this risk., There is a monthly assurance pack produced which goes to the CHC review meetings. The CHC monthly assurance report tracks FNC reviews. There are monthly meetings held at place level where this risk is discussed. There are individual borough plans setting out how boroughs will clear the overdue reviews.	No gaps in controls	There are minimal vacancies across the place based teams. Individual borough plans in place and teams are working towards reducing the backlogs.	Place CHC leads have been asked to provide individual borough trajectories where necessary. The CHC monthly performance report shows that overdue FNC reviews have been increasing overall
601	Ceri Jacob	Andrew Bland	ICB Change Programme – Capacity risks	Operational- relating to the effective day to day running of the ICB organisation (MOR)	13 - 15	There is a risk that the ICB will face significant and increasing capacity challenges throughout implementation of the ICB change programme. A cautious approach to recruitment throughout a prolonged period of uncertainty then a lengthy change programme, plus the impact of staff leaving through voluntary redundancy across April and May has/will have a tangible effect on ICB workforce capacity. This increases the risk of burnout for staff covering extended portfolios and threatens delivery of statutory responsibilities, ICB objectives and priority areas of work.	5	4	20	5	4	20	4	4	16	Programme governance structure in place (SMT, Transition Committee, Operations Group). Proposals tested through scenarios with a detailed transition plan being developed. Agreement to cease some non-priority work to focus on statutory and "must-do" areas. Reappraisal of consultation timeline aligned to issue of national guidance and review by ICB Board and the ability to more easily secure fixed term contracts and secondments for additional time-limited capacity. Change Programme Group with joint SROs meets weekly. Participation in London Transition Group to ensure safe transfer of functions. Directors have confirmed how they will manage gaps resulting from the first round of VR; outputs of this exercise have been reviewed collectively to identify interface issues. Directors will be asked to review again given the numbers of staff leaving via VR in April and May., Feedback from staff across the change programme and consultation has flagged anxieties around capacity gaps. Alignment in structures and ways of working with South West London ICB may give some additional capacity in some areas. Development of future ICB operating model and supporting DD programme programme.	None	Programme governance structure in place (SMT, Transition Committee, Operations Group).	None
602	Ceri Jacob	Andrew Bland	ICB Change Programme – Staff morale and wellbeing	Operational- relating to the effective day to day running of the ICB organisation (MOR)	13 - 15	There is a risk of low staff morale, health and wellbeing impacts, and wider disengagement as a result of the ICB change programme. Increased uncertainty around future structures, consultation length, and job security has led to a potential for increased staff anxiety and change fatigue. This may lead to higher sickness absence, reduced productivity, disruption to business as usual delivery, and disengagement from the consultation process.	5	4	20	5	4	20	5	4	20	Weekly CEO-led all-staff briefings with executive team in attendance. Dedicated intranet and MS Teams space to support transparent communications. Access to HR/OD support, including psychological support and wellbeing offers. Regular "Ask HR" sessions and targeted staff training/support.	None	Change Programme Group (MOR Ops Group) meeting weekly; monitors feedback received through consultation and has requested measures of morale and wellbeing i.e. long-term sickness rates, use of EAP. Ongoing monitoring of staff feedback through HR channels. Analysis of consultation feedback from staff has noted some issues of morale; to be addressed. Long-term sickness absence has not increased significantly through the change programme.	None currently identified
651	Pin Bhandal - Associate Director of ICT	Nisha Wheeler - Deputy Chief Digital Information Officer	Digital - Disruption to ICT/Digital systems affecting the ICB and providers systems.	Data and Information Management	7 - 9	There is a risk of significant disruptions to the ICT and digital systems across out provider and/or supply settings. This may be caused by external factors such as cyber attacks, gaps in security controls, human factors (including phishing and social engineering) or change initiatives implemented by the NHS, healthcare and service providers. The consequences of this occurring is significant disruption or delay to the provision of clinical services potentially affecting patients health and care. This may cause financial and reputational damage, particularly if a data breach leads to penalties from the Information Commissioners Officer or service user negligence claims. Subsequently resource levels would likely be significantly impacted by this level of disruption.	4	3	12	4	3	12	3	3	9	Accessible and secured current backups of data and information needed to recover operation of your essential function(s) following an adverse impact to network and information systems. Up-to-date incident response plan that is grounded in a thorough risk assessment that takes account of network and information systems supporting the operation of your essential function(s) and covers a range of incident scenarios. DSPT compliance indicates that monitoring is in place to detect potential security problems and to track the ongoing effectiveness of protective security measures. NHS SEL ICB Data Protection Impact Assessment process includes capturing details of any new flows as new projects, systems and changes are implemented throughout the ICB. Using and sharing information - use and share information for direct care. B + You lawfully and appropriately use and share information for purpose outside of direct care. SEL ICB has various mechanisms in place to ensure security defences remain effective and to detect cyber security events and incidents which may potentially impact essential functions. Such controls are in place that it limits the access to the designated areas/files/premises or incorporates different levels of accessibility. Access controls are monitored or revoked where necessary, change advisory board process is in place to support review of any proposed system changes proposed by partners or digital solutions providers which are then risk and impact reviewed with necessary mitigation actions identified and / or rollback process agreed as necessary prior to any approvals taking place. Secure design, configuration and management processes have been established. Vulnerability Management Processes are in place to track, manage and prioritise activity. Systems are designed to ensure appropriate levels of resilience and recovery plans are in place. Supply Chain -DSPT compliance indicates that for some organisations processes to risk assess the supply chain are in place and there is an understanding of how suppliers may impact the delivery of our essential functions. Asset Management - - DSPT compliance indicates that assets underpinning essential services are identified, prioritised and have clear ownership assigned. SEL ICB have implemented information assurance policies, processes and procedures, Security Culture - processes are in place to ensure staff have appropriate awareness, knowledge and skills to carry out their roles., Relevant committees oversee information risk. There is clear direction, roles and responsibilities, and decision making with respect to information risk. There is an established assurance plan, including external assurance, and risk management framework that supports our risk management approach. Regular audits are completed to ensure that the appropriate access and controls are in place for user accounts.	The SEL ICB Cyber incident response plan testing exercises are still in planning for 2025-26 and not yet completed,. Inclusion of Digital /ICT risk management approach to be linked into Corporate Risk management framework	2024-25 DSPT/CAF compliance. Cyber Essentials and Cyber Essentials Plus accreditation achieved, Governance structure and policies in place (IG Framework and suite of policies and procedures), DCB1596 - NHS E-mail Security Standard, Cyber Security Operation Centre members - 24/7 Monitoring capabilities.	ICT Disaster recovery /Cyber incident planning exercise to be completed for 2025-26. Risk management Framework - Links of Digital/Cyber security risk approach to be incorporated (in progress with Assurance team), RSM internal audit - Pending outcomes of 2025-26 DSPT/CAF internal audit assurance taking place in March 2026. 2025-26 DSPT/CAF submission due end June 2026.
663	Sarah Osborn / Harriet Agyepong	Annabel Appleby	Operational Planning Performance Targets	Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that the ICB is unable to deliver its 2026/27 operational plan commitments in relation to: •Cancer access and waiting times standards, particularly 62-day treatment standard •Referral to Treatment times (RTT) for elective care •Patient Tracking Lists (PTL) Waiting List) size reductions •Diagnostic activity levels, to support cancer, and elective performance •Community health waiting list reductions •Mental health, particularly Talking Therapies outcomes, access to IPS and reducing the number of out of area placements •General Practice activity and urgent appointments seen on the same day •Urgent and Emergency care - urgent community response Failure to achieve these performance standards may result in delays to diagnosis and treatment, leading to poorer clinical outcomes for patients, increased system pressure, and reputational and regulatory risk for the ICB and partner organisations. Similarly, failure to meet performance/activity targets could result in our inability to achieve medium term planning targets in 3 to 5 years, resulting in delays in transforming our system to achieve better health outcomes for the population of South East London. Non-delivery will also lead to increased assurance from region / national teams which risks shifting the focus of ICB operational and leadership teams.	4	4	16	4	4	16	2	3	6	The 2026/27 operational plan includes agreed system-wide trajectories to improve cancer performance, including the Faster Diagnosis Standard (FDS) and the 62-day treatment standard. System oversight is provided through the South East London Cancer Alliance and SEL ICB Cancer Executive, alongside regular regional performance review meetings. Targeted support arrangements remain in place, NHSE London oversight for RTT, diagnostics and UEC at local acute trusts. Quality and safety are monitored through ongoing surveillance, including harm reviews for patients impacted by delays. SEL has used part of the Strategic Investment Fund (SIF) as a resource to support the trust's in delivering the PTL reductions for the SEL population. However it should be noted that some ICB patients will be waiting at trusts outside SEL. SEL has developed a 5-year strategic commissioning plan which sets a clear course of action for all care pathways across the ICB. All areas have targeted work in the 3 fundamental areas set out in the NHS 10-Year Health Plan focusing on Hospital to Community, Analogue to Digital and Sickness to Prevention. Ith Plan focusing on Hospital to Community, The Strategic Investment Fund will be used across the ICB to support the delivery of our operational plans. The ICB is reliant on providers for delivery of many operational planning targets and works closely with partners to understand progress and support resolution of issues.	Whilst plans are in place, the 26/27 operational targets are stretching and significant work is required across all areas to achieve targets. The ICB has limited influence on how non local trusts deliver on these expectations, as they will be receiving their steer from their host ICB.	Our plans have been signed off by the Board and approved with conditions by NHSE. ICB will continue to be assured through NHSE oversight arrangements in year.	none

Risk ID	Risk Owner	Risk Sponsor	Risk Title	Risk Type	Risk Appetite	Risk Description	Initial Likelihood	Initial Consequence	Initial Rating	Current Likelihood	Current Consequence	Current Rating	Target Likelihood	Target Consequence	Target Rating	Control Summary	Gaps in Control Summary	Assurance in Place	Gaps in Assurance
642	Gita Prasad	Diana Braithwaite	Neurodiversity Assessment Pathways	Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments. This is due to sustained increases in demand, historical backlogs, and limited diagnostic workforce capacity. The delays adversely affect children and adults, increase reliance on private providers through 'Right to Choose', and create financial pressures for the ICB arising from non-contracted activity. Prolonged waits also undermine public confidence and impact delivery of national and local improvement commitments for mental health and neurodevelopmental services.	4	3	12	4	4	16	2	3	6	SEL Commissioning leads on the ASD and ADHD diagnostic pathways are developing an Assessment Hub to support priority screening and support for patients referred for a diagnosis. Locally, Bexley has expanded access to pre-and post-diagnostic support for ADHD and autism to support CYP and families while they wait for a diagnosis and post diagnosis. Oxleas our provider has sub-contracted an independent provider Healios to increase capacity and support with increased demand for autism assessments. Clear targets identified by the ICB with SLaM to reduce 52+week waiting times, SEL -wide neurodevelopmental improvement programme established under the CYP MH and Wellbeing Partnership Board to oversee ASD and ADHD diagnostic pathways, waiting times, and consistency of the core offer across SEL boroughs / places. New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays. Targeted capacity investment including non recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays. Targeted capacity investment including non-recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis. SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. Exploring the opportunity to join arrangements with other boroughs to ensure residents have equity of access to medication and review pathways, Investment into waiting list initiatives, and waiting well initiatives in place.	Workforce capacity across community paediatrics and specialist diagnostic teams remains below demand. Limited ability to influence activity and quality within private 'Right to Choose' pathways. Data completeness and standardisation across providers and places not yet consistent. Funding for additional diagnostic capacity remains non-recurrent and therefore unsustainable without future investment commitments.	This has been raised as a concern across the local partnership and work is underway to consider how we can collectively support CYP based on presenting need rather requiring a formal diagnosis. Pre-and- post diagnostic workshops are available and have been scheduled. Other than ADHD prescribing, CYP can access health services without a diagnosis and waiting times for most health services are within national targets. The pilot assessment hub is due to start in Q3 for 2025/26 and will support with expediting access to assessments for ADHD and autism and alleviate some of the demand on the core commissioned pathway. Oxleas has increased output for autism assessments and is working to streamline their processes to meet increased demand. Oversight through the SEND Improvement Board, Place SEND Partnerships, and the SEL CYP MH and Wellbeing Partnership Board. Monthly contract and performance meetings with kay providers. Regular reporting through ICB performance and finance structures on diagnostic activity, spend and trajectories. Periodic deep dives and review sessions through SEL CYPMH Delivery Group and borough governance. Autism Partnership Board reporting into Learning Disability and Autism Oversight Board and Mental Health Oversight and Co-ordination Board when appropriate. The diagnostic pathways for adults and CYP are being monitored at both Place and SEL levels.	Demand is still outstripping capacity and data indicates demand has significantly increased in recent months. This is likely to impact CYP who would require ADHD medication the most as other treatment pathways can be referred to without a diagnosis. Staff sickness in community paediatrics may further compound capacity concerns and negatively impact waiting times further. Quarter Three performance report on the ASD diagnostic pathways shows adverse performance with the number of ASD assessments completed decreasing from 172 in Q2 to 146 in Q3 and the average waited time of those seen increasing. Therefore overall risk is raised to 16.

Conditional Format

Cell	
Cell	Initial Rating Less than or equal to 3
Cell	Initial Rating Between 4 And 6
Cell	Initial Rating Between 8 And 12
Cell	Initial Rating Greater than or equal to 15
Cell	Current Rating Less than or equal to 3
Cell	Current Rating Between 4 And 6
Cell	Current Rating Between 8 And 12
Cell	Current Rating Greater than or equal to 15
Cell	Target Rating Less than or equal to 3
Cell	Target Rating Between 4 And 6
Cell	Target Rating Between 8 And 12
Cell	Target Rating Greater than or equal to 15

Risk ID	Risk Owner	Risk Sponsor	Risk Title	Risk Type	Risk Appetite	Risk Description	Initial Likelihood	Initial Consequence	Initial Rating	Current Likelihood	Current Consequence	Current Rating	Target Likelihood	Target Consequence	Target Rating	Control Summary	Gaps in Control Summary	Assurance in Place	Gaps in Assurance
509	Sean Rafferty	James Postgate	Neurodevelopmental diagnostic pathways (autism and ADHD) - CYP and adults	Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments. This is due to sustained increases in demand, historical backlogs, and limited diagnostic workforce capacity. The delays adversely affect children and adults, increase reliance on private providers through 'Right to Choose', and create financial pressures for the ICB arising from non-contracted activity. Prolonged waits also undermine public confidence and impact delivery of national and local improvement commitments for mental health and neurodevelopmental services.	3	3	9	4	4	16	2	2	4	CYP ASD/ADHD Project established led by a project manager with buy-in across Oxleas, Bromley Healthcare and Bromley Council. Clear targets identified by the ICB with SLaM to reduce 52+week waiting times, Targeted capacity investment including non-recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis. SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. Exploring the opportunity to join arrangements with other boroughs to ensure residents have equity of access to medication and review pathways	Limited ability to influence activity and quality within private 'Right to Choose' pathways. Data completeness and standardisation across providers and places not yet consistent.	Project Manager in place, Action plan developed and submitted to reduce waiting times, Governance through the CYP Mental Health and Wellbeing Partnership Board, Oversight through the SEND Improvement Board, Place SEND Partnerships, and the SEL CYP MH and Wellbeing Partnership Board. Monthly contract and performance meetings with key providers. Regular reporting through ICB performance and finance structures on diagnostic activity,spend and trajectories, Periodic deep dives and review sessions through SEL CYPMH Delivery Group and borough governance. Autism Partnership Board reporting into Learning Disability and Autism Oversight Board and Mental Health Oversight and Co-ordination Board when appropriate.	Further work between Bromley Healthcare and Oxleas CAMHS on joint decision making

Conditional Format

Cell	
Cell	Initial Rating Less than or equal to 3
Cell	Initial Rating Between 4 And 6
Cell	Initial Rating Between 8 And 12
Cell	Initial Rating Greater than or equal to 15
Cell	Current Rating Less than or equalto 3
Cell	Current Rating Between 4 And 6
Cell	Current Rating Between 8 And 12
Cell	Current Rating Greater than or equalto 15
Cell	Target Rating Less than or equal to 3
Cell	Target Rating Between 4 And 6
Cell	Target Rating Between 8 And 12
Cell	Target Rating Greater than or equal to 15

Risk ID	Risk Owner	Risk Sponsor	Risk Title	Risk Type	Risk Appetite	Risk Description	Initial Likelihood	Initial Consequence	Initial Rating	Current Likelihood	Current Consequence	Current Rating	Target Likelihood	Target Consequence	Target Rating	Control Summary	Gaps in Control Summary	Assurance in Place	Gaps in Assurance
635	Jenny Lamprell, Rana Amin, Roneeta Campbell	Lisa Wilson, David Borland	Neurodevelopmental diagnostic pathways (autism and ADHD) - CYP and adults	Strategic commitments and delivery priorities Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments. This is due to sustained increases in demand, historical backlogs, and limited diagnostic workforce capacity. The delays adversely affect children and adults, increase reliance on private providers through 'Right to Choose', and create financial pressures for the ICB arising from non-contracted activity. Prolonged waits also undermine public confidence and impact delivery of national and local improvement commitments for mental health and neurodevelopmental services.	5	5	25	4	4	16	2	3	6	Greenwich is just formalising the Autism Strategy, and as part of the newly established integrated commissioning team, a performance tracker on the waiting times and financial impact will be created. Data analytics will need to be supported by Ockleas as the local provider for ADHD/ASD diagnosis to track the impact of these delays, as well as the financial impact. SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories, SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories, Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis, New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays, SEL -wide neurodevelopmental improvement programme established to oversee ASD and ADHD diagnostic pathways, waiting times, and consistency of the core offer across SEL boroughs / places	No data for ADHD (waiting list or post-diagnostic). Autism, no register required within QOF ,akin to LD or SMI,and so very reliant on each practice to code accurately, hence the clinical variation. Whilst national RTC is in place, no control over volume and diagnosis asked for. Cannot control GP-shared care with private contractors. Workforce capacity across community specialist diagnostic teams remains below demand. Limited ability to influence activity and quality within private 'Right to Choose' pathways, Data completeness and standardisation across providers and places not yet consistent, Workforce capacity across community paediatrics and specialist diagnostic teams remains below demand.	Oversight through the SEND Improvement Board, Place SEND Partnerships, and the SEL CYP MH and Wellbeing Partnership Board. Monthly contract and performance meetings with key providers. Regular reporting through ICB performance and finance structures on diagnostic activity, spend and trajectories. Periodic deep dives and review sessions through SEL CYPMH Delivery Group and borough governance. Autism Partnership Board reporting into Learning Disability and Autism Oversight Board and Mental Health Oversight and Co-ordination Board when appropriate	Inconsistent and incomplete BI reporting across places pending full implementation of the SEL-wide dashboard, Limited independent verification of data accuracy and trajectory modelling, Assurance over 'Right to Choose' activity and spend still under development.

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Risk ID	Risk Owner	Risk Sponsor	Risk Title	Risk Type	Risk Appetite	Risk Description	Initial Likelihood	Initial Consequence	Initial Rating	Current Likelihood	Current Consequence	Current Rating	Target Likelihood	Target Consequence	Target Rating	Control Summary	Gaps in Control Summary	Assurance in Place	Gaps in Assurance
129	Laura M Griffin	Integrated Director for Children and Young People	Diagnostic waiting times for neurodiversity assessments - children and young people	Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that waiting time targets for children and young people waiting for an autism or ADHD assessment is unacceptably long. This is caused by high demand and recovery from Covid-19. This impact's on the ICB's ability to ensure waiting time targets are met and could affect the organisations reputation. This could also have an adverse affect on CYP who are waiting for a diagnosis.	4	2	8	4	4	16	2	2	4	Transformation funding proposal from Evelina London is going through contract management meetings route in order to build paediatric capacity to manage this. The additional capacity in place is overseen by Evelina Contract Management meeting - any issues escalated and managed there. Initial trajectory of referrals has been now seen as incorrect as referrals have continued to increase, a number of mitigatory steps in place now to re-define trajectory and reduction of waiting list. Total number of CYP on this list across L&S exceeds 1000. This will be monitored monthly at place through 23/24. Saturday clinics are also now in place as well as additional trained staff. Additional consultants in-post to increase diagnostic capacity.	Covid impact on finances means that transformation schemes will not all be fully funded however proposal to continue this during 23/23 has been submitted. Covid impact on the diagnostic itself meant that there were inherent delays. face to face appointments are necessary. This is an international issue.	Bi-monthly contract monitoring meetings with Evelina. Monthly ADHD meetings with Evelina and SLAM. Monthly reporting of position now coming direct from Evelina to Place, Council also monitoring this indicator. Regular meetings with local management team to develop and standardise EPIC report, Initiative to address longest wait times tracked in local performance, Ongoing oversight of diagnostic performance by the Lambeth Together Assurance Group.	No Gaps in assurance at this time.

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644	Paul Creech LBL Commissioning Lead	Ceri Jacob, Place Executive Lead	Neurodevelopmental diagnostic pathways (autism and ADHD) - CYP and Adults	Strategic commitments and delivery priorities: Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments. This is due to sustained increases in demand, historical backlogs, and limited diagnostic workforce capacity. The delays adversely affect children and adults, increase reliance on private providers through 'Right to Choose', and create financial pressures for the ICB arising from non-contracted activity. Prolonged waits also undermine public confidence and impact delivery of national and local improvement commitments for mental health and neurodevelopmental services.	4	4	16	4	4	16	4	3	12	SEL-wide neurodevelopmental improvement programme established under the CYP MH and Wellbeing Partnership Board to oversee ASD and ADHD diagnostic pathways, waiting times, and consistency of the core offer across SEL boroughs / places. New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays. Targeted capacity investment including non-recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis. SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. SEL-wide neurodevelopmental improvement programme established under the CYP MH and Wellbeing Partnership Board to oversee ASD and ADHD diagnostic pathways, waiting times, and consistency of the core offer across SEL boroughs / places. New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays. Targeted capacity investment including non-recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis. SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. Exploring the opportunity to join arrangements with other boroughs to ensure residents have equity of access to medication and review pathways. SLAM is working with community and voluntary organisations to develop a dedicated ADHD pathway, with potential to support autism services. Working with mental health school teams to support the identification of children for autism assessments.	Workforce capacity across community paediatrics and specialist diagnostic teams remains below demand. Limited ability to influence activity and quality within private 'Right to Choose' pathways. Data completeness and standardisation across providers and places not yet consistent. Funding for additional diagnostic capacity remains non-recurrent and therefore unsustainable without future investment commitments. Delivery of Autism wait times is being monitored by NHSE as part of the Trusts delivery plan and to bring down wait times to within 18 weeks. Transformation work on the autism referral and assessment pathway to reduce waiting times and diversify the workforce to increase capacity.	Oversight through the SEND Improvement Board, Place SEND Partnerships, and the SEL CYP MH and Wellbeing Partnership Board. Monthly contract and performance meetings with key providers. Regular reporting through ICB performance and finance structures on diagnostic activity, spend and trajectories. Periodic deep dives and review sessions through SEL CYPMH Delivery Group and borough governance.	Inconsistent and incomplete BI reporting across places pending full implementation of the SEL-wide dashboard. ADHD limited independent verification of data accuracy and trajectory modelling. Assurance over 'Right to Choose' activity and spend still under development.

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520	Russell Jones - Assistant Director, Integrated Commissioning	Claire Bolgard, Acting Director of Integrated Commissioning	Neurodevelopmental diagnostic pathways (autism and ADHD) - CYP and adults	Strategic commitments and delivery priorities. Implementation of ICB strategic commitments, approved plans, and delivery priorities	10 - 12	There is a risk that residents experience excessively prolonged waiting times for autism and ADHD diagnostic assessments. This is due to sustained increases in demand, historical backlogs, and limited diagnostic workforce capacity. The delays adversely affect children and adults, increase reliance on private providers through 'Right to Choose', and create financial pressures for the ICB arising from non-contracted activity. Prolonged waits also undermine public confidence and impact delivery of national and local improvement commitments for mental health and neurodevelopmental services.	4	2	8	4	4	16	2	2	4	Additional administrative support to bring additional capacity to wait list management, regular monitoring meetings, clear targets identified by the ICB with SLaM to reduce 52+week waiting times. SEL-wide neurodevelopmental improvement programme established under the CYP MH and Wellbeing Partnership Board to oversee ASD and ADHD diagnostic pathways, waiting times, and consistency of the core offer across SEL boroughs / places. New integrated diagnostic pathway from April 2025 enabling movement between ADHD and Autism assessments, reducing duplication and re-referral delays. Targeted capacity investment including non-recurrent and recurrent funding to providers to expand assessment capacity, weekend clinics, and workforce recruitment initiatives. Waiting well and early support offers publicised through local offers and all-age autism services to provide information, advice and support before diagnosis, SEND Improvement Board oversight with joint leadership from local authorities and Directors of Children's Services to drive delivery of local improvement plans and monitor performance trajectories. Exploring the opportunity to join arrangements with other boroughs to ensure Southwark residents have equity of access to medication and review pathways, SEL Referral Management centre implementation	Workforce capacity across community paediatrics and specialist diagnostic teams remains below demand. Limited ability to influence activity and quality within private 'Right to Choose' pathways. Data completeness and standardisation across providers and places not yet consistent. Funding for additional diagnostic capacity remains non-recurrent and therefore unsustainable without future investment commitments.	Oversight through the SEND Improvement Board, Place SEND Partnerships, and the SEL CYP MH and Wellbeing Partnership Board. Monthly contract and performance meetings with key providers. Regular reporting through ICB performance and finance structures on diagnostic activity, spend and trajectories. Periodic deep dives and review sessions through SEL CYPMH Delivery Group and borough governance. Autism Partnership Board reporting into Learning Disability and Autism Oversight Board and Mental Health Oversight and Co-ordination Board when appropriate.	Inconsistent and incomplete BI reporting across places pending full implementation of the SEL-wide dashboard. Limited independent verification of data accuracy and trajectory modelling. Assurance over 'Right to Choose' activity and spend still under development.

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South East London Integrated Care System

NHS South East London

ICB Board Meeting in Public

Title	**Overall Committees Report**						
Meeting date	22 July 2026		Agenda item Number	8	Paper Enclosure Ref	G	
Author	Simon Beard, Associate Director for Corporate Operations						
Executive lead	Tosca Fairchild (Chief of Staff)						
Paper is for:	Update	x	Discussion		Decision	X	
Purpose of paper	The purpose of the paper is to highlight to the Board any DECISIONS referred to the Board from ICB Committees, to provide INFORMATION on any decisions made under derogation by those committees, and to provide INFORMATION on activity of the committee meetings.						
Summary of main points	The Overall Committees paper provides an overview to the Board members of the activity and decision making that has taken place at the ICB committees which report directly to the Board in the period since the last Board meeting held in public. In particular the Board is asked to note: • Decisions referred to the Board for approval, detailed in section 4. • Decisions made by committees, under their own delegated authority. The Board is asked to consider the decisions referred for approval and to note the other activity that has taken place during the period.						
Potential conflicts of Interest	Where conflicts have been identified with any items discussed at a committee, action has been taken to mitigate the conflict in line with the ICBs Standards of Business Conduct policy.						
Relevant to these boroughs	Bexley	X	Bromley	x	Lewisham	x	
	Greenwich	x	Lambeth	x	Southwark	x	
Equalities Impact	No equality impacts identified						
Financial Impact	Any financial impacts are identified in the relevant papers.						
Public Patient Engagement	This paper is being presented to a Board meeting held in public for the purposes of transparency.						
Committee engagement	Discussions at other committees are detailed in the attached paper.						
Recommendation	The Board is asked to: • Approve the decisions recommended by its committees • Note the committee decisions and committee activities detailed.						

Overall Report of the ICB Committees

ICB Board 22 July 2026

1. Introduction

1.1 The purpose of this report is to provide a summary of the activity that has taken place within the Committees that report directly to the Board since the last meeting on 8 April 2026. In addition the ICS benefits from two provider collaboratives and whilst no formal delegation has been made to them from the ICB this paper provides an update on their key activities over this same period.

1.2 The report highlights:

- Decisions recommended to the Board from Committees, in line with the ICBs Scheme of Reservation and Delegation.
- A summary of items discussed at the Committees during the period being reported.
- Report of activities taking place in the Local Care Partnerships of South East London.
- Report of activities taking place in the South East London provider collaboratives and community services provider network.

2. Summary of Meetings

2.1 ICB Committees

	Committees								
	Integrated Performance Committee	**Quality and Safeguarding Committee**	**Audit & Risk Committee**	**Remuneration Committee**	**Greenwich Charitable Funds Committee**	**Clinical and Care Professional Committee**	**People Committee**	**Digital Committee**	**Executive Committee**
Meeting date	-	20 April 2026	23 April 2026	7 May 2026	1 June 2026	-	23 March 2026	12 May 2026	1 April 2026
	-	-	16 June 2026	15 May 2026	-	-	18 June 2026	-	15 April 2026
	-	-	9 July 2026	-	-	-	-	-	29 April 2026

	Transition Committee
Meeting date	3 March 2026
	28 April 2026
	29 May 2026
	29 June 2026

Joint executive/ SMT		
7 May 2026	28 May 2026	18 June 2026
14 May 2026	4 June 2026	25 June 2026
21 May 2026	11 June 2026	2 July 2026

	Local Care Partnerships					
	Bexley	**Bromley**	**Greenwich**	**Lambeth**	**Lewisham**	**Southwark**
Meeting date	26 March 2026	26 March 2026	29 April 2026	5 March 2026	26 March 2026	26 March 2026
	28 May 2026	18 June 2026	-	11 June 2026	28 May 2026	28 May 2026

3. Summary of the Principal Role of ICB Committees

Committee	Principal role of the committee	Chair
Integrated Performance Committee	Oversight and assurance of delivery of the ICS four aims through the objectives and deliverables set out in the range of ICP and ICB strategic plans. The Committee will monitor how delivery across different parts of the system contributes to the ICS's overall strategic work and direction, seeking to ensure efforts are aligned across the system.	Paul Najsarek, Non-Executive Member
Quality and Safeguarding Committee	Acts as a focal point for the collective oversight and strategic direction of safeguarding and quality matters across SEL Integrated Care System. Responsible for overseeing the delivery of high-quality care, ensuring compliance with safeguarding legislation, promoting the safety and wellbeing of vulnerable populations and fostering continuous improvement in health services. This is aimed at supporting improved health outcomes, reduced inequalities and enhanced patient experience.	Anu Singh, Non-Executive Member
Audit & Risk Committee	Responsible for delegated approval of annual accounts, providing an objective view of the ICB's compliance with statutory responsibilities, arranging appropriate audit, and oversight / assurance on the adequacy of governance, risk management and internal control processes across the ICB.	Peter Matthew, Non-Executive Member
Greenwich Charitable Funds Committee	Responsible for discharging its duties as a corporate trustee. Referred to as the Greenwich Healthier Communities Fund.	Peter Matthew, Non-Executive Member
Clinical and Care Professional Committee	Responsible for bringing together clinicians, care professionals and south east London residents to ensure the ICB has robust care, patient and public engagement, population health management, and leadership in place to shape and that the ICB's plans are demonstrably influenced by the outputs of its engagement work.	Dr Toby Garrood, ICB Medical Director
People Committee	Responsible for the design, development and delivery of plans related to the health and care workforce in South East London. This includes meeting any national targets and ensuring sufficient and consistent strategies across the ICS for equality, diversity and inclusion and staff health and wellbeing.	Dr Ify Okocha, Partner Member

Digital Committee	The Digital Committee is constituted of members from across the SEL Integrated Care System partnership and provides leadership to the development of strategic priorities for digital and analytics, including ensuring digital capabilities are utilised to reduce inequalities.	Dr Toby Garrood, ICB Medical Director
Executive Committee	The Committee provides a platform for the executive directors of NHS South East London Integrated Care Board (SEL ICB) to discuss key issues relating to the strategy, operational delivery and performance of SEL ICB, and related Integrated Care System or wider issues upon which the executive team should be briefed or develop a proposed approach. From 1 May 2026, the Executive Committee ceased to meet, replaced by a combined SEL/SWL senior management team meeting held weekly.	Andrew Bland, ICB Chief Executive
Transition Committee	The purpose of the Committee is to provide assurance and oversight of the ICB SEL Change Programme for the ICB Board, ensuring a safe and coherent transition, managing local risks, tracking progress and overseeing the development of organisational design and implementation of the change process, including the transfer of functions to providers over time.	Georgina Fekete, Non-Executive Member
Local Care Partnerships	Responsible for convening local system partners to develop plans to meet the needs of the local population, reduce inequalities and optimise integration opportunities. The ICB will delegate responsibility for the delivery of specified out of hospital care objectives and outcomes, including the management of the associated budget. A representative from each LCP will be a member of the ICB.	Dr Sid Deshmukh (Bexley) Dr Andrew Parson & Cllr Colin Smith (co-chairs, Bromley) Iain Dimond (Greenwich) Dr Di Aitken & Cllr Nanda Manley-Browne (co-chairs, Lambeth) Vanessa Smith & Fiona Derbyshire (co-chairs, Lewisham) Dr Nancy Kuchemann & Cllr Evelyn Akoto (co-chairs, Southwark)

4. Recommendations to the Board for Decision / Approval

4.1 The ICB Board is asked to approve the revised Section 75 agreement with London Borough of Lewisham (see appendix A), as required in the Scheme of Reservation and Delegation.

4.2 The ICB Board is asked to approve the revised Terms of Reference to the Lambeth Together Local Care Partnership Board, which have been amended to reflect a revised frequency for meetings held in public from every two months to quarterly. A revised terms of reference document is attached as appendix B.

4.3 The ICB Board is asked to approve the revised Terms of Reference for the Greenwich Charitable Funds Committee, drafted to reflect the changes to the ICBs organisational structure and designed to enable management of the fund at Place level. A revised terms of reference document is attached as appendix C.

5. Decisions made by Committees or Sub-Committees Under Delegation

5.1 Below is a summary of decisions taken by committees under delegation from the Board.

No.	Committee name	Meeting date	Items for Board to note
1.	Executive Committee	15 April 2026	• The Committee approved a reviewed standards of business conduct policy. • The committee received an update on the development of an architecture to support population health management, and approved use of its strategic investment fund to further develop the work.
2.	Audit & Risk Committee	23 April 2026	• The committee approved the submission of the draft annual report and accounts for 2025/26 to NHS England.
3.	Executive Committee	29 April 2026	• The committee approved a model to achieve consistent delivery of mental health support to all schools in south east London, through an expansion of mental health support teams
4.	Remuneration Committee	7 May 2026	• The Remuneration Committee approved the list of proposed redundancies under the second round of Voluntary Redundancy offered to ICB staff.
5.	Remuneration Committee	15 May 2026	• The Remuneration Committee approved the remuneration for the joint posts of Chief Finance & Compliance Officer and Chief Nurse & Quality Officer. As these are joint posts, similar approval was discussed by the Remuneration Committee of NHS South West London ICB.
6.	Remuneration Committee [by email]	22 May 2026	• The Remuneration Committee approved by email correspondence the remuneration for the joint post of Chief Commissioning Officer. As this is a joint post, similar approval was discussed by the Remuneration Committee of NHS South West London ICB.
7.	Remuneration Committee [by email]	12 June 2026	• The Remuneration Committee approved by email correspondence the remuneration for the joint post of Medical Director. As this is a joint post, similar

			approval was discussed by the Remuneration Committee of NHS South West London ICB.
8.	Audit & Risk Committee	16 June 2026	• Following feedback from internal and external auditors, the Committee members approved submission of the final audited annual report and accounts and associated documents to NHS England.
9.	Audit & Risk Committee	9 July 2026	• Approval of Risk Management Framework (pending agreement of quorate membership)

6. Agenda Items of Note

6.1 Below is a summary of other significant actions and items of note for Board information.

No.	Committee name	Meeting date	Items discussed
1.	Transition Committee	3 March 2026	• The committee were updated on progress with the planned transfer of various functions from the ICB to NHS providers in line with the ICB blueprint. • The committee considered its changing role following the agreement of SWL and SEL ICBs to move to a clustered arrangement, and how it could deliver its remit of overseeing the transition process.
2.	People Committee	23 March 2026	• The meeting primarily focused on future arrangements for the Committee and a shorter linked item on a proposal for a SEL ICS Staff Partnership Forum. • With regards to future committee arrangements, members received: ○ a report summarising the engagement from the January forum to co-design adjustments ○ a refreshed set of principles, goals and construct for the People Committee ○ key changes to the SEL People Programme Governance ○ discussion prompts which were covered in the session. • Members endorsed the report adding that it reflects the richness of the discussions. There was collective approval of the proposed direction of travel for the committee with an acknowledgement that the ICB Governance will inform further checks and review in due course. • It was agreed that a small number of issues will require continued attention in implementation of the new arrangements; seeking to remain agile overall, ensuring system workforce risks kept at pace with timely interventions and further developing alignment with social care and boroughs. • Overall, the People Committee approved changes which mean a move to quarterly forum with strategic workforce topics, deep dives into 'wicked issues' and spotlight sessions. The next meeting will be in June, focused on Equality, Diversity and Inclusion. • A proposal to establish a SEL ICS Partnership Forum was discussed, noting most London systems have a forum which reports into the London Region SPF. It was

			noted that the forum is not designed to duplicate, but to come together on issues affecting members, noting the significant workforce implications of key policies and further developing our partnership working between health and care organisations, trade unions and leadership. The Committee approved the plan to deliver a refreshed TOR and test a 6 monthly forum.
3.	Executive Committee	1 April 2026	• The Committee received from the CEO and executives • The Committee heard an update on bids submitted for Innovate UK funding for innovative approaches proposed for the Obesity management pathway including community pharmacy based weight management and lifestyle support. • The committee discussed arrangements for the senior management team of both SEL ICB and SWL ICB to meet as a joint SMT to conduct business going forwards. • The committee considered and discussed the latest performance report. • The committee considered and discussed the latest financial report.
4.	Executive Committee	15 April 2026	• The Committee received from the CEO and executives • The committee heard an update on the ICBs response to a letter from NHS England asking for details of its future strategic commissioning approach and proposals on key priority areas. • The committee noted reports on the public sector equalities duty, equality objectives and Equality delivery system 2022 which helped the ICB meet its statutory obligations. • The committee received a paper on the prevention of violence in NHS workplace, noting the complex issue with impact on staff, costs, outcomes for patients and productivity and the efforts made so far to address the issue working across the system. • The committee received a report summarising the ICBs engagement work in partnership with people and communities.
5.	Quality & Safeguarding Committee	20 April 2026	• Committee members received the patient experience report, noting a 19.5% increase in caseload in Q3 of 2025/26, with further increases in Q4. High volumes of primary care complaints were noted, with 55% of ICB-led complaints relating to CHC. • A report on Learning Disability, Autism and SEND was delivered, noting achievement of targets and ongoing work around ADHD/ASD assessment pathways and the

			children and young people neurodiversity assessment hub. Oliver McGowan training had now transferred to providers. • The Local Maternity and Neonatal System report highlighted some ongoing work on the Maternity Incentive Scheme compliance, an increase in London-wide capping of out-of-area maternity referrals, and ongoing discussions on transition of maternity oversight from the ICB to NHSE regional office. • The Infection Prevention and Control report focussed on Meningitis B and the ICB support provided to the response. • Medicines Optimisation discussed the key national patient safety highlights around an overuse of short-acting beta agonists in asthma, and key decisions and guideline approvals from the Integrated Medicines Optimisation Committee. • The Quality and Safety report covered an update on a Data Transfer Service error incident, never event reporting, local CQC outcomes, and system learning from audits. The committee discussed the development of a framework for sharing learning across the system. • The Committee members received a report on Safeguarding training compliance, safeguarding supervision, outcomes of an NHSE assurance programme, safeguarding workplan updates and themes from safeguarding statutory reviews. • The quality, safety and safeguarding risk register was discussed.
6.	Audit & Risk Committee	23 April 2026	• Committee members received a report from Grant Thornton on progress against the external audit plan, noting progress to timetable. A positive outcome from the work on the Mental Health Investment Standard 2024/25 was also reported. • Internal audit provided a progress report against audit plan, noting a partial assurance opinion on a deep dive into the CHC appeals process, and reasonable assurance for the financial controls and Board Assurance Framework audits. Concern about the partial assurance outcome was expressed and further actions requested to seek assurance. Ongoing discussions concerning the 2026/27 audit programme would be required. RSM also confirmed a substantive assurance outcome in the draft Head of Internal Audit Opinion report and updated on progress reported against agreed management actions.

			• The ICBs anti-fraud advisors reported good progress against the 2025/26 workplan and sought approval of the 2026/27 plan. • The committee approved submission of the draft annual report and accounts to NHS England. • The Board Assurance Framework and latest position for the corporate risk register was reviewed and noted. • The Chief Finance Officer confirmed no debt write offs, losses or special payments, the approval of three tender waivers, and highlighted the sign off in the period of a Grant Agreement.
7.	Transition Committee	28 April 2026	• The committee received an update of the latest developments of the programme and reviewed risks. • The committee considered the draft proposal to retain a small strategic estates programme but to transfer some its function to be delivered by local providers, with clear reporting lines to the ICB. • The committee heard some of the measures put in place to ensure that the change process was non-discriminatory for staff and the metrics to measure its impact with respect to equalities. • The committee received an update on work to prepare for the programme of organisational development which would be required to support the ICB and its staff as it moved to its new strategic commissioning role.
8.	Executive Committee	29 April 2026	• The committee considered and discussed the latest financial report. • The committee received detailed information on the corporate objective related to improving cancer screening, noting variable performance across Breast, Bowel and Cervical screening programmes but progress in addressing key areas of inequalities.
9.	Remuneration Committee	7 May 2026	• The Remuneration Committee approved the list of proposed redundancies under the second round of Voluntary Redundancy offered to ICB staff.
10.	Joint Executive SMT meeting	7 May 2026	• Discussed the new SMT arrangements and agreed that it should be referred to as the executive.

			• Noted changes to regional executive meetings and the national intention of each region having an Office of Pan-ICB commissioning. • Discussed an update on Hospice care in SEL and SWL noting concerns about financial sustainability across providers and mitigating actions. • Considered a national planning letter inviting ICBs to set out their local approaches to using payor functions. • Were updated on the forthcoming new National Oversight Framework for 2026/27 • Received update on the ICB change programme including updates to structures in response to consultation feedback and progress with voluntary redundancy.
11.	Digital Committee	12 May 2026	• The Committee received a presentation on the digital estates portal, aiming to capture key systems and data deployed in each NHS organisation. • The Committee discussed the need to extend a service management arrangement with LGT to manage the London Care Record on behalf of SEL organisations. • The members considered the concept of Digital Communities of Practice and agreed further work to develop a proposal. • An update was provided on the work of the frontline productivity programme, considering areas such as ambient voice technology and digital medicine. • A paper was presented on solutions for the replacement of the Urgent and Emergency Care RAIDR programme, which is used by the System Co-ordination Centre to maintain oversight of provider activity and capacity. The committee endorsed the recommendation to extend the use of the current RAIDR system.
12.	Joint Executive SMT meeting	14 May 2026	• The group noted the recent Local elections and discussed how best to support and engage with new councillors and administrations on the work of the ICB. • Noted the process to develop referral guidelines for primary care hip and knee pain • Approved an approach in relation to the Richmond Obstructive Sleep Apnoea service, noting completion of an EQIA and programme of engagement with patients. • Discussed branding and communications across the ICB and considered approaches which provided the most clarity and opportunity to make the most of resources. • Discussed the issues in relation to SEL neurodevelopmental services and agreed to receive a proposal.

			• Noted an expectation on ICBs to participate in a national review of SEND and notd progress already underway. • Noted a proposed change in the employment of clinical leads in south east London intended to signal the value placed on these leads and clarify employment rights and responsibilities. • received an update on the management response following the ICB change consultation being finalised.
13.	Remuneration Committee	15 May 2026	• The Remuneration Committee approved the remuneration for the joint posts of Chief Finance & Compliance Officer and Chief Nurse & Quality Officer.
14.	Joint Executive SMT meeting	21 May 2026	• Confirmed approval of SEL Speciality Primary Care Guidelines for orthopaedic hip and knee pain • noted national and London context, discussing alignment across ICBs • Received an update on change programme with good progress on role matching and discussion on EQIA • Agreed a preference for joint work to engage with NHS England on performance whilst retaining clear separation of organisational positions. • Agreed a proposal to proceed with procuremnt of Mental Health Support Teams in SWL schools
15.	Joint Executive SMT meeting	28 May 2026	• Received an update on support to hospices in SWL • Considered a proposed response to an NHSE request for assurance on meeting £19ph • Considered the SWL corporate risk register, Performance report, Quality report an Finance report. • Discussed the SWL submission on neighbourhood health centres
16.	Transition Committee	29 May 2026	• The committee received an update of the latest developments of the programme and reviewed risks. • The committee considered a proposal to transfer the management of the estates function to two local trusts and recommended that it proceed to COSOP stage.

17.	Greenwich Charitable Funds Committee	1 June 2026	• The Committee members considered a proposal to revise the Committees terms of reference, which would be submitted to the ICB Board as part of a separate paper containing a wider package of governance reform as part of the ICB reshaping. • The annual accounts for 2025/26 were presented, with the committee accepting the accounts as accurate and agreeing for submission to the CFO for sign off. • The members received an update on the funding provided in 2025/26, noting £1.4m of funds were granted to support VCSE organisations to build capacity develop partnerships, improve skills base, and improve wellbeing in the local community. £1.25m of grants was anticipated to be released in 2026/27. • The committee received a proposal to update its delivery strand funds, with three options for consideration. Option A: Pooled budget to provide a neighbourhood community chest, which would allow for larger grants to be pooled with other organisations and S75 agreement, was supported.
18.	Joint Executive SMT meeting	4 June 2026	• Received an update on the SWL ICB change programme, and discussed the recently released management response and start of role matching. • Received updates on both SWL and SEL ICB on the expectation that ICBs should participate in the national SEND maturity assessment. •
19.	Joint Executive SMT meeting	11 June 2026	• Approved the submission of a expression of interest for SWL in relation to 'steps to safety' a programme to support violence reduction against women and girls. • Received an update on Community Diagnostic Centres and efforts underway to boost capacity. • Received an update on developing proposals for hosting of neighbourhood teams in Lewisham • Received an update on progress against agreed use of the Strategic investment Fund in SEL • received and endorsed the SEL Board Assurance Framework

20.	Audit & Risk Committee	16 June 2026	• Committee members received the annual report from the ICBs internal auditors, and the external audit report and audit recommendations from the external audit of the ICB annual report and accounts for the year ending 31 March 2026.
21.	Joint Executive SMT meeting	18 June 2026	• Considered a position regarding SEL Clinical Professional Care Leads (CCPLs), and agreeing an approach to the treatment of these roles as part of the ICB change programme. • Received an update on the ICB change programme, noting staff concerns and impact on morale and the importance of clear and consistent communication. • Considered an CEO update on local issues and staff moves. • The group considered a procurement decision relating to ADHD services. Approval was given to award a contract to the provider that met the required criteria. It was agreed that the unsuccessful applicant would receive feedback and be invited to reapply in due course.
22.	People Committee	18 June 2026	• The People Committee held a focussed session on Equality, Diversity and Inclusion, with external speakers.
23.	Joint Executive SMT meeting	25 June 2026	• Updates from regional and national meetings were discussed including introdution of a Single Patient Record and proposed NHS Online model. • Cost modelling for updated NICE guidance for type 2 diabetes was considered, with discussion recognising the clear benefits but also the cost to the system, and the need for adequate equality impact assessment. • Discussed a paper on the urology service at Queen Elizabeth's Hospital in Woolwich outlining extensive system wide enagagement on clinical, workforce and performance pressures associated with the service and agreement of mitigating actions. • Discussed a proposed increase in the scope of delegation to the South London Mental Health and Community Partnership Complex Care Programme • Noted the Finance Report, Corporate Report and Change Managment update for SWL ICB.
24.	Transition Committee	29 June 2026	• The committee were advised of the intended future joint configuration and the chair acknowledged the contribution of existing members.

			• The committee received an update on the programme so far noting that the functional transfers were on track, staff had been issued letters following the matching process and a provider had been identified for some CSU services. The committee discussed the considerable number of appeals by staff. • The committee received a report on the impact of VR2 on the demographic of staff noting generally marginal changes. • The committee received a paper proposing the transfer of neighbourhood development functions to a host, noting the proposal would be developed further. • The committee reflected on the operation of the committee so far.
25.	Joint Executive SMT meeting	2 July 2026	• A proposed piece of work on the ICBs' operating model was announced, which would inform wider organisational development work. • An indicative view, based on availabel information and guidance, was presented on how the SEL and SWL ICBs might perform relative to the new NHS Oversigth Framework for ICB performance. • A proposal for diagnostic work on the issue of high and increasing demand for neurodevelopmental services and associated costs was supported. • Agreement was secured to offer nationally available online training sessions on strategic commissioning to staff in both ICBs. • An update on work on user involvement and community engagement in the context of the clustered ICBs, including use of innovative tools to provide a database of engagement feedback to inform ICB decision making.
26.	Audit & Risk Committee	9 July 2026	• This was a handover meeting between the outgoing and incoming Audit and Risk Committee chairs. • Committee members discussed arrangements across South East London and South West London and agreed more discussion to identify audit efficiencies would be helpful. • Internal audit presented a progress update paper, noting some discussions were ongoing around audit dates and content for the 2026/27 workplan, and presented a summary of management follow up actions.

			• The anti-crime report was received from TIAA, noting progress was as expected. • The deputy Chief Nurse for the ICB attended to discuss actions undertaken with CHC following a review of process. • A paper on revised governance arrangements as the SEL/SWL clustering arrangements progressed was presented. • The members considered the latest Board Assurance Framework and corporate risk registers following a full review for the start of the new financial year. • The refreshed Risk Management Framework and review of risk appetite was discussed. • The CFO reported on tender waivers received and confirmed no debt write-offs, special payments or losses, or grant agreements had been approved in the quarter.

Appendix 1

Bexley Local Care Partnership – Bexley Health and Wellbeing Partnership

1. Recommendations to the Board for Decision / Approval

1.1 No items are referred to the Board for decision or approval in this period.

2. Decisions made by Bexley Health and Wellbeing Partnership Under Delegation

2.1 Below is a summary of decisions taken by the Bexley Health and Wellbeing Partnership under delegation from the Board:

No.	Meeting date	Agenda item	Items discussed
1.	28 May 2026	Better Care Fund Assurance Return 2026/27	The Bexley Wellbeing Partnership Committee received a report on the Better Care Fund (BCF) Assurance Return 2026/27, which sets out how pooled funding will be used to support integrated and preventative care across the borough. The report highlighted continued focus on maintaining stability in core services, alongside progress towards neighbourhood-based care and alignment with local and national priorities. **The Bexley Wellbeing Partnership Committee:** **Endorsed** the Better Care Fund Assurance Return 2026/27 and authorised delegated approval of any required amendments to secure national approval.

3. Agenda Items of Note

3.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items discussed
1.	26 March 2026	Performance Assurance Report	The Bexley Wellbeing Partnership Committee received a performance update, noting strong performance across dementia diagnosis, talking therapies, continuing healthcare, learning disabilities and autism health checks, and cancer screening. Areas below trajectory included adult talking therapies access, childhood immunisations, hypertension management and flu vaccination rates. Members noted data lag in quarterly reporting and highlighted positive partnership working, including improved SMI health check uptake through targeted engagement. **The Bexley Wellbeing Partnership Committee:** • Noted the report is for information and assurance to the Bexley Wellbeing Partnership Committee.
2.	26 March 2026	Bexley New GP Premium 2026/29	The Bexley Wellbeing Partnership Committee received a report on the proposed redesign of the Bexley GP Premium (2026–2029). GPs withdrew from the discussion due to declared financial conflicts of interest. The proposal sets out an outcomes-focused model aligned to prevention, population health and reducing inequalities, targeting key cohorts and supported by c.£2.2m annual investment. Members sought assurance on patient outcomes, access impacts, value for money and evaluation. It was confirmed that the scheme enhances, rather than duplicates, core GP services and will be subject to formal performance monitoring. **The Bexley Wellbeing Partnership Committee:** **(i) Endorsed** commissioning of the redesigned GP Premium, including KPIs. • **Noted** alignment to local and national strategic priorities.
3.	26 March 2026	South East London Integrated Care	The Bexley Wellbeing Partnership Committee received a report outlining a multi-agency framework across health, education, social care and the voluntary sector, focused on

		System: Children & Young Peoples Integrated Neighbourhood Teams Framework	prevention, early identification and coordinated care. Members discussed education engagement, pressures in neurodevelopmental pathways, and the need for clear outcomes, KPIs and governance arrangements, noting these are in development. **The Bexley Wellbeing Partnership Committee:** • **Endorsed** the Children & Young People Integrated Neighbourhood Team Framework.
4.	28 May 2026	*Let's Talk* About Digital Access	The 'Let's Talk' discussion was on digital services and how we are working to ensure access to them as the world becomes more online. Bexley is making good progress in digital transformation, with high and growing use of the NHS App, strong uptake of Universal Care Plans and positive early results from the *getUbetter* App. Together, these tools are helping residents to access services more easily, manage aspects of their own care and support a wider shift towards prevention, community-based care and more joined-up service delivery.

Appendix 2

Bromley Local Care Partnership – One Bromley

1. Recommendations to the Board for Decision/Approval

1.1 There were no recommendations made to the Board during this period.

2. Decisions made by Bromley LCP Under Delegation

2.1 No decisions were taken by the One Bromley LCP under delegation from the Board during this period.

3. Agenda Items of Note

3.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items discussed
1.	26 March 2026	Item 6 – Our Year 2025/26	• Dr Angela Bhan presented the report, reflecting on the achievements of the One Bromley Local Care Partnership over the last year and noting the considerable progress made in year three of the five year strategy to improve services and the health of the Bromley population. • It was noted that significant challenges remain; however there has been strong focus on prevention, urgent and emergency care and making the best use of combined resources. • There was discussion of the mobilisation of integrated neighbourhood teams, to include work on children's care, development of frailty programmes and improving access to primary care, vaccinations, the Mental Health and Wellbeing Strategy

			2025-30, medicines optimisation and encouraging young people to come forward to take up careers in health and care. • The Board noted the report.
2.	26 March 2026	Item 7 – Month 10 SEL ICB Finance Report	• The One Bromley Local Care Partnership Board received the Month 10 Finance Report noting a breakeven position for both SEL ICB as a whole and the Bromley Place budget for the 2025/26 financial . Financial planning for 2026-27 was underway.
3.	26 March 2026	Item 8 – Partnership Report	• The report highlighted the opening of the Bromley Health and Wellbeing Centre. This was attended by the Mayor of Bromley, a local MP and many other colleagues from the One Bromley system. • The Board noted the Partnership Report.
4.	26 March 2026	Item 9 – Bromley Primary Care Group Report	• The Board noted the Bromley Primary Care Group Report.
5.	26 March 2026	Item 10 – Bromley Performance, Quality and Safeguarding Group Report	• The Board noted the Bromley Performance, Quality and Safeguarding Report.
6.	26 March 2026	Item 11 – Procurement and Contracts Group Report	• The Board noted the Bromley Procurement and Contracts Group Report.
7.	18 June 2026	Summary of Items	At the time of writing, the June One Bromley Local Care Partnership Board meeting has not yet taken place. The agenda includes discussion of the following items: • Strategic Investment Fund • Neighbourhoods Update • Winter Evaluation 2025-26 and Recommendations • Better Care Fund • Communications and Engagement Annual Report 2025-26 • Month 12 Finance Report

			• Partnership Report • Bromley Primary Care Group Report • Bromley Performance, Quality and Safeguarding Group Report • Bromley Procurement and Contracts Group Report • A full summary will be provided for the next SEL ICB Board meeting in October.

Appendix 3

Greenwich Local Care Partnership – Healthier Greenwich Partnership (HGP)

1. Recommendations to the Board for Decision / Approval

1.1 There were no decisions referred to the Board for decision or approval at the meeting held on

2. Decisions made by Healthier Greenwich Partnership LCP Under Delegation

No.	Meeting date	Agenda item	Items for Board to note
1.			

3. Agenda Items of Note

3.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items discussed
1.	29 April 2026	Agenda item 6 Shared Successes	• Members and partners shared success stories from their organisations which included: • Successful bowel cancer community engagement with the Plumstead Nepalese community • Progress of the Tobacco and vapes Bill which is an important national public health development

			• Work linked to the South East London Women and Girls Health Hubs have been shortlisted by the NHS Excellence Awards – Patient Involvement and Choice category • A recent GIRFT review on mental health pathways by LGT and Oxleas has shown improved collaboration and a strong focus on patient experience • The Royal Borough of Greenwich has agreed a transfer of land for the development of modern extra care housing which supports older residents with independent living
2.	29 April 2026	Agenda item 7 Positive Partnership Story: S106 Funding awarded to hospice	• The partnership received a report advising that the Greenwich & Bexley Community Hospice had been awarded £390,000.00 of S106 funding to support the redevelopment of the inpatient unit. • It was noted that this is not only a significant achievement but also the first example of a hospice receiving S106 funding. • The members were advised that there has been national interest in this model and Greenwich will be sharing their learnings from the hospice and wider system engagement.
4.	29 April 2026	Agenda item 8 Age Well update	• The partners received a report that provided an update on 'Age Well' which noted that home care redesign is progressing and support for unpaid carers has improved • It was noted that the DHACT service has supported more that 2,000 people in the first year • Partners were advised that the Transfer of Care hub at Queen Elizabeth hospital has strengthened discharge planning and is contributing to reducing delays to discharge
5.	29 April 2026	Agenda item 9 Healthier Greenwich Charitable Funds update	• The members noted that the fund is now in it's third year of delivery and 161 projects have been awarded to 142 organisations in all wards in Greenwich • The partners noted that the Delivery Strand timetable for 26/27 is being reviewed to ensure it aligns with neighbourhood working and the Royal Borough of Greenwich grants programme

Appendix 4

Lambeth Local Care Partnership – Lambeth Together

1. Recommendations to the Board for Decision / Approval

1.1 Below is a summary of decisions recommended to the Board for approval.

No.	Meeting date	Agenda item	Items discussed
1.	11 June 2026	Lambeth Together Business Planning and Governance	Members of the Partnership Board recommend the updated Terms of Reference for the Lambeth Together Care Partnership Board.

2. Decisions made by Lambeth Together Care Partnership Under Delegation

2.1 Below is a summary of decisions taken by the Lambeth Together Care Partnership under delegation from the Board.

No.	Meeting date	Agenda item	Items for Board to note
1.	5 March 2026	Lambeth Learning Disabilities Intensive Support Team	Members of the Partnership Board: • Endorsed the ongoing development and implementation of the Lambeth Intensive Support Team, a function hosted by NHS South London and Maudsley and NHS Guy's and St Thomas', preventing placement breakdowns and hospital admissions through rapid, flexible, community-based responses for people with learning disabilities who may also be autistic. • Supported the partnership working with Lambeth Adults Social Care and South East London Integrated Care Board (Lambeth) to continue to mobilise around jointly agreed system objectives
2.	5 March 2026	Lambeth Together Primary Care	Members of the Partnership Board:

		Commissioning Committee (PCCC)	• Ratified decisions made at the Primary Care Commissioning Committee on 21 January 2026 and the Extraordinary Primary Care Commissioning Committee on 14 January 2026.
3.	5 March 2026	Lambeth Together Business Planning 2026/27	Members of the Partnership Board: • Noted the progress made since the last Partnership Board meeting. • Provided feedback and approved the draft activities for 2026/27 outlined within the appendix of the board pack. • Approved the timeframes and remaining actions leading to the final Lambeth Together Health and Care Plan refresh for 2026/27.
4.	11 June 2026	Lambeth Together Business Planning and Governance	Members of the Partnership Board: • Approved the 2025/26 annual review and refresh of Our Health, Our Lambeth. • Approved the Lambeth Together Assurance and Oversight proposal.
5.	11 June 2026	Children and Young People's (CYP) Alliance Governance Change	Members of the Partnership Board ratified the decisions made at the Children and Young People's Alliance Board meeting on 21 May 2026: • Approve the proposed Phase 2 development of the Children and Young Person • Alliance Board. • Confirm the Board as the central strategic forum for children and young people's • place-based delivery. • Extend Board oversight to the Families First Partnership programme, alongside • existing safeguarding and operational arrangements. • Review and strengthen Board membership to reflect children's social care, policing, • SEND, education, family hubs, neighbourhood delivery and wider place governance. • Refresh and formalise reciprocal governance arrangements with key children and • young people's partnerships. • Develop coordinated governance arrangements with neighbourhood and integrator • delivery forums. • Ensure children-related decisions and financial proposals are considered through the • CYP Alliance before wider sign-off routes. • Retain flexibility to establish additional governance arrangements where required

			• across the wider children and young people's system. • Agree the proposed implementation and engagement timetable through May and June 2026.
6.	11 June 2026	Lambeth Together Primary Care Commissioning Committee (PCCC) Update	Members of the Partnership Board noted the content of the discussions held at the Extraordinary Primary Care Commissioning Committee on 22 April 2026 as well as the Primary Care Commissioning Committee on 13 May 2026 and ratified decisions made: • Protected learning time (PLT) procurement and contract award: The Committee approved the completion of the most suitable provider route for the PLT clinical cover contract and approved the contract valued at £79,125 for one year, with an option to extend for a further year to be offered to Lambeth Healthcare Ltd • Re-procurement of a practice contract: The Committee noted the update. • Financial Arrangements for Delegated Primary Care budget 26/27: The Committee noted the update. • Proposal for Primary Care (PC) Support Offer: The Committee approved the structure of the programme to manage the PC Support Offer implementation in Lambeth and approved the delegation of authority to the Quality Operational Group to make decisions in line with the PC Support Offer process and on practices eligible for financial support. • Lambeth Offer Shared Care (SC) Scheme 26/27: The Committee approved the SC Medicines scheme within the Lambeth Offer contract for implementation in July 2026, approved the decrease in Enhanced Prevention funding to support the funding of the SC Medicine scheme and approved the decommissioning of the immunomodulatory scheme would be starting in July. • Lambeth Offer: Enhanced Prevention Scheme: The Committee approved the Enhanced Prevention scheme for inclusion in the Lambeth Offer for 26/27, subject to confirmation of the correct funding figures. • Request for change to Streatham Common catchment area: The Committee noted the update with updated action to return in July 2026. • Enhanced access for Fiveways PCN proposal: The Committee approved the subcontract change under the enhanced access for Fiveways proposal. The decision

			initiates a three-month transition period and ensures continued compliance with the enhanced access service obligations. • Capacity and Access Improvement Payment (CAIP) and Capacity and Access Support Payment (CASP) Payments: The Committee approved the release of CAIP payment in full to PCNs for 25/26 financial year.

3. Agenda Items of Note

3.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items discussed
1.	5 March 2026	Lambeth Together Care Partnership - Place Executive Lead Report	Members of the Partnership Board received an update on key developments since the last Lambeth Together Care Partnership Board meeting in public on 8 January 2026, including decisions made under the South East London Integrated Care Board scheme of delegation.
2.	5 March 2026	Lambeth Together Assurance Group (LTAG) Update	Members of the Partnership Board noted the update report from the Lambeth Together Assurance Sub-Group and the and the associated Integrated Assurance Report presented on 27 January 2026.
3.	5 March 2026	Living Well Network Alliance (LWNA) Progress Update	Members of the Partnership Board: • Noted the proposed changes to neighbourhood working in our Living Well Centres, South London and Maudsley (SLaM) NHS Foundation Trust's £675k voluntary, community and social enterprise (VCSE) tendering and highlights from our 2024/25 Progress Report. • Heard from Marcus, the subject of Lambeth's Mosaic Clubhouse Evening Sanctuary film.
4.	11 June 2026	Place Executive Lead Update	Members of the Partnership Board received an update on key developments since the last Lambeth Together Care Partnership Board meeting in Public on 5 March 2026.

5.	11 June 2026	Lambeth Together Assurance Group (LTAG) Update	Members of the Partnership Board were asked to note the report from the Lambeth Together Assurance Sub-Group and the associated Integrated Assurance Report presented on 12 May 2026.
6.	11 June 2026	Sexual Health Programme: HIV in Lambeth	Members of the Board were asked to: • Identify support to distribute Pre-Exposure Prophylaxis (PrEP) campaign materials. • Support staff (including non-health care staff) to undertake training to increase knowledge of HIV. • Encourage Lambeth organisations and stakeholders to sign up to HIV Confident and undertake HIV Confident Training.

Appendix 5

Lewisham Local Care Partnership – Lewisham Health & Care Partnership

1. Recommendations to the Board for Decision / Approval

1.1 No items are referred to the Board for decision or approval in this period.

2. Decisions made by Lewisham Health & Care Partnership Under Delegation

2.1 Below is a summary of decisions taken by the Lewisham LCP under delegation from the Board.

No.	Meeting date	Agenda item	Items for Board to note
1.	26 March 2026	**(4) Terms of Reference for the Lewisham Primary Care Group**	Revised Terms of Reference for the Lewisham Primary Care Group were presented for approval. The purpose of the changes was to strengthen multidisciplinary input by ensuring community pharmacy is represented in planning and decision making. **The LCP Board approved the Terms of Reference.**
2.	26 March 2026	**(5) Grounded Report: Community-Led Reflections and Solutions to Systemic Health Inequalities**	The members received a presentation on The Grounded Report, which was commissioned to assess delivery of the Birmingham and Lewisham African & Caribbean Health Inequalities Review (BLACHIR) recommendations and to understand how system changes are being experienced by communities. The report set out four priority areas for action: delivering services through trusted community settings; sustained investment in health literacy and prevention; increasing Black leadership and decision-making power; and embedding continuous, community-led engagement, supported by phased recommendations and key system enablers **The LCP Board approved the Grounded Report.**

3. Agenda Items of Note

3.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items discussed
1.	26 March 2026	**(3) Place Executive Lead (PEL) report**	The Place Executive lead delivered an update report, noting: • The key dates for the ICB staff consultation and second round of the voluntary redundancy scheme. • The availability of funding through the Sustainability Investment Fund to support development of neighbourhoods, including delivery of the SEL frameworks for frailty, multiple long-term conditions and Children and Young People, and implementation of the SEL ICB Prevention Framework. • The National Neighbourhood Framework has now been published, with Lewisham adopting a broad, community-wide approach, noting it was in a strong position to adopt.
2.	26 March 2026	**(6) ICB/VCS Infrastructure Support Impact Summary**	The LCP members received an update from The Lewisham Black and Voluntary Network (LBVN) on the deployment of ICB funding received over the last 18 months to support partnership working and strengthening of the LBVN's own infrastructure, including additional fundraising capacity. The investment had improved sustainability and enhanced the contribution of LBVN and its members to neighbourhood working, prevention and health equity priorities. Continued funding would enable this work to be sustained and expanded.
3.	26 March 2026	**(7) Lewisham VCS Neighbourhood Contract**	The Lewisham VCS Neighbourhood Contract was discussed, outlining the shift to a neighbourhood-based model with increased investment in the north and south of the borough, including a small transition grant to support existing frontline services. The model focuses on infrastructure and capacity-building, strengthening small and grassroots organisations and community networks, alongside social prescribing to connect residents to local support. Delivery learning would inform future service development and partnership working.
4.	26 March 2026	**(8) LCP Assurance report – Focus on Flu vaccinations**	The members discussed the challenges which remain in achieving physical health checks for people with serious mental illness and initial discussions with SLaM regarding data sharing and the potential impact of health checks on long term outcomes. It was noted that performance is comparable with other boroughs, reflecting significant effort from teams, however further improvement was required. Other areas requiring work to improve performance included cervical

			screening, and flu vaccination uptake particularly among some ethnic groups. In response, extensive outreach had been undertaken to improve access, including vaccination delivery in community venues, shopping centres and through local partners, alongside wide-scale communications activity and VCSE organisations had been commissioned to deliver targeted engagement activity.
5.	26 March 2026	**(9) Q3 Quality Summary**	A quality report was delivered, noting progress on the Quality Impact Assessments (QIA), completion of three patient safety incident investigations in the quarter, and ongoing system wide improvement work on never events. In addition, a maternity led quality improvement programme, training materials and an animation were nearing completion. Ongoing analysis of quality alerts identified key themes, with Learning from Deaths and medication safety remaining a focus. Positive and negative patient experience feedback continued to inform system improvement work and the Patient Safety Incident Response Framework (PSIRF) pilot in general practice had concluded, with learning being embedded into business as usual arrangements and Care Quality Commission (CQC) preparedness. A public health deep dive on termination of pregnancy highlighted higher rates among Black Caribbean women with work needed to consider how to address this health inequality.
6.	26 March 2026	**(10) Finance update**	A finance update for Month 10 was presented, noting a break-even position YTD and on a forecast outturn basis at month 10 for the borough delegated budgets, with the LCP achieving a 5% efficiency target for 2025/26. The ICB and ICS financial positions were also reported.
7.	28 May 2026	**(3) Place Executive Lead (PEL) Report**	The Place Executive Lead delivered an update report, noting: • Ongoing progress with the NHS Change Programme. • The development of neighbourhood health centres, with proposals submitted to NHS England. Lewisham would take a networked approach, making best use of existing estate and community assets rather than relying solely on single-site developments.
8.	28 May 2026	**(4) Strategic Investment Fund (SIF)**	The group received an update on the funding available to Lewisham and its proposed allocation. Ring fenced strategic investment funding totalling £1.6m was available in 2026/27 (part-year), rising to £2.14m recurrent from 2027/28. Funding was aligned to neighbourhood health priorities and SEL strategic commissioning objectives.

9.	28 May 2026	**(5) LCP performance update – April 2026 and Focus on Cancer screening**	**LCP performance update – April 2026 and Focus on Cancer screening** An update on cancer screening programmes, including bowel, breast and cervical screening, as well as the rollout of the lung cancer screening programme, was delivered. Key highlights included strong progress in breast cancer screening, stable performance for bowel cancer screening and challenges in delivering cervical screening, particularly among younger cohorts.
10.	28 May 2026	**(6) Risk Register**	**Risk Register** The members reviewed the LCP risk register, noting there were no new risks and that risks relating to financial efficiencies and the population health platform had been closed following progress against actions. The only risk that had deteriorated related to preventable diseases, linked to vaccination uptake and herd immunity.
11.	28 May 2026	**(7) Q4 Adults Safeguarding Report**	The Q4 safeguarding report highlighted a continued increase in safeguarding concerns within Lewisham, reflecting both rising demand and increased awareness and reporting. The Safeguarding Adult Reviews and Domestic Homicide Reviews evidence that there are both social, economic and environmental reasons that determine significant inequalities in health outcomes amongst Black African and Black Caribbean communities, both locally and nationally. The Board noted the extensive programme of training and workforce development, including a focus on violence against women and girls, domestic abuse, and complex safeguarding issues.

Appendix 6

Southwark Local Care Partnership – Partnership Southwark

1. Recommendations to the Board for Decision / Approval

1.1 No items were referred to the Board for decision or approval in this period.

2. Decisions made by Partnership Southwark Under Delegation

2.1 No items were decided under delegation from the Board in this period.

3. Agenda Items of Note

3.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items discussed
1.	26 March 2026	Kings College Hospital – 5-Year Strategy (2026-2031)	Colleagues presented the refresh of KCH's five-year strategy, noting extensive engagement had shown the current values and vision were unclear and overly internally focused. In response, new values—caring, inclusive, collaborative, and innovative—alongside a revised purpose and vision have been proposed, supported by measurable objectives and a new improvement method to better link strategy to daily work. Members welcomed the changes but highlighted the need to demonstrate clear impact, strengthen partnerships and neighbourhood integration, and continue inclusive engagement, with next steps focused on further refinement ahead of approval in May.

2.	26 March 2026	CYP Mental Health Action Plan	The Board discussed the refreshed Children and Young People's Mental Health action plan, noting rising demand, long waiting times, and system challenges including workforce shortages, funding constraints, and fragmented pathways. Members heard that a recent multi-agency workshop had informed a revised plan focused on simplifying pathways, reducing duplication, and establishing clear priorities such as leadership and governance, neurodevelopmental pathways, early intervention, single front door triage, and improved data use. While the direction was welcomed, the Board emphasised the need for clear delivery ownership, stronger governance, and confidence that changes—particularly around access and duplication—would reduce waiting times, with a target of four weeks noted against current waits exceeding 52 weeks for some; next steps include circulating the full plan, developing a risk log, and clarifying governance arrangements.
3.	26 March 2026	Funding Differently	Katherine de Krester (Projects Manager, Community Southwark) and Jason Charles (Grants Manager, United St Saviour's Charity) presented an evaluation of the Funding Differently pilot, noting its positive impact in improving access to funding for VCSE organisations through a simplified application process and strong involvement of the sector in decision-making. Rebecca and Terry-King Emmanuel (Co-founders, JoshyBoy Legacy) and Kemi Olafare (Volunteer, Peckham Soup Kitchen) shared examples of how the approach supports grassroots organisations, particularly those led by communities experiencing inequalities, and helps build capability, partnerships, and understanding of funding processes. The model and its focus areas—including mental health, community connection, and youth—were welcomed. The Board highlighted the need to balance data collection with proportionality, strengthen evaluation of impact, and consider how learning could inform wider funding approaches across the system. Next steps include sharing evaluation reports and continuing development of the model, including exploring multi-year funding.

4.	26 March 2026	Strategic Director for Health & Care and Place Executive Lead Report	The Board discussed system updates, noting progress on neighbourhood development, including plans for Neighbourhood Health Hubs and the mobilisation of neighbourhood teams from April. Members acknowledged significant financial pressures across the system, including reductions in council funding and ongoing challenges within the Better Care Fund, with a commitment to reflect these risks in the shared risk register. Updates were also provided on the closure of Trafalgar Practice, and new initiatives such as the Work Well programme. Reports from the Integrated Governance and Assurance Committee highlighted financial challenges and performance oversight. The report from the Southwark Neighbourhood Transformation Board detailed emerging cohort-based approaches, with the Board supporting the proposed direction while recognising the need for continued clarity on impacts for residents and system partners.
5.	28 May 2026	Ageing Well INTs x COPSINS	The Board heard about and discussed the development of the Ageing Well integrated partnership model, noting progress in bringing together VCS partners and neighbourhood teams to address frailty through a more proactive, preventative approach that combines social and clinical interventions. Updates highlighted outreach activity, training to build a shared understanding of frailty, and the introduction of a link worker role to strengthen integration and access to services, with early work identifying unmet need and supporting more joined-up care. Members welcomed the focus on prevention, reducing isolation, and improving pathways, but noted challenges around sustainability of non-recurrent funding, gaps in support for more complex needs such as dementia, and the need for clearer outcomes data to demonstrate impact and inform future commissioning.
6.	28 May 2026	Neighbourhood Update from the Integrator	The Board discussed the Southwark Integrator and progress in establishing Integrated Neighbourhood Teams (INTs), noting a focus on supporting residents with the highest levels of need through a “team of teams” approach led by neighbourhood-based clinical leadership. Updates highlighted progress in recruitment, planned mobilisation from July, and development of three key cohorts

			(Children and Young People, multiple long-term conditions, and frailty), alongside Strategic Investment Fund (SIF) proposals centred on prevention, mental health, and system integration. Members welcomed the direction of travel, particularly the emphasis on prevention, data use, and partnership working, but stressed the need for clearer communication of impact for residents. The Board also highlighted the importance of engaging partners, including primary care and the new council administration, ensuring using data and patient feedback to guide delivery and demonstrate outcomes.
7.	28 May 2026	Strategic Director for Health & Care and Place Executive Lead Report	The Board discussed key system updates, noting recent achievements including awards for the CHILDs programme and the Harold Moody Health Centre, alongside ongoing work on the primary–secondary care interface and VCS engagement in neighbourhood programmes. Members were updated on the draft SEND reform submission. The board was also updated on ICB organisational changes, including workforce reductions and a shift towards a more strategic commissioning model, requiring reprioritisation of resources. Performance updates highlighted improvements in Talking Therapies and primary care access, but ongoing challenges in childhood immunisations, and SMI health checks, with further analysis planned. Financial reporting confirmed a small underspend but continued pressures, particularly in mental health and prescribing costs. Discussions at SNTB emphasised neighbourhood alignment, population health management, and the development of CYP cohorts, which the Board formally endorsed.

Appendix 7

Acute Provider Collaborative

1. Key decisions made by the Acute Provider Collaborative (APC)

1.1 There are no key decisions to highlight for the period 21 March 2026 to 8 June 2026.

2. Agenda Items of Note

2.1 Below is a summary of other significant actions and items of note from the APC for the period 21 March 2026 to 8 June 2026, for Board information.

No.	Meeting	Meeting date	Agenda item	Items discussed
1.	APC Committee in Common APC Executive	9 June 2026 17 April and 15 May 2026	Acute Services Review	The APC Executive and Committee in Common meetings have provided an opportunity for acute trust leaders to take a step back and reflect together on the progress of the work, feeding back into the approach via the established governance routes (eg Steering Group etc)
2.	APC Executive	15 May 2026	APC funding allocation and workplan	The APC Executive approved a revised funding allocation and interim workplan for the APC, pending the outcome of the Carnall Farrar review.
3.	APC Committee in Common	9 June 2026	Digital including EPIC	Trust leaders are increasingly looking to the transformative potential of EPIC implementation across all three trusts following LGT Go-Live in April 2027 and are keen to build a shared vision across the three trusts of the key opportunities this will offer.
4.	Multiple Executive Advisory Group and sub-groups	March to June	Acute Service Review (Carnall Farrar work)	The SSG lead for this work and the Carnall Farrar team have continued to attend a number of EAG and subgroup meetings as this work has progressed, with governance structure for the Acute Service Review linking to the APC governance structure at multiple points, eg Finance & Estates Group supporting financial analysis, Performance & Planning for collective engagement with BI and performance leads.

5.	Clinical Delivery Group	3 June 2026	Pre-op testing MRSA	A programme of work has been under way led by the APC Anaesthetic & Peri-operative Medicine network to align the approach to pre-operative testing, particularly MRSA testing. The Clinical Delivery Group are very supportive of this approach which has benefits for patient experience and for staff and also reduces the cost of testing. The impact of the proposed alignment will be monitored over the next few months.

Appendix 8

Mental Health Collaborative

1. Key decisions made by the Mental Health Collaborative

1.1 Below is a summary of the key decisions made by the mental health collaborative in the period since the last report of 23 April 2026.

No.	Meeting date	Agenda item	Items for Board to note
1.	May 26	Strengthening substance misuse services in NHS secure services.	NHS secure services (also known as forensic mental health services) are for people with severe mental health conditions who pose a significant risk to themselves or others. These specialised NHS services are provided by the South London Partnership provider collaborative (Oxleas, South London and Maudsley and South West London and St Georges). At the May meeting, the provider collaborative agreed an 'invest to save' business case to strengthen substance misuse services across all secure care inpatient wards. This aims to reduce substance-related harm, improved treatment engagement, and incur fewer delays to recovery and hospital discharge. An evaluation of the impact of the investment will be undertaken over the next 18 months.

2. Agenda Items of Note

2.1 Below is a summary of other significant actions and items of note for Board information.

No.	Meeting date	Agenda item	Items for Board to note
1.	May 26	NHS 111 Mental Health Service	The NHS111 Mental Health Service is provided by SL&M for residents across south London. Each SLP Trust has agreed to integrate its stand-alone mental health crisis lines into NHS111 Mental Health and this commenced with SL&M in April and will be followed by Oxleas in July, and SWLSTGs in October. A test and learn approach is being used to evaluate the impact of each integration, particularly in relation to call responsiveness. Establishing a single point of access into mental health crisis support for all south London residents is already providing rich data such as differing levels of demand and presenting conditions and has the potential to inform service planning across system, Place and neighbourhoods.
		Areas of collaborative work	The partnership has identified a series of new innovation areas and one of these is collaboration across psychosis pathways, led by the three Chief Medical Officers. A workshop with clinical leads from all three Trusts is being held on 3rd July with the overall aim of reducing unwarranted variation, improving physical healthcare outcomes, and developing opportunities for learning in relation to clinical effectiveness.
		Future strategic priorities	There are four mental health provider collaboratives across south London: Secure Care, Adult Eating Disorders, CAMHS Tier 4 and Perinatal. ICBs have invited SLP Trusts to set out their strategic priorities for the next 3 years for each collaborative and these will be used to support new contractual arrangements that must be in place from April 2027.

Dated **1 April 2026**

THE LONDON BOROUGH OF LEWISHAM

and

NHS SOUTH EAST INTEGRATED CARE BOARD

FRAMEWORK PARTNERSHIP AGREEMENT PURSUANT TO SECTION 75 NHS ACT 2006 RELATING TO THE COMMISSIONING OF HEALTH AND SOCIAL CARE SERVICES GOVERNING THE BETTER CARE FUND

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THIS AGREEMENT is made on day of 1 April 2026

PARTIES

(1) **THE LONDON BOROUGH OF LEWISHAM of Laurence House, Catford, London SE6 4RU (the "Council")**

(2) **NHS SEL INTEGRATED CARE BOARD (Lewisham) of Laurence House, Catford, London SE6 4RU (the "ICB")**

each a "**Partner**" and together the "**Partners**".

BACKGROUND

(A) The Council has responsibility for commissioning and/or providing social care services on behalf of the population of the borough of Lewisham.

(B) The ICB has the responsibility for commissioning health services pursuant to the 2006 Act in the borough of Lewisham.

(C) The Better Care Fund has been established by the Government to provide funds to local areas to support the integration of health and social care and to seek to achieve the National Conditions. It is a requirement of the Better Care Fund that the ICB and the Council establish a pooled fund for this purpose.

(D) Section 75 of the 2006 Act gives powers to local authorities and integrated care boards to establish and maintain pooled funds out of which payment may be made towards expenditure incurred in the exercise of prescribed local authority functions and prescribed NHS functions.

(E) The purpose of this Agreement is to set out the terms on which the Partners have agreed to collaborate and to establish a framework through which the Partners can secure the future position of health and social care services which are the subject of this Agreement through lead or joint commissioning arrangements and through which the Partners will pool funds and/or align budgets as set out in this Agreement.

(F) The aims and benefits of the Partners in entering into this Agreement are to:

a) improve the quality and efficiency of the Services;

b) meet the National Conditions;

c) make more effective use of resources through the establishment and maintenance of a pooled fund for revenue expenditure on the Services; and

d) achieve better health and wellbeing outcomes for users of Lewisham's health and care services

(G) The Partners are entering into this Agreement in exercise of the powers referred to in Section 75 of the 2006 Act and/or Section 65Z5 of the 2006 Act as applicable, to the extent that exercise of these powers is required for the Partners to comply with their obligations under this Agreement.

1 DEFINED TERMS AND INTERPRETATION

1.1 In this Agreement, save where the context requires otherwise, the following words, terms and expressions shall have the following meanings:

2000 Act means the Freedom of Information Act 2000.

2004 Regulations means the Environmental Information Regulations 2004.

2006 Act means the National Health Service Act 2006.

Affected Partner means, in the context of Clause 24, the Partner whose obligations under the Agreement have been affected by the occurrence of a Force Majeure Event

Agreed Sharing Mechanisms means the technical measures described in paragraph 5 of Appendix 1 (Data Sharing Protocol) of Schedule 8, being the means by which the Partners shall transmit Personal Data between each other.

Agreement means this agreement including its Schedules and Appendices.

Annual Report means the annual report produced by the Partners in accordance with Clause 20 (Review).

Approved Expenditure means any expenditure approved by the Partners in writing or as set out in the Scheme Specification in relation to an Individual Scheme over and above any Contract Price, Permitted Expenditure or agreed Third Party Costs.

Authorised Officers means an officer of each Partner appointed to be that Partner's representative for the purpose of this Agreement.

BCF Quarterly Report means the quarterly report produced by the Partners and provided to the Health and Wellbeing Board.

Better Care Fund means the Better Care Fund as described at **NHS England » Better Care Fund** as relevant to the Partners.

Better Care Fund Plan means the plan agreed by the Partners for the relevant Financial Year setting out the Partners' plan for the use of the Better Care Fund as attached as Schedule 6.

Better Care Fund Requirements means any and all requirements on the ICB and the Council in relation to the Better Care Fund set out in Law and guidance published by the Department of Health and Social Care and NHS England.

Change in Law means the coming into effect or repeal (without re-enactment or consolidation) in England of any Law, or any amendment or variation to any Law, or any judgment of a relevant court of law which changes binding precedent in England after the Commencement Date.

Commencement Date means 00:01 hrs on 1 April 2026.

Confidential Information means information, data and/or material of any nature which any Partner may receive or obtain in connection with the operation of this Agreement and the Services and:

(a) which comprises Personal Data or Sensitive Personal Data or which relates to any patient or his treatment or medical history;

(b) the release of which is likely to prejudice the commercial interests of a Partner or the interests of a Service User respectively; or

(c) which is a trade secret.

Contract Price means any sum payable under a Service Contract as consideration for the provision of goods, equipment or services as required as part of the Services and which, for the avoidance of doubt, does not include any Default Liability.

Controller has the meaning given to it in the Data Protection Legislation.

Data Protection Contact means the person appointed by each Partner and identified in paragraph 9 of Appendix 1 (Data Sharing Protocol) of Schedule 8.

Data Protection Legislation means all applicable data protection and privacy legislation in force from time to time in the UK including the UK GDPR, the Data Protection Act 2018 and the Privacy and Electronic Communications (EC Directive) Regulations 2003 and any guidance and codes of practice issues by any Regulatory or Supervisory Body from time to time.

Data Sharing Protocol means the data sharing protocol set out in Appendix 1 of Schedule 8 (Data Sharing Protocol).

Data Subject has the meaning given to it in the Data Protection Legislation.

Default Liability means any sum which is agreed or determined by Law or in accordance with the terms of a Service Contract to be payable by any Partner(s) as a consequence of (i) breach by any or all of the Partners of an obligation(s) in whole or in part) under a Service Contract or (ii) any act or omission of a third party for which any or all of the Partners are, under the terms of the relevant Service Contract, liable to the Provider.

Financial Contributions means the financial contributions made by each Partner to a Pooled Fund or Non-Pooled Fund in any Financial Year.

Financial Year means each financial year running from 1 April in any year to 31 March in the following calendar year.

Force Majeure Event means one or more of the following:

(a) war, civil war (whether declared or undeclared), riot or armed conflict;

(b) acts of terrorism;

(c) acts of God;

(d) fire or flood;

(e) industrial action;

(f) prevention from or hindrance in obtaining raw materials, energy or other supplies; or

(g) any form of contamination or virus outbreak,

in each case where such event is beyond the reasonable control of the Partner claiming relief.

Functions means the NHS Functions and the Health-Related Functions.

Health-Related Functions means those of the health-related functions of the Council, specified in Regulation 6 of the Regulations as relevant to the commissioning of the Services and which may be further described in the relevant Scheme Specification.

Host Partner means for each Pooled Fund the Partner that will host the Pooled Fund and for any Non-Pooled Fund the Partner that will host the Non-Pooled Fund.

Health and Wellbeing Board means the Health and Wellbeing Board established by the Council pursuant to Section 194 of the Health and Social Care Act 2012.

ICB Statutory Duties means the duties of the ICB pursuant to Sections 14Z32 to 14Z44 of the 2006 Act.

Indirect Losses means loss of profits, loss of use, loss of production, increased operating costs, loss of business, loss of business opportunity, loss of reputation or goodwill or any other consequential or indirect loss of any nature, whether arising in tort or on any other basis.

Individual Scheme means one of the schemes which has been agreed by the Partners to be included within this Agreement using the powers under Section 75 of the 2006 Act as documented in a Scheme Specification.

Information Commissioner has the meaning given to it in the Data Protection Legislation.

Integrated Commissioning means arrangements by which both Partners commission Services in relation to an Individual Scheme on behalf of each other in exercise of both the NHS Functions and Health-Related Functions through integrated structures.

Joint (Aligned) Commissioning means a mechanism by which the Partners jointly commission a Service. For the avoidance of doubt, a joint (aligned) commissioning arrangement does not involve the delegation of any functions pursuant to Section 75 of the 2006 Act.

Law means:

(a) any statute or proclamation or any delegated or subordinate legislation;

(b) any enforceable community right within the meaning of Section 2(1) European Communities Act 1972;

(c) any guidance, direction or determination with which the Partner(s) or relevant third party (as applicable) are bound to comply to the extent that the same are published and publicly available or the existence or contents of them have been notified to the Partner(s) or relevant third party (as applicable); and

(d) any judgment of a relevant court of law which is a binding precedent in England.

Lead Commissioning Arrangements means the arrangements by which one Partner commissions Services in relation to an Individual Scheme on behalf of the other Partner in exercise of both the NHS Functions and the Health-Related Functions.

Lead Partner means the Partner responsible for commissioning an Individual Service under a Scheme Specification.

Losses means all damage, loss, liabilities, claims, actions, costs, expenses (including the cost of legal and/or professional services), proceedings, demands and charges whether arising under statute, contract or at common law but excluding Indirect Losses and "Loss" shall be interpreted accordingly.

Month means a calendar month.

National Conditions mean the national conditions as set out in the National Guidance as are amended or replaced from time to time.

National Guidance means any and all guidance in relation to the Better Care Fund as issued from to time to time by NHS England, the Ministry of Housing, Communities and Local Government, the Department of Health and Social Care, and the Local Government Association either collectively or separately.

NHS Functions means those of the NHS functions listed in Regulation 5 of the Regulations as are exercisable by the ICB as are relevant to the commissioning of the Services and which may be further described in each Scheme Specification.

NHS Standard Contract means the contract published by NHS England which must be used by the ICB when commissioning clinical services.

Non-Pooled Fund means the budget detailing the financial contributions of the Partners which are not included in a Pooled Fund in respect of a particular Service as set out in the relevant Scheme Specification.

Non-Recurrent Payments means funding provided by a Partner to a Pooled Fund in addition to the Financial Contributions pursuant to arrangements agreed in accordance with Clause 8.4.

Overspend means any expenditure from a Pooled Fund or Non-Pooled Fund in a Financial Year which exceeds the Financial Contributions for that Financial Year.

Partner means each of the ICB and the Council, and references to "**Partners**" shall be construed accordingly.

Partnership Board means the Section 75 Agreement Management Group responsible for review of performance and oversight of this Agreement as set out in Clause 19.2 and Schedule 2 or such other governance arrangements as the Partners agree.

Partnership Board Quarterly Reports means the reports that the Pooled Fund Manager shall produce and provide to the Partnership Board on a Quarterly basis.

Permitted Budget means in relation to a Service where the Council is the Provider, the budget that the Partners have set in relation to the particular Service.

Permitted Expenditure has the meaning given in Clause 7.4.

Personal Data has the meaning given to it in the Data Protection Legislation.

Personal Data Breach has the meaning given to it in the Data Protection Legislation.

Pooled Fund means any pooled fund established and maintained by the Partners as a pooled fund in accordance with the Regulations.

Pooled Fund Manager means such officer of the Host Partner which includes a Section 113 Officer for the relevant Pooled Fund as nominated by the Host Partner from time to time to manage the Pooled Fund in accordance with Clause 8.

Processing has the meaning given to it in the Data Protection Legislation, and the terms “Process” and “Processed” shall be construed accordingly.

Processor has the meaning given to it in the Data Protection Legislation.

Provider means a provider of any Services commissioned under the arrangements set out in this Agreement.

Quarter means each of the following periods in a Financial Year:

1 April to 30 June

1 July to 30 September

1 October to 31 December

1 January to 31 March

and "**Quarterly**" shall be interpreted accordingly.

Regulations means the means the NHS Bodies and Local Authorities Partnership Arrangements Regulations 2000 No 617 (as amended from time to time).

Regulatory or Supervisory Body means any statutory or other body having authority to issue guidance, standards or recommendations with which the relevant Partner must comply or to which it must have regard, and includes the Information Commissioner.

Scheme means Individual Scheme.

Scheme Specification means a specification setting out the arrangements for an Individual Scheme agreed by the Partners to be commissioned under this Agreement.

Services means such health and social care services as agreed from time to time by the Partners as commissioned under the arrangements set out in this Agreement and more specifically defined in each Scheme Specification.

Service Contract means an agreement entered into by one or more of the Partners in exercise of its obligations under this Agreement to secure the provision of the Services in accordance with the relevant Individual Scheme.

Service Provider means a provider of Services under a Service Contract with one or both of the Partners.

Service Users means those individuals for whom the Partners have a responsibility to commission the Services.

Shared Personal Data means any Personal Data (including Special Category Personal Data) of any Service User(s) or Staff that fall(s) within any of the categories of Personal Data specified in paragraph 3 (Categories of Personal Data) of Appendix 1 (Data Sharing Protocol) of Schedule 8 and means in particular such data as any Disclosing Partner makes available under this Agreement and that a Receiving Partner receives under this Agreement.

Special Category Personal Data means Personal Data that falls within the scope of special categories of Personal Data specified in Article 9 of the UK GDPR.

Third Party Costs means all such third party costs (including legal and other professional fees) in respect of each Individual Scheme as a Partner reasonably and properly incurs in the proper performance of its obligations under this Agreement and as agreed by the Partnership Board.

UK GDPR has the meaning given to it in section 3(10) (as supplemented by section 205(4)) of the Data Protection Act 2018.

Underspend means any expenditure from the Pooled Fund in a Financial Year which is less than the aggregate value of the Financial Contributions for that Financial Year.

Working Day means 8.00am to 6.00pm on any day except Saturday, Sunday, Christmas Day, Good Friday or a day which is a bank holiday (in England) under the Banking & Financial Dealings Act 1971.

1.2 In this Agreement, all references to any statute or statutory provision shall be deemed to include references to any statute or statutory provision which amends, extends, consolidates or replaces the same and shall include any orders, regulations, codes of practice, instruments or other subordinate legislation made thereunder and any conditions attaching thereto. Where relevant, references to English statutes and statutory provisions shall be construed as references also to equivalent statutes, statutory provisions and rules of law in other jurisdictions.

1.3 Any headings to Clauses, together with the front cover and the index are for convenience only and shall not affect the meaning of this Agreement. Unless the contrary is stated, references to Clauses and Schedules shall mean the clauses and schedules of this Agreement.

1.4 Any reference to the Partners shall include their respective statutory successors, employees and agents.

1.5 In the event of a conflict, the conditions set out in the Clauses to this Agreement shall take priority over the Schedules.

1.6 Where a term of this Agreement provides for a list of items following the word "including" or "includes", then such list is not to be interpreted as being an exhaustive list.

1.7 In this Agreement, words importing any particular gender include all other genders, and the term "person" includes any individual, partnership, firm, trust, body corporate, government, governmental body, trust, agency, unincorporated body of persons or association and a reference to a person includes a reference to that person's successors and permitted assigns.

1.8 In this Agreement, words importing the singular only shall include the plural and vice versa.

1.9 In this Agreement, "staff" and "employees" shall have the same meaning and shall include reference to any full or part time employee or officer, director, manager and agent.

1.10 Subject to the contrary being stated expressly or implied from the context in these terms and conditions, all communication between the Partners shall be in writing.

1.11 Unless expressly stated otherwise, all monetary amounts are expressed in pounds sterling but in the event that pounds sterling is replaced as legal tender in the United Kingdom by a different currency then all monetary amounts shall be converted into such other currency at the rate prevailing on the date such other currency first became legal tender in the United Kingdom.

1.12 All references to the Agreement include (subject to all relevant approvals) a reference to the Agreement as amended, supplemented, substituted, novated or assigned from time to time.

2 TERM

2.1 This Agreement shall come into force on the Commencement Date.

2.2 This Agreement shall continue until it is terminated in accordance with Clause 22.

2.3 The duration of the arrangements for each Individual Scheme shall be as set out in the relevant Scheme Specification or if not set out, for the duration of this Agreement unless terminated earlier by the Partners in accordance with Clause 22.2.

3 GENERAL PRINCIPLES

3.1 Nothing in this Agreement shall affect:

3.1.1 the liabilities of the Partners to each other or to any third parties for the exercise of their respective functions and obligations (including the Functions); or

3.1.2 any power or duty to recover charges for the provision of any services (including the Services) in the exercise of any local authority function.

3.2 The Partners agree to:

3.2.1 treat each other with respect and an equality of esteem;

3.2.2 be open with information about the performance and financial status of each; and

3.2.3 provide early information and notice about relevant problems.

3.3 For the avoidance of doubt, the aims and outcomes relating to an Individual Scheme may be set out in the relevant Scheme Specification.

4 PARTNERSHIP FLEXIBILITIES

4.1 This Agreement sets out the mechanism through which the Partners will work together to commission the Services. This may include one or more of the following commissioning mechanisms:

4.1.1 Lead Commissioning Arrangements;

4.1.2 Integrated Commissioning;

4.1.3 Joint (Aligned) Commissioning; and/or

4.1.4 the establishment of one or more Pooled Funds,

in relation to Individual Schemes (the "**Flexibilities**").

4.2 Where there are Lead Commissioning Arrangements and the ICB is Lead Partner the Council delegates to the ICB and the ICB agrees to exercise, on the Council's behalf, the Health-Related Functions to the extent necessary for the purpose of performing its obligations under this Agreement in conjunction with the NHS Functions.

4.3 Where there are Lead Commissioning Arrangements and the Council is Lead Partner, the ICB delegates to the Council and the Council agrees to exercise on the ICB's behalf the NHS Functions to the extent necessary for the purpose of performing its obligations under this Agreement in conjunction with the Health-Related Functions.

4.4 Where the powers of a Partner to delegate any of its statutory powers or functions are restricted, such limitations will automatically be deemed to apply to the relevant Scheme Specification and the Partners shall agree arrangements designed to achieve the greatest degree of delegation to the other Partner necessary for the purposes of this Agreement which is consistent with the statutory constraints.

5 FUNCTIONS

5.1 The purpose of this Agreement is to establish a framework through which the Partners can secure the provision of health and social care services in accordance with the terms of this Agreement.

5.2 This Agreement shall include such Functions as shall be agreed from time to time by the Partners as are necessary to commission the Services in accordance with their obligations under this Agreement.

5.3 The Scheme Specifications for the Individual Schemes included as part of this Agreement at the Commencement Date are set out in Schedule 1 Part 2.

5.4 Where the Partners wish to add a new Individual Scheme to this Agreement a Scheme Specification for each Individual Scheme shall be completed and approved by the Partnership Board in accordance with the variation procedure set out in Clause 30 (Variations). Each new Scheme Specification shall be substantially in the form set out in Schedule 1 Part 1.

5.5 The Partners shall not enter into a Scheme Specification in respect of an Individual Scheme unless they are satisfied that the Individual Scheme in question will improve health and well-being in accordance with this Agreement.

6 COMMISSIONING ARRANGEMENTS

General

6.1 The Partners shall comply with the commissioning arrangements as set out in the relevant Scheme Specification and their obligations in Schedule 4 (Joint Working Obligations).

6.2 The Partners shall comply with all relevant legal duties or, and guidance applicable to, both Partners in relation to the Services being commissioned.

6.3 Each Partner shall keep the other Partner and the Partnership Board regularly informed of the effectiveness of the arrangements including the Better Care Fund and any Overspend or Underspend in a Pooled Fund or Non-Pooled Fund.

6.4 Where there are Integrated Commissioning or Lead Commissioning Arrangements in respect of an Individual Scheme then prior to any new Service Contract being entered into the Partners shall agree in writing:

6.4.1 how the liability under each Service Contract shall be apportioned in the event of termination of the relevant Individual Scheme; and

6.4.2 whether the Service Contract should give rights to third parties (and in particular if a Partner is not a party to the Service Contract to that Partner, the Partners shall consider whether or not the Partner that is not to be a party to the Service Contract should be afforded any rights to enforce any terms of the Service Contract under the Contracts (Rights of Third Parties) Act 1999 and if it is agreed that such rights should be afforded the Partner entering the Service Contract shall ensure as far as is reasonably possible that such rights that have been agreed are included in the Service Contract and shall establish how liability under the Service Contract shall be apportioned in the event of termination of the relevant Individual Scheme.

6.5 The Partners shall comply with the arrangements in respect of Joint (Aligned) Commissioning as set out in the relevant Scheme Specification, which shall include where applicable arrangements in respect of the Service Contracts.

Integrated Commissioning

6.6 Where there are Integrated Commissioning arrangements in respect of an Individual Scheme:

6.6.1 the Partners shall work in cooperation and shall endeavour to ensure that Services in fulfilment of the NHS Functions and Health-Related Functions are commissioned with all due skill, care and attention; and

6.6.2 both Partners shall work in cooperation and endeavour to ensure that the relevant Services as set out in each Scheme Specification are commissioned within each Partners Financial Contribution in respect of that particular Service in each Financial Year.

Appointment of a Lead Partner

6.7 Where there are Lead Commissioning Arrangements in respect of an Individual Scheme the Lead Partner shall comply with its obligations under Schedule 4 Part 1 and:

6.7.1 exercise the NHS Functions in conjunction with the Health-Related Functions as identified in the relevant Scheme Specification;

6.7.2 endeavour to ensure that the NHS Functions and the Health-Related Functions are funded within the parameters of the Financial Contributions of each Partner in relation to each particular Service in each Financial Year;

6.7.3 commission Services for individuals who meet the eligibility criteria set out in the relevant Scheme Specification;

6.7.4 contract with Provider(s) for the provision of the Services on terms agreed with the other Partner;

6.7.5 comply with all relevant legal duties and guidance of both Partners in relation to the Services being commissioned;

6.7.6 where Services are commissioned using the NHS Standard Contract, perform the obligations of the “Commissioner” and “Co-ordinating Commissioner” with all due skill, care and attention and where Services are commissioned using any other form of contract to perform its obligations with all due skill and attention;

6.7.7 undertake performance management and contract monitoring of all Service Contracts including (without limitation) the use of contract notices where Services fail to deliver contracted requirements;

6.7.8 make payment of all sums due to a Provider pursuant to the terms of any Service Contract; and

6.7.9 keep the other Partner and Partnership Board regularly informed of the effectiveness of the arrangements including the Better Care Fund and any Overspend or Underspend in a Pooled Fund or Non-Pooled Fund.

7 ESTABLISHMENT OF A POOLED FUND

7.1 In exercise of their respective powers under Section 75 of the 2006 Act, the Partners have agreed to establish and maintain such pooled funds for revenue expenditure as agreed by the Partners.

7.2 At the Commencement Date there shall be a single Pooled Fund in respect of this Agreement.

7.3 Each Pooled Fund shall be managed and maintained in accordance with the terms of this Agreement.

7.4 Subject to Clause 7.5, it is agreed that the monies held in a Pooled Fund may only be expended on the following:

7.4.1 the Contract Price;

7.4.2 where the Council is to be the Provider, the Permitted Budget;

7.4.3 Third Party Costs where these are set out in the relevant Scheme Specification or as otherwise agreed in advance in writing by the Partnership Board; and

7.4.4 Approved Expenditure as set out in the relevant Scheme Specification or as otherwise agreed in advance in writing by the Partnership Board,

("**Permitted Expenditure**").

7.5 The Partners may only depart from the definition of Permitted Expenditure to include or exclude other revenue expenditure with the express written agreement of the Partnership Board.

7.6 For the avoidance of doubt, monies held in the Pooled Fund may not be expended on Default Liabilities unless this is agreed by all Partners in accordance with Clause 7.5.

7.7 Pursuant to this Agreement, the Partners agree to appoint a Host Partner for each of the Pooled Funds set out in the Scheme Specifications. The Host Partner shall be the Partner responsible for:

7.7.1 holding all monies contributed to the Pooled Fund on behalf of itself and the other Partners;

7.7.2 providing the financial administrative systems for the Pooled Fund;

7.7.3 appointing the Pooled Fund Manager; and

7.7.4 ensuring that the Pooled Fund Manager complies with their obligations under this Agreement.

8 POOLED FUND MANAGEMENT

8.1 When introducing a Pooled Fund, the Partners shall agree which officer of the Host Partner shall act as the Pooled Fund Manager for the purposes of Regulation 7(4) of the Regulations.

8.2 The Pooled Fund Manager for each Pooled Fund shall have the following duties and responsibilities:

8.2.1 the day-to-day operation and management of the Pooled Fund;

8.2.2 ensuring that all expenditure from the Pooled Fund is in accordance with the provisions of this Agreement and the relevant Scheme Specification;

8.2.3 maintaining an overview of all joint financial issues affecting the Partners in relation to the Services and the Pooled Fund;

8.2.4 ensuring that full and proper records for accounting purposes are kept in respect of the Pooled Fund;

8.2.5 reporting to the Partnership Board as required by this Agreement and by the Partnership Board;

8.2.6 ensuring action is taken to manage any projected under or overspends relating to the Pooled Fund in accordance with this Agreement;

8.2.7 preparing and submitting to the Partnership Board Quarterly Reports (or more frequent reports if required by the Partnership Board) and an annual return about the income and expenditure from the Pooled Fund together with such other information as may be required by the Partners and the Partnership Board to monitor the effectiveness of the Pooled Fund and to enable the Partners to complete their own financial accounts and returns. The Partners agree to provide all necessary information to the Pooled Fund Manager in time for the reporting requirements to be met including (without limitation) comply with any reporting requirements as may be required by relevant National Guidance; and

8.2.8 preparing and submitting reports to the Health and Wellbeing Board as may be required by it and any relevant National Guidance including (without limitation) supplying Quarterly Reports referred to in Clause 8.2.7 above to the Health and Wellbeing Board.

8.3 In carrying out their responsibilities as provided under Clause 8.3, the Pooled Fund Manager shall:

8.3.1 have regard to National Guidance and the recommendations of the Partnership Board; and

8.3.2 be accountable to the Partners for delivery of those responsibilities.

8.4 The Partnership Board may agree to the viring of funds between Pooled Funds or amending the allocation of the Pooled Fund between Individual Schemes.

9 NON-POOLED FUNDS

9.1 Any Financial Contributions agreed to be held within a Non-Pooled Fund will be notionally held in a fund established solely for the purposes agreed by the Partners. For the avoidance of doubt, a Non-Pooled Fund does not constitute a pooled fund for the purposes of Regulation 7 of the Partnership Regulations.

9.2 When introducing a Non-Pooled Fund in respect of an Individual Scheme, the Partners shall agree:

9.2.1 which Partner if any shall host the Non-Pooled Fund; and

9.2.2 how and when Financial Contributions shall be made to the Non-Pooled Fund.

9.3 The Host Partner of the relevant Non-Pooled Fund will be responsible for establishing the financial and administrative support necessary to enable the effective and efficient management of the Non-Pooled Fund, meeting all required accounting and auditing obligations.

9.4 Both Partners shall ensure that any Services commissioned using a Non-Pooled Fund are commissioned solely in accordance with the relevant Scheme Specification.

9.5 Where there are Joint (Aligned) Commissioning arrangements, both Partners shall work in cooperation and shall endeavour to ensure that:

9.5.1 the NHS Functions funded from a Non-Pooled Fund are carried out within the ICB Financial Contribution to the Non-Pooled Fund for the relevant Service in each Financial Year; and

9.5.2 the Health-Related Functions funded from a Non-Pooled Fund are carried out within the Council's Financial Contribution to the Non-Pooled Fund for the relevant Service in each Financial Year.

10 FINANCIAL CONTRIBUTIONS

10.1 The Financial Contributions of the ICB and the Council to any Pooled Fund or Non-Pooled Fund for the Financial Year of operation shall be as set out in the Schedule 5.

10.2 The Financial Contribution of the ICB and the Council to any Pooled Fund or Non-Pooled Fund for each subsequent Financial Year of operation shall be subject to review by the Partners. Financial contributions for each scheme, including new schemes, will be agreed by both parties 3 months before the start of each financial year.

10.3 Financial Contributions will be paid as set out in Schedule 3.

10.4 With the exception of Clause 13, no provision of this Agreement shall preclude the Partners from making additional contributions of Non-Recurrent Payments to a Pooled Fund from time to time by mutual agreement. Any such additional contributions of Non-Recurrent Payments shall be explicitly recorded in Partnership Board minutes and recorded in the budget statement as a separate item.

11 NON-FINANCIAL CONTRIBUTIONS

11.1 Unless set out in a Scheme Specification or otherwise agreed by the Partners, each Partner shall provide the non-financial contributions for any Service that they are Lead Partner or as required in order to comply with its obligations under this Agreement in respect of the commissioning of a particular Service.

11.2 Each Scheme Specification shall set out non-financial contributions of each Partner including staff (including the Pooled Fund Manager), premises, IT support and other non-financial resources necessary to perform its obligations pursuant to this Agreement (including, but not limited to, management of Service Contracts and the Pooled Fund(s)).

12 RISK SHARE ARRANGMENTS, OVERSPENDS AND UNDERSPENDS

Risk share arrangements

12.1 The Partners have agreed risk share arrangements as set out in Schedule 3, which provide for risk share arrangements arising within the commissioning of services from the Pooled Funds as set out in National Guidance.

Overspends in Pooled Fund

12.2 Subject to Clause 12.3, the Host Partner for the relevant Pooled Fund shall manage expenditure from a Pooled Fund within the Financial Contributions and shall use reasonable endeavours to ensure that the expenditure is limited to Permitted Expenditure.

12.3 The Host Partner shall not be in breach of its obligations under this Agreement if an Overspend occurs provided that it has used reasonable endeavours to ensure that the only expenditure from a Pooled Fund has been in accordance with Permitted Expenditure and it has informed the Partnership Board in accordance with Clause 12.4.

12.4 In the event that the Pooled Fund Manager identifies an actual or projected Overspend the Pooled Fund Manager must ensure that the Partnership Board is informed as soon as reasonably possible and the provisions of the relevant Scheme Specification and Schedule 3 shall apply.

Overspends in Non-Pooled Funds

12.5 Where in Joint (Aligned) Commissioning Arrangements either Partner identifies an actual or projected Overspend in relation to a Partner's Financial Contribution to a Non-Pooled Fund, that Partner shall as soon as reasonably practicable inform the other Partner and the Partnership Board.

12.6 Where there is a Lead Commissioning Arrangement the Lead Partner is responsible for the management of the Non-Pooled Fund. The Lead Partner shall as soon as reasonably practicable inform the other Partner and the Partnership Board in the event that the Lead Partner identifies an actual or projected Overspend.

Underspend

12.7 In the event that expenditure from any Pooled Fund or Non-Pooled Fund in any Financial Year is less than the aggregate value of the Financial Contributions made for that Financial Year or where the expenditure in relation to an Individual Scheme is less than the agreed allocation to that particular Individual Scheme, the Partners shall agree how the monies shall be spent, carried forward and/or returned to the Partners and the provisions of Schedule 3 shall apply. Such arrangements shall be subject to the Law and the standing orders and standing financial instructions (or equivalent) of the Partners.

13 CAPITAL EXPENDITURE

13.1 Except as provided in Clause 13.2, neither Pooled Funds nor Non Pooled Funds shall normally be applied towards any one-off expenditure on goods and/or services, which will provide continuing benefit and would historically have been funded from the capital budgets of one of the Partners.

13.2 The Partners agree that capital expenditure may be made from a Better Care Fund Pooled Fund where this is in accordance with National Guidance. If a need for capital expenditure to be funded from any Pooled Fund relating to a non-Better Care Fund Scheme or from any Non Pooled Fund is identified this must be in accordance with the Partners' respective statutory powers and agreed by the Partners in writing.

14 VAT

The Partners shall agree the treatment of each Pooled Fund for VAT purposes in accordance with any relevant guidance from HM Revenue and Customs.

15 AUDIT AND RIGHT OF ACCESS

15.1 The Partners shall promote a culture of probity and sound financial discipline and control. The Host Partner shall arrange for the audit of the accounts of the relevant Pooled Fund in accordance with the Regulations and the Local Audit and Accountability Act 2014.

15.2 All internal and external auditors and all other persons authorised by the Partners will be given the right of access by them to any document, information or explanation they require from any employee, member of the relevant Partner in order to carry out their duties. This right is not limited to financial information or accounting records and applies equally to premises or equipment used in connection with this Agreement. Access may be at any time without notice, provided there is good cause for access without notice.

15.3 The Partners shall comply with relevant NHS finance and accounting obligations as required by relevant Law and/or National Guidance.

16 LIABILITIES AND INSURANCE AND INDEMNITY

16.1 Nothing in this Agreement shall affect:

16.1.1 the liability of the Council to the Service Users in respect of the Health-Related Functions; or

16.1.2 the liability of the ICB to the Service Users in respect of the NHS Functions.

16.2 Subject to Clause 16.3, and 16.4, if a Partner (the "Indemnified Partner") incurs a Loss arising out of or in connection with this Agreement (including a Loss arising under an Individual Scheme) as a consequence of any act or omission of another Partner (the "Indemnifying Partner") which constitutes negligence, fraud or a breach of contract in relation to this Agreement or any Service Contract then the Indemnifying Partner shall be liable to the Indemnified Partner for that Loss and shall indemnify the First Partner accordingly.

16.3 Clause 16.2 shall only apply to the extent that the acts or omissions of the Indemnifying Partner contributed to the relevant Loss. Furthermore, it shall not apply if such act or omission occurred as a consequence of the Indemnifying Partner acting in accordance with the instructions or requests of the Indemnified Partner or the Partnership Board.

16.4 If any third party makes a claim or intimates an intention to make a claim against either Partner which may reasonably be considered as likely to give rise to liability under this Clause 16, the Indemnified Partner will:

16.4.1 as soon as reasonably practicable give written notice of that matter to the Indemnifying Partner specifying in reasonable detail the nature of the relevant claim;

16.4.2 not make any admission of liability, agreement or compromise in relation to the relevant claim without the prior written consent of the Indemnifying Partner (such consent not to be unreasonably conditioned, withheld or delayed); and

16.4.3 give the Indemnifying Partner and its professional advisers reasonable access to its premises and personnel and to any relevant assets, accounts, documents and records within its power or control so as to enable the Indemnifying Partner and its professional advisers to examine such premises, assets, accounts, documents and records and to take copies at their own expense for the purpose of assessing the merits of, and if necessary defending, the relevant claim.

16.5 Each Partner shall ensure that they maintain policies of insurance (or equivalent arrangements through schemes operated by the National Health Service Litigation Authority) in respect of all

potential liabilities arising from this Agreement and in the event of Losses shall seek to recover such Loss through the relevant policy of insurance (or equivalent arrangement).

16.6 Each Partner shall at all times take all reasonable steps to minimise and mitigate any loss for which one Partner is entitled to bring a claim against the other Partner pursuant to this Agreement.

Conduct of Claims

16.7 In respect of the indemnities given in this Clause 16:

16.7.1 the Indemnified Partner shall give written notice to the Indemnifying Partner as soon as is practicable of the details of any claim or proceedings brought or threatened against it in respect of which a claim will or may be made under the relevant indemnity;

16.7.2 the Indemnifying Partner shall at its own expense have the exclusive right to defend conduct and/or settle all claims and proceedings to the extent that such claims or proceedings may be covered by the relevant indemnity provided that where there is an impact upon the Indemnified Partner, the Indemnifying Partner shall consult with the Indemnified Partner about the conduct and/or settlement of such claims and proceedings and shall at all times keep the Indemnified Partner informed of all material matters; and

16.7.3 the Partners shall each give to the other all such cooperation as may reasonably be required in connection with any threatened or actual claim or proceedings which are or may be covered by a relevant indemnity.

17 STANDARDS OF CONDUCT AND SERVICE

17.1 The Partners will at all times comply with Law and ensure good corporate governance in respect of each Partner (including the Partners respective standing orders and standing financial instructions).

17.2 The Council is subject to the duty of Best Value under the Local Government Act 1999. This Agreement and the operation of the Pooled Fund is therefore subject to the Council's obligations for Best Value and the other Partners will co-operate with all reasonable requests from the Council which the Council considers necessary in order to fulfil its Best Value obligations.

17.3 The ICB is subject to the ICB Statutory Duties and these incorporate a duty of clinical governance, which is a framework through which they are accountable for continuously improving the quality of its services and safeguarding high standards of care by creating an environment in which excellence in clinical care will flourish. This Agreement and the operation of the Pooled Funds are therefore subject to ensuring compliance with the ICB Statutory Duties and clinical governance obligations.

17.4 The Partners are committed to an approach to equality and equal opportunities as represented in their respective policies. The Partners will maintain and develop these policies as applied to service provision, with the aim of developing a joint strategy for all elements of the Services.

18 CONFLICTS OF INTEREST

18.1 The Partners shall comply with the agreed policy for identifying and managing conflicts of interest as set out in Schedule 7.

19 GOVERNANCE

19.1 Overall strategic oversight of partnership working between the Partners is vested in the Health and Wellbeing Board, which for these purposes shall make recommendations to the Partners as to any action it considers necessary.

19.2 The Partners have established a Partnership Board to oversee this agreement for the BCF. This Board reports to the Health & Wellbeing Board.

19.3 The Partnership Board is based on a joint working group structure. Each member of the Partnership Board shall be an officer of one of the Partners and will have individual delegated responsibility from the Partner employing them to make decisions which enable the Partnership Board to carry out its objects, roles, duties and functions as set out in this Clause 19 and Schedule 2.

19.4 The terms of reference of the Partnership Board shall be as set out in Schedule 2 as may be amended or varied by written agreed from time to time.

19.5 The Partnership Board shall be responsible for the overall approval of the Individual Schemes and the financial management.

19.6 The Health and Wellbeing Board shall be responsible for ensuring compliance with the Better Care Fund Plan and the strategic direction of the Better Care Fund.

19.7 Each Scheme Specification shall confirm the governance arrangements in respect of the Individual Service and how that Individual Service is reported to the Partnership Board and Health and Wellbeing Board.

19.8 Each Partner has secured internal reporting arrangements to ensure the standards of accountability and probity required by each Partner's own statutory duties and organisation are complied with.

19.9 The Partnership Board shall be responsible for the overall approval of the Individual Schemes and the financial management set out in Clause 12 and Schedule 3. The Partnership Board will report to the Health and Wellbeing Board in accordance with its terms of reference.

19.10 The Health and Wellbeing Board shall be responsible for ensuring compliance with the Better Care Fund Plan and the strategic direction of the Better Care Fund.

19.11 Each Scheme Specification shall confirm the governance arrangements in respect of the Individual Scheme and how that Individual Scheme is reported to the Partnership Board and Health and Wellbeing Board.

20 REVIEW

20.1 The Partners shall produce a BCF Quarterly Report which shall be provided to the Health and Wellbeing Board in such form and setting out such information as required by National Guidance and any additional information required by the Health and Wellbeing Board or NHS England.

20.2 Save where the Partnership Board agree alternative arrangements (including alternative frequencies) the Partners shall undertake an annual review ("**Annual Review**") of the operation of this Agreement, any Pooled Fund and Non-Pooled Fund and the provision of the Services within 3 Months of the end of each Financial Year.

20.3 Subject to any variations to this process agreed by the Partnership Board, Annual Reviews shall be conducted in good faith.

20.4 The Partners shall within [20] Working Days of the annual review prepare an Annual Report including the information as required by National Guidance and any other information required by the Health and Wellbeing Board. A copy of this report shall be provided to the Health and Wellbeing Board and Partnership Board.

20.5 In the event that the Partners fail to meet the requirements of the Better Care Fund Plan and NHS England the Partners shall provide full co-operation with NHS England to agree a recovery plan.

21 COMPLAINTS

The Partners' own complaints procedures shall apply to this Agreement. The Partners agree to assist one another in the management of complaints arising from this Agreement or the provision of the Services.

22 TERMINATION & DEFAULT

22.1 This Agreement may be terminated by any Partner giving not less than three [3] Months' notice in writing to terminate this Agreement provided that such termination shall not take effect prior to the termination or expiry of all Individual Schemes.

22.2 Each Individual Scheme may be terminated in accordance with the terms set out in the relevant Scheme Specification provided that the Partners ensure that the Better Care Fund Requirements continue to be met.

22.3 If a Partner ("**Relevant Partner**") fails to meet any of its obligations under this Agreement, the other Partner may by notice require the Relevant Partner to take such reasonable action within a reasonable timescale as the other Partner may specify to rectify such failure. Should the Relevant Partner fail to rectify such failure within such reasonable timescale, the matter shall be referred for resolution in accordance with Clause 23.

22.4 Termination of this Agreement (whether by effluxion of time or otherwise) shall be without prejudice to the Partners' rights in respect of any antecedent breach and the provisions of Clauses 22.1 and 22.2.

22.5 Upon termination of this Agreement for any reason whatsoever the following shall apply:

22.5.1 the Partners agree that they will work together and co-operate to ensure that the winding down and disaggregation of the integrated and joint activities to the separate responsibilities of the Partners is carried out smoothly and with as little disruption as possible to Service Users, the Services, the employees, the Partners and third parties, so as to minimise costs and liabilities of each Partner in doing so;

22.5.2 where either Partner has entered into a Service Contract which continues after the termination of this Agreement, both Partners shall continue to contribute to the Contract Price in accordance with the agreed contribution for that Service prior to termination and will enter into all appropriate legal documentation required in respect of this;

22.5.3 the Lead Partner shall make reasonable endeavours to amend or terminate a Service Contract (which shall for the avoidance of doubt not include any act or omission that would place the Lead Partner in breach of the Service Contract) where the other Partner requests the same in writing Provided that the Lead Partner shall not be required to make any payments to the Provider for such amendment or termination unless the Partners shall have agreed in advance who shall be responsible for any such payment;

22.5.4 where a Service Contract held by a Lead Partner relates all or partially to services which relate to the other Partner's Functions then provided that the Service Contract allows the other Partner may request that the Lead Partner assigns the Service Contract in whole or part upon the same terms mutatis mutandis as the original Service Contract;

22.5.5 the Partnership Board shall continue to operate for the purposes of functions associated with this Agreement for the remainder of any contracts and commitments relating to this Agreement; and

22.5.6 termination of this Agreement shall have no effect on the liability of any rights or remedies of either Partner already accrued, prior to the date upon which such termination takes effect.

22.6 In the event of termination in relation to an Individual Scheme the provisions of Clause 22.6 shall apply in relation to the Individual Scheme (as though references as to this Agreement were to that Individual Scheme).

23 DISPUTE RESOLUTION

23.1 In the event of a dispute between the Partners arising out of this Agreement, either Partner may serve written notice of the dispute on the other Partner, setting out full details of the dispute.

23.2 The Authorised Officer shall meet in good faith as soon as possible and in any event within seven (7) days of notice of the dispute being served pursuant to Clause 23.1, at a meeting convened for the purpose of resolving the dispute.

23.3 If the dispute remains after the meeting detailed in Clause 23.2 has taken place, the Partners' respective chief executives or nominees shall meet in good faith as soon as possible after the relevant meeting and in any event with fourteen (14) days of the date of the meeting, for the purpose of resolving the dispute.

23.4 If the dispute remains after the meeting detailed in Clause 23.3 has taken place, then the Partners will attempt to settle such dispute by mediation in accordance with the CEDR Model Mediation Procedure or any other model mediation procedure as agreed by the Partners. To initiate a mediation, either Partner may give notice in writing (a **"Mediation Notice"**) to the other requesting mediation of the dispute and shall send a copy thereof to CEDR or an equivalent mediation organisation as agreed by the Partners asking them to nominate a mediator. The mediation shall commence within twenty (20) Working Days of the Mediation Notice being served. Neither Partner will terminate such mediation until each of them has made its opening presentation and the mediator has met each of them separately for at least one (1) hour. Thereafter, paragraph 14 of the Model Mediation Procedure will apply (or the equivalent paragraph of any other model mediation procedure agreed by the Partners). The Partners will co-operate with any person appointed as mediator, providing him with such information and other assistance as he shall require and will pay his costs as he shall determine or in the absence of such determination such costs will be shared equally.

23.5 Nothing in the procedure set out in this Clause 23 shall in any way affect either Partner's right to terminate this Agreement in accordance with any of its terms or take immediate legal action.

24 FORCE MAJEURE

24.1 Neither Partner shall be entitled to bring a claim for a breach of obligations under this Agreement by the other Partner or incur any liability to the other Partner for any losses or damages incurred by that Partner to the extent that a Force Majeure Event occurs and it is prevented from carrying out its obligations by that Force Majeure Event.

24.2 On the occurrence of a Force Majeure Event, the Affected Partner shall notify the other Partner as soon as practicable. Such notification shall include details of the Force Majeure Event, including evidence of its effect on the obligations of the Affected Partner and any action proposed to mitigate its effect.

24.3 As soon as practicable, following notification as detailed in Clause 24.2, the Partners shall consult with each other in good faith and use all best endeavours to agree appropriate terms to mitigate the effects of the Force Majeure Event and, subject to Clause 24.4, facilitate the continued performance of the Agreement.

24.4 If the Force Majeure Event continues for a period of more than sixty (60) days, either Partner shall have the right to terminate the Agreement by giving fourteen (14) days written notice of termination to the other Partner. For the avoidance of doubt, no compensation shall be payable by either Partner as a direct consequence of this Agreement being terminated in accordance with this Clause 24.

25 CONFIDENTIALITY

25.1 In respect of any Confidential Information a Partner receives from another Partner (the "**Discloser**") and subject always to the remainder of this Clause 25, each Partner (the "**Recipient**") undertakes to keep secret and strictly confidential and shall not disclose any such Confidential Information to any third party, without the Discloser's prior written consent provided that:

25.1.1 the Recipient shall not be prevented from using any general knowledge, experience or skills which were in its possession prior to the Commencement Date; and

25.1.2 the provisions of this Clause 25 shall not apply to any Confidential Information which:

(a) is in or enters the public domain other than by breach of the Agreement or other act or omission of the Recipient; or

(b) is obtained by a third party who is lawfully authorised to disclose such information.

25.2 Nothing in this Clause 25 shall prevent the Recipient from disclosing Confidential Information where it is required to do so in fulfilment of statutory obligations or by judicial, administrative, governmental or regulatory process in connection with any action, suit, proceedings or claim or otherwise by applicable Law.

25.3 Each Partner:

25.3.1 may only disclose Confidential Information to its employees and professional advisors to the extent strictly necessary for such employees to carry out their duties under the Agreement;

25.3.2 will ensure that, where Confidential Information is disclosed in accordance with Clause 25.3.1, the recipient(s) of that information is made subject to a duty of confidentiality equivalent to that contained in this Clause 25; and

25.3.3 shall not use Confidential Information other than strictly for the performance of its obligations under this Agreement.

26 FREEDOM OF INFORMATION AND ENVIRONMENTAL INFORMATION REGULATIONS

26.1 The Partners agree that they will each cooperate with each other to enable any Partner receiving a request for information under the 2000 Act or the 2004 Regulations to respond to a request promptly and within the statutory timescales. This cooperation shall include but not be limited to finding, retrieving and supplying information held, directing requests to other Partners as appropriate and responding to any requests by the Partner receiving a request for comments or other assistance.

26.2 Any and all agreements between the Partners as to confidentiality shall be subject to their duties under the 2000 Act and 2004 Regulations. No Partner shall be in breach of Clause 26 if it makes disclosures of information in accordance with the 2000 Act and/or 2004 Regulations.

27 OMBUDSMEN

The Partners will co-operate with any investigation undertaken by the Health Service Commissioner for England or the Local Government Commissioner for England (or both of them) in connection with this Agreement.

28 INFORMATION SHARING

28.1 The Partners agree to exercise their rights and obligations under this Agreement at all times in compliance with their respective obligations under the Data Protection Legislation.

28.2 Without prejudice to the obligation under Clause 28.1, the Partners will comply with the Data Sharing Protocol set out in Schedule 8, as agreed and amended between the Partners from time to time.

29 NOTICES

29.1 Any notice to be given under this Agreement shall either be delivered personally or sent by first class post or electronic mail. The address for service of each Partner shall be as set out in Clause 29.3 or such other address as each Partner may previously have notified to the other Partner in writing. A notice shall be deemed to have been served if:

29.1.1 personally delivered, at the time of delivery;

29.1.2 posted, at the expiration of forty eight (48) hours after the envelope containing the same was delivered into the custody of the postal authorities; or

29.1.3 if sent by electronic mail, at the time of transmission and a telephone call must be made to the recipient warning the recipient that an electronic mail message has been sent to him (as evidenced by a contemporaneous note of the Partner sending the notice) and a hard copy of such notice is also sent by first class recorded delivery post (airmail if overseas) on the same day as that on which the electronic mail is sent.

29.2 In proving such service, it shall be sufficient to prove that personal delivery was made, or that the envelope containing such notice was properly addressed and delivered into the custody of the postal authority as prepaid first class or airmail letter (as appropriate), or that the electronic mail was properly addressed and no message was received informing the sender that it had not been received by the recipient (as the case may be).

29.3 The address for service of notices as referred to in Clause 29.1 shall be as follows unless otherwise notified to the other Partner in writing:

29.3.1 if to the Council, addressed to the Director of Adult Health & Social Care

Tel: 020 8314 8107

Email: denise.radley@lewisham.gov.uk

and

29.3.2 if to the ICB, addressed to the Lewisham Place Executive Lead

Tel: 07927582875

Email: ceri.jacob@selondonics.nhs.uk

30 VARIATION

30.1 No variations to this Agreement will be valid unless they are recorded in writing and signed for and on behalf of each of the Partners.

31 CHANGE IN LAW

31.1 The Partners shall ascertain, observe, perform and comply with all relevant Laws, and shall do and execute or cause to be done and executed all acts required to be done under or by virtue of any Laws.

31.2 On the occurrence of any Change in Law, the Partners shall agree in good faith any amendment required to this Agreement as a result of the Change in Law subject to the Partners using all reasonable endeavours to mitigate the adverse effects of such Change in Law and taking all reasonable steps to minimise any increase in costs arising from such Change in Law.

31.3 In the event of failure by the Partners to agree the relevant amendments to the Agreement (as appropriate), the Clause 23 (Dispute Resolution) shall apply.

32 WAIVER

No failure or delay by any Partner to exercise any right, power or remedy will operate as a waiver of it nor will any partial exercise preclude any further exercise of the same or of some other right to remedy.

33 SEVERANCE

If any provision of this Agreement, not being of a fundamental nature, shall be held to be illegal or unenforceable, the enforceability of the remainder of this Agreement shall not thereby be affected.

34 ASSIGNMENT AND SUBCONTRACTING

The Partners shall not sub-contract, assign or transfer the whole or any part of this Agreement, without the prior written consent of the other Partners, which shall not be unreasonably withheld or delayed. This shall not apply to any transfer to a statutory successor of all or part of a Partner's statutory functions.

35 EXCLUSION OF PARTNERSHIP AND AGENCY

35.1 Nothing in this Agreement shall create or be deemed to create a partnership under the Partnership Act 1890 or the Limited Partnership Act 1907, a joint venture or the relationship of employer and employee between the Partners or render either Partner directly liable to any third party for the debts, liabilities or obligations of the other.

35.2 Except as expressly provided otherwise in this Agreement or where the context or any statutory provision otherwise necessarily requires, neither Partner will have authority to, or hold itself out as having authority to:

35.2.1 act as an agent of the other;

35.2.2 make any representations or give any warranties to third parties on behalf of or in respect of the other; or

35.2.3 bind the other in any way.

36 THIRD PARTY RIGHTS

Unless the right of enforcement is expressly provided, no third party shall have the right to pursue any right under this Agreement pursuant to the Contracts (Rights of Third Parties) Act 1999 or otherwise.

37 ENTIRE AGREEMENT

37.1 The terms herein contained together with the contents of the Schedules constitute the complete agreement between the Partners with respect to the subject matter hereof and supersede all previous communications representations understandings and agreement and any representation promise or condition not incorporated herein shall not be binding on any Partner.

37.2 No agreement or understanding varying or extending or pursuant to any of the terms or provisions hereof shall be binding upon any Partner unless in writing and signed by a duly authorised officer or representative of the parties.

38 COUNTERPARTS

This Agreement may be executed in one or more counterparts. Any single counterpart or a set of counterparts executed, in either case, by all Partners shall constitute a full original of this Agreement for all purposes.

39 GOVERNING LAW AND JURISDICTION

39.1 This Agreement and any dispute or claim arising out of or in connection with it or its subject matter or formation (including non-contractual disputes or claims) shall be governed by and construed in accordance with the laws of England and Wales.

39.2 Subject to Clause 23 (Dispute Resolution), the Partners irrevocably agree that the courts of England and Wales shall have exclusive jurisdiction to hear and settle any action, suit, proceedings, dispute or claim, which may arise out of, or in connection with, this Agreement, its subject matter or formation (including non-contractual disputes or claims).

This Agreement has been entered into on the date stated at the beginning of it.

Signed by Denise Radley, Director of Adult Health & Social Care for and on behalf of **LEWISHAM COUNCIL**

_____

Authorised Signatory

Signed by Andrew Bland, Chief Executive Officer **NHS SOUTH EAST LONDON INTEGRATED CARE BOARD**

_____

Authorised Signatory

SCHEDULE 1 – SCHEME SPECIFICATION

Part 1 – SERVICES SCHEDULE

1 OVERVIEW OF INDIVIDUAL SERVICE

- This Scheme Specification sets out the arrangements agreed by the Partners for the commissioning and delivery of the Service described in Schedule 1 Part 2.
- The Scheme forms part of the Better Care Fund Plan for the relevant Financial Year and is commissioned under the Flexibilities permitted by this Agreement.

2 AIMS AND OUTCOMES

The aims of the Scheme are to:

- support the integration of health and social care in Lewisham;
- improve outcomes for Service Users through coordinated, person-centred care;
- reduce avoidable hospital admissions and facilitate timely discharge;
- promote independence and reduce reliance on long-term care;
- address health inequalities through neighbourhood-based delivery.

The expected outcomes include:

- improved experience of care;
- reduced non-elective admissions;
- improved discharge flow and reduced delays;
- increased independence following reablement;
- reduced long-term care home admissions.

3 COMMISSIONING, CONTRACTING, ACCESS

- The commissioning arrangements for this Scheme shall be undertaken in accordance with Clause 6 of this Agreement and shall follow the commissioning model set out in this Scheme Specification (Lead Commissioning, Integrated Commissioning or Joint (Aligned) Commissioning).
- Where Lead Commissioning applies, the Lead Partner shall commission the Service on behalf of both Partners and shall enter into and manage the relevant Service Contracts in accordance with Schedule 4 (Joint Working Obligations).
- Where Integrated Commissioning applies, the Partners shall jointly plan, design and commission the Service through agreed integrated structures, ensuring alignment of NHS Functions and Health-Related Functions.
- Where Joint (Aligned) Commissioning applies, each Partner shall retain responsibility for its own commissioning decisions but shall coordinate activity to ensure consistency of approach and delivery.
- Access to the Service shall be based on the eligibility criteria set out in this Scheme Specification and shall be applied consistently by both Partners and all Providers.
- The Partners shall ensure that Service Users have equitable access to the Service regardless of the route of referral, and that referral pathways are clearly defined and communicated to all relevant stakeholders.
- The Partners shall ensure that any Service Contracts entered into under this Scheme include clear requirements relating to access, referral processes, waiting times, and service responsiveness.

4 FINANCIAL CONTRIBUTIONS

- The Financial Contributions of each Partner to this Scheme shall be as set out in this Scheme Specification and in Schedule 3 (Financial Arrangements, Risk Share and Overspends).

Running Balances	Income	Expenditure
DFG	£1,952,454	£1,952,454
NHS Minimum Contribution	£33,317,619	£33,317,619
Local Authority Better Care Grant	£18,433,042	£18,433,042
Additional LA Contribution	£797,115	£797,115
Additional NHS Contribution	£0	£0
Total	**£54,500,230**	**£54,500,230**

- Financial resources in subsequent Financial Years to be determined in accordance with the Agreement.

5 FINANCIAL GOVERNANCE ARRANGEMENTS

Management of the Pooled Fund

- There will be one pooled fund for the Lewisham Better Care Fund. The Host Partner for the pooled fund will be the Council and the Pooled Fund Manager, being an officer of the council, is the Strategic Business Partner – Adult Social Care & Health

Financial Management

- Before the start of the financial year the Partners will agree an expenditure plan and the financial contribution of each party. For each element the plan will indicate which party will be the Lead Commissioner (or whether there will be joint commissioning). All BCF expenditure will require the approval of the Partnership Board.

- Payments to and from the Pooled Fund will be made from and recorded on the Council's financial systems.

- Except where agreed otherwise payments to providers for services that are part of the BCF will be made by the Partner holding the contract. These will be identified and agreed by the Partners at least yearly before the start of each financial year. The Council will invoice the ICB Lewisham monthly for 1/12th of the agreed annual contribution less all planned payments made by the ICB Lewisham in relation to ICB Lewisham let contracts delivering the Schemes.

- In addition, the Pooled Budget Manager may agree budget lines for which invoices will be paid by the ICB Lewisham and where reimbursement will be sought by the ICB Lewisham from the Pooled Fund.

- By the 10th of each month the ICB Lewisham shall notify the Council of (a) details of payments made for which it is seeking reimbursement and (b) estimated payments for the remainder of the year.

- The Partnership Board will be provided with regular financial reports showing the use of the Pooled Fund against planned activity and will report to the Health and Wellbeing Board as appropriate.

- The Council will produce monthly monitoring reports showing spend to date and projected full year spend for the Schemes and projected variances against agreed budgets. These will be reported to the Partnership Board by the 15th of the following month. Council finance staff will identify any actual or potential overspends and bring these to the attention of
- Scheme managers immediately.

- Changes to allocations/ budgets will be considered/agreed by the Partnership Board.

- The Pooled Budget Manager will inform the Partnership Board of all actual and potential overspends and underspends against the agreed budgets. The Partnership Board will agree the use of these underspends on a non-recurring basis and the approach to each overspend. The Pooled Budget Manager will amend the agreed expenditure plan/budget accordingly.

- Each Partner will make financial records available to the other.

- Where budgets have been allocated against specific activity or contracts, the responsible officer will notify the Pooled Budget Manager of any likely under or overspends as soon as they become likely. Overspends above £10,000 will require the approval of Partnership Board. Overspends below £10,000 will require the approval of the Pooled Budget Manager.

- Allocation of overheads to the Pooled Budget will require the approval of the Partnership Board.

VAT

- The VAT regime applicable to the Council will apply where payments for services are made by the Council. The VAT regime applicable to the ICB Lewisham will apply where payments are made by the ICB Lewisham.

ACCOMMODATION AND OTHER-NON FINANCIAL CONTRIBUTIONS

The Partners shall make available to each other such rooms and hot desking facilities (including the use of desks, space, shelving, information technology and voice and data equipment and services and meeting rooms) as shall be reasonably required for the purposes of this Agreement at their respective premises as agreed by the Partners from time to time and the Partners hereby grant to each other non-exclusive licences for such purpose.

6 ASSURANCE AND MONITORING

The Health and Well Being Board ("HWB")
This is a partnership body comprising, amongst others, elected members of the Council, Council officers, the Director of Public Health, voluntary organisations and a representative of the NHS. It will provide a framework and direction within which the Council and ICB will deliver the Scheme objectives and outcomes.

Partnership Board
The Partnership Board shall be known as the Section 75 Management Agreement Group. The Section 75 Agreement Management Group is made up of representatives of the Partners and its purpose is to oversee this Agreement and the operation of the Pooled Fund. It will be responsible for the monitoring of assurance and monitoring against the Schemes.

7 INTERNAL APPROVALS

Following approval of the plan by the Health and Wellbeing Board, the Section 75 Agreement shall be subject to formal approval and execution in accordance with the respective governance and decision-making requirements of the Local Authority, through Mayor and Cabinet, and the SEL ICB Board.

Part 2 – AGREED SCHEME SPECIFICATIONS

- The Individual Schemes commissioned under this Agreement are those set out in Schedule 6 (Better Care Fund Plan).
- The Scheme Specifications, including scheme descriptions, financial contributions, commissioning arrangements, providers, and performance measures, shall be as set out in Schedule 6 and the associated Better Care Fund numerical return for the relevant Financial Year.
- For the avoidance of doubt, Schedule 6 forms the agreed Scheme Specifications for the purposes of Clause 6 and this Schedule.

SCHEDULE 2 – GOVERNANCE

1 Partnership Board

1.1 The membership of the Partnership Board will be as follows:

Core members

- Place Executive Lead, NHS South East London ICB (Lewisham)
- Associate Director of Finance, NHS South East London ICB (Lewisham)
- Executive Director, Adult Social Care & Health, Lewisham Council
- Executive Director of Corporate Resources, Lewisham Council (or officer with S151 decision-making responsibility)
- Executive Director for Children and Young People's Services, Lewisham Council (where CYP S75 and related matters are discussed)

The Partnership Board may agree to invite one or more individuals to attend any meeting or part of a meeting. Such attendees may participate in discussions but may not participate in decision making.

2 Role of Partnership Board

3 The Partnership Board shall:

3.1.1 Provide strategic direction on the Individual Schemes

3.1.2 receive the financial and activity information;

3.1.3 review the operation of this Agreement and performance manage the Individual Services;

3.1.4 agree such variations to this Agreement from time to time as it thinks fit;

3.1.5 review and agree annually a risk assessment;

3.1.6 review and agree annually revised Schedules as necessary;

3.1.7 request such protocols and guidance as it may consider necessary in order to enable teach Pooled Fund Manager to approve expenditure from a Pooled Fund;

3.1.8 cooperate with the Pooled Fund Manager in meeting reporting requirements in accordance with relevant National Guidance;

4 Reporting

The Partnership Board shall report directly to the Health and Wellbeing Board on a Quarterly basis in accordance with relevant National Guidance.

5 Partnership Board Support

The Partnership Board will be supported by officers from the Partners from time to time.

6 Meetings

6.1 The Partnership Board will meet Quarterly at a time to be agreed within following receipt of each Quarterly report of the Pooled Fund Manager.

6.2 The quorum for meetings of the Partnership Board shall be a minimum of one representative from each of the Partner organisations. Members of the Partnership Board may participate in Partnership Board meetings by telephone or video conference and shall count towards the quorum in doing so.

6.3 Decisions of the Partnership Board shall be made unanimously. Where unanimity is not reached then the item in question will in the first instance be referred to the next meeting of the Partnership Board. If no unanimity is reached on the second occasion it is discussed then the matter shall be dealt with in accordance with the dispute resolution procedure set out in the Agreement.

6.4 Where a Partner is not present and has not given prior written notification of its intended position on a matter to be discussed, then those present may not make or record commitments on behalf of that Partner in any way.

6.5 Minutes of all decisions shall be kept and copied to the Authorised Officers within [seven (7)] days of every meeting.

7 Delegated Authority

7.1 The Partnership Board is authorised within the limited of delegated authority for its members (which is received through their respective organisation's own financial scheme of delegation) to:

7.1.1 authorise commitments which exceed or are reasonably likely to lead to exceeding the contributions of the Partners to the aggregate contributions of the Partners to any Pooled Fund; and

7.1.2 authorise a Lead Partner to enter into any Service Contract necessary for the provision of Services under an Individual Scheme

8 Information and Reports

Each Pooled Fund Manager shall supply to the Partnership Board on a Quarterly basis the financial and activity information as required under the Agreement.

9 Post-termination

The Partnership Board shall continue to operate in accordance with this Schedule following any termination of this Agreement but shall endeavour to ensure that the benefits of any Service Contracts are received by the Partners in the same proportions as their respective contributions at that time.

10 **Review**

These terms of reference will be reviewed on an annual basis by the Partners.

Date of the next review: 31 March 2027.

SCHEDULE 3: FINANCIAL ARRANGEMENTS, RISK SHARE AND OVERSPENDS

1.1 Unless the context otherwise requires, the defined terms used in this Schedule shall have the same meanings as set out in Clause 1 of the main body of Agreement.

1.2 Subject to any contrary provision in the relevant Scheme Specification, the Parties agree that Overspends or Underspends shall be managed in accordance with this Schedule 3.

Financial Contributions

1.3 The allocated funding for the Better Care Fund in 2026-27 is. The NHS Minimum Contribution is £33,317,619, of which the contribution to adult social care is £13,004,674. The Local Authority Better Care Grant has been pooled into the BCF and totals £18,433,042. The DFG allocation is £1,952,454.

Risk Share

1.1 Before the start of each year the Partners shall agree a value for a Risk Contingency for that year. This will be used to address agreed risks as required by National Guidance.

1.2 The sum identified as risk contingency will be deducted from the Total Contributions as shown in paragraph 3 of this schedule and the balance used to fund individual schemes.

1.3 Application of this sum will be agreed by the Partnership Board.

1.4 Should there be a balance left in risk contingency for a particular year the Partnership Board will consider and agree one of the options in the 'Underspend section.

Pooled Fund Management

1.5 There will be one pooled fund for the Lewisham Better Care Fund. This will be known as the overall "Lewisham Integrated Pooled Fund."

1.6 The Host Partner for the pooled fund will be the Council and the Pooled Fund Manager, being an officer of the council, is the Strategic Business Partner – Adult Social Care & Health

Overspend

2 The Partnership Board shall consider what action to take in respect of any actual or potential Overspends.

3 The Partnership Board shall, acting reasonably and having taken into consideration all relevant factors (including where appropriate the Better Care Fund Plan and any agreed outcomes and any other budgetary constraints) agree appropriate action in relation to Overspends which may include the following:

3.1 whether there is any action that can be taken in order to contain expenditure;

3.2 whether there are any Underspends that can be vired from any other fund maintained under this Agreement; and

3.3 how any Overspend shall be apportioned between the Partners, such apportionment to be just and equitable taking into consideration all relevant factors.

4 The Partners agree to co-operate fully in order to establish an agreed position in relation to any Overspends.

5 Subject to any continuing obligations under any Service Contract entered into by either Partner, either Partner may give notice to terminate a Service or Individual Scheme where the Scheme Specification provides and where the Service does not form part of the Better Care Fund Plan.

Underspends

6. Where an underspend is projected on a Scheme the Partnership Board will consider and agree one of the following options for the use of the funding released:

- Underspending balance to be carried forward to the next financial year and added to the Pooled Fund for the next financial year
- underspend used to offset or partly offset overspends elsewhere in the programme
- underspend reallocated to support new (or expanded) projects on a temporary basis
- underspend reallocated to support additional expenditure on the Schemes
- underspend to support existing service arrangements before transformational restructuring agreed by the Partners.

SCHEDULE 4 JOINT WORKING OBLIGATIONS

Part 1 – LEAD PARTNER OBLIGATIONS

Terminology used in this Schedule shall have the meaning attributed to it in the NHS Standard Contract save where this Agreement or the context requires otherwise.

1 The Lead Partner shall notify the other Partner if it receives or serves:

1.1 a Change in Control Notification;

1.2 a Notice of an Event of Force Majeure;

1.3 a Contract Query; and/or

1.4 Exception Reports

And provide copies of the same to the other Partner.

2 The Lead Partner shall provide the other Partner with copies of any and all:

2.1 CQUIN Performance Reports;

2.2 Finance and Activity Reports;

2.3 Review Records;

2.4 Remedial Action Plans;

2.5 JI Reports;

2.6 Service Quality Performance Reports;

3 The Lead Partner shall consult with the other Partner before attending:

3.1 an Activity Management Meeting;

3.2 Contract Management Meeting;

3.3 Review Meeting;

and, to the extent the Service Contract permits, raise issues reasonably requested by a Partner at those meetings.

4 The Lead Partner shall not:

4.1 permanently or temporarily withhold or retain monies pursuant to the Withholding and Retaining of Payment Provisions;

4.2 vary any Provider Plans (excluding Remedial Action Plans);

4.3 agree (or vary) the terms of a Joint Investigation or a Joint Activity Review;

4.4 give any approvals under the Service Contract;

4.5 agree to or propose any variation to the Service Contract (including any Schedule or Appendices);

4.6 suspend all or part of the Services;

4.7 serve any notice to terminate the Service Contract (in whole or in part);

4.8 serve any notice;

4.9 agree (or vary) the terms of a Succession Plan;

without the prior approval of the other Partner (acting through the [Partnership Board]) such approval not to be unreasonably withheld or delayed.

5 The Lead Partner shall advise the other Partner of any matter which has been referred for dispute and agree what (if any) matters will require the prior approval of the other Partner as part of that process.

6 The Lead Partner shall notify the other Partner of the outcome of any Dispute that is agreed or determined by Dispute Resolution

7 The Lead Partner shall share copies of any reports submitted by the Service Provider to the Lead Partner pursuant to the Service Contract (including audit reports) with the other Partner.

Part 2 – OBLIGATIONS OF THE OTHER PARTNER

Terminology used in this Schedule shall have the meaning attributed to it in the NHS Standard Contract save where this Agreement or the context requires otherwise.

1 Each Partner shall (at its own cost) provide such cooperation, assistance and support to the Lead Partner (including the provision of data and other information) as is reasonably necessary to enable the Lead Partner to:

1.1 resolve disputes pursuant to a Service Contract;

1.2 comply with its obligations pursuant to a Service Contract and this Agreement;

1.3 ensure continuity and a smooth transfer of any Services that have been suspended, expired or terminated pursuant to the terms of the relevant Service Contract;

2 No Partner shall unreasonably withhold or delay consent requested by the Lead Partner.

3 Each Partner (other than the Lead Partner) shall:

3.1 comply with the requirements imposed on the Lead Partner pursuant to the relevant Service Contract in relation to any information disclosed to the other Partner;

3.2 notify the Lead Partner of any matters that might prevent the Lead Partner from giving any of the warranties set out in a Service Contract or which might cause the Lead Partner to be in breach of warranty.

SCHEDULE 5 – PERFORMANCE ARRANGEMENTS

1.1 The Partnership Board oversees Better Care Fund Performance.

1.2 The Partnership Board will be provided with performance reports and will report to the Health and Wellbeing Board as appropriate.

1.3 The Partnership Board will consider performance against the BCF schemes and identify where action or improvement is needed.

1.4 The national metrics are those which are set out in the BCF national guidance. The BCF national metrics are also regularly reported to the Health and Wellbeing Board. The metrics for 26/27 are set out in the BCF plan which is attached in Schedule 6.

1.5 Returns are submitted to NHS England reporting against the key requirements and conditions of the Fund.

Better Care Fund 2026-27
Narrative return

Introduction and guidance

This return has been designed to enable ICBs and local authorities, working with Health and Wellbeing Boards (HWBs), to submit information which demonstrates how their plans for the Better Care Fund (BCF) meet the national conditions and planning requirements for 2026-27. Completing and submitting the BCF narrative return is a required part of the overall BCF submission process. Planning leads should ensure that all questions within this narrative return are fully addressed.

This year, the length of the narrative return has been reduced. This reflects feedback on the benefits of a more focused BCF assurance process. In completing the return, HWBs, ICBs and local authorities may wish to develop more detailed joint plans for BCF expenditure for their own use and/or draw on other joint plans.

Each question in the return has a suggested length of around a page (around 500 words) and we would generally expect the overall submission to be around 2500 words. These act as a guide to support a more focused assurance process rather than strict limits.

The narrative provided in this return should align with the expenditure plans and the ambitions for the national metrics set out in your BCF excel numerical return.

When completing the narrative return, please use the following documents for guidance and support, these can be found on the BCF Exchange:

- **Planning Principles:** outlines what good practice looks like in relation to each narrative question and aligns with the relevant national conditions.

- **Metrics Handbook:** provides the formal technical specifications for the national metrics within the framework, including the rationale, methodology, required data inputs and worked examples.

Submission Requirements:

- Each HWB area must have its own BCF excel numerical return, but a single narrative BCF return covering multiple HWBs may be submitted where this reflects local integrated working arrangements.

- Each HWB area included in a combined narrative return should provide clarity and state any specific details relevant to the separate HWBs within the narrative

questions (and more words may be required for this than a single HWB return). Local authorities, ICBs and HWBs for each area should formally sign off the shared narrative return and their individual numerical excel BCF return.

- The deadline for completing this narrative return is **19 May 2026.**
- Please submit this return to both: england.bettercarefundteam@nhs.net and your regional better care manager(s).

Submission details

Adapt as necessary	**HWB area 1**	**HWB area 2**
HWB	**Lewisham**	
ICB	**SEL (Lewisham)**	
ICB		
ICB		

1. **Please provide a short statement setting out the rationale for using BCF funding to maximise delivery of integrated and preventative care linked to the relevant areas of neighbourhood health and social care services.**

The Better Care Fund (BCF) is a core enabler of Lewisham's neighbourhood health model, supporting integrated, preventative care and helping residents remain independent for longer. Lewisham has established four neighbourhood teams that proactively support residents with long-term conditions, frailty, and complex needs through integrated, person-centred care. A strong Lifestyle Medicine approach also supports residents to better manage their health and wellbeing, alongside proactive patient lists developed by the Lewisham Population Health Team to identify those who would benefit from early intervention and preventative support.

In 2026-27, the BCF in Lewisham will support four neighbourhood hubs (North, East, West and South), underpinned by Integrated Neighbourhood Teams (INTs). The INTs bring together NHS providers, Mental Health providers, Local Authority and Social Care, Primary Care Networks and VCSE organisations to deliver coordinated care focused on prevention, early intervention, and proactive management of people at risk of deterioration.

Lewisham's 2026–27 BCF Plan has consolidated existing schemes as part of the INT model and introduced two new 'Neighbourhood Health Development' schemes (Schemes 39 and 40) to support the integrated neighbourhood model.

The 2026-2027 Lewisham's BCF Plan responds to key borough pressures including:

Deprivation and health inequalities

Lewisham is the 63rd most deprived local authority in England[1], with significant inequalities in outcomes across communities. The borough has a highly diverse population, with 48.5% of residents from Black and Minority Ethnic communities[2]. The Birmingham Lewisham African Caribbean Health Inequalities Review (2022) identified persistent disparities in access and outcomes for Black African and Black Caribbean residents including higher prevalence of conditions such as hypertension, diabetes and stroke, which are associated with increased risk of frailty, hospital admission and longer-term care needs. BCF funding supports

[1] https://www.observatory.lewisham.gov.uk/wp-content/uploads/2025/09/Picture_of_Lewisham_2025_updated_September_2025.pdf

[2] How life has changed in Lewisham: Census 2021

targeted, neighbourhood-based and VCSE-led interventions to improve early access, prevention, and culturally appropriate care.

BCF Scheme 37 Example: **Lewisham's Main Grants Programme**

Lewisham's new Main Grants Programme (2026–2029) has now incorporated a Neighbourhood Grant Fund. The programme enables VCSE organisations to deliver integrated, neighbourhood-focused services across the borough. A key requirement is to demonstrate inclusive practice and actively reduce inequalities, supporting earlier intervention and improved engagement with underserved communities. The introduction of the VCSE sector role has created an opportunity to test the role within INT delivery, including enabling VCSE attendance and participation in INT multidisciplinary team meetings.

A growing, ageing population with increasing complexity

Lewisham has reviewed intermediate care demand and capacity through the development of its Intermediate Care Strategy (2025–2040)[3] and associated workforce and demand modelling. The review identified a continued increase in demand for both home-based and bed-based intermediate care services, driven by population growth, increasing frailty and complexity of need, with the borough's population aged 65+ projected to increase by 58.2% by 2043.

In line with national "Home First" principles, Lewisham has progressively shifted the balance of provision towards home-based intermediate care, whilst maintaining bed-based capacity for residents requiring intensive rehabilitation and nursing support following hospital discharge. Current home-based provision includes Enablement, Discharge to Assess, Community Therapies, Urgent Community Response and the NHS @ Home virtual ward service.

The strategy sets out plans to further increase home-based intermediate care capacity through the Transfer of Care Hub, with improved multidisciplinary working, greater integration between health and social care teams, and increased use of proactive and preventative community-based interventions to avoid hospital admission and support earlier discharge. This includes strengthening pathways that support rehabilitation and reablement within a person's own home wherever appropriate.

[3] Not yet in the public domain

At the same time, the strategy recognises the ongoing need for bed-based intermediate care for residents who are medically optimised but unable to safely return home immediately. Current provision is for 14 commissioned intermediate care beds with future modelling estimating a requirement for up to 20 beds during summer periods and 23 beds during winter periods by 2040. The strategy commits to optimising the use of bed-based intermediate care as part of a wider rehabilitation continuum, ensuring bed-based provision is targeted towards those with the greatest rehabilitation need and linked to effective step-down and home-based rehabilitation pathways to maximise independence and reduce long-term care dependency.

BCF Scheme 1 Example: Lewisham Step-Down Beds (via Alexander Care Home)

The scheme provides short term support for patients who are medically optimised but require up to 6 weeks bed-based rehabilitation before they can return home. The service provides rehabilitation, reablement, and intensive social care assessment to facilitate recovery and plan for longer term care needs. It provides integrated health and social care support for people leaving hospital who are not yet ready to return home. Co-ordination across hospital teams, social care, community services and primary care enables timely discharge, shared decision-making and ongoing monitoring. The model provides flexible, in borough capacity that supports neighbourhood-based pathways and helps residents remain close to home and family.

A significant unpaid carer population

In 2021, Lewisham had 19,957 unpaid carers, with almost 10,000 providing over 20 hours of care per week[4]. Without early identification and support, carers are at risk of breakdown, increasing demand on statutory services.

BCF Scheme 4 Example: Carers Information, Advice and Support

Following an action in Lewisham's All-Age Unpaid Carer Action Plan 2025-28 to support and identify carers in new care developments, Lewisham's Long Term Conditions INT Assessment has been updated to include carer identification and signposting. By embedding this into routine assessments, it improves wellbeing for both carers and the people they care for, leading to better overall health outcomes.

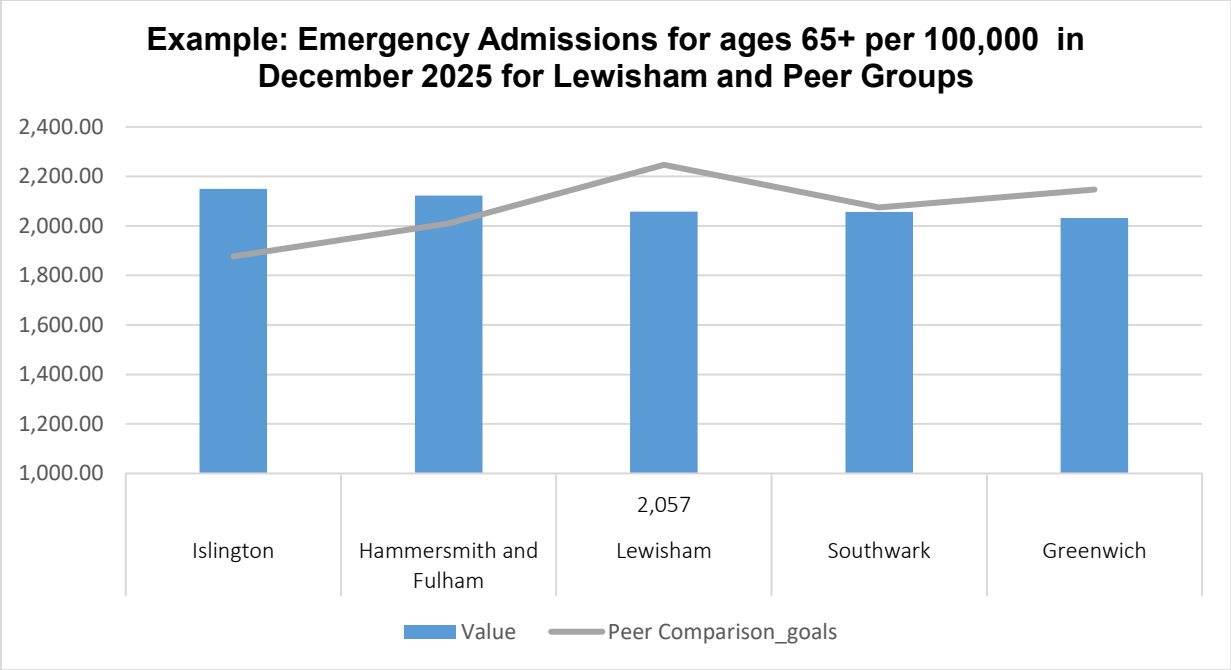
[4] https://lewisham.moderngov.co.uk/documents/s123252/Unpaid%20Carers%20Report.pdf

2. **Please provide a brief explanation of the rationale for how you have set out goals for the metrics of non-elective admissions (for those 65 years old and over) and delayed discharges. Please also set out how you will monitor and drive progress in preventing avoidable long-term care home admissions and improving outcomes from reablement, including through any locally agreed goals for long term admissions to residential care and nursing homes.**

Lewisham's goals for non-elective admissions (65+) and Discharge Ready Date (DRD) have been developed using a combination of historic performance, population trends, benchmarking, and provider planning assumptions. These have been informed through joint discussions with Lewisham and Greenwich NHS Trust, South East London Integrated Care Board performance team, and London Borough of Lewisham.

Reducing Non-Elective Admissions (65+) per 100,000 population

In 2025/26, Lewisham performed well against comparator group averages for non-elective admissions per 100,000 population aged 65+, although performance fluctuated during peak winter periods, reflecting increased frailty demand and system pressures. Demand has continued to rise, driven by growth in the 65+ population and increasing complexity of need.



Provider operational plans from key acute partners, including Lewisham and Greenwich NHS Trust, have been used as an important input into trajectory setting. These have been triangulated against local data and system intelligence

to inform an estimated 5.7% reduction in non-elective admissions. This reflects analysis of historic trends, including seasonal fluctuations during winter months, alongside the anticipated impact of service changes and a potential risk of in-year coding changes (e.g. ED Type 5 activity, existing SDEC shifting into non-elective admissions) which could impact negatively on non-elective admissions.

Our projections are supported by local community-based initiatives, including Integrated Neighbourhood Teams (INTs), which focus on people with long-term conditions and complex needs (with frailty services currently in development), and the Lewisham NHS @ Home Virtual Ward (Lewisham and Greenwich NHS Trust), launched in October 2025. The Virtual Ward is now fully operational, delivering hospital-level care to patients in their own homes. The service will support a minimum of 70 patients per month, with a flexible operating capacity of up to 25 virtual beds per day. Based on current modelling, it is estimated that one acute hospital bed day could be released for every 2.5 virtual ward bed days delivered. In addition, the Virtual Ward now includes a proactive strand focused on high-attending respiratory patients, aiming to reduce non-elective admissions through earlier intervention, prevention, and improved long-term condition management.

Improving Discharge Ready Dates (DRD)

In 2025/26, Lewisham did not achieve the target for discharge ready dates. Performance was particularly challenging between September 2025 and March 2026, with the target achieved in only one month during this period (November 2025). However, improvements in the accuracy and completeness of Discharge Reporting documentation across providers have contributed to an increase in the number of recorded discharge delays. While this has adversely affected reported performance, it provides a more accurate understanding of the factors impacting patient flow and enables the system to identify, monitor and respond more effectively to the causes of delayed discharge.

The Lewisham plan for DRD performance aligns with that of the host provider, reflecting a collective agreement across system partners to align discharge planning approaches with those set out in provider operational plans wherever possible. The improved quality of reporting provides enhanced intelligence to support targeted improvement activity and strengthen system oversight of discharge delays.

Notwithstanding the impact of improved reporting, the system continues to experience significant operational pressures in supporting the discharge of

patients with complex needs. A particular challenge relates to a small cohort of patients with highly complex discharge requirements whose cases can result in prolonged delays. These include patients affected by severe hoarding, where discharge is dependent on specialist property clearance and coordinated multi-agency support. Although relatively few in number, such cases can have a disproportionate impact on overall discharge delay performance.

For 2026/27, the discharge delays target has been revised from 91% to 86% to reflect the increased accuracy and completeness of DRD reporting and to ensure alignment with provider plans. This revision does not reflect a deterioration in underlying system performance, but an increase in accuracy and completeness of recording.

New initiatives have also been introduced to support patients with complex discharge needs (Schemes 39 and 40), providing dedicated support for hoarding clearances to prevent barriers to discharge. These measures are expected to reduce delays associated with complex cases and contribute to improved performance over time.

To further understand the factors driving discharge delays and identify additional opportunities for improvement, the Section 75 Agreement Management Group has commissioned a deep-dive review of discharge delays as a key action for 2026/27. The findings will inform future service development and system-wide improvement activity, supporting continued progress in reducing delays, improving patient flow and achieving better outcomes for residents. Progress and findings from this review will be incorporated into future performance reporting.

We anticipate that these actions will strengthen oversight of discharge performance throughout 2026/27, enabling the system to better understand and address the causes of delay and supporting the development of more ambitious performance targets for 2027/28.

Preventing Long Term Admissions

Lewisham’s goals for long-term care home admissions have been developed through analysis of local demand, available placements, and the ambition to support residents to remain independent at home wherever possible.

Our residential and nursing home placements are subject to monthly panel scrutiny, ensuring a “Home First” approach is applied consistently. Decisions are only agreed where all community alternatives have been fully explored, including care at home and extra care provision.

Progress against locally agreed targets to reduce long-term admissions is monitored monthly, with trend analysis used to challenge variation and support system-wide improvement.

Improving Outcomes from Reablement

Reablement is delivered through the joint Transfer of Care Hub (TOCH), maximising enablement through short-term support, equipment and wraparound care. Outcomes are monitored monthly, including independence at discharge, reduction in ongoing care needs, and avoidance or reduction of care packages. Reablement outcomes are monitored closely, and this will continue throughout 2026-27.

Lewisham's ambition is to increase the proportion of people aged 65+ who remain at home 12 weeks after discharge from hospital into reablement services. There is a transformation project underway to look at in house and purchased enablement care additionally further work to look at the use of Telecare to support people longer. Performance will be monitored monthly, with a focus on pathway-level variation and continuous service improvement.

For all metrics, performance against plan will be monitored, and we will seek to rebase plans if activity shifts are significant.

3. **Please provide a short explanation of the planned impact of BCF funding on achievement of goals**

BCF schemes will support the delivery of integrated health and social care services, enabling system partners to improve capacity, flow and coordination across pathways, directly contributing to the achievement of BCF 26-27 goals. Examples of the impact this will have has been provided through recent reviews of the schemes below:

Example of BCF Scheme	Target Population	Impact for FY 26-27 on the achievement of BCF goals
Lewisham Integrated Medicines Optimisation Service: supporting people to manage their own medicines, enabling them to live their lives as independently as possible (scheme 18)	Older/ frail patients and those with long-term conditions.	Patients are supported to achieve timely discharge from hospital and coordinated community-based support to maximise gain from safe use of medicines and prevent avoidable admissions/ readmissions. In 2026/27, this is expected to deliver approximately 54 avoided admissions and support around 1300 patients with medicines support needs, 420 patients of which are through the Enablement Discharge to Assess (D2A) pathway. Note: over the last couple of years, LIMOS has been trialling new approaches, such as implementing a triage system to manage increasing referral rates without additional service capacity and enable prioritisation of urgent cases to manage risk. In 2025; this resulted in an increase in the proportion of patients seen within two weeks to 28%, and within 30 days to 84%. Implementation of electronic prescribing by independent prescribing pharmacists also improves access to care for older frail patients where prompt medication changes needed to help keep them well and at home.
High Intensity User Service (HIS):	Vulnerable patients,	The service decreases unnecessary hospital admissions and calls to

supports patients who frequently rely on urgent and emergency services (scheme 29)	including those facing homelessness, housing instability, drug and alcohol dependency, mental health challenges, and other complex health needs	emergency services, with patients benefitting from more effective and coordinated care. From a recent sample of patients, there was an average reduction in UEC attendances of 78% during case management (a reduction from 183 pre and 35 during the HIU intervention).
The Community Falls Service: supports people who have had or at risk of falls (scheme 5)	Older people (65+) living with frailty.	The service proactively addresses falls risk and reduces hospital admissions to acute or long-term care services. It supports independence and delays discharge into long-term care. A recent audit found that 47% of patients were no longer classified as a falls risk at 25 weeks, based on the Timed Up and Go assessment. This will continue to be monitored in 26-27.
Transfer of Care Hub (schemes 9-11)	People being discharged from hospital.	Over the last year, we have been trialling new processes and bringing together 6x different health and social care pathways/teams that support hospital discharge and six weeks of enablement under one service. Pre-launch, clients on an enablement pathway would wait between 3 and 5 weeks on an enablement pathway to receive rehabilitation. Post launch, nearly 70% of people are visited by a therapist within 72 hours. The model is also releasing senior social work time to support very complex cases (which are increasing). The model is continuing to develop and test new ways of working.

Collectively, these interventions contribute directly to BCF metric improvement by reducing avoidable non-elective admissions, improving the timeliness of hospital discharge, and increasing the proportion of people who regain independence following reablement. For example, HIS focuses on prevention

reduce crisis escalation through supporting those with complex needs, while the Transfer of Care Hub improves discharge flow and reablement outcomes.

Lewisham will continue to develop and test new models of care throughout 2026–27 to strengthen delivery against these goals, including closer alignment with Integrated Neighbourhood Teams (INTs).

4. **Please outline how ICBs and local authorities have confidence that the services funded through the BCF represent value for money, and how they will seek to raise the productivity of services.**

A comprehensive deep dive review of all 2025–26 BCF schemes has been undertaken to assess performance, cost-effectiveness, and alignment to strategic priorities. This has established a baseline understanding of service demand, activity, outputs and outcomes, which is being used to align delivery to neighbourhood health priorities for 2026–27.

Value for Money

Value for money is assessed through a structured approach across inputs, outputs, and outcomes, including cost, activity, quality, and system impact (e.g. hospital admissions avoided, reduced length of stay, and reduced demand on long-term care). This is underpinned by routine financial and performance monitoring through BCF reporting processes. It is proposed that the *Place System Delivery & Assurance Group* (see Question 5) will review and monitor value for money through these processes throughout 2026-27.

Some examples of a BCF schemes demonstrating value for money have been outlined below:

Scheme 5: Community Falls

The Falls Service supports older people (65+) living with frailty by proactively addressing falls risk and reducing avoidable escalation to acute or long-term care services, supports independence and delays discharge into long-term care. Prevention activity is estimated to save Lewisham and Greenwich NHS Trust **£194,000–£242,500** in acute and hospital care costs, alongside wider benefits including improved quality of life, increased independence, and delayed entry into long-term care. Wider social care savings are also anticipated, in line with evidence from NICE (2013) and Public Health England (2018).

Part of Scheme 36: Care Home Liaison

The purpose of the Care Home Liaison Nurse role is to serve as a liaison and champion for older adult care home providers in Lewisham. The nurse supports the placement assessment period and up to two weeks post-placement, rapidly addressing clinical queries or requests for additional information about potential residents. The role has resulted in length of stay for pathway three clients with the average length of stay decreased from 80 days to an average of 65 days which equates to **£47,340** cost avoidance for the wider system.

Productivity

Partners have identified a range of opportunities to improve productivity across BCF-funded services, all underpinned by the development of neighbourhood-based working as the core operating model for system transformation. The establishment of four neighbourhood models (North, East, South and West) in Lewisham is enabling a fundamental shift towards more proactive, coordinated and place-based care delivered through multidisciplinary teams.

Wider productivity initiatives include:

- **System transformation and discharge improvement**: The Enablement and Discharge Improvement Programme across the council and acute is improving throughput, reducing delays, and supporting more consistent, equitable pathways. This includes earlier therapy-led intervention, reduced reliance on high-cost packages of care (including double-handed care) and extending enablement approaches beyond acute discharge.

- **Digitisation and information flow**: Implementation of a shared Electronic Patient Record (EPR) across acute and community services is improving information sharing, reducing duplication, and supporting more coordinated decision-making.

- **Workforce optimisation**: Improved skill mix and multidisciplinary working, including the Transfer of Care Hub model, is reducing duplication of assessments and ensuring the right professional input at the right time.

Confidence

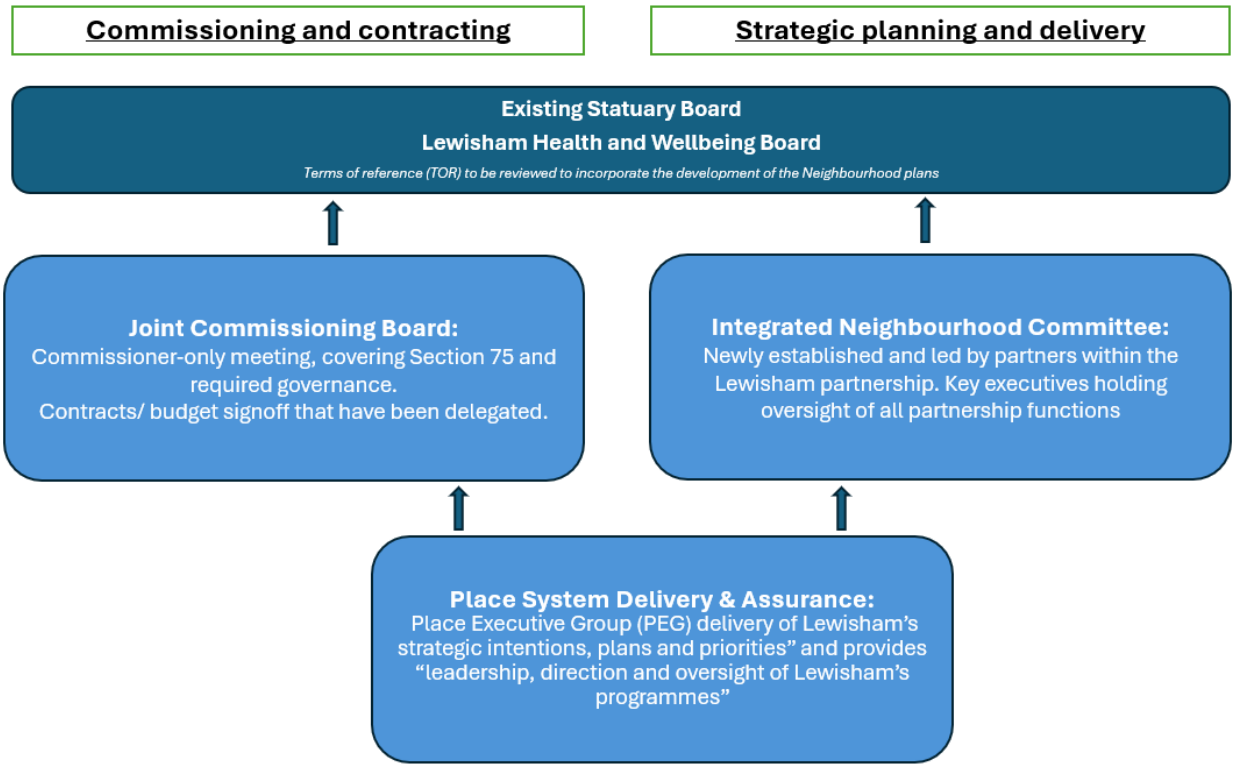
Confidence in value for money and productivity is supported through Lewisham's proposed joint governance arrangements (set out in Question 5). This includes the

proposed *Place System Delivery & Assurance Group* which will oversee financial performance, service delivery, and scheme impact across the portfolio, enabling regular review of performance, identification of underperformance, and corrective action, including reallocation of resources where required.

5. **Please outline your robust joint governance for managing the expenditure of BCF funding, including assessing impact of funding, value for money and continuous improvement.**

In 2026/27, as part of a key milestone in delivering neighbourhood health, the Government's 10 Year Health Plan, London's Target Operating Model, and Care for Change programme will be supported locally through Lewisham's Health & Care Partnership (the Integrator). This will provide the core infrastructure to enable effective integrated neighbourhood team working as it develops, ensuring services are tailored to local community needs and operate seamlessly across organisational boundaries. Its role is critical in creating cohesive, proactive, targeted, and sustainable services that place individuals and communities at the centre of care. In Lewisham, this is a partnership between the London Borough of Lewisham, Lewisham & Greenwich NHS Trust, primary care, South London and Maudsley NHS Foundation Trust, and the voluntary and community sector.

To support this arrangement, Lewisham is developing revised governance arrangements for the Better Care Fund (BCF), based on strong partnership working between the Local Authority and SEL ICB (Lewisham). It is proposed that oversight of the BCF will be provided through a Joint Commissioning Board, incorporating Section 75 requirements and operating under delegated authority from the Health and Wellbeing Board. This will ensure strategic alignment, financial accountability, and shared management of risk across partners. It is intended that the Board will sit alongside the Integrated Neighbourhood Committee, which oversees partnership activity, including the development of neighbourhood health plans aligned to BCF priorities.



As part of the proposed governance arrangements, the *Place System Delivery & Assurance Group* will aim to strengthen operational oversight, with a focus on monitoring delivery, assessing impact, and driving continuous improvement. The Group will include representation from across all BCF schemes including Lewisham & Greenwich Trust. It will facilitate shared learning across schemes, ensure alignment with BCF priorities and metrics, and undertake targeted deep dives into areas of variation or underperformance. This includes identifying system bottlenecks such as delays in hospital discharge and coordinating collective action to address them.

It is proposed that value for money will be assessed through a combination of performance monitoring, outcome measurement, and benchmarking against comparator areas using national datasets through the *Place System Delivery & Assurance Group*. Schemes are expected to demonstrate measurable impact, including reducing avoidable hospital admissions, improving discharge pathways, and supporting independence. Where schemes are not delivering expected outcomes, it is proposed that the *Place System Delivery & Assurance Group* will make recommendations to the Joint Commissioning Board to review, adapt, or reallocate funding to maximise impact.

Continuous improvement is further supported through service transformation and innovation, including a shift towards home-based care, investment in prevention and early intervention, and the use of assistive technology. Engagement with service users and stakeholders is embedded across delivery, ensuring that learning and feedback inform ongoing service development.

Example of Service User Involvement: Unpaid Carers

Carers Forums (In-Person and Online)

The Council and ICB host bi-monthly in-person Carers Forums at Catford Civic Centre. These forums provide a dedicated space for unpaid carers to connect, share feedback, and engage directly with service leads.

In addition, an online evening Carers Forum has been established to support working carers or those unable to travel. This flexible approach has widened participation and improved accessibility.

Forum discussions cover a range of topics related to the carers' offer. For example, carers highlighted the need to strengthen support during the hospital discharge process which has now been implemented.



A dedicated BCF Delivery Manager provides day-to-day oversight of the programme and ensures alignment with wider system priorities and national reporting requirements. This role also supports the proposed new governance arrangements, including the Joint Commissioning Board by providing analysis, recommendations, and escalation of risks where required.

Together, these arrangements ensure transparent decision-making, effective financial management, and a strong focus on impact, value for money, and continuous improvement across the BCF programme.

Better Care Fund Numerical Return

Governance and Sign off

Health and Wellbeing Board:	Lewisham
Confirmation that the plan has been signed off by Health and Wellbeing Board ahead of submission – Plans should be signed off ahead of submission.	No
If no indicate the reasons for the delay.	The BCF Plan was not signed off by the Health and Wellbeing Board prior to submission due to the election. It was signed off at the Health and Wellbeing Board on 15 June 2026.
If no please indicate when the HWB is expected to sign off the plan:	Mon 15/06/2026 << Please enter using the format, DD/MM/YYYY

Submitted by:	Lise Hansen
Role and organisation:	Better Care Fund Delivery Manager
E-mail:	Lise.Hansen@Lewisham.gov.uk
Contact number:	7816076618
Documents submitted (please select from drop down) In addition to this template the HWB are submitting the following:	Narrative

	Role:	al title (e.g. Dr, Cllr, Prof)	First-name:	Surname:	E-mail:	Organisation
Health and wellbeing board chair(s) sign off	Health and wellbeing board chair	Mayor	Liam	Shrivastava	liam.shrivastava@lewisham.gov.uk	
	Health and wellbeing board chair					
Named accountable person	Local authority chief executive	Ms	Jennifer	Daothong	Jennifer.Daothong@Lewisham.gov.uk	
	ICB chief executive 1	Mr	Andrew	Bland	Andrew.Bland@selondonics.nhs.uk	South East London ICB
	ICB chief executive 2 (where required)					
	ICB chief executive 3 (where required)					
Finance sign off	LA section 151 officer	Mr	David	Austin	David.Austin@Lewisham.gov.uk	
	ICB finance director 1	Mr	Chris	Dance	Chris.Dance@selondonics.nhs.uk	South East London ICB (Lewisham and
	ICB finance director 2 (where required)					

Area assurance contacts	Local authority director of adult social services	Ms	Denise	Radley	Denise.Radley@Lewisham.gov.uk	
	DFG lead	Ms	Modester	Anucha	Modester.Anucha@Lewisham.gov.uk	
	ICB place lead 1	Ms	Ceri	Jacob	Ceri.Jacob@selondonics.nhs.uk	South East London (Lewisham)
Please add any additional key contacts who have been responsible for completing the plan	ICB place lead 2 (where required)					
	ICB place lead 3 (where required)					

Better Care Fund 2026-27 Numerical Template

4. Expenditure

Selected Health and Wellbeing Board: Lewisham

	2026-27		
Running Balances	Income	Expenditure	Balance
DFG	£1,952,454	£1,952,454	£0
NHS Minimum Contribution	£33,317,619	£33,317,619	£0
Local Authority Better Care Grant	£18,433,042	£18,433,042	£0
Additional LA Contribution	£797,115	£797,115	£0
Additional NHS Contribution	£0	£0	£0
Total	**£54,500,230**	**£54,500,230**	**£0**

Required spend on adult social care from NHS minimum allocations

	2026-27	
	Minimum required spend	Planned Spend
Adult Social Care services spend from the NHS minimum allocations	£13,004,674	£13,004,674

Number	Category of scheme	Description of scheme	Source of funding	Adult Social Care Spen	Expenditure for 2026-27 (£)
1	Long-term residential/nursing home care	Care Home Beds	Local Authority Better Care Grant	Yes	£2,987,779
2	Wider local support to promote prevention and independence	Care Home Beds - Discharge	NHS Minimum Contribution	Yes	£733,781
3	Wider local support to promote prevention and independence	Care Home Beds - Transitions	Local Authority Better Care Grant	Yes	£3,152,500
4	Wider local support to promote prevention and independence	Carers, Information, Advice & Support	NHS Minimum Contribution	Yes	£658,646
5	Wider local support to promote prevention and independence	Community Falls	NHS Minimum Contribution	No	£408,593
6	Discharge support and infrastructure	Continuing Healthcare	NHS Minimum Contribution	No	£683,166
7	Urgent community response	Coordinated Care - 111	NHS Minimum Contribution	No	£1,815,716
8	Housing related schemes	Housing Adaptations & Support	DFG	Yes	£1,952,454
9	Discharge support and infrastructure	Discharge Flow - TOCH: telephone triage services for urgent healthcare needs available 24 hours per day.	NHS Minimum Contribution	No	£2,061,546
10	Home-based intermediate care (short-term home-based rehabilitation, reablement and	Discharge Staffing -TOCH	Local Authority Better Care Grant	Yes	£1,971,000
11	Discharge support and infrastructure	Discharge Flow - TOCH: staffing costs including Lead Operational Manager, Senior Social Workers and Social Workers.	NHS Minimum Contribution	No	£498,750
12	Home-based intermediate care (short-term home-based rehabilitation, reablement and	Discharge Staffing - Enablement	NHS Minimum Contribution	Yes	£2,834,560

13	Home-based intermediate care (short-term home-based rehabilitation, reablement and	Enablement Staffing (LGT)	NHS Minimum Contribution	No	£2,507,705
14	Assistive technologies and equipment	Equipment & Assistive Technologies	NHS Minimum Contribution	No	£2,113,447
15	Assistive technologies and equipment	Equipment & Assistive Technologies (Linkline)	NHS Minimum Contribution	Yes	£366,874
16	Housing related schemes	Housing Support	NHS Minimum Contribution	No	£354,538
17	Discharge support and infrastructure	Integrated System Support: Integrated support staff supporting the development of Neighbourhood Health in Lewisham (including development of new Frailty INT).	NHS Minimum Contribution	No	£677,845
18	Wider local support to promote prevention and	LIMOS (medication management)	NHS Minimum Contribution	No	£699,145
19	Wider local support to promote prevention and	Neighbourhood Teams	Local Authority Better Care Grant	Yes	£1,253,500
20	Wider local support to promote prevention and	Neighbourhood Teams	NHS Minimum Contribution	Yes	£6,171,556
21	Wider local support to promote prevention and	Neighbourhood Teams	Additional LA Contribution	Yes	£797,115
22	Wider local support to promote prevention and	Neighbourhood Teams	NHS Minimum Contribution	No	£2,694,219
23	Home-based intermediate care (short-term home-based	Packages of Care - Discharge	Local Authority Better Care Grant	Yes	£1,914,000
24	Home-based intermediate care (short-term home-based	Packages of Care - Community	Local Authority Better Care Grant	Yes	£6,079,924
25	Home-based intermediate care (short-term home-based	Packages of Care - Discharge double handed	NHS Minimum Contribution	Yes	£1,239,257
26	Home-based intermediate care (short-term home-based	Packages of Care - Discharge double handed	Local Authority Better Care Grant	Yes	£400,000
27	Home-based intermediate care (short-term home-based	Packages of Care - Enablement	NHS Minimum Contribution	Yes	£1,000,000
28	Discharge support and infrastructure	Therapists/social workers recharge LBL	NHS Minimum Contribution	No	£620,803
29	Wider local support to promote prevention and	High Intensity User Service	NHS Minimum Contribution	No	£168,756
30	Wider local support to promote prevention and	Packages of Care - Community	NHS Minimum Contribution	No	£1,061,598
31	Wider local support to promote prevention and	Primary Care Home Support	NHS Minimum Contribution	No	£966,587
32	Evaluation and enabling integration	Programme Support	NHS Minimum Contribution	No	£121,066
33	Evaluation and enabling integration	Programme Support	Local Authority Better Care Grant	No	£65,839
34	Wider local support to promote prevention and	Self-managed Support	NHS Minimum Contribution	No	£162,290
35	Bed-based intermediate care (short-term bed-based	Step down beds	Local Authority Better Care Grant	Yes	£402,000
36	Discharge support and infrastructure	Voluntary sector hospital discharge	NHS Minimum Contribution	No	£141,357
37	Wider local support to promote prevention and	Voluntary sector prevention	NHS Minimum Contribution	No	£509,000
38	Discharge support and infrastructure	Timely support from hospital	NHS Minimum Contribution	No	£1,886,633
39	Wider local support to promote prevention and independence	Neighbourhood Health Development: includes a new Project Officer role, a handyperson service in Lewisham and funding towards hoarding cleans. This is split across 39 & 40	Local Authority Better Care Grant	Yes	£206,500
40	Wider local support to promote prevention and independence	Neighbourhood Health Development: includes a new Project Officer role, a handyperson service in Lewisham and funding towards hoarding cleans. This is split across 39 & 40	NHS Minimum Contribution	No	£160,185

Better Care Fund 2026-27 Numerical Template

5. Metrics for 2026-27

Selected Health and Wellbeing Board: Lewisham

5.1 Non-Elective admissions

		Apr 25 Actual	May 25 Actual	Jun 25 Actual	Jul 25 Actual	Aug 25 Actual	Sep 25 Actual	Oct 25 Actual	Nov 25 Actual	Dec 25 Actual	Jan 26 Actual	Feb 26 Actual	Mar 26 Actual
Non elective admissions to hospital for people aged 65 and over per 100,000 population	Rate	1,993	2,009	1,736	2,025	1,800	1,832	1,816					
	Number of admissions 65+	620	625	540	630	560	570	565					
	Population of 65+*	31,107	31,107	31,107	31,107	31,107	31,107	31,107					
		Apr 26 Plan	May 26 Plan	Jun 26 Plan	Jul 26 Plan	Aug 26 Plan	Sep 26 Plan	Oct 26 Plan	Nov 26 Plan	Dec 26 Plan	Jan 27 Plan	Feb 27 Plan	Mar 27 Plan
	Rate	1,813	1,884	1,649	1,951	1,855	1,787	1,730	1,668	1,704	1,787	1,620	1,755
	Number of admissions 65+	564	586	513	607	577	556	538	519	530	556	504	546
	Population of 65+	31,107	31,107	31,107	31,107	31,107	31,107	31,107	31,107	31,107	31,107	31,107	31,107

Source: https://digital.nhs.uk/supplementary-information/2025/non-elective-inpatient-spells-at-english-hospitals-occurring-between-01-04-2020-and-30-11-2024-for-patients-aged-18-and-65

5.2 Discharge delays

*Dec Actual onwards are not available at time of publication

	Apr 25 Actual	May 25 Actual	Jun 25 Actual	Jul 25 Actual	Aug 25 Actual	Sep 25 Actual	Oct 25 Actual	Nov 25 Actual	Dec 25 Actual	Jan 26 Actual	Feb 26 Actual	Mar 26 Actual
Average length of discharge delay for all acute adult patients (this calculates the % of patients discharged after their DRD, multiplied by the average number of days)	0.42	0.76	0.67	0.78	0.50	1.08	0.83	0.80				
Proportion of adult patients discharged from acute hospitals on their discharge ready date	94.0%	91.3%	91.2%	91.0%	92.9%	90.7%	90.2%	89.4%				
For those adult patients not discharged on DRD, average number of days from DRD to discharge	7.0	8.7	7.6	8.7	7.1	11.7	8.4	7.6				
	Apr 26 Plan	May 26 Plan	Jun 26 Plan	Jul 26 Plan	Aug 26 Plan	Sep 26 Plan	Oct 26 Plan	Nov 26 Plan	Dec 26 Plan	Jan 27 Plan	Feb 27 Plan	Mar 27 Plan
Average length of discharge delay for all acute adult patients	0.98	0.97	0.97	0.96	0.97	0.96	0.95	0.94	0.93	0.92	0.92	0.92
Proportion of adult patients discharged from acute hospitals on their discharge ready date	85.7%	85.6%	85.6%	85.6%	85.5%	85.5%	85.6%	85.7%	85.7%	85.8%	85.7%	85.7%
For those adult patients not discharged on DRD, average number of days from DRD to discharge	6.85	6.77	6.74	6.70	6.67	6.63	6.60	6.56	6.52	6.48	6.44	6.40

Source: https://www.england.nhs.uk/statistics/statistical-work-areas/discharge-delays/discharge-ready-date/

5.3 Admissions to residential and nursing care homes

		Rolling 12 month total until end of quarter date indicated							
		Actual Ending 31-12-2024	Actual Ending 31-03-2025	Actual Ending 30-06-2025	Actual Ending 30-09-2025	2026-27 Plan Ending 30-06-2026	2026-27 Plan Ending 30-09-2026	2026-27 Plan Ending 31-12-2026	2026-27 Plan Ending 31-03-2027
Long-term admissions to residential and nursing care homes for people aged 65 and over per 100,000 population	Rate	578.6	514.4	488.6	485.4	479.0	475.8	472.6	466.1
	Number of admissions	180	160	152	151	149	148	147	145
	Population of 65+*	31,107	31,107	31,107	31,107	31,107	31,107	31,107	31,107

*Population of people aged 65 and above are based on the latest available mid-year estimates from the ONS

National Condition	Planning requirement	Assurance statement	Yes/No to assurance statement	Where the planning requirement or assurance statement is not met, please note the actions in place towards meeting the requirement	Timeframe for resolution
National Condition 1: effectively support the delivery of integrated and preventative care ICBs and local authorities must develop joint plans, agreed by health and wellbeing boards, outlining how ICBs and local authorities intend to use BCF funding to deliver more integrated and preventative care, linked to the relevant areas of neighbourhood health and social care services.	ICBs and local authorities must have considered how to use the BCF most effectively to support the delivery of more integrated and preventative services, particularly supporting those with more complex health and social care needs. This must include setting out how the funding will be used to develop the quality, efficiency and outcomes from intermediate care.	Named ICB and local authority chief executives and named HWB chair must confirm that BCF expenditure is agreed and aligned with wider strategic objectives for neighbourhood health and social care.	Yes		
	ICBs and local authorities must set out plans that: - show reasonable progress in the metrics of non-elective admissions (for people aged 65 and over) and delayed discharges - show how they will monitor and drive progress in preventing avoidable long-term care home admissions and improving outcomes from reablement - include the specific contribution of BCF-funded services.				
	ICBs and local authorities must demonstrate that their plans for the use of the BCF represent value for money and improve overall productivity				
National Condition 2: comply with expenditure and grant conditions ICBs and local authorities must comply with all national grant and funding conditions and deliver in accordance with their approved return. ICBs must maintain the NHS minimum contribution to adult social care and pool NHS BCF contributions into a section 75 (of the NHS Act 2006) pooled fund.	ICBs and local authorities must pool their designated minimum contribution (in the case of ICB partners) and the Local Authority Better Care Grant and DFG (in the case of local authority partners). ICBs and local authorities are able to voluntarily pool additional funding through the BCF where they consider this is likely to lead to an improvement in the services being funded.				
	The NHS minimum contribution to adult social care must be met and maintained by the ICB in line with the published BCF allocations. This represents an increase of 4.4% in each health and wellbeing board area. Local authorities must comply with the grant conditions of the Local Authority Better Care Grant and the DFG, including the pooling of funding.	ICBs and local authorities confirm compliance with BCF national grant and funding conditions, and that they will deliver in accordance with approved spend and BCF numerical return, including maintaining the NHS minimum contribution to adult social care.	Yes		
		ICBs and local authorities confirm they will pool funds through Section 75 agreements by 30th September 2026.	Yes		
National Condition 3: - effective governance, reporting and engagement ICBs and local authorities must comply and engage with BCF planning, governance and reporting requirements including adherence to any assurance and oversight processes.	ICBs and local authorities must have effective joint governance is in place to ensure local accountability for delivery of outcomes, including reviewing performance against plan objectives and local goals, and taking action if necessary to bring delivery back on track.				
	ICBs, local authorities and health and wellbeing boards are required to engage with BCF reporting, oversight and support processes	ICBs and local authorities confirm full compliance with BCF planning and reporting requirements and will adhere to the BCF oversight and support processes.	Yes		

SCHEDULE 7 – POLICY FOR THE MANAGEMENT OF CONFLICTS OF INTEREST

1. The Council and the ICB (Lewisham) jointly recognise that each operates in a complex practice, policy and political environment and that from time to time this complexity could give rise to situations where the wider interests of one Partner may create an actual or perceived conflict of interest in respect of delivery of the Better Care Fund plan.

2. Both Partners also recognise that the complexity of the environment in which each operates means that it is incumbent on each Partner to ensure that in planning any investment or disinvestment decisions and/or policy or practice changes any potential impact on Better Care Fund plan delivery is considered and appropriate mitigation sought during the planning of change. In so doing, the Partners wish to reduce the likelihood of conflicts of interest arising inadvertently.

3. The Partners undertake to use best endeavours to minimise the risk of any such conflicts arising, and to minimise the adverse impact should such conflicts (actual or perceived) arise. At all times when addressing any actual or perceived conflicts the Partners will have due regard to the terms of this agreement, and the partnership approach underpinning it, and in particular to the General Principles set out in Clause 3.2 of the Agreement.

4. The Authorised Officers will, in the first instance, seek to resolve any actual or perceived conflict of interest that arises during the term of this Agreement through discussion. While this can be managed informally, a record of the actual or perceived conflict, and of the agreed means of resolving, should be kept by the Authorised Officers and reported to the next available Partnership Board meeting for noting.

5. In circumstances the Authorised Officers are unable to resolve the conflict of interest through informal discussion the Dispute Resolution procedure set out at Clause 23 of the Agreement shall be followed.

SCHEDULE 8 – INFORMATION SHARING

INTRODUCTION

1.1 This document is an Information Sharing Protocol (for the purpose of this protocol, the terms data and information are synonymous). The aim of this document is to facilitate appropriate sharing of personal, sensitive and non personal data between the Partner Organisations, so that members of the public receive the services they need.

1.2 Organisations involved in providing services to the public have a legal responsibility to ensure that their use of personal data is lawful, properly controlled and that an individual's rights are respected. This balance between the need to share personal data to provide quality services, and the protection of confidentiality, is often a difficult one to achieve.

1.3 The legal situation regarding the protection and use of personal data can be unclear. This situation may lead to information not being readily available to those who have a genuine need to know in order for them to do their job properly.

1.4 There are fewer constraints on the sharing of non-personal data, that is, data which either does not identify a living individual or, when combined with other information that is in or may come into the organisation's possession, will not identify a living individual.

1.5 Each partner to this protocol should ensure that all of their staff who are affected by it are aware of its contents, and the obligations that the protocol, and any information sharing agreements (ISAs) which are created between the organisations signed up to it, place on them.

1.6 Each partner should also ensure that revisions to the protocol and ISAs raised in it are signed in good time, which should be before any sharing takes place.

SCOPE

1.7 This overarching protocol sets out the principles for information sharing between Partner Organisations.

1.8 This protocol sets out the rules that all people working for or with the Partner Organisations must follow when using and sharing information.

1.9 This Protocol will be further extended to include other public sector, private and voluntary organisations working in Partnership to deliver services.

1.10 The specific purpose for the use and sharing of information will be defined in the Information Agreements, which will be specific to the Partner Organisations sharing that information.

APPENDIX 1 – DATA SHARING PROTOCOL

1.	**Permitted Purposes**	The Partners acknowledge and agree that they will hold and share Shared Personal Data for the purposes of carrying out their respective obligations as described in the Agreement. In particular, the Partners shall use the Shared Personal Data for the purposes of: • commissioning, planning and delivering the Services under the NHS Functions and Health-Related Functions; • managing and administering the Pooled Fund and any Non-Pooled Funds, including financial reporting, audit and oversight; • contract management, quality assurance and performance monitoring of Service Providers delivering the Individual Schemes; • supporting the provision, coordination and continuity of health and social care to Service Users, including assessment, care planning, review and case management; • facilitating safe and timely hospital discharge, reablement and intermediate care pathways; • meeting statutory duties relating to safeguarding of adults and children; • complying with the Better Care Fund Requirements, including reporting, monitoring and evaluation; • responding to requests from regulatory or supervisory bodies where required by Law. Where a request relates to data shared between the parties, each party shall, where reasonably necessary, provide the other with assistance and relevant information without undue delay and within sufficient time to meet statutory deadlines, including Subject Access Requests, within the statutory time limits under UK GDPR. and • any other purpose necessary and proportionate to the performance of this Agreement, provided such purpose is consistent with the Data Protection Legislation.
2.	**Data Retention**	Personal Data shall be retained only for the period necessary to fulfil the purposes of each scheme under the Better Care Fund Plan 2026–27. Where statutory or regulatory retention requirements apply (including the NHS Records Management Code of Practice and applicable local authority statutory retention schedules), these shall take precedence. At the end of the applicable retention period, Personal Data shall be securely deleted or anonymised in a manner that ensures it can no longer be identified, in accordance with the parties' established information governance and secure disposal practices.
3.	**Categories of Personal Data**	See Appendix 2
4.	**Legal Bases for Processing**	The Parties shall process Personal Data in accordance with Article 6(1)(e) UK GDPR, where processing is necessary for the performance of a task carried out in the public interest or in the exercise of official authority. Where Special Category Data is processed, the Parties shall rely on Article 9(2)(h) UK GDPR and Schedule 1, Part 1, Paragraph 2 of the Data Protection Act 2018,

		where processing is necessary for the provision, management or administration of health or social care services.
5.	**Agreed Sharing Mechanisms**	The Partners agree that Shared Personal Data shall only be transmitted, accessed and stored using secure mechanisms that comply with the Data Protection Legislation and each Partner's information governance and security policies. 5.1 The Partners shall only transmit Shared Personal Data using the secure technical and organisational measures agreed between them and set out in this paragraph 5 ("Agreed Sharing Mechanisms"). 5.2 The Agreed Sharing Mechanisms are: (a) secure email systems compliant with the Data Security and Protection Toolkit, including @nhs.net, @nhs.uk, @gov.uk and other accredited secure domains; (b) the shared Electronic Patient Record (EPR) system used by the Partners, with role-based access controls and audit functionality; (c) secure encrypted file transfer solutions approved by both Partners, including multi-factor authentication and time-limited access; (d) digital platforms used by Integrated Neighbourhood Teams for multidisciplinary case discussions, where approved by both Partners' information governance teams; (e) NHS-approved virtual ward and remote monitoring systems used for the delivery of the Services. 5.3 Where Shared Personal Data is accessed through shared systems, each Partner shall ensure that access is restricted to authorised Staff, audit logs are maintained, and system permissions reflect internal policies and professional codes of conduct. 5.4 Where Shared Personal Data is transmitted outside shared systems, the Partners shall ensure that all files are encrypted, Personal Data is minimised, and transfers are recorded in accordance with internal information governance procedures. 5.5 No additional mechanism for transmitting Shared Personal Data shall be used unless approved by both Partners' Data Protection Contacts. 5.6 The Partners shall review the Agreed Sharing Mechanisms annually or sooner where required by a Change in Law, system changes, or recommendations from a Regulatory or Supervisory Body.
6.	**Security**	6.1 Each Partner shall ensure that all Shared Personal Data is protected by appropriate technical and organisational measures in accordance with the Data Protection Legislation and each Partner's information governance and security policies. 6.2 The Partners shall ensure that Shared Personal Data is:

		(a) stored and transmitted securely using the Agreed Sharing Mechanisms; (b) accessed only by authorised Staff for the Permitted Purposes; (c) protected through encryption, access controls and audit logging; (d) held on secure systems that meet the Partners' minimum security standards; and (e) disposed of securely in line with each Partner's retention and disposal policies. 6.3 Each Party shall notify the other without undue delay and in any event within 24 hours of becoming aware of a Personal Data Breach affecting Shared Personal Data. The Parties acknowledge that this notification supports compliance with the requirement to notify the Information Commissioner's Office within 72 hours where applicable.
7.	**Data Quality**	7.1 Each Partner shall take reasonable steps to ensure that all Shared Personal Data is accurate, complete, up to date and relevant for the Permitted Purposes. 7.2 Where a Partner becomes aware that Shared Personal Data it has disclosed is inaccurate or incomplete, it shall notify the other Partner promptly and provide corrected information as soon as reasonably practicable. 7.3 Each Partner shall ensure that appropriate data quality checks, validation processes and audit arrangements are in place for Shared Personal Data held within its systems. 7.4 The Partners shall cooperate to resolve any data quality issues that may affect the delivery of the Services or the accuracy of Shared Personal Data.
8.	**Data Minimisation**	Each Partner shall ensure that the Shared Personal Data are adequate, relevant and limited to what is necessary for the Permitted Purposes. 8.1 Shared Personal Data shall be limited to what is adequate, relevant and necessary for the Permitted Purposes and shall not be excessive. 8.3 Where aggregated, anonymised or pseudonymised data is sufficient, it shall be used in preference to identifiable Personal Data. 8.4 Each Partner shall ensure that Staff are aware of and apply data minimisation principles when accessing, sharing or Processing Shared Personal Data.
9.	**Data Security & Protection Toolkit Assessment**	The Parties must complete and publish an annual information governance assessment in accordance with, and comply with the mandatory requirements of, the NHS Data Security and Protection Toolkit.

10.	**Data Protection Contacts**	**ICB Data Protection Contact:** Name: Katie Nalkin Email: Katie.Nalkin@selondonics.nhs.uk **Council Data Protection Contact:** Name: Stephen Williams Email: dpo@lewisham.gov.uk
11.	**Review Date/Frequency**	The Partners agree to review the Protocol at least annually.

APPENDIX 2 – CATEGORIES OF PERSONAL DATA AND LEGAL BASIS FOR PROCESSING

Data Subjects	Category of data	Legal basis for processing relied upon by the ICB and Council
Service Users, Service User Family or Friends, Other Carers, Referrers, Other NHS or Social Care Staff involved in delivery of Services	Personal Data Full name Date of birth Address, postcode Telephone number(s) Email Address National Insurance Number NHS Number Carer / Guardian contact details Special Category Personal Data referred to below	**[Purpose 1: Oversight of the Services]** Article 6(1)(e) UK GDPR – *processing is necessary for the performance of a task carried out in the public interest or in the exercise of official authority vested in the controller.* The Council and the NHS organisation act as independent controllers in respect of Shared Personal Data processed under this Agreement. Each Party determines its own purposes and lawful basis for processing in accordance with UK GDPR, whilst cooperating as necessary for the Permitted Purposes set out in Appendix 1. Relevant Statutory Functions: - National Health Service Act 2006, including: • Sections 3, 3A and 3B – duties relating to the provision and commissioning of health services; • Section 14Z34–14Z44 – statutory duties of Integrated Care Boards, including planning, performance, quality, and financial oversight; • Section 75 – partnership arrangements between NHS bodies and local authorities. Care Act 2014, including: • Sections 1–2 – promoting individual wellbeing and preventing, reducing or delaying needs; • Sections 8–13 – assessment, eligibility and care planning duties; • Sections 6–7 – cooperation and integration duties between local authorities and NHS bodies;

Data Subjects	Category of data	Legal basis for processing relied upon by the ICB and Council
		• Section 80 – market oversight and quality assurance. Health and Social Care Act 2012, including: • Section 195 – duties of Health and Wellbeing Boards to promote integrated working; • Section 194 – establishment and oversight functions of the Health and Wellbeing Board. Better Care Fund Requirements issued under the NHS Act 2006 and subsequent national guidance, including duties relating to: • performance monitoring; • reporting; • assurance; • oversight of pooled fund arrangements. Local Government Act 1972 and 2000 – general powers relating to governance, scrutiny and performance oversight of local authority functions.
	<u>Special Category Personal Data</u> To the extent that the following contain health data:- Inward and Outward Referrals Assessments and Outcomes Observations Diagnoses Medical histories Test results Prescriptions Treatments Adverse Events Safeguarding information Where necessary for the purposes of delivering services that meets Service Users' needs and assessing equality of access and outcomes:- Racial or ethnic origin Religious or philosophical belief Information concerning sex life or sexual orientation	**<u>[Purpose 1: Oversight of the Services]</u>** Article 9(2)(h) UK GDPR – *processing is necessary for the purposes of… medical diagnosis, the provision of health or social care or treatment or the management of health or social care systems on the basis of domestic law or pursuant to contract with a health professional and subject to the conditions and safeguards referred to in paragraph 3* Article 9(3) UK GDPR – *Personal data referred to in paragraph 1 may be processed for the purposes referred to in point (h) of paragraph 2 when those data are processed by or under the responsibility of a professional subject to the obligation of professional secrecy under domestic law or rules established by national competition bodies or by another person also subject to an obligation of secrecy under Union or domestic law or rules established by national competent bodies.* See also s.10 and Schedule 1 to the Data Protection Act 2018

NHS
South East London

Integrated Care Board

Lambeth Local Care Partnership Committee (Lambeth Together)

Terms of Reference

11 June 2026

1. Introduction

1.1. The NHS South East London Integrated Care Partnership (ICP) Board provides a forum for NHS leaders and local authorities to come together, as equal partners, with important stakeholders from across the system and community. The Partnership sets strategic direction, provides leadership and support of key South East London-wide programmes, and holds system partners to account for delivery.

1.2. The NHS South East London Integrated Care Board (ICB) is an NHS statutory organisation that brings together partners involved in planning and providing NHS services, working together to meet the health needs of the population within South East London and deliver the Integrated Care Partnership's strategy.

1.3. Lambeth Local Care Partnership (LCP) committee [the "committee"] is established as a committee of the NHS South East London ICB and its executive powers are those specifically delegated in these terms of reference. These terms of reference can only be amended by the ICB Board.

1.4. These terms of reference set out the role, responsibilities, membership, and reporting arrangements of the committee under its terms of delegation from the ICB Board.

1.5. All members of staff and members of the ICB are directed to co-operate appropriately with any requests made by the Local Care Partnership committee.

2. Purpose

2.1. **'**Lambeth Together**'** seeks to ensure that partners can design, plan, deliver and evaluate their work together to improve services and outcomes for residents, with a particular focus on tackling health inequalities and ensuring equitable access to care and support for all communities. The partner organisations represented through the core members of the Lambeth Together Care Partnership Board may opt to bring their formal delegations to the decisions of the Board. Lambeth Council will act through the delegated authority of Cabinet and Executive Leads.

2.2. The committee is responsible for the effective discharge and delivery of the Place-based functions as agreed by the ICB. The committee is responsible for ensuring:

a. The Place[1] contribution to the ICB's agreed overall planning processes including the effective planning and delivery of place based services to meet the needs of the local population, with a specific focus on community based care and integration across primary care, community services and social care, managing the place delegated budget, taking action to meet agreed performance, quality and health outcomes, ensuring proactive and effective communication and engagement with local communities and developing the Local Care Partnership to ensure it is able to collaborate and deliver effectively, within the partnership and in its interactions with the wider Integrated Care System (ICS).

b. The Local Care Partnership can secure the delivery of the ICS's strategic and operational plan as it pertains to place, and the core objectives established by the LCP for their population and delegated responsibilities.

c. The Local Care Partnership plays a full role in securing at place the four key national objectives of ICSs, aligned to ICB wide objectives and commitments as appropriate.

d. The representation and participation of the Local Care Partnership in the wider work of the ICS and Integrated Care Board, contributing to the wider objectives and work of the ICS as part of the overall ICS leadership community.

3. Duties

3.1. **Place-based leadership and development:** Responsibility for the overall leadership and development of the Local Care Partnership to ensure it can operate effectively and with maturity, work as a collective and collaborative partnership and secure its delegated responsibilities with appropriate governance and processes.

3.2 **Community engagement:** Responsibility for development, relationship-building activities and meaningful local community and resident engagement to ensure that services are responsive to the needs of all residents. The Local Care Partnership also needs to support the Place Executive Lead to effectively represent the Partnership's views, while also considering the needs of the wider ICS, and fostering strong, ongoing partnerships with local communities to ensure their voices are heard in decision-making.

3.3. **Planning:** Responsibility for ensuring an effective place contribution to ICP/ICB wide strategic and operational planning processes. Ensuring that the Local Care Partnership develops and secures a place based strategic and operational plan to secure agreed outcomes and which is aligned with the Health and Wellbeing strategic plan and underpinned by the Joint Strategic Needs Assessment (JSNA) and a Section 75 agreement. The LCP must ensure the agreed plan is driven by the needs of the local population, uses evidence and feedback from communities and professionals, takes account of national, regional and system level planning requirements and outcomes, and is reflective of and can demonstrate the full engagement and endorsement of the full Local Care Partnership.

3.4. **Delivery:** Responsibility for ensuring the translation of agreed system and place objectives into tangible delivery and implementation plans for the Local Care Partnership. The LCP will ensure the plans are locally responsive, deliver value for money and support quality improvement. The LCP will develop a clear and agreed implementation path, with the resource required whilst ensuring the financial consequences are within the budget of the LCP and made available to enable delivery.

[1] Within Integrated Care Systems (ICSs), 'Place' refers to a smaller geographic footprint, which in most cases is based on local authority boundaries. South East London ICS is made up of six places, co-terminous with the six borough councils.

3.5. **Monitoring and management of delivery:** Responsible for ensuring robust but proportionate mechanisms are in place to support the effective monitoring of delivery, performance and outcomes against plans, evaluation and learning and the identification and implementation of remedial action and risk management where this is required. This should include robust expenditure and action tracking, ensure reporting into the ICS or ICB as required, and ensure local or system discussions are held proactively and transparently to agree actions and secure improvement where necessary.

3.6. **Governance:** Responsible for ensuring good governance is demonstrably secured within and across the Local Care Partnership's functions and activities as part of a systematic accountable organisation that adheres to the ICB's statutory responsibilities and adheres to high standards of public service, accountability and probity (aligned to ICB governance and other requirements). Responsibility for ensuring the LCP complies with all legal requirements, that risks are proactively identified, escalated and managed.

4. Accountabilities, authority and delegation

4.1. The Local Care Partnership Committee is accountable to the Integrated Care Board of the SEL Integrated Care System.

4.2. The partner organisations represented through the core members of the Lambeth Together Care Partnership Board may opt to bring their formal delegations to the decisions of the Board. Lambeth Council will act through the delegated authority of Cabinet and Executive Leads.

5. Membership and attendance

5.1. Core members of the committee will include representatives of the following organisations including specific postholders, where named.

Clinical and Professional Membership

a. Lambeth Together Care Partnership Board Co-Chair - Clinical lead
b. Director of Public Health, Lambeth Council
c. Director of Adult Social Care (DASS), Lambeth Council
d. Corporate Director Children, Families and Education (DCS), Lambeth Council
e. Chair of Lambeth GP Clinical Cabinet
f. Delivery Alliance Clinical and Care Professional Lead(s) x 3

Community Membership

a. Cabinet Member(s) for Healthier Communities and Lambeth Together Care Partnership Board Co-Chair
b. Programme Director, Black Thrive
c. Lambeth Together Lay member
d. Patient & Public Voice representatives x 2

Executive Membership

a. Place Executive Lead - Corporate Director, Integrated Health and Care
b. Executive, Guys and St Thomas' NHS Foundation Trust
c. Executive, Kings College Hospital NHS Foundation Trust

d. Executive, South London and the Maudsley NHS Foundation Trust
e. Managing Director, GP Federation
f. CEO, Age UK

5.2. Non-voting members. Additional members are included in all committee meetings and activity, as key partners of the Lambeth Together Care Partnership, but do not hold a vote and are not part of the quoracy.

a. Lambeth Healthwatch
b. Lambeth Local Medical Committee.

6. Chair of meeting

6.1. The meeting will be co-chaired by the Cabinet Member for Healthier Communities and an LCP clinical lead as appointed by Board members, and subject to regular review.

6.2. At any meeting of the committee the co-chairs shall preside.

6.3. If one co-chair is temporarily absent on the grounds of conflict of interest, the other co-chair shall preside, or, in the case that they also may not, then a person chosen by the committee members shall preside.

7. Quorum and conflict of interest

7.1. The quorum of the committee is at least 50% of core members.

7.2. Each core member has one vote, unless otherwise specified in section 5.1 by virtue of multiples of the same role.

7.3. In the event of quorum not being achieved, matters **deemed by the chair to be 'urgent'** can be considered outside of the meeting via email communication.

7.4. The committee will operate with reference to NHS England guidance and national policy requirements and will abide by the ICB's standards of business conduct. Compliance will be overseen by the chair.

7.5. The committee agrees to enact its responsibilities as set out in these terms of reference in accordance with the Seven Principles of Public Life set out by the Committee on Standards in Public Life (the Nolan Principles).

7.6. Members will be required to declare any interests they may have in accordance with the ICB Conflict of Interest Policy. Members will follow the process and procedures outlined in the policy in instances where conflicts or perceived conflicts arise.

8. Decision-making

8.1. The aim of the committee will be to achieve consensus decision-making wherever possible. If a vote is required, the core members, including the co-chairs, are the voting members of the Local Care Partnership. Core members are expected to have a designated deputy who will attend the formal Local Care Partnership with delegated authority as and when necessary.

8.2. The partner organisations represented through the members of the Lambeth Together Care Partnership Board may opt to bring their formal delegations to the decisions of the

Board. Lambeth Council will act through the delegated authority of Cabinet and Executive Leads.

9. Frequency

9.1. The committee will meet once per quarter (in public) with ability to have a private session as Part B in addition to this.

9.2. All members will be expected to attend all meetings or to provide their apologies in advance should they be unable to attend.

9.3. Members are responsible for identifying a suitable deputy should they be unable to attend a meeting. **Arrangements for deputies'** attendance should be notified in advance to the committee Chair and meeting secretariat.

9.4. Nominated deputies will count towards the meeting quorum as per the protocol specified in the ICS constitution, which means individuals formally acting-up into the post listed in the membership shall count towards quoracy and deputies not formally acting-up shall not.

10. Reporting

10.1. Papers will be made available five working days in advance to allow members to discuss issues with colleagues ahead of the meeting. Members are responsible for seeking appropriate feedback.

10.2. The committee will report on its activities to ICB Board. In addition, an accompanying report will summarise key points of discussion; items recommended for decisions; the key assurance and improvement activities undertaken or coordinated by the committee; and any actions agreed to be implemented.

10.3. The minutes of meetings shall be formally recorded and reported to the NHS ICB Board and made publicly available.

11. Committee support

11.1. The LCP will provide business support to the committee. The meeting secretariat will ensure that draft minutes are shared with the Chair for approval within five working days of the meeting. Draft minutes with the Chair's approval will be circulated to members together with a summary of activities and actions within five working days of the meeting.

12. Review of Arrangements

12.1. The committee shall undertake a self-assessment of its effectiveness on at least an annual basis. This may be facilitated by independent advisors if the committee considers this appropriate or necessary.

NHS
South East London

NHS South East London Integrated Care Board
Charitable Funds Committee

Terms of Reference

Proposed from July 2026

1. Introduction

These Terms of Reference set out the role, responsibilities, membership, and reporting arrangements of the Charitable Funds Committee.

2. Purpose

NHS South East London Integrated Care Board (ICB) is the Corporate Trustee of NHS Greenwich Charitable Funds, Charity number: 1097722. The ICB Board serves as its agent in the administration of the charitable funds. The funds are known in Greenwich as 'Greenwich Healthier Communities Fund'

Declaration of Trust, dated 25 March 2003, as affected by Statutory Instrument 2004 No. 1643 as amended on 17 Apr 2020.

The charitable objects are 'For any charitable purpose or purposes relating to the National Health Service, for the Health & Wellbeing of the residents and staff in the population served by the Royal Borough of Greenwich.'

The Charitable Funds Committee has been established by the Board to provide oversight of the annual plan, and delivery or the grant making strategy.

The Scheme of Reservation and Delegation sets out that role of the Corporate Trustee enacted through the Chief Finance & Compliance Officer, with oversight of the day to day control and management of the charitable funds enacted through the Place Executive Lead (Greenwich).

The key purpose of the Committee is to support these two key roles in helping to govern, manage, regulate and plan the finances, accounts, investments, assets, business and all affairs of the charity, including the authorisation of expenditure.

3. Scope

The scope of the Committee will be the responsibility for charitable funds, where the NHS South East London Integrated Care Board (ICB) is the Corporate Trustee.

4. Duties

Within the budget, priorities and spending criteria determined by the ICB as Trustee and consistent with the requirements of the Charities Act 2016 (or any modification of that Act) to apply the charitable funds in accordance with their respective governing documents.

To ensure that the ICB policies and procedures for charitable funds investments are followed. To make decisions involving the sound investment of charitable funds in a way which both preserves their capital value and produces proper return consistent with prudent investment and ensuring compliance with:

- Trustee Act 2000
- The Charities Act 2016
- Terms of the Funds' Governing documents

To ensure the approval and submission of the annual accounts and Trustees' report in accordance with the Charities Act.

To monitor the ICB's scheme of delegation for expenditure for the levels in accordance with policy and delegated limits.

To monitor income, expenditure and investments in relation to charitable funds.

To receive proposals for major expenditure and to approve charitable fund bids in accordance with the schedule of matters delegated to officers

To consider strategy in relation to the charity and charitable funds, with a view to making recommendations to the ICB Board as Corporate Trustee, ensuring that the outcomes are delivered against the charitable objects.

To ensure appropriate advice is sought in relation to the health & wellbeing needs of the population of Greenwich, so that this informs the strategy and evaluation of impact, being mindful of the inequalities within the borough.

To ensure appropriate evaluation of the Greenwich Healthier Communities Fund grant giving, through independent evaluation partners, and that learning is built into future grant giving approach, as well as shared across SEL ICB to inform the partnership working with Voluntary Sector and Faith Communities across the ICS.

The Committee will determine the strategy and policies for fundraising, including

whether the ICB should undertake major fundraising appeals, establishing the appropriate framework to ensure that any appeal is properly managed. To monitor fundraising performance and compliance with fundraising regulations. This may include fundraising partnership opportunities with Royal Borough of Greenwich, and opportunity to formally collaborate and pool funding using established Section 75 agreement, and develop sustainable funding flows.

To identify and monitor risks in relation to the charity and charitable funds, including investments and grants made.

To ensure that training needs of Committee members are identified and met.

5. Accountabilities, authority and delegation

The Committee is a committee of the ICB Board. The Committee will provide a report to the ICB Board a minimum of once each year.

The Committee shall have the authority to appoint an investment manager to advise it on investment matters and may delegate day-to-day management of some of all of the investments to that investment manager. In exercising this power the Committee must ensure that:

- The scope of the power delegated is clearly set out in writing and communicated with the person or persons who will exercise it.
- There are in place adequate internal controls and procedures which ensure that the power is being exercised properly and prudently.
- It regularly reviews the performance of the person or person's exercising the delegated power.
- Where an investment manager is appointed, that the person is regulated under the Financial Services and Markets Act 2000.
- It establishes and maintains an approved list of counter parties for investment activities.
- It will obtain appropriate professional advice to support its investment activities.
- It will regularly review investments to see if other opportunities or investment managers offer a better return.

The Committee shall also have the authority to appoint a partner organisation that can support the Charity in the formation and implementation of a strategy to distribute funds held. In exercising this power the Committee must ensure that:

- The scope of the power delegated is clearly set out in writing and communicated with the person or persons who will exercise it.

- There are in place adequate internal controls and procedures which ensure that the power is being exercised properly and prudently.

- It regularly reviews the performance of the person or person's exercising the delegated power.

- It will obtain appropriate professional advice to support.

6. Membership and attendance

The core membership of the Committee will be:

- A Non-Executive Director, ICB
- Chief Financial & Compliance Officer, ICB
- Greenwich Place Executive Director, ICB
- Director of Public Health, Royal Borough of Greenwich

In addition, the Committee will have the following in attendance:

- Charity Manager (nominated Greenwich representative to work with Charity Advisor on regular coordination)
- Charity Advisor
- Associate Director of Finance, Greenwich & Lewisham
- Director of Neighbourhood Development, Greenwich, ICB
- Director of Voluntary Sector Collaboration & Partnerships (SE London ICS)
- Subject matter leads will be invited to the Committee as required

7. Chair of meeting

The Committee will be chaired by the Non-Executive Director. In the event that the Non-Executive Director is unable to attend, the Committee will be chaired by one of the Core Members.

If the chair is temporarily absent on the grounds of conflict of interest, then another core member shall preside.

8. Quorum and conflict of interest

The quorum of the Committee is at least 50% of core members.

The Committee will operate with reference to NHS England guidance and national policy requirements and will abide by the ICB's standards of business conduct. Compliance will be overseen by the chair of the Committee.

The Committee agrees to enact its responsibilities as set out in these terms of reference in accordance with the Seven Principles of Public Life set out by the Committee on Standards in Public Life (the Nolan Principles).

Committee members will be required to declare any interests they may have in accordance with the ICB's Conflict of Interest Policy (included within the Standards of Business Conduct Policy). Members will follow the process and procedures outlined in the policy in instances where conflicts or perceived conflicts arise.

9. Decision-making

Where a decision is required, it is expected that this will be reached by consensus. Where a vote is required to decide a matter, each core member may cast a single vote. In the event of equal votes, the chair will have a casting vote.

By exception and outside of formal meetings, decisions may be reached via email, for example approval of a funding bid, providing appropriate information is available to Committee members to consider the recommendation.

10. Frequency

The Committee will meet a minimum of two times over the course of a year.

All members will be expected to attend all meetings or to provide their apologies in advance should they be unable to attend.

Members are responsible for identifying a suitable deputy should they be unable to attend a Committee meeting which needs to be agreed with the chair, and notified to the meeting secretariat, in advance.

Nominated deputies will count towards the meeting quorum if attendance has been agreed by the Committee chair.

11. Reporting

Papers will be made available five working days in advance to allow members to discuss issues with colleagues ahead of the Committee. Members are responsible for seeking appropriate feedback.

Regular updates (at least annually) from the Committee will be provided to the Healthier Greenwich Partnership, so wider system partners are able to contribute to and be informed on the charity's activity and plans.

An annual report will be provided to the ICB Board.

12. Group support

The group will be supported by the Greenwich Place team.

The meeting secretariat will ensure that draft minutes are shared with the chair for approval within three working days of the meeting. Draft minutes with the chair's approval will be circulated to members together with a summary of activities and actions within five working days of the meeting.

13. Review of Arrangements

The Committee shall undertake a self-assessment of its effectiveness on at least an annual basis. This may be facilitated by independent advisors if the Committee considers this appropriate or necessary.

These terms of reference shall be reviewed by the Committee chair on an annual basis, in the context of the self-assessment and any changing business requirements.

SEL and SWL ICB Joint Transition Committee

Chair's Report to the SEL and SWL ICB Boards

1. Purpose

To provide an update on the first Joint Transition Committee, to alert, advise and assure the Board.

2. Executive Summary

The first meeting of the Joint Transition Committee agreed its purpose and reviewed the status of the SEL/SWL ICB change programmes. Overall, the change programmes were reported as being on track. The Committee focused its discussions on the appeals process, requesting more detail on the themes driving the appeals and actions being taken, as well as the impact of the change programmes on the ICBs' business as usual.

3. Key issues to be aware of

Date of Meeting: 7 July 2026	**Chair**: Georgina Fekete

Alert

- There are no issues.

Advise

1) Programme update - Appeals to the Job Matching Process

- Following the completion of the role matching process, the organisation entered an appeal period where staff could challenge the decision made regarding their status. This process is expected to complete by the 17th of July. Both organisations will then commence the selection and recruitment period.
- A process will be put in place to formally capture learning across both ICBs.
- The Committee requested a report on the appeals and rematching processes, the outcome and subsequent learning at its next meeting on 4 August 2026.

2) Programme Update – impact on business-as-usual (BAU)

- Risks to BAU were highlighted including the impact of voluntary redundancy on existing structures, capacity, staff morale and anxiety levels and staff trust in the process.
- Some directorates are already having to manage vacancies and associated capacity pressures resulting from the first round of voluntary redundancy, for example finance and medicines optimisation.
- All Directors are asked to proactively manage gaps in the existing structure due to voluntary redundancy and to identify any interface issues with other directorates.

Assure

1) Defining the scope and ways of working of the Committee

- The Committee's purpose is to provide assurance and oversight of the SEL and SWL ICB Change Programmes for the ICB Boards, per the Model ICB Blueprint v.1.0.
- The responsibilities divide into 2 broad areas: i) oversight of the *structure* as agreed by the Board and ii) oversight of the *process* of implementation (including financial requirements, risks, alignment with HR and other policies, equality impact assessments, timeliness)
- The Committee will recommend to the Board the transfer of agreed functions to providers, assure the delivery of the organisations' transition plans and feed into learning.
- The responsibility for the future operating model remains the responsibility of both ICB Boards.

- The Committee is time-bound. A review will take place in January 2027 to determine if it has delivered its duties with a view to wrapping up at the end of FY 2026/27.
- A Terms of Reference for the Committee has been drafted following the meeting and is contained in the Board documents for the July 2026 meetings.

2) Programme update - Transfer of Functions to providers

- Prior to setting up a Joint Transition Committee, the SEL Transition Committee reviewed the transfer of the estates function (to Oxleas and GSTT) and recommended it proceed to COSoP (Cabinet Office Statement of Practice guidelines for transfer of employees from one public sector employer to another). They also reviewed an early draft of the proposal to transfer the Lewisham Neighbourhood Development Team to Lewisham and Greenwich Trust (as the Integrator partnership host). The proposal will be further reviewed in August and other proposed services to be transferred will be first reviewed in draft by the Committee in September 2026.

4. Recommendation

The Board is asked to note the items the Committee 'advises' above.

Prepared by:

Georgina Fekete, Chair, Ben Luscombe, SRO SWL ICB and Ceri Jacobs, SRO SEL ICB

13 July 2026

South East London Integrated Care System

NHS South East London

Board Meeting in Public

Title	**Performance Report**					
Meeting date	22 July 2026	Agenda item Number	9	Paper Enclosure Ref	H	
Author	ICB Risk and Assurance and ICB Performance teams					
Executive lead	Annabel Appleby, Acting Joint Director of Planning and Director – Acute and Specialised Services					
Paper is for:	Update	x	Discussion	x	Decision	
Purpose of paper	The report provides the Board with a summary of performance across a range of national performance metrics. It is intended to ensure Board members are appraised of progress against key operational planning commitments and understand the areas of challenge, risk and improvement focus. The report supports the Board's oversight of delivery by setting out the performance position together with a summary analysis across a range of key performance areas, including urgent and emergency care, cancer, referral to treatment, diagnostics, primary care, mental health, community services and continuing healthcare. Where performance is below trajectory, the paper outlines the agreed recovery actions and improvement trajectories.					
Summary of main points	The report shows progress in a number of areas. The planning trajectory for the percentage of 12-hour delays from arrival in A&E was met in March 2026. This means that the target was achieved for 5 out of 6 months of the second half of 2025/26. April saw further reductions in the number of 12 hour waits and acute bed occupancy. Performance against the faster diagnosis standard broadly remained in line with the agreed trajectory during 2025/26, with some monthly fluctuations, and the 80% target was achieved at year end. SEL also performs well on use of a faecal immunochemical test (FIT) to support referral for lower gastrointestinal (LGI) suspected cancer. GP appointment volumes remained above plan in March 2026, with good uptake of *Pharmacy First* clinical conditions and hypertension services in community pharmacy. The health check targets for people with a learning disability and autistic people was consistently above planned trajectory and the year end target was achieved.					

	National CHC performance standards are being met in most months in south east London for the proportion of CHC assessments completed within 28 days and the number of incomplete referrals over 12 weeks. There has been improvement in Talking Therapies performance with targets, covering improvement, recovery and access, achieved at the end of 2025/26. At the same time, significant pressures remain. These include in referral to treatment times (RTT), cancer, diagnostics, urgent and emergency care, mental health crisis presentations and long waits for inpatient beds, waits for community paediatrics, and the number of Learning Disability and Autism (LDA) inpatients. Some challenges also persist in securing mental health access targets. There is an enhanced focus on the changes needed to optimise the management of long waiters, and the number of 65-week-waits has reduced significantly from a peak in the first six months of 2025/26. The paper outlines the recovery actions underway across these areas. Note: The format and metrics covered in this report are being revised in line with the requirements of the 2026/27 Operational Plan expectations. This will be reflected in future versions of this report.					
Potential conflicts of Interest	None identified.					
Relevant to these boroughs	Bexley	x	Bromley	x	Lewisham	x
	Greenwich	x	Lambeth	x	Southwark	x
Equalities Impact	Not directly applicable to the production of this paper.					
Financial Impact	Not directly applicable to the production of this paper.					
Public Patient Engagement	Not directly applicable to the production of this paper.					
Committee engagement	ICB Executive, 9 July 2026					
Recommendation	The Board is asked to note the update and consider any implications for system assurance and delivery oversight.					

South East London Integrated Care System

NHS South East London

SEL System Performance Summary July 2026

SEL ICB Board - 22 July 2026

South East London Integrated Care System

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Summary of YTD position – July 2026 (1 of 3):

Area	YTD summary and key issues
Urgent & Emergency Care (UEC)	• Performance remained broadly in line with improvement trajectories for the 4-hour performance standard during the first three quarters of 2025/26. However, during the final quarter, performance has been below planned levels and we did not achieve the national expectation at year end. This is reflective of the continued pressures in emergency departments. April is showing an improved position, and the published SEL 'footprint' performance, which includes activity from stand alone Urgent Treatment Centres, was 77.1%, an improvement on the 75.5% reported in March. • The target for the percentage of 12-hour delays (from arrival) was met for five out of six months during the second half of 2025/26. April saw a small reductions in the number of 12 hour waits in EDs and acute bed occupancy. An increase in the percentage of patients discharged on their Discharge Ready Date (DRD) dates was also reported in April. Whilst these improvements are positive, it is too early to tell if improved performance will continue. • Demand, capacity and flow challenges continue across physical and mental health, with ongoing improvement work across the care pathway. This includes front door management and use of alternatives to emergency departments, implementation of 'Criteria to Admit' (CTA) across sites and in-hospital management, including same day emergency care and improving lengths of stay for patients.
Cancer	• System level performance against the 28 day Faster Diagnosis Standard (FDS) broadly remained in line with the agreed trajectory during 2025/26, with some monthly fluctuations, and the 80% target was achieved at year end. The backlog position has also improved • The SEL system continues to perform well on use of a faecal immunochemical test (FIT) to support referral for lower gastrointestinal (LGI) suspected cancer. The proportion of LGI referrals accompaniment by a FIT result increased each month during 25/26 and remains above the planned trajectory. • 62-day performance, however, remained a challenge for the system during 2025/26. The final month of the year saw a strong performance improvement however the 75% target was not achieved. • The system focus continues to be improving pathway efficiency, inter-trust transfers and treatment capacity. Focused support is being provided to address service and provider level specialty challenges.
Referral to Treatment Times (RTT)	• South east London achieved the planned improvement trajectory for referral to treatment under 18 weeks during 2025/26. However, long waiters remain in a number of challenged specialties. • 65-week-waits rose during the first half of 2025/26. There is an enhanced focus in SEL on the system and process changes needed to optimise the management of long waiters. The number of 65-week-waits fell substantially during the second half of 2025/26 and in March, there were 64 65-week-waits which is down from a peak of 792 in August 2025. RTT transformation is a major focus in 2026/27
Diagnostics	• Diagnostic performance remains an area of significant system challenge. Performance during 2025/26 was affected by sustained pressure on key modalities such as non-obstetric ultrasound, MRI, echocardiography and audiology. • Recovery plans are being actioned with additional capacity through insourcing and outsourcing, and the implementation of clinical decision support tools to assist with demand management. There is a continued focus on maximising the available capacity in the SEL Community Diagnostic Centres and reviewing the consistency of the direct access offer across SEL.

Summary of YTD position – July 2026 (2 of 3):

Area	YTD summary and key issues
Mental health including crisis and flow	• Mental health services continue to experience significant pressure, particularly across crisis pathways, with high mental health presentations to EDs and demand for admissions. • The number of out of area placements fell during the second half of 2025/26 with a large reduction in the final month of the year due to the resolution of a known data quality issue. The final position was 10 against a year end trajectory of 4. • All providers are delivering internal flow improvement plans, and system-wide work is underway to reduce ED delays and support timely discharge, focusing on reducing length of stay, purposeful admission, stepping down patients and providing alternatives to admissions where appropriate. • The urgent Children and Young People (CYP) eating disorders waiting time standard was met in March 2026, however the routine target was missed again. This is being driven by staffing issues. • SEL Talking Therapy performance improved in March 2026 and all Talking Therapy targets, covering improvement, recovery and access, were achieved. • Performance has remained below the planned trajectories for the number of people accessing perinatal and children and young people service. Perinatal Access and CYP Access. Actions are in place to support improvement in these areas.
Primary care access	• Appointment levels have exceeded plan during 2025/26, noting some variation in performance above and below monthly planned trajectories. Appointments totalled 850,382 in March 2026 , which is above the operating plan trajectory of 844,075. • Support for practices to meet online consultation requirements and to deliver total triage is ongoing. This helps ensure that patients are directed to the right appointment, in the most appropriate setting, with the most suitable practitioner to meet their needs. • A focused support and training offer is being developed in collaboration with the Workforce Development Hub to help practices meet the 2026/27 national contractual target of ensuring 90% of clinically urgent appointments are seen on the same day within the practice.
Community pharmacy: pharmacy first clinical consultations, hypertension and oral contraception	• Take-up of Pharmacy First, hypertension and contraception services has been strong across most boroughs, with 92% of SEL community pharmacies providing all three services as of March 2026. Total service activity has surpassed the operating plan targets every month in 2025/26. • During 2025/26, there have been approximately 99k consultations for the seven clinical conditions, approximately 148k for hypertension consultations (blood pressure and ambulatory monitoring), and approximately 27k oral contraception consultations (initiation and ongoing supply) up to February 2026. There have also been approximately 15k emergency hormonal contraception consultations after going live on 29th October 2025. • Ongoing work is focused on improving referral pathways, data quality and promoting services in community pharmacy.

Summary of YTD position – July 2026 (3 of 3):

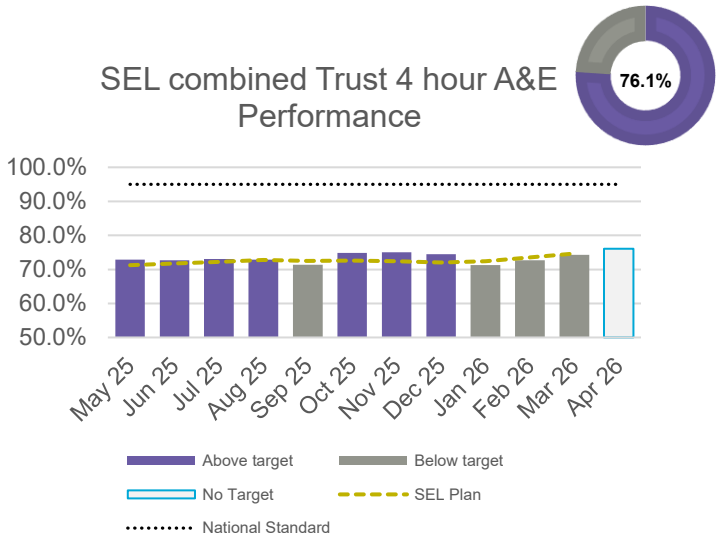
Area	YTD summary and key issues
Community waits and urgent community response (UCR)	• SEL UCR services continue to meet or exceed waiting time targets, though delivery of the referral rate target continues to present a challenge. Work continues with providers and referrers to establish changes to be made to current services to enable them to better respond to referrals and to increase the number of referrals. • Community wait lists remain high and the number of people waiting over 52 weeks has remained above trajectory during 2025/26 and continues to be a significant challenge particularly in community paediatrics, driven primarily by neurodiversity waits. There was a significant increase in September 2025 due to a change in the way that one provider is reporting ADHD and ASD. Providers have identified actions to tackle the backlogs and progress has been made in addressing the number of long waits at a provider service level. An SEL wide approach will be developed to transform current delivery models based on population need and demand trends.
Virtual ward	• SEL Virtual Ward capacity was below plan for April. Average utilisation was, however, above plan at 83% against a target of 80%. Whilst a number of Providers reported utilisation of over 100%, three Providers had lower performance due to data gaps in returns for individual wards. • An SEL wide acuity tool to manage patient admission into virtual wards has now been developed and agreed through the SEL Virtual Ward Community of Practice. This will ensure that all adult services are using the same criteria to assess the complexity of patients and appropriateness to admit to a VW bed. Further work will be done with Children's Services Leads to develop a CYP tool to standardise access to Paediatric Virtual Wards.
Continuing healthcare (CHC)	• CHC performance improved across 2024/25 and into 2025/26, with delivery of the national standards being met most months for the proportion of CHC assessments completed within 28 days and the number of incomplete referrals over 12 weeks. • April 2026 performance (using local reporting) against the number of referrals completed within the 28-day timeframe was 85% which remains above the national target of 80%. The number of incomplete referrals over 12 weeks remained at zero which is in line with the national standard.
Learning disability and autism	• Admissions in April and May were lower than in previous months and discharges have been achieved so SEL is currently within the Q1 target number of people admitted to hospital. Ongoing use of the Dynamic Support Register, alongside Care Education Treatment Reviews (CETRs), remains critical to reducing admission risk through earlier community intervention and ensuring the right care and support are in place. Out of Area Placements (OAPs) in acute mental health units are being reviewed to repatriate or discharge people to the community. • Annual Health Check completion remained strong at the end of March 2026, and the target was achieved across south east London and in all SEL boroughs individually. The target during 2026/27 has been updated to also focus on the number of people receiving an action plan as well as a health check.

Urgent & Emergency Care

South East London Integrated Care System

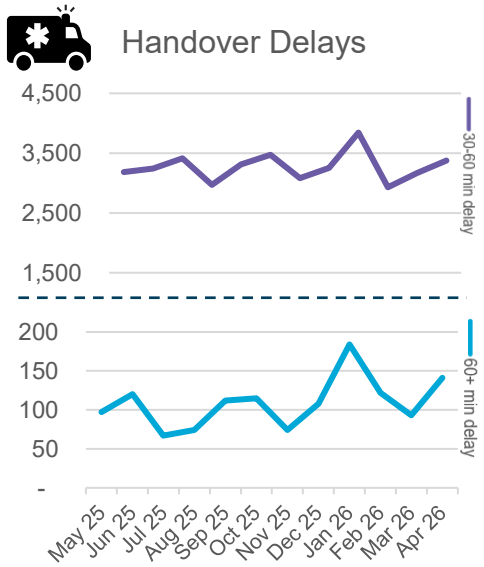
Notes and Issues

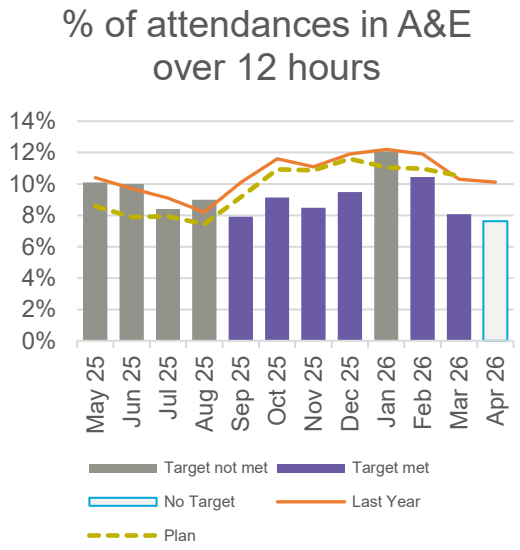
- ED performance: April SitRep data showed performance of 76.1% across the three acute Trusts. Published performance for SEL in April (including standalone Urgent Treatment Centres activity) was 77.1%, an improvement on the 75.5% reported in March.
- 30-60 minute ambulance handovers increased again in April with 3,376 delays compared to 3,168 in March. The number of 60+ minute waits increased in April with 141 reported.
- The percentage of 12 hours waits (from arrival) reduced in April.
- Bed occupancy levels fell slightly compared with the previous month. The target for the percentage of patients discharged on their discharge ready day (DRD) was missed in March with reported performance of 89.3% vs. 92% target.
- SEL missed the target set for average LoS post DRD with reported performance of 8.2 days against a target of 7.4 days.



Recovery Actions

- Local systems/providers have plans in place for 2026/27 to support the delivery of agreed improvement trajectories. Improvement actions continue to focus on:
  - Front door management – use of alternatives to ED, ED triage and streaming, redirection, use of admission avoidance, Mental Health crisis pathway, hospital handovers.
  - In-hospital management – same day emergency care, length of stay improvement (FLOW)
- Continued focus on reducing discharge delays.
- All Trusts have completed an assessment against the Model ED requirements to identify any areas that may require improvement.
- Better Care Fund (BCF) plans have been submitted to NHS England for review. The borough-based plans include schemes to support safe, effective and efficient discharge.

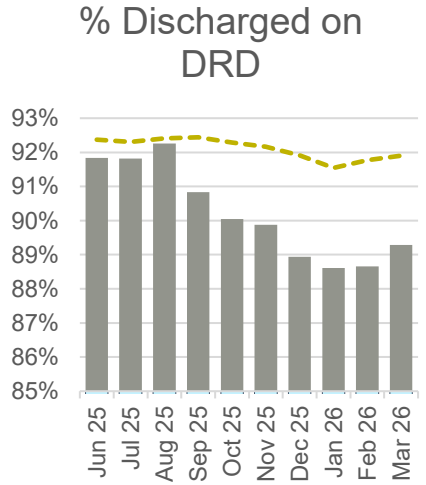


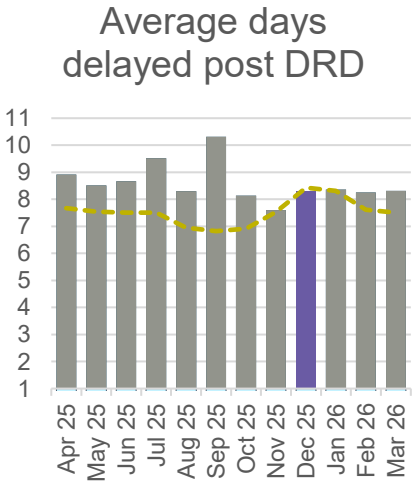




94.8% of G&A beds are currently occupied

88% at GSTT
98% at KCH
97% at LGT





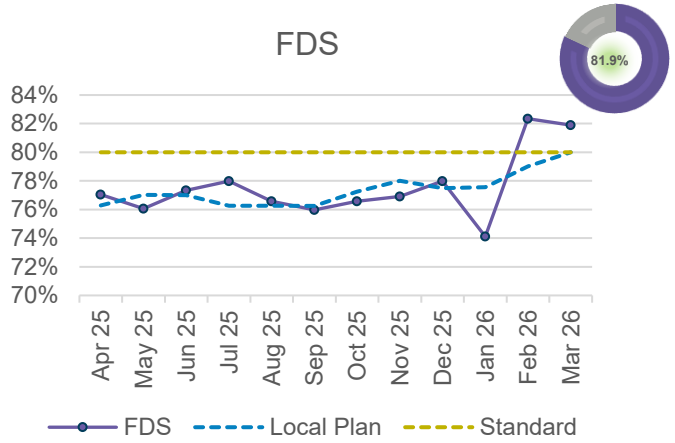
Linked to Risk: 663

South East London Integrated Care System

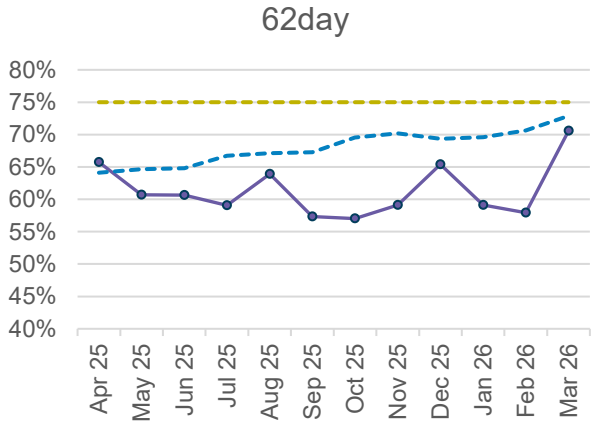
Cancer

Notes and Issues

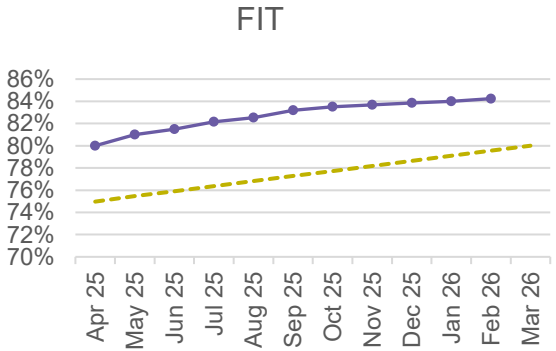
- Faster Diagnosis Standard (FDS) performance has finished the year above trajectory target.
- 62-day performance remains a challenge for the system. Timely and effective Inter-Trust Transfer are a critical focus to improve performance, along with treatment capacity. Specific drivers are the lung and urology pathways. However, strong improvement was seen at the end of the year, with the final trajectory being missed by 2.5%.
- Backlog position has now improved and is more in line with national expectations.
- The system continues to perform well against the Faecal Immunochemical Test (FIT) targets. FIT is used to support referral for suspected lower gastrointestinal cancer.



Faster Diagnosis Standard (Standard Target 80%)
Percentage of patients receiving a communication of diagnosis for cancer or a ruling out of cancer, or a decision to treat if made before a communication of diagnosis within 28 days



(Standard Target 75%)
% of patients with first treatment within 62 days of urgent GP referral

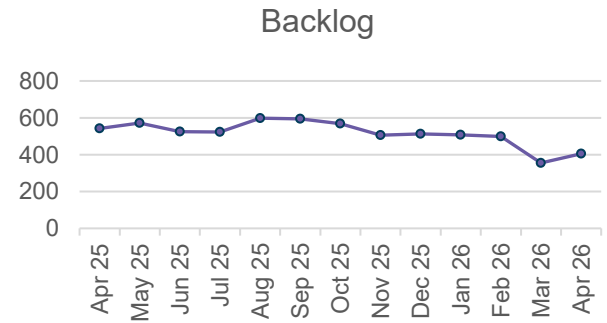


Percentage of lower gastrointestinal two week wait (fast track) cancer referrals accompanied by a **faecal immunochemical test** result, with the result recorded either in the twenty-one days leading up to the referral, or in the fourteen days after the referral

Recovery Actions

- Streamline cancer pathways and optimise diagnostics.
- Ensure timely communication of diagnoses and rule-outs within 28 days.
- Promote utilisation of rapid diagnostic clinics, FIT testing, teledermatology, and personalised stratified follow-up.
- Improve early diagnosis, patient experience, and resource utilisation.
- Proactively mitigate potential risks related to workforce, technology systems, and industrial action.
- Participate in national trials and programs to contribute to advancements in cancer detection and management
- Allocate necessary funding to support key actions.
- Patient Tracking List (PTL) validation and review including clinical PTL review.
- Increased theatre capacity.
- Cancer recognised as priority pathway for available capacity.

Please note that this is the only metric on this page that is not included in the Sytem's operational plan for 25/26. We have included this for information purposes.



Cancer 62 day pathways waiting 63 days or more after an urgent suspected cancer referral at the end of the reporting period

Linked to Risk: 663

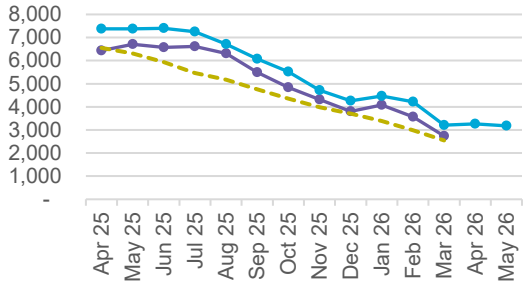
Referral to Treatment: Long Waiters

Latest Published month	Mar 26
Previous Published month	Feb 26
Latest weekly data	10/05/26
Reporting on:	SEL Trusts
Plan	-------

Notes and Issues

- The main focus for 2026/27 will be delivering improvement in the overall 18-week performance, with the intention to return to constitutional standards of 92% by March 29
- Alongside this, oversight of long waiters continues with a shift in focus to 52-week waiters.
- While 65 ww breaches improved significantly at the end of 25/26, there has been an uptick in waiters at the beginning of 26/27.

52 week waits



There are currently **3,179** patients at SEL Trusts who have been waiting more than 52 weeks to start treatment.

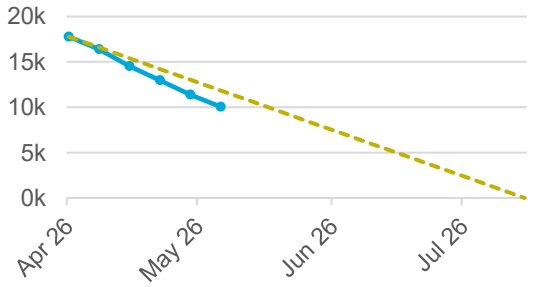
RTT Patients still waiting Mar 26

	52+	65+	78+	104+
This month	2,739	64	2	-
Plan	× 2,547	× -	× 0	✓ 0
Last month	▼ -828 (-30.2%)	▼ 152 (-237.5%	▼ 21 (-1050.0%	◀▶ 0 ()
Latest week	3,179	220	12	1

Recovery Actions

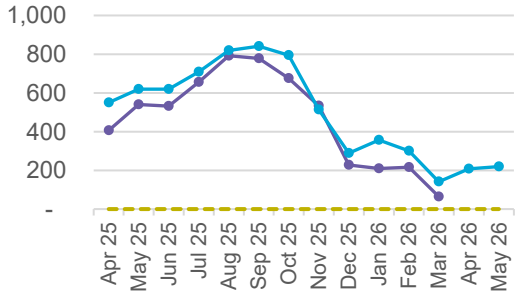
- Service transformation will need to be imbedded in services over the course of 26/27 to make constitutional standards deliverable and sustainable.
- Reductions are expected in overall Patient Tracking List (PTL) size through
  - Validation
  - Pathways changes
  - Additional capacity through outsourcing and insourcing
- Strategic Investment Funds are being used to support delivery.

52+ Cohort

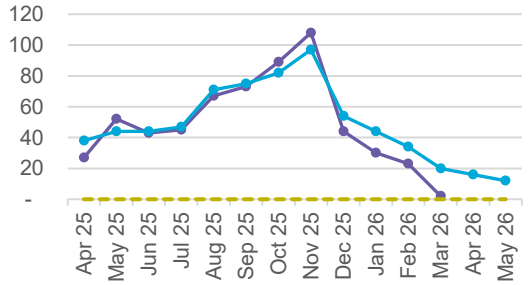


52+ Cohort: (total pathways which will breach 52-weeks by 30/06/2026 if not seen)

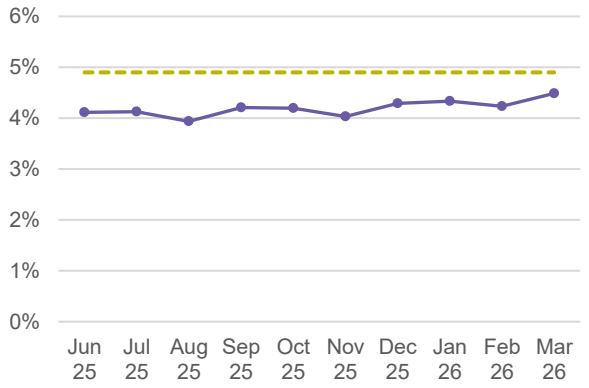
65 week waits



78 week waits



PIFU



Linked to Risk: 663

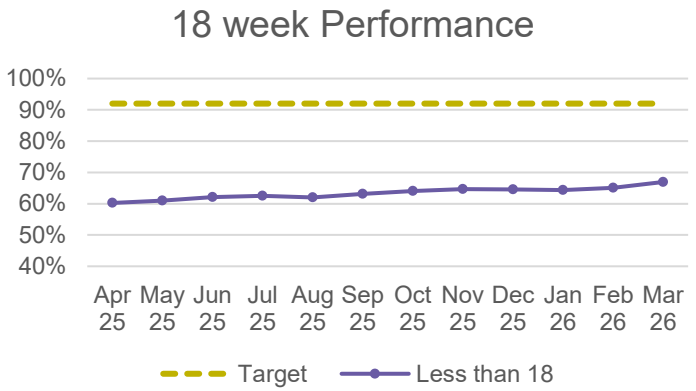
Referral to Treatment: Demand Management

South East London Integrated Care System

Latest Published month	Mar 26
Previous Published month	Feb 26
Latest weekly data	10/05/26
Reporting on:	SEL Trusts
Plan	

Notes and Issues

- South east London achieved the planned improvement trajectory for referral to treatment under 18 weeks during 2025/26.
- Trusts have been set organisation specific targets for improvements in the 18-wk performance in 2026/27, as milestones to delivering the constitutional standard of 92% by March 29.
- Reductions in total PTL size during 2026/27 has also been included in ICB targets for the Medium Term Plan.
- Advice and Guidance (A&G) through electronic referral continues to perform well, with improved provision and timely responses. However, the diversion rate has continued to decline, which may reflect changes in referral or triage behaviour.
- There is an ongoing focus on triage which is required to improve provision and identify opportunities for improving the rate of appropriate diversion.

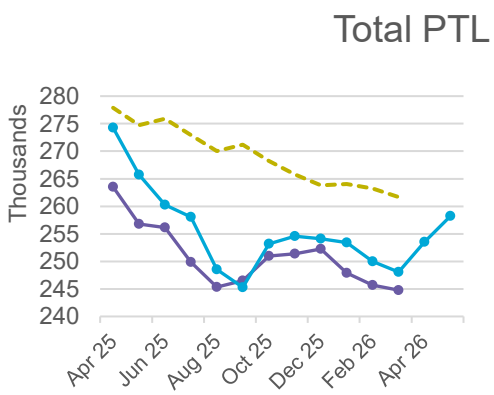


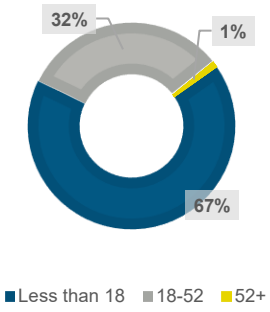
Recovery Actions

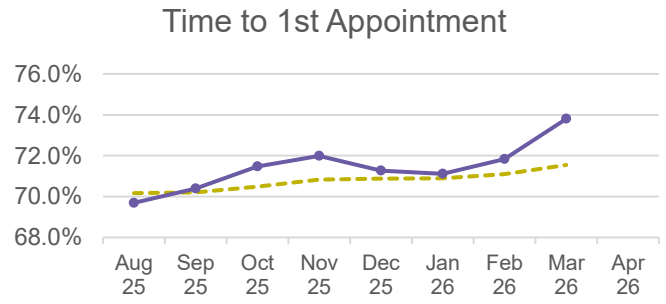
- Improved use of advice services and a priority focus on increasing and improving triage as the most evidence-based intervention (EBI) for demand management.
- Outpatient transformation, including straight to test, to improve waiting times at the beginning of RTT pathways.
- The focus on A&G, triage, booking processes, improving Did Not Attend (DNA) rates and scaling Patient Initiated Follow Up (PIFU) will continue during 2026/27.

RTT Patients still waiting Mar 26

	Total	<18 ww	18 week perf
This month	244,778	163,860	66.94%
Plan	✓ 277,882		× 92%
Last month	▼ -923 (-0.4%)	▲ 3889 (2.4%)	▲ 1.8% (2.7%)
Latest week	258,253	173,681	67.25%







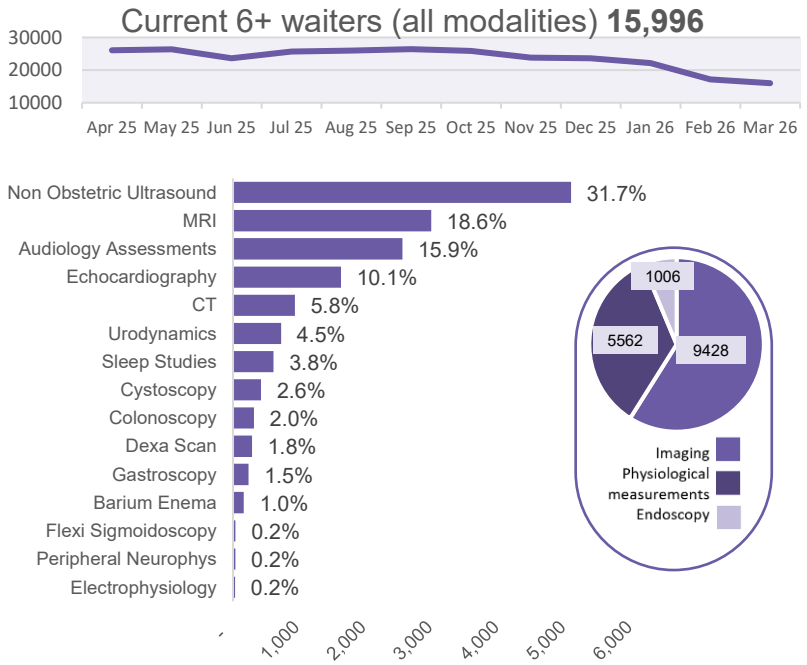
DEMAND MANAGEMENT METRICS	Current	Mar-25	National	Trend
eRS Advice & Guidance				
Provision	42%	40%	nk	▲
Responded in <48hrs	63%	61%	65%	▲
Diversion Rate	74%	79%	65%	▬
Consultant Connect				
Provision	67	69	n/a	▼
Answer Rate (calls)	53%	43%	64%	▲
Answer Rate (messages)	99%	99%	99%	▲
Diversion Rate (calls)	19%	20%	61%	▬
Diversion Rate (messages)	69%	68%	50%	▬
Referral Triage				
Provision	39.3%	27.9%	16-68%*	▲
Responded in <48hrs	56%	64%	nk	▼
Diversion Rate	15%	13%	14%	▲
**range of London systems. SEL is 2nd highest in London*				

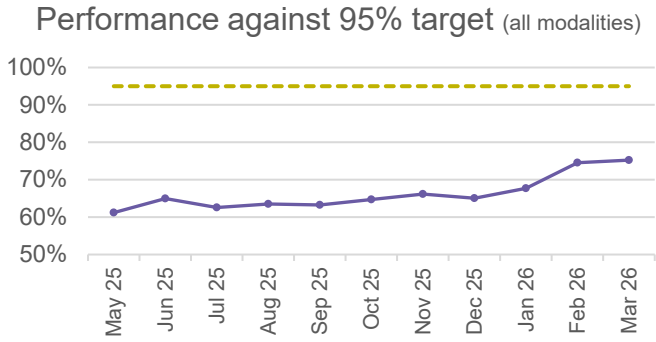
Linked to Risk: 663

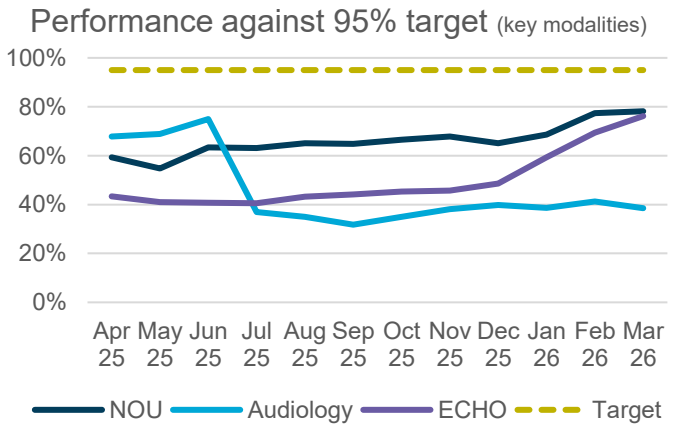
Diagnostics

Notes and Issues

- Interim targets have been set for trusts as milestones to delivering the constitutional standard of less than 1% of patients waiting more than 6 weeks for diagnostic by March 29.
- In addition, ICBs have been set activity targets for diagnostic activity to support the delivery of the diagnostic target and 18-week performance.
- Key modalities that remain challenged include non-obstetric ultrasound (NOUS), MRI, echocardiology and audiology
- There have been reductions in the number of patients waiting more the 13 weeks. However, more work is required in this area.







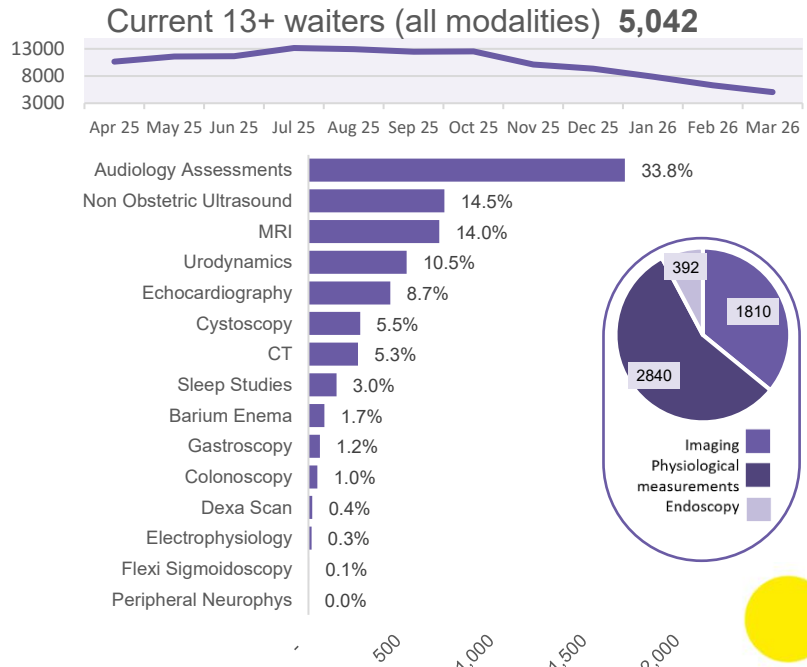
25/26 CDC Activity Mar 26 (cumulative)

Diagnostic Tests	Activity
Non-obstetric Ultrasound	14,343
Magnetic Resonance Imaging	12,142
Computed Tomography	12,012
Other Imaging	7,176
Other tests	2,992
Other Respiratory	2,650

Diagnostic Tests	Activity
Electrocardiography	1,300
Echocardiography	1,257
Sleep Studies	1,054
Other Physiology	167
Other Pathology	-

Recovery Actions

- Clinical and administrative validation of the overall diagnostic PTL.
- Implementing a clinical decision support tool to assist with demand management.
- Additional capacity through maximising on-site capacity, in- and outsourcing.
- As part of the operational planning process local trajectories for further reducing 13-week waiters were agreed.
- The Acute Provider Collaborative is leading SEL wide demand and capacity reviews for imaging as part of their work on system sustainability.
- Maximising the available capacity at the Community Diagnostic Centres and reviewing the consistency of the Direct Access offer across SEL



Linked to Risk: 663

Mental Health

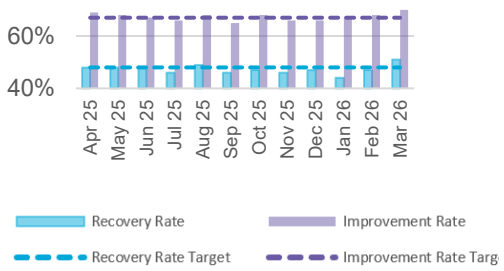
Actual
Target

Notes and Issues

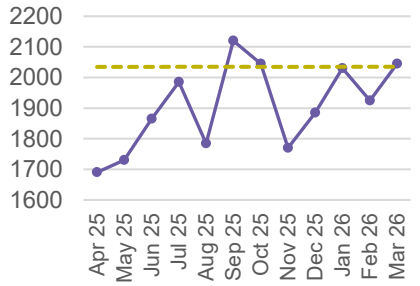
- The waiting times target for urgent Children and Young People (CYP) eating disorder referrals was met in March but performance continues to fall below target for routine referrals. As previously reported, the under-performance is driven by staffing issues. The position is expected to improve in Q1 2026/27.
- CYP access performance remains below target in March, although there has been another small increase in reported access. There has been a change in how one of the trusts is reporting CYP ADHD and ASD which has a more significant impact on overall performance across SEL than anticipated.
- The number of people completing a course of Talking Therapies treatment increased in March with reported performance above the monthly target. Both the improvement and recovery targets were met in this reporting month.
- Perinatal access continues to perform below trajectory in March with reported performance of 1,755 against a year-end target of 1,918.
- Q4 SMI PHC performance has been published and shows an increase in the number of completed health checks. However, performance is below the year-end trajectory.

Talking Therapies (IAPT)

Improvement[1] & Recovery[2] Rates



Number Discharged

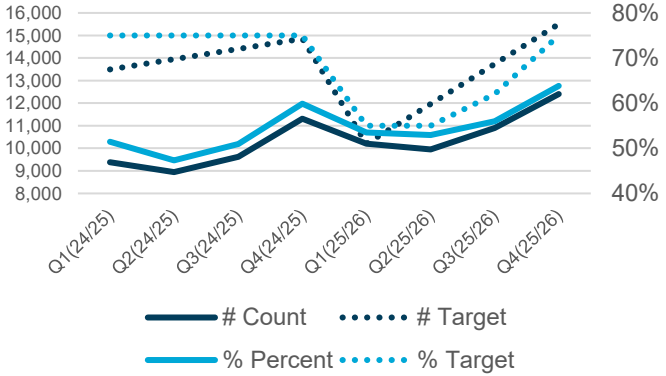


1. Reliable improvement rate for those completing a course of treatment.

2. Reliable recovery rate for those completing a course of treatment and meeting caseness

3. Number of patients discharged having received at least 2 treatment appointments in the reporting period

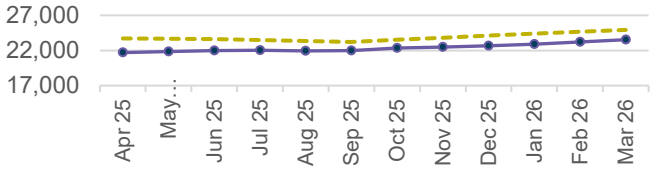
SMI Physical Health Checks



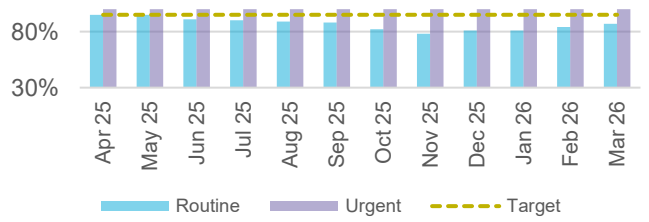
Recovery Actions

- Continued support available to ensure all providers can submit data.
- Data Quality Improvement Plans (DQIPs) embedded in the contracts for the two major mental health providers in south east London. DQIPs are reviewed and updated regularly.
- Local improvement plans will be reviewed for 2026/27 to ensure actions are in place to increase the number of Physical Health Checks undertaken for people with SMI.
- All Talking Therapies services and Individual Placement and Support (IPS) services are implementing delivery plans to support performance improvement against the targets set for 2026/27.

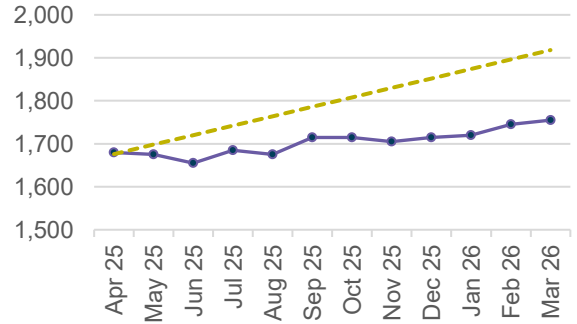
No. young people accessing NHS funded MH services



CYP Eating Disorders: % achieving standard



Number of people in contact with perinatal services

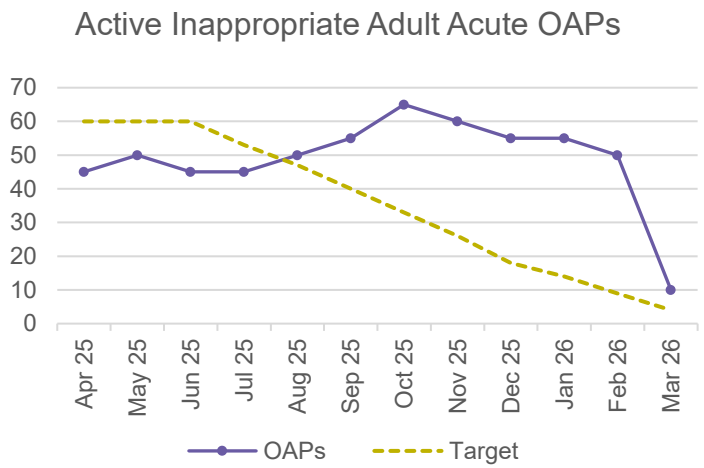


Linked to Risk: 663

Mental Health Crisis & Flow

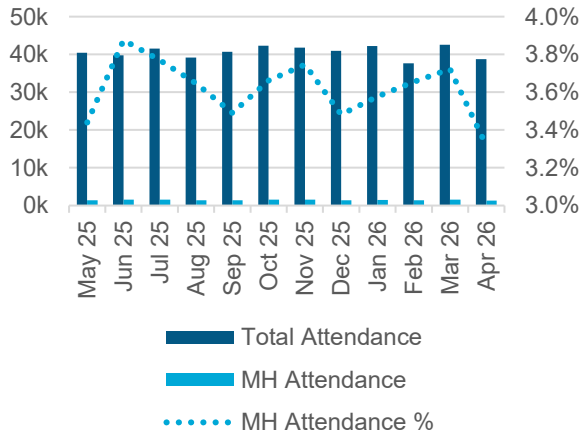
Notes and Issues

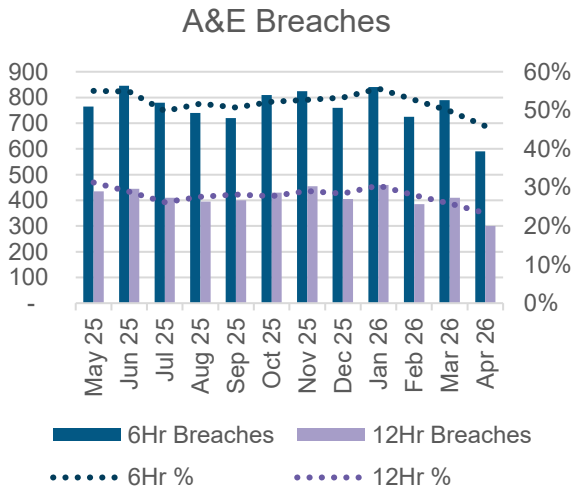
- Mental Health emergency pressures continue with some very challenging days for providers again reported over recent months.
- A significant reduction in Out of Area Placements (OAPs) was reported in March; the SEL position is now 10 active placements against a year-end plan of 4. The reduction is due to the resolution of a known data quality issue.
- A&E data shows that the proportion of MH presentations in ED in April reduced slightly to around 3.3%. 46% of MH patients waited more than 6 hours in ED and 23% more than 12 hours.
- A&E breaches remain disproportionately high for MH patients. A reduction in the number of 6 and 12 hour waits for mental health patients was seen in April
- SEL met the length of stay target in February with performance of 48 days against a target of 52.2.

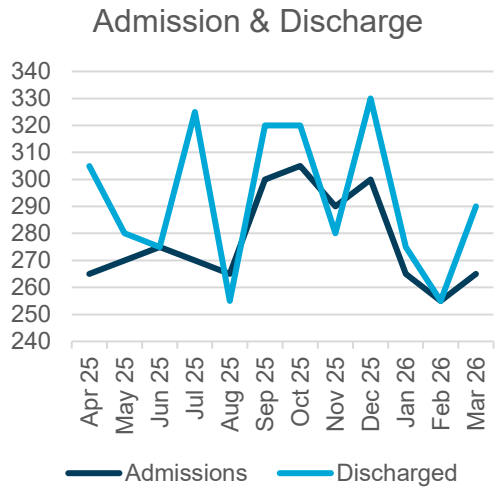


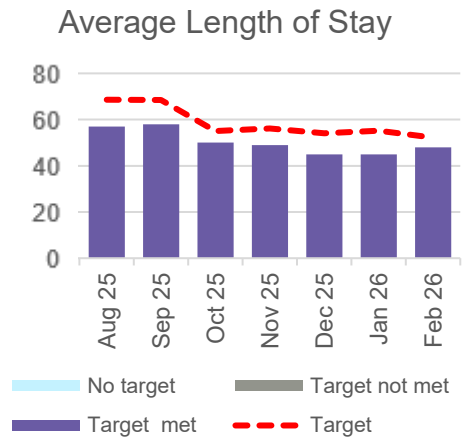
Recovery Actions

- There continues to be a focus from all system partners on expediting discharges for those patients that are clinically ready and reducing the number of long delays in ED for MH patients
- Mental health providers continue to deliver their internal flow improvement plans, focusing on reducing length of stay, purposeful admission, stepping down patients and providing alternatives to admissions where appropriate.
- Continued focus on reducing the number of long delays in ED for MH patients.
- Enhanced monthly monitoring of OAPs commenced in April with ICBs co-ordinating a system wide manual submission.









Linked to Risk: 663

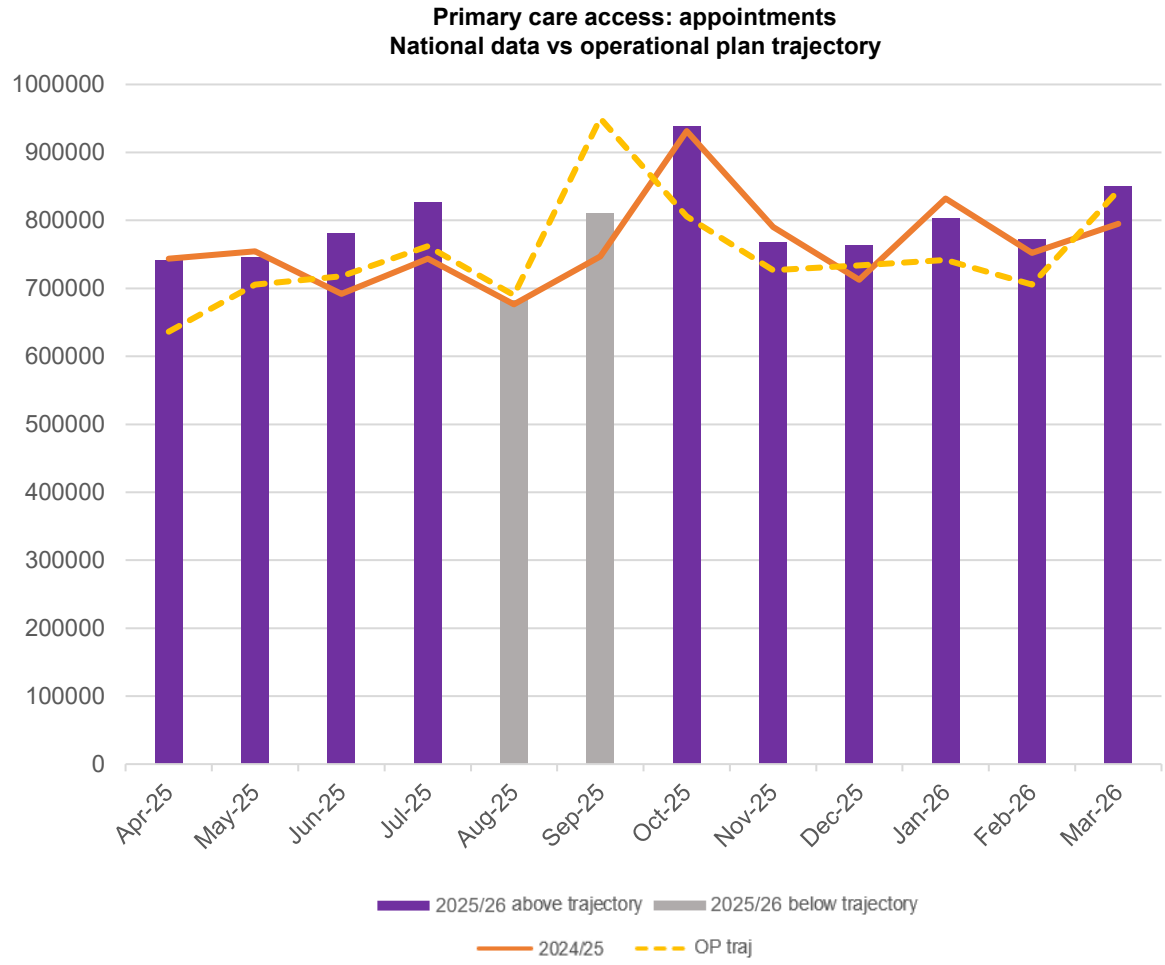
Primary care access (1 of 2)

Notes and Issues

- Appointments totalled 850,382 in March 2026 against the operating plan target of 844,075.
- The analysis of the latest available patient experience data has been completed, and practices with the highest levels of unwarranted variation have been identified. Place-based teams continue to lead on this work, prioritising which practices require support and agreeing targeted interventions.

Recovery actions

- A practical support offer for general practice sustainability has been agreed, building on the sector's views of what is needed now and in the future. The offer aims to promote equity of provision, access, experience, and outcomes across general practice.
- Support for practices to meet online consultation requirements and to deliver total triage is ongoing. This helps ensure that patients are directed to the right appointment, in the most appropriate setting, with the most suitable practitioner to meet their needs.
- A focused support and training offer is being developed in collaboration with the Workforce Development Hub to help practices meet the national contractual target of ensuring 90% of clinically urgent appointments are seen on the same day within the practice. This will be delivered through a programme of webinars and targeted learning opportunities, supporting practices to understand the requirement and how to demonstrate compliance through appropriate recording and coding.



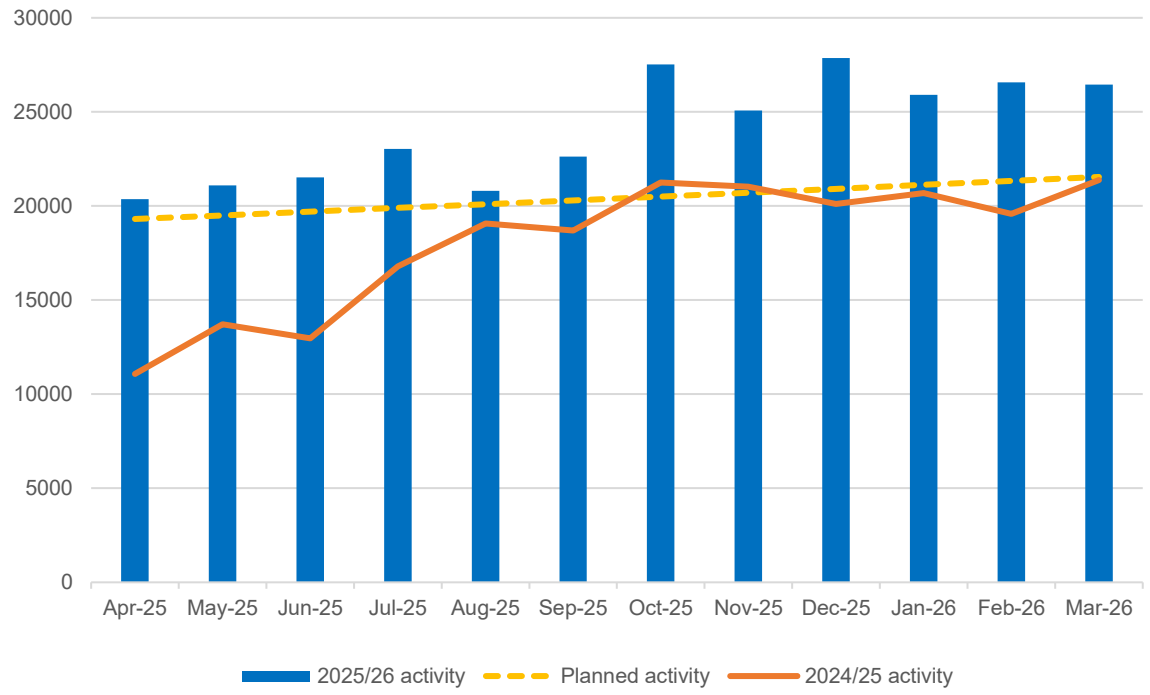
Linked risk: 664 Lew 528

Community Pharmacy: Pharmacy First Clinical Consultations, Hypertension and Oral Contraception

Notes and Issues

- In March 26, 308 pharmacies (of 321) were providing Pharmacy First services, with 306 providing hypertension screening and 304 providing oral contraception
- During 2025/26:
  - 99,163 clinical pathway consultation have been conducted by SEL community pharmacies. This is a 49% increase from 2024/25.
  - 147,792 hypertension consultations have been conducted by SEL community pharmacies. This is an 8% increase from 2024/25.
  - 26,927 contraception consultations have been conducted by SEL community pharmacies (initiation and ongoing consultations only). This is a 107% increase from 2024/25.
- Updated oral contraception service, which include emergency hormonal contraception (EHC), went live on 29 October. There have been 14,983 EHC consultations across SEL since the service being available.
- In March 26, 298 SEL pharmacies were providing all 3 advance services and qualified for additional monthly threshold payments.
- Antimicrobial Stewardship: of all clinical pathway consultations from Mar 24-Mar 26, 66% resulted in an antibiotic supply. Data specific to urinary tract infection (UTI) treatment indicates no concerning increase in antibiotic supply from community pharmacy.
- The number of GP practices referring into the services has increased compared to previous months. The latest data (February) shows that 17 GP practices have not referred into any of the three services. This is an improvement from 21 in December 2025.

SEL: Total number of consultations (hypertension, oral contraception & Pharmacy First clinical pathways)



Note: The chart/data on this page uses indicative management information from the NHS BSA – Manage Your Service (MYS).

Recovery Actions

- Sharing of resources, webinars and toolkit roll out for practices and pharmacies to improve referrals to community pharmacy for all three services.
- Improving data sharing across teams, to drive referrals from practices.
- Digital resources being promoted to increase uptake for contraception consultations in community pharmacy e.g. messaging from GP practices
- Digital IT facilitators supporting GP practices to increase use of referral tools available to Community Pharmacy
- Working closely with SEL Local Pharmaceutical Committee (LPC) to support and provide training to community pharmacy, in particular those that are not already providing all three services
- Community Pharmacy Neighbourhood Leads (CPNL) proactively engaging with local pharmacies to support with provision of the three services.
- Shared learning between peers; often via borough Pharmacy Network Meetings.

UCR and community waits

Notes and Issues

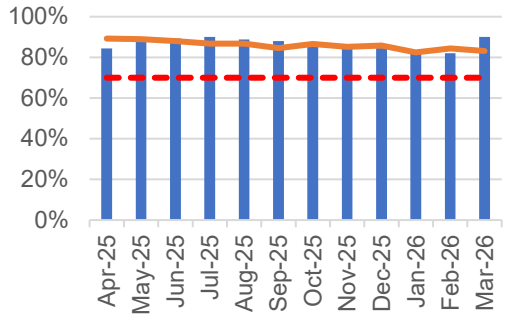
- March Urgent Community Response (UCR) performance data shows SEL providers exceeding the 2- hour and 2-day response standards.
- The latest complete month of reporting (February 2026) for standardised rate of referral was 98 per 100,000 population. This is below the target of 180.
- The total number of patients on the Community Services waiting list (for services in scope) was 37,415 – an increase of 650 on the previous month.
- Of the total number of patients waiting, 22,831 (61%) have been waiting less than 18 weeks for a first appointment.
- Services contributing most to overall wait numbers are: Community Paediatrics (44%), MSK (9%), Podiatry/Podiatric Surgery (9%) and Paed SLT (7%).

Long waiters:

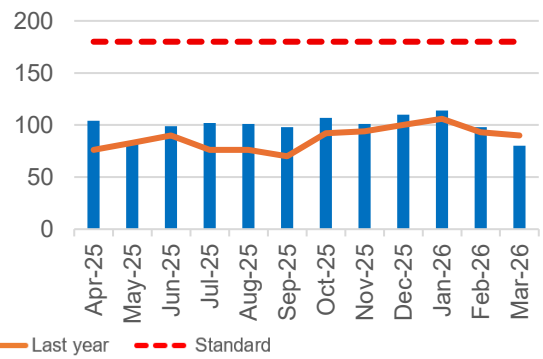
- The number of patients waiting over 52 weeks for a first appointment has increased from 6,433 to 6,523. Of the 5,163 patients waiting 52-104 weeks 5,093 (98.6%) were in Community Paediatrics, with fewer than 10 patients across Looked after Children (LAC), Paediatric Speech and Language Therapy (SLT) and Paed Physio. In adult services 60 were in Podiatry/Podiatric Surgery, with small numbers in Physiotherapy and MSK.
- The number of patients waiting over 104 weeks increased from 1,273 to 1,360. This is primarily driven by Community Paediatrics, which accounts for 99.8% of the list.

Linked to Risk: 663

Combined SEL Trust level UCR performance (Published data)**

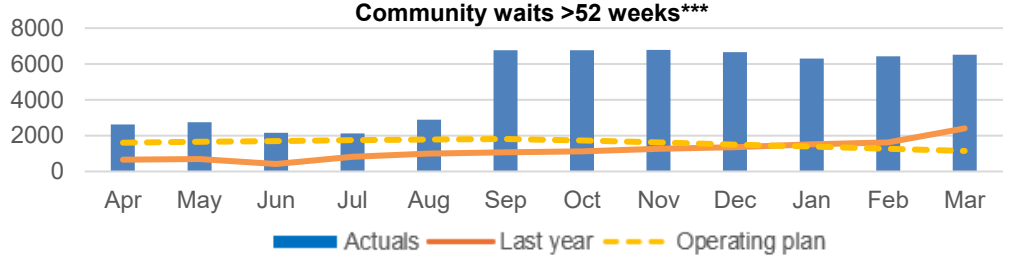


Standardised rate of referral per 100,000 population**



**Latest month is provisional data which excludes GSTT

Community waits >52 weeks***



***April 24 to Feb 25 data excludes GSTT.

Note: Some service data is only published at national level to prevent risk of disclosing sensitive information.

SEL Waiting List Breakdown (March 26)

Weeks	Number of waiters
0-1 weeks	**3,093**
>1-2 weeks	**2,916**
>2-4 weeks	**4,271**
>4-12 weeks	**9,625**
>12-18 weeks	**2,926**
>18-52 weeks	**8,051**
>52-104 weeks	**5,163**
>104 weeks	**1,360**

Recovery Actions

UCR:

- SEL UCR services continue to meet or exceed compliance targets,
- Work has commenced with Provider (supported by the Community Services Providers Network CPN)) to identify all 2- hour crisis response activity across community services to better understand the gap between SEL reported standardised referral rate and the national target. National guidance changed in 24/25 to allow for all 2-hour response activity to be counted as UCR. Ensuring this happens in south east London will provide a more accurate performance position.

Community wait list:

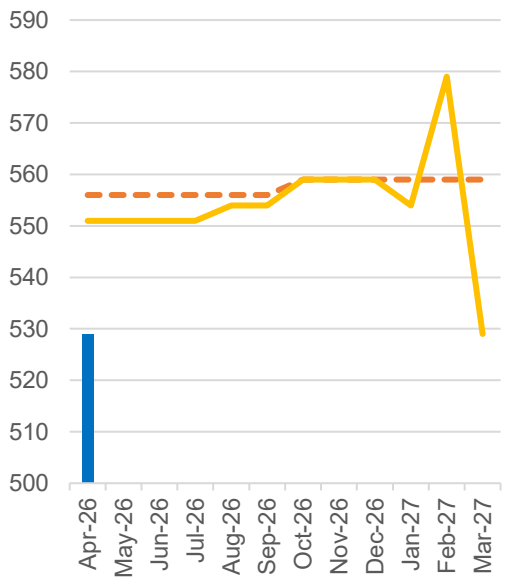
- Long waits in Community Paediatrics continue to present the largest challenge for all SEL providers. The overall number of waiters has remained largely consistent as capacity fails to keep pace with referral demand.
- Provider improvement plans are focussed on adding more capacity in services, redesigning pathways and developing demand and capacity capability.
- All providers are undertaking additional reporting to NHSE for long waits
- Providers are also evolving workforce models in response to rising demand. Workforce growth across SEL is increasingly focused on multidisciplinary expansion rather than solely increasing consultant capacity. Providers are investing in psychology, speech and language therapy, CNS and advanced practice roles to improve pathway flexibility. Multidisciplinary Team (MDT) expansion may improve resilience where Consultant led models are vulnerable to bottlenecks
- 2 service lines showing further sustained improvements in reducing longs waits are Podiatry/Podiatric Surgery - which has reduced from 92 to 60 this month and Paediatric Speech and Language Therapy.

Virtual Wards

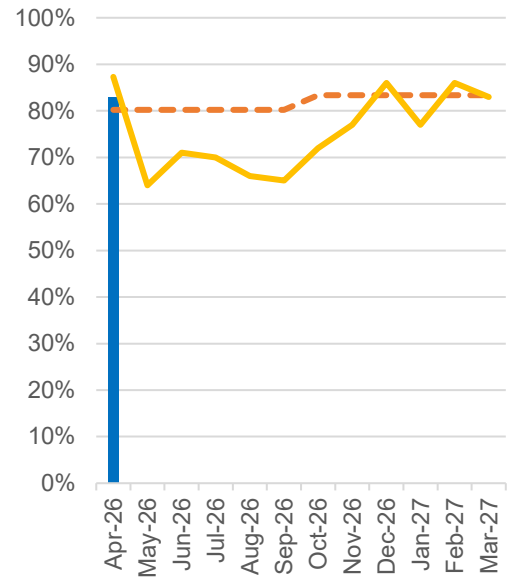
Notes and Issues

- SEL Virtual Ward capacity is below plan at 529 beds with a target of 556.
- The average utilisation for April was 83%, which is above the plan of 80%. Lower than target utilisation was reported for Bromley Healthcare (70%), GSTT (65%) and St Christopher's (45%). Some missing ward data was noted this month.
- There remains a reporting gap for one provider due to transitional reporting arrangements.

Capacity (beds) vs plan*



Occupancy vs plan*





Average of snapshots April 2026

April 2026	Av. Capacity	Average Utilisation
SEL actuals*	529	83%
SEL Plan	556	80%

*SEL average capacity and utilisation for March 2026 onwards are based on the consolidated positions across the providers. There is a known gap for one provider following transition to new service. National sitrep dashboard includes capacity from the previous provider - this has been excluded from the SEL figures calculated above.

2025/26 Plans and Actions

- Work to improve the utilisation of VWs is ongoing with a standardised acuity tool now developed and agreed for use in all SEL services. The tool is a 5 tier system for adult UCR and VWs with considerations for digital integration and frailty scoring.
- The tool will support teams to better identify patients as those requiring sub-acute care and enable compliance with the national framework.
- A programme of work is in place to share best practice regarding oxygen weaning in at-home and Virtual Ward settings between south east London teams and external providers, reviewing current pathways, challenges, and the potential for standardisation and cross-borough collaboration.
- To support a comprehensive review of Virtual Ward pathways, a series of deep-dive sessions are being undertaken to ensure all services are included and pathway mapping is completed. This analysis is critical in identifying opportunities to optimise service delivery and strengthen integration with Urgent Community Response (UCR) and frailty Integrated Neighbourhood Team (INT) programmes. It will also support the development of a consistent approach to future Virtual Ward pathways across South East London.

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Linked to Risk: Gre 474

NHS Continuing Healthcare

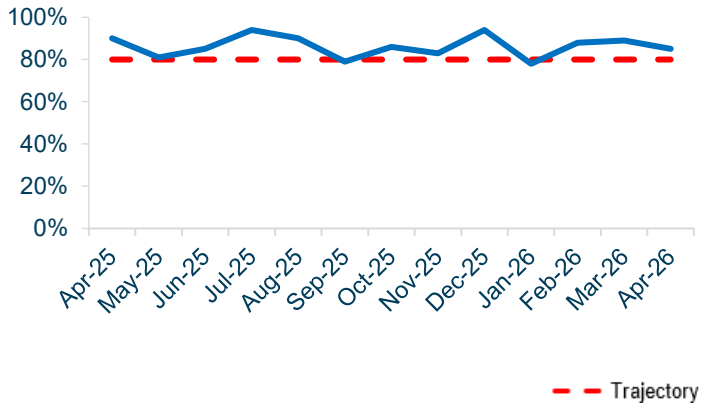
Notes and Issues

- April performance (local reporting) against the number of referrals completed within the 28-day time frame is 85%. This is a decrease from the March performance of 89% but remains above the national target of 80%.
- Incomplete referrals over 12 weeks: SEL reported zero 'long waits' over 12 weeks in April which meets the national requirement.
- The quarter 4 statutory reported position for SEL was 83% for 28-day performance which is above the national target. There were zero incomplete referrals over 12 weeks which meets the national target.

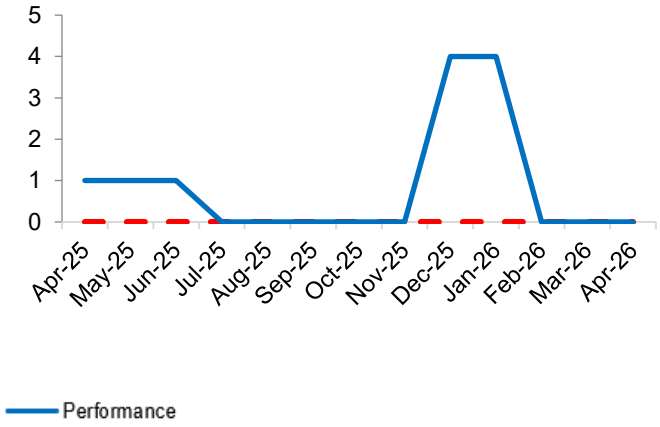
Standard CHC and Fast Track Reviews:

- There is variation in the number of overdue standard CHC and fast track reviews across the six boroughs.
- The number of individuals waiting for a Standard CHC review increased to 206 from 183.
- There are 88 overdue fast track reviews. This is a decrease from the previous month of 106.
- Funded Nursing Care Reviews (FNC): The number of FNC Reviews decreased from 770 to 756 in April.

CHC assessments completed within 28 days
Local monthly tracking



Incomplete referrals over 12 weeks
Local monthly tracking



Trajectory — Performance

Quarterly statutory reported position

	CHC assessments in an acute setting		% assessments completed in 28 days		Incomplete referrals over 12 weeks	
	Q4	Target	Q4	Target	Q4	Target
Bexley	0%	0%	79%	80%	0	0
Bromley	0%	0%	86%	80%	0	0
Greenwich	0%	0%	95%	80%	0	0
Lambeth	0%	0%	100%	80%	0	0
Lewisham	0%	0%	58%	80%	0	0
Southwark	0%	0%	77%	80%	0	0
SEL	0%	0%	83%	80%	0	0

Note: monthly reporting is in place as an 'early warning' and means that data issues can be identified and addressed within the quarter. Monthly and quarterly data may not align.

Recovery Actions

- Boroughs continue to work to agreed trajectories to reduce the number of patients waiting for Standard CHC, fast track and Funded Nursing Care Reviews, as a significant number remain overdue.

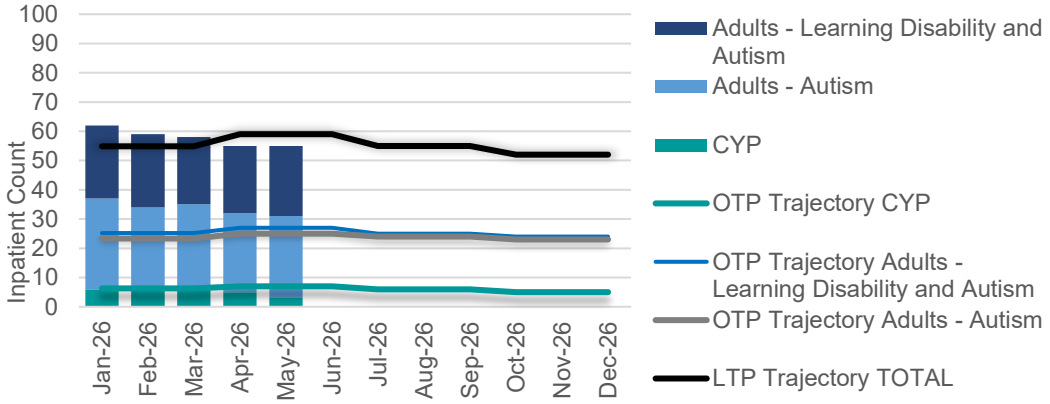
Linked to Risk: SEL 468

Learning disability and autism (LDA)

Notes and Issues

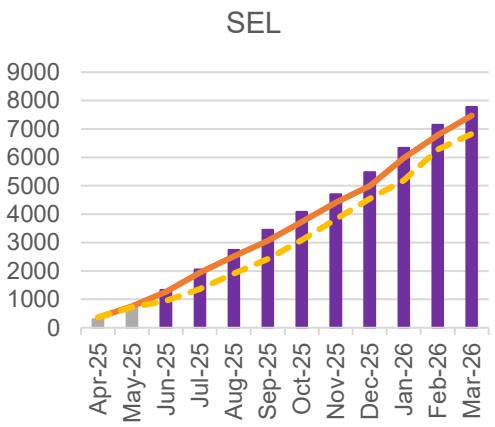
- At the end of April 2026, there were 55 inpatients, 4 below the Q1 target. There were 50 adults (27 autism only diagnoses) in non-secure and secure units. SEL was also achieving the target for the number of children and young people.
- As part of the SEL Mental Health, Learning Disability and Autism Inpatient Quality Transformation Plan (MHLDA QTP), Out of Area Placements are being reviewed to ensure that patients are repatriated to SEL or discharged into the community. There are currently fewer than 10 LDA patients out of area in mental health beds. This does not include secure patients who are mainly restricted and overseen by the Provider Collaborative.
- Continued increase in demand for autism assessments for both adults and children and young people across all boroughs.
- Based on end of March 2026 data, SEL has achieved overall completion rate of 82% (7,771 checks delivered) of the people on GP LD registers receiving an annual health check, demonstrating continued good progress.

LDA Inpatient Position



Note: Data correct as of 27 May 2026.

LD AHCs: SEL and Borough Level Position



	March 26 performance		2025/26 Plan
	%	Count	
Bexley	79%	1021	908
Bromley	87%	1153	939
Greenwich	84%	1440	1248
Lambeth	81%	1445	1279
Lewisham	77%	1584	1473
Southwark	84%	1128	976
SEL	82%	7771	6823

Recovery Actions

- Community autism specialist services to support autistic only people have been developed to prevent admission and support community placements. Along with existing services commissioned from MH providers, these services will support the continued reduction in admission rate. The service specification has been agreed, and providers are working to mobilise services during Q2 2026/27.
- Intensive Support Teams (ISTs) for people with a learning disability were also secured and recruitment commenced in Q3 2025/26, ensuring that there is IST coverage in all SEL boroughs to support discharge and prevent admissions.
- Housing, care and support work is in development to support discharge and prevent admissions.
- Work continues with providers to identify an action plan to address the high numbers of people on waiting lists and long waiting times for autism assessment, as well as requirements to meet demand in the longer term. This includes work on Right to Choose and accreditation of services.
- Digital Dynamic Support Registers (DSRs) launched in August 2025 across SEL to support admission prevention and improve utilisation of Care Education Treatment Reviews (CETRs). The system is now being used to capture case notes for Keyworking and Case Management.
- LDA specialist prescribing directly supports patients, primary care, annual health checks and the LeDeR Programme. The One Stop STOMP clinic is doing exceptional work in SEL.

Linked to Risks: SEL 389, SEL 390, SEL 391, Gre 615, Gre 635, Bex 642, Bro 509, Lam 129, Lew 644, Sou 520

South East London Integrated Care System

NHS South East London

ICB Board in Public

Title	**Month 2 ICB Financial Update**						
Meeting date	22 July 2026		Agenda item Number	**10**	Paper Enclosure Ref	**I**	
Author	David Maloney, Director of Corporate Finance						
Executive lead	Dinah McLannahan, Chief Finance and Compliance Officer						
Paper is for:	Update	**x**	Discussion	**x**	Decision		
Purpose of paper	To provide an update to the Board of the financial position of the ICB as at month 2.						
Summary of main points	A paper summarising the ICB's financial position at month 2 is being presented. The key headline is that the ICB is reporting a year to date (YTD) **break-even** position. This is in line with its annual financial plan. Whilst it is very early in the financial year for activity trends to emerge, the Board is asked to note that mental health cost per case activity (of which a significant element relates to ADHD and ASD activity) continues to represent a key area of financial pressure for the ICB. The overall overspend on mental health services at month 2 is just under £1.0m.						
Potential conflicts of Interest	Not applicable						
Relevant to these boroughs	Bexley	**x**	Bromley	**x**	Lewisham	**x**	
	Greenwich	**x**	Lambeth	**x**	Southwark	**x**	
Equalities Impact	Not applicable						
Financial Impact	As set out in the attached finance report.						
Public Patient Engagement	Not applicable						
Committee engagement	ICB committees receive regular updates on the financial position.						
Recommendation	The Board is asked to note the report and discuss any actions in relation to the financial position.						

NHS
South East London

SEL ICB Finance Report

Month 2 2026/27

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1. Key Financial Indicators
2. Executive Summary
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1. Key Financial Indicators

NHS South East London

- The below table sets out the ICB's performance against its main financial duties on both a year to date (YTD) and forecast basis.
- As at month 02, the ICB is reporting a year to date (YTD) and forecast out-turn (FOT) **break-even position** against its revenue resource limit (RRL) and financial plan.
- **All boroughs are reporting that they will deliver a minimum of financial balance at the year-end.**
- The ICB is showing a **break-even** position for YTD and forecast out-turn position against the **running cost allowance (RCA)**. Any YTD underspends against running cost and programme corporate budgets are being used to mitigate the reduction to the ICB's overall allocation **(£21,400k)** in respect of the Change Programme.
- The ICB is reporting that YTD its efficiencies plan is on track and the forecast is for the annual plan to be delivered.
- **All financial duties have been delivered for the year to month 02 period.**

Key Indicator Performance

	Year to Date		Forecast		
	Target	Actual	Target	Actual	
	£'000s	£'000s	£'000s	£'000s	
Expenditure not to exceed income	989,201	989,201	5,950,393	5,950,393	
Operating Under Resource Revenue Limit	989,201	989,201	5,950,393	5,950,393	
Not to exceed Running Cost Allowance	5,342	5,342	31,717	31,717	
Month End Cash Position (expected to be below target)	5,563	3,231			
Operating under Capital Resource Limit	n/a	n/a	n/a	n/a	
95% of NHS creditor payments within 30 days	95.0%	100.0%			
95% of non-NHS creditor payments within 30 days	95.0%	99.7%			
Mental Health Investment Standard (Annual)			560,171	561,171	

2. Executive Summary

NHS South East London

- This report sets out the month 2 financial position of the ICB. The financial reporting is based upon the final plan submission. This included a **planned break-even position** for the ICB. The ICB's financial allocation as at month 2 is **£5,950,393k**. In month, the ICB has received an additional **£48,587k** of allocations. These are as detailed on the following slide. **As at month 2, the ICB is reporting a year to date (YTD) break-even position.**
- Due to the routine time lag, the ICB has not yet received any 2627 prescribing data. After the usual accrual for two months of estimated prescribing expenditure, the ICB is reporting a **break-even position YTD across PPA and non PPA**. Once information is available the usual slide will be presented.
- Given that reporting for dental, ophthalmic and community pharmacy is subject to same time lag, a YTD breakeven position is reported. Again, when information is available, the usual slide will be included within the monthly reporting pack.
- The CHC financial position is **£126k overspent** at month 2. The boroughs which are most impacted are Lewisham and Bexley, further work will be undertaken to validate the clients in month to ensure the reporting is correct and any mitigations can be identified. The YTD position for **Mental Health services** is an overall **overspend of £974k.** This is generated by pressures on cost per case services and Right to Choose costs with all boroughs impacted. **ADHD and ASD assessments** continue to be a significant financial pressure for all boroughs. Additional analysis of these MH pressures is being undertaken in month as well as identifying potential mitigations against these pressures. Work is due to be undertaken around the ASD and ADHD pathways, and progress regarding this work will be followed up on and reported back in future reports.
- Two places are reporting a small overspends YTD at month 2 – **Bromley (£161k – driven by MH CPC and Right to Choose overspends) and Greenwich (£122k – driven by MH CPC and Right to Choose overspends), with a break-even position being forecast by all.** All Places have been tasked to identify additional mitigations to offset financial risks.
- In reporting this month 2 position, the ICB has delivered the following financial duties:
  - An **overall break-even** YTD financial position.
  - Breakeven against its management costs allocation. YTD underspends **(£1,680k)** are being used to offset the allocation reduction **(£21,400k)** due to the national assumption that staff restructures arising from the Change Programme were implemented by 1st April 2026 which is not true for the ICB. The redundancy costs of the programme were included as either provisions (Voluntary Redundancy Scheme 2 and any compulsory Redundancies) or accruals (Voluntary Redundancy Scheme 1) in the ICB's 2025/26 annual accounts, so will not be a charge against the 2026/27 position.
  - Delivering all targets under the **Better Practice Payments code**;
  - Subject to the usual annual review, delivered its commitments under the **Mental Health Investment Standard**; and
  - Delivered the **month-end cash position**, well within the target cash balance.
- As at month 2 the ICB is forecasting an overall **break-even position** against its annual financial plan.

3. Revenue Resource Limit (RRL)

NHS South East London

	Bexley	Bromley	Greenwich	Lambeth	Lewisham	Southwark	South East London	Total SEL ICB
	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s
ICB Start Budget (as per submitted plan)	**109,388**	**208,005**	**130,682**	**145,879**	**119,148**	**113,778**	**5,074,926**	**5,901,806**
M2 Internal Adjustments								
Planning Adjustments to Boroughs	538	527	837	788	312		-3,004	**0**
Transfer to Lewisham for S117 cost pressures					1,600		-1,600	**0**
M2 Allocations								
Indicative budget still pending at M2 (SDF)							(9,967)	**(9,967)**
Cost increase on ICB programme funded expenditure (Recurrent)	121	566	173	30	42	60	22,966	**23,958**
DDRB - Cost increase on ICB programme funded expenditure (Recurrent)	84	390	119	20	29	42	15,832	**16,516**
Cost increase on ICB specialised commissioning funded expenditure (Recurrent)							6,035	**6,035**
DDRB - cost increase on Specialised Commissioning expenditure (Recurrent)							3,966	**3,966**
Non-recurrent support for NHS providers (host basis)							2,887	**2,887**
DDRB - Non-recurrent support for NHS providers (host basis)							1,895	**1,895**
Additional FNC allocation							1,165	**1,165**
Childrens Hospice Grant allocations 2026/27							840	**840**
Cost increase on ICB primary care funded expenditure							412	**412**
Running Costs Allowance Pay Award Impact							320	**320**
Various Minor Allocations under £250k							560	**560**
M2 Budget	**110,132**	**209,488**	**131,811**	**146,717**	**121,131**	**113,880**	**5,117,233**	**5,950,393**

- The table sets out the Revenue Resource Limit (RRL) at month 2.
- The start allocation of **£5,901,806k** is consistent with the Operating Plan submissions.
- In month, the ICB has received an additional **£48,587k** of allocations, giving a total allocation of **£5,950,393k** at month 2**.**
- Indicative SDF funding of **£9,967k** which was included in the planning submission is still pending as at month 2.
- Additional allocations received in month 2 included funding to cover the impact of 2026/27 pay awards for the ICB and ICS partners totalling **£55,989k**, an additional FNC allocation of **£1,165k,** Children's Hospice Grant funding of **£840k**, and other smaller adjustments totalling **£560k.**
- Further allocations both recurrent and non-recurrent will be received as per normal throughout the year each month.

4. Budget Overview

NHS South East London

	M02 YTD								
	Bexley	Bromley	Greenwich	Lambeth	Lewisham	Southwark	PCD Team	South East London	Total SEL CCG
	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s
Year to Date Budget									
Acute Services	113	1,129	993	79	111	12	559,972	-	**562,409**
Community Health Services	4,390	16,406	6,121	5,319	5,320	6,125	54,276	-	**97,958**
Mental Health Services	1,904	2,668	1,883	4,175	1,764	2,076	112,515	874	**127,858**
Continuing Care Services	4,440	4,927	5,348	5,928	4,229	3,436	-	-	**28,307**
Prescribing	6,327	8,298	6,375	7,129	7,517	6,031	-	553	**42,230**
Other Primary Care Services	264	356	319	664	306	137	-	2,621	**4,667**
Other Programme Services	83	-	-	-	-	121	3,220	1,151	**4,574**
Programme Wide Projects	-	-	-	-	4	-	-	637	**642**
Delegated Primary Care Services	8,259	11,830	10,568	16,043	12,080	12,901	-	440	**72,120**
Delegated Primary Care Services DPO	-	-	-	-	-	-	11,248	26,009	**37,257**
Corporate Budgets - staff at Risk	-	-	-	-	-	-	-	-	**-**
Corporate Budgets	530	733	623	813	578	750	-	7,153	**11,179**
Total Year to Date Budget	**26,309**	**46,347**	**32,230**	**40,150**	**31,909**	**31,588**	**741,231**	**39,438**	**989,201**

	Bexley	Bromley	Greenwich	Lambeth	Lewisham	Southwark	PCD Team	South East London	Total SEL CCG
	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s
Year to Date Actual									
Acute Services	113	1,129	993	79	111	12	559,797	-	**562,234**
Community Health Services	4,074	16,249	6,076	5,319	5,245	6,125	54,299	-	**97,388**
Mental Health Services	1,996	2,985	2,046	4,335	1,794	2,144	112,665	867	**128,833**
Continuing Care Services	4,746	4,927	5,351	5,768	4,274	3,368	-	-	**28,434**
Prescribing	6,327	8,298	6,375	7,129	7,517	6,031	-	553	**42,230**
Other Primary Care Services	264	356	319	664	306	137	-	2,621	**4,667**
Other Programme Services	-	-	-	-	-	121	3,220	875	**4,215**
Programme Wide Projects	-	-	-	-	4	-	-	637	**642**
Delegated Primary Care Services	8,259	11,830	10,568	16,043	12,080	12,901	-	440	**72,120**
Delegated Primary Care Services DPO	-	-	-	-	-	-	11,251	26,009	**37,260**
Corporate Budgets - staff at Risk	-	-	-	-	-	-	-	-	**-**
Corporate Budgets	530	733	623	813	578	750	-	7,153	**11,179**
Total Year to Date Actual	**26,308**	**46,508**	**32,352**	**40,150**	**31,908**	**31,588**	**741,232**	**39,156**	**989,201**

	Bexley	Bromley	Greenwich	Lambeth	Lewisham	Southwark	PCD Team	South East London	Total SEL CCG
	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s	£'000s
Year to Date Variance									
Acute Services	0	(0)	(0)	0	0	0	175	-	**175**
Community Health Services	316	157	45	(0)	75	(0)	(23)	-	**570**
Mental Health Services	(93)	(318)	(163)	(160)	(30)	(68)	(149)	6	**(974)**
Continuing Care Services	(306)	(0)	(4)	160	(45)	68	-	-	**(126)**
Prescribing	-	-	-	-	-	-	-	-	**-**
Other Primary Care Services	0	0	(0)	0	0	-	-	(0)	**0**
Other Programme Services	83	-	-	-	-	-	(0)	275	**359**
Programme Wide Projects	-	-	-	-	-	-	-	(0)	**(0)**
Delegated Primary Care Services	-	-	-	-	-	0	-	-	**0**
Delegated Primary Care Services DPO	-	-	-	-	-	-	(3)	-	**(3)**
Corporate Budgets - staff at Risk	-	-	-	-	-	-	-	-	**-**
Corporate Budgets	(0)	0	0	(0)	0	0	-	0	**0**
Total Year to Date Variance	**1**	**(161)**	**(122)**	**(0)**	**0**	**0**	**(0)**	**282**	**(0)**

- As at month 2, the ICB is reporting a YTD **break-even position**, albeit with **pressures in specific budgets.** The main area of financial pressure is in **mental health services.**
- Due to the routine time lag, the ICB has not yet received any 2627 prescribing data. After the usual accrual for two months of estimated prescribing expenditure, the ICB is reporting a breakeven position YTD across PPA and non PPA budgets.
- The **CHC** financial position is **£126k** overspent at month 2. The boroughs which are most impacted are Lewisham and Bexley, further work will be undertaken to validate the clients in month to ensure the reporting is correct and any mitigations can be identified.
- The YTD position for **Mental Health services** is an overall overspend of **£974k**. This is generated by pressures on cost per case services and Right to Choose costs with all boroughs impacted. **ADHD and ASD assessments** continue to be a significant financial pressure for all boroughs. Additional analysis of these MH pressures is being undertaken in month, as well as identifying potential mitigations.
- The ICB is now settling the Voluntary Redundancy 1 claims which are being charged against year end accruals and shortly costs for the Voluntary Redundancy 2 scheme will be coming through which will be offset against a year end provision, so neither of these schemes will impact upon the 2026/27 financial position.
- Two places are reporting a small overspends YTD at month 2 – **Bromley (£161k – driven by MH CPC and Right to Choose overspends) and Greenwich (£122k – driven by MH CPC and Right to Choose overspends)**, with a break-even position being forecast by all. All places have been tasked to identify additional mitigations to offset any financial risks, to ensure delivery of their financial plans.

5. NHS Continuing Healthcare

NHS South East London

- As of Month 02, the Continuing Healthcare (CHC) budget is reporting an overall year-to-date **overspend of £126k**, although cost pressures continue to vary across boroughs. **Bexley and Lewisham** are currently reporting **modest overspends**.
- **Bexley** is the largest contributor to the overall overspend, with a year-to-date variance of **£306k above budget and a forecast outturn overspend of £500k**. This is primarily driven by **high-cost palliative care and Personal Health Budget (PHB) clients** and includes a **£763k full-year contingency within the reported position** for anticipated provider price increases. **Lewisham** is reporting an **overspend of £44k**, mainly attributable to **Funded Nursing Care**.
- Places continue to review client packages to ensure correct packages of care are in place and to ensure equity and value for money.
- To support consistent management of provider price uplifts, an ICB-wide panel has been established to review all requests exceeding 1.5%. The uplift panel meets weekly to review and agree provider requests.
- In summary, the boroughs are currently forecasting delivery of their annual plans. However, given the nature of CHC costs, it is still too early to forecast with certainty. Overall, there are no significant changes in the financial position compared with the previous financial year.

6. Mental Health Investment Standard (MHIS) – 2026/27

NHS South East London

Mental Health Investment Standard (excluding LD and Dementia) and Delegated Specialised Commissioning Mental Health Investment Standard:	Expected Sign	2_1Achieve01 MHIS Achieved per plans submitted 31/03/2026 Desc 31/03/2027 Year Ending TEXT	2_1PLAN%01 2026/27 allocation growth Plan 31/03/2027 Year Ending %	2_1AuditedPY 2025/26 Outturn Actual 31/03/2026 Year Ending £'000	2_1TARGET01 Target MHIS spend 2026/27 Target 31/03/2027 Year Ending £'000	2_1ACT02 FOT 2026/27 Forecast 31/03/2027 Year Ending £'000	2_1VAR%01 Excess / Shortfall in 2026/27 MHIS Delivery % Actual 31/03/2027 Year Ending %	2_1VAR02 Excess / Shortfall in 2026/27 MHIS Delivery Actual 31/03/2027 Year Ending £'000	2_1Achieve02 MHIS Achieved in 2026/27? Desc 31/03/2027 Year Ending TEXT	2_1POP01 Projected Population 2026/27 YTD 31/03/2027 Year Ending Number	2_1Achieve03 SCMHIS Achieved per plans submitted 31/03/2026 Desc 31/03/2027 Year Ending TEXT	2_1SCMHISAuditedPY 2025/26 SCMHIS Outturn Actual 31/03/2026 Year Ending £'000	2_1TARGET02 Target SCMHIS spend 2026/27 Target 31/03/2027 Year Ending £'000	2_1ACT03 SCMHIS FOT 2026/27 Forecast 31/03/2027 Year Ending £'000	2_1SCMHISVAR%01 Excess / Shortfall in 2026/27 SCMHIS Delivery % Actual 31/03/2027 Year Ending %	2_1SCMHISVAR02 Excess / Shortfall in 2026/27 SCMHIS Delivery Actual 31/03/2027 Year Ending £'000	2_1SCMHISAchieve02 SCMHIS Achieved in 2026/27? Desc 31/03/2027 Year Ending TEXT	2_1COMM01 Commentary for Variances Desc 31/03/2027 Year Ending TEXT
MHIS Achievement	+/-	Yes	2.03%	550,303	560,171	561,171	0.18%	1,000	Yes	2,757,608	Yes	91,032	92,573	92,587	0.01%	13	Yes	

Summary

- SEL ICB is required to deliver the Mental Health Investment Standard (MHIS) by increasing spend over 2025/26 outturn by a **minimum of the growth uplift of 2.03%.** The spend will be subject to independent review as part of the annual accounts audits effective from the year 2025/26 where the audit has just concluded.

- There are two elements to the MHIS target for 2026/27, as there were in the previous year:
  - The core MHIS target of £560,171; and
  - Plus, there is a separate MHIS target for Delegated Specialised Commissioning of £92,573k where responsibility has been transferred to the ICB from NHSE for services delivered through contracts managed by the South London Partnership (the Mental Health Provider Collaborative).

- MHIS excludes:
  - spending on Learning Disabilities and Autism (LDA) and Dementia (Non MHIS eligible).
  - out of scope areas include ADHD and the physical health elements of continuing healthcare/S117 placements.
  - spend on SDF and other non-recurrent allocations, noting that the majority of SDF funding has been transferred into the ICB baseline.

- **As at Month 2 we are forecasting core MHIS delivery of £561,171k, exceeding the target by circa £1,000k (0.18%) plus Specialised Commissioning MHIS delivery of £92,587k, exceeding the target by £14k. This is summarised in the above table, with further details available in the Month 2 IFR submitted to NHS England.**

6. Mental Health Investment Standard (MHIS) – 2026/27

NHS South East London

Risks and Mitigations

- We continue to see growth in mental health cost per case spend, in terms of client numbers, cost and complexity, for example on S117 placements. Mitigating actions include ensuring that timely client reviews are undertaken, reviewing and strengthening joint funding panel arrangements and developing new services and pathways.

- Learning disability placements costs continue to grow in some boroughs, with an increase in the complexity of some care packages being seen. Mitigating actions include reviewing LD cost per case activity across health and social care to understand care package costs, planning for future patient discharges to agree funding approaches, developing new services to prevent admissions and seeking to implement risk share agreements.

- ADHD is outside the MHIS definition and is therefore excluded from this reported position. There is, however, continued, significant and increasing independent sector spend on both ADHD and ASD services. A referral management centre was set up in year in 2025/26, however, the impact of this is to be assessed. A SE London wide piece of work on ADHD and ASD assessments is to be undertaken in 2026/27, with the scope still in discussion.

  The following actions which began last year are continuing to be undertaken:

  - increasing local provider capacity to reduce waiting times.
  - working with local providers across adult and CYP ADHD services to review and transform care pathways to create sustainable services.
  - undertaking an accreditation process to ensure the quality and VFM of independent sector providers.
  - working to agree contracts with high value independent sector providers to attempt to mitigate financial risk and ensure quality.

South East London Integrated Care System

NHS South East London

ICB Board Meeting in Public

Title	**Quality and Nursing**					
Meeting date	1st July 2026	Agenda item Number	**11**	Paper Enclosure Ref	**J**	
Author	**Annette Fogarty, Associate Director of Quality and Patient Safety** **Elizabeth Aitken, Clinical and Care Professional Lead, Quality**					
Executive lead	**Lizzie Wallman, Deputy Chief Nurse** **Elaine Clancy, Chief Nursing and Quality Officer**					
Paper is for:	Update	**X**	Discussion		Decision	
Purpose of paper	To provide an overview of quality and nursing across the ICS for Quarter 4					
Summary of main points	**Quality and Patient Safety:** In Q4, 13 Patient Safety Incident Investigations and 1 Never Event was reported. There is ongoing work through quality improvement projects across the Trusts that have reported Never Events linked to Patient Safety Incident Response Plans. 439 Quality Alerts were raised, with the trending themes reported include appointment/referral, transfer of care, discharge, communication and diagnostics. Improvement work is underway, including early detection pathways, diagnostic accuracy, and medication safety through improved escalation, clearer processes, and more robust system controls. **Safeguarding:** During Q4, safeguarding activity focused on strengthening system intelligence and commissioning insight, particularly through the use of the Serious Violence Duty dataset. Progress was made through the continued development of the Families First Programme and the effective triangulation of learning from the deaths of children and young people with partner agencies to drive system learning. Embedding lived experience within commissioning and service design, alongside strengthened safeguarding training, robust compliance monitoring, and 100% supervision attendance, providing strong assurance of workforce capability and governance oversight. **All Age Continuing Care (AACC):** The team continue to make progress against national performance standards, with timely eligibility decisions exceeding targets and robust governance arrangements in place to support oversight. However, areas of reduced assurance remain, including overdue reviews, a high rate of Independent Review Panel overturns, and sustained complaints, with improvement actions underway to strengthen decision quality, consistency, and learning. **Local Maternity and Neonatal System (LMNS)**: During Q 4 there is a broad programme of improvement and transformation aligned to national reforms, including implementation of new care bundles and system-wide workforce and quality initiatives. Performance improvements are evident in areas such as earlier maternity booking, alongside extensive work to enhance equity, service user experience, and workforce development. However, ongoing national reviews and increasing service demand highlight the need for continued focus on safety, capacity, and system coordination. **Infection Prevention and Control (IPC):** The most recent surveillance data shows the SEL ICS position is above trajectory for cases of E. coli, Clostridioides difficile,						

	Klebsiella spp., P. aeruginosa, and methicillin-resistant staphylococcus aureus (MRSA) bacteraemia cases. **Learning Disabilities and Autism (LDA):** Progress continued in Q4 to reduce reliance on inpatient care and strengthen quality and safety for people with a learning disability and autistic people through improved community services, governance and medicines optimisation. **Special Educational Needs and Disabilities (SEND)**: In response to the Schools White Paper and national SEND reforms, SEL partners have developed Local Area SEND Reform Plans, supported by national advisers. These plans have now been agreed and signed off by the Chief Executive.					
Potential conflicts of Interest	Nil known					
Relevant to these boroughs	Bexley	X	Bromley	x	Lewisham	x
	Greenwich	x	Lambeth	x	Southwark	x
Equalities Impact	This paper has considered the potential impact on individuals and groups with protected characteristics. No adverse impacts have been identified, and the proposed quality improvements are expected to promote equitable access and outcomes for all service users.					
Financial Impact	There is no financial impact associated with this paper					
Public Patient Engagement	Patient engagement is outlined in the paper					
Committee engagement	Quality and Safeguarding Committe					
Recommendation	The Board are asked to note the content of the report					

South East London Integrated Care System

NHS South East London

Board Report

Quality and Nursing

July 2026

We are a partnership of NHS commissioners and providers, the boroughs of Bexley, Bromley, Greenwich, Lambeth, Lewisham and Southwark and the voluntary and community sector

1. Introduction

This report provides an overview of key performance updates from the Quality and Nursing Directorate across South East London Integrated Care Board (SEL ICB) for Quarter 4. It covers essential areas, including Quality and Safety, Safeguarding, All Age Continuing Care (AACC), the Local Maternity and Neonatal System (LMNS), Infection Prevention and Control (IPC), Learning Disabilities and Autism (LDA), and Special Education Needs and Disabilities (SEND).

2. Quality and Nursing Updates

2.1 Quality and Patient Safety

During Q4, a total of 13 Patient Safety Incident Investigations (PSIIs) were commissioned, and 1 Never Event was recorded. There have been 20 Never Events reported across the system year to date. This number has changed as a data quality issue was identified whereby the ICB did not have full oversight of a provider's Never Events, resulting in an incorrect reported total. This has now been reviewed and corrected. The Never Event reported this quarter related to retained foreign object involving a retained instrument (malleable retractor) following surgery, once it was identified the Trust protocol was followed and the instrument was removed. There is ongoing work through quality improvement projects across the Trusts that have reported Never Events linked to Patient Safety Incident Response Plans. Key actions include; dissemination of learning from invasive-procedure Never Events (including retained objects guidance), piloting a revised WHO checklist in selected areas, and alignment with work on cultural improvements:

- Clearly document the "right site" using site-specific information, particularly where pathways vary.
- Review and respond promptly to patient safety incidents.
- Ensure learning leads to SMART safety actions that are implemented and monitored.
- Sustain learning through audit, standardisation, and a strong safety culture.

Quality Alerts (QAs) have been closely monitored, and the quality team meet on a weekly basis to review and theme the QAs reported across the system. A total of 439 quality alerts were reported in Q4. The top themes reported include appointments/referrals, transfer of care, discharge, communication and diagnostics. Similarly, in Q3, during Q4, Accident and Emergency services continued to be the most prevalent service across all acute providers in reported QAs - an expected finding given the higher volume of received patients. Themes and trends from QAs are also fed into the System Interface Group led by the ICB Medical Director. A new South East London (SEL) standard for discharge summaries has been proposed to help address known system issues. The

team are also updating the current version of the Consensus Document to include reference to mental health.

There has been targeted improvement work across a number of providers in SEL from QAs:

Delayed escalation of persistent symptoms (Primary Care):

- Finding: Repeated presentations with ongoing symptoms were not escalated, leading to delayed diagnosis.
- Learning/Actions: Introduce clear escalation triggers (e.g. cough >6 weeks or repeat attendances), implement proactive follow-up systems (e.g. GP-initiated reviews and tracking of outstanding actions), and expand use of direct access diagnostics such as chest X-ray pathways.

Diagnostic test request and sample labelling error (Primary Care / Pathology):

- Finding: Reuse of historical test requests led to manual processing and mislabelled samples, resulting in incorrect results recorded.
- Learning/Actions: Generation of new electronic test requests for all repeat tests, reinforce this through practice-level process reviews, and implement targeted communications (e.g. staff bulletin) to highlight risks of reprinting forms.

Care provider response to medication overdose (Community Care):

- Finding: Unclear escalation responsibility, duplicate calls to 111, and carers leaving patients before clinical advice.
- Learning/Actions: Introduce a single-point-of-contact protocol (carer on-site responsible for escalation), mandate carers remain with patients until clinical advice is received and deliver refresher training on overdose management and escalation pathways, supported by spot checks.

Dispensing error (Community Pharmacy):

- Finding: Look-alike, sound-alike (LASA) medication error resulted in patient harm.
- Learning / Actions: Physically segregate LASA medications on shelves, introduce warning labels and visual prompts in dispensing areas, update LASA risk lists, and reinforce SOP compliance through team briefings and safety huddles.
- Finding: Incorrect strength dispensed due to similar packaging, distractions, and missed final check.
- Learning/Actions: Separate storage of high-risk medicines by strength, introduce high-alert labelling for anticoagulants, reinforce mandatory final accuracy checks, provide targeted training on high-risk medicines, and review staffing/workflow during peak periods to reduce interruptions.

During the reporting period, Lewisham and Greenwich NHS Trust achieved an overall rating of Good, with Safe rated Requires Improvement, while Guy's and St Thomas' NHS

Foundation Trust maintained an overall Good rating with Outstanding performance in Urgent and Emergency care and elements of leadership and patient experience; both reports provide positive assurance with a continued focus required on safety.

2.2 Safeguarding

The safeguarding update provides the Board with a summary update on the delivery of the ICB Safeguarding Workplan, outlining progress against key priorities and providing assurance on safeguarding activity and outcomes.

Serious Violence Duty (Including VAWG):
The ICB Serious Violence Dataset, drawing on Emergency Care Data Set (ECDS) and inpatient activity data, now provides partners with improved access to high level intelligence on hospital contacts relating to serious violence across the SEL Integrated Care System (ICS). The availability of a comprehensive, shared dataset has strengthened system wide insight and supports more informed commissioning and safeguarding decision making. SEL ICB central team was selected to present Emergency Department Serious Violence data at the Public Health Violence Reduction Network meeting, highlighting system leadership and contributing to regional learning on the health response to serious violence."

Families First Programme (FFP):
The Families First Partnership Programme is a national reform programme aimed at strengthening early help, integrating family support, and delivering a timelier multi-agency child protection response.
Lewisham is one of a small number of nationally designated Pathfinder sites, providing early learning to support wider system rollout. Lewisham has made positive progress across a number of areas, using its Pathfinder status to test and develop new models of practice. This includes advancing integrated approaches to Family Help, strengthening early intervention, and progressing work to align safeguarding pathways across partners. The programme benefits from established multi-agency relationships and a clear commitment to a Think Family approach.

While all boroughs are progressing towards the national FFP ambitions, variation in system maturity and pace of co-design continues, with ongoing work strengthening multi-agency collaboration and supporting consistent development across the SEL footprint. Progress across SEL ICB is informing the development of Multi Agency Child Protection Teams, with regional collaboration helping to shape sustainable models, clarify place-based delivery, and identify commissioning and resourcing requirements for future implementation.

Learning from Deaths for Children and Young People:
Has been effectively triangulated with partner agencies, including our colleagues in Public Health with identified themes driving system learning, informing practice, and supporting targeted improvements progressed throughout the quarter.

Commissioning:
The ICB continues to embed the voice of the child through commissioning, procurement, and contract management processes across all ages including Children Looked After, Care Leavers, ensuring that lived experience informs service design and delivery. The ICB remains committed to a 'Making Safeguarding Personal' for adults and 'Think Family' approach, supported through system learning, partnership working, and the dissemination of best practice to strengthen outcomes for adults, children and families.

Training and Supervision:
The ICB central safeguarding team has significantly strengthened delivery of statutory safeguarding training requirements through the implementation of a safeguarding training and development strategy. This has enabled delivery across all levels of safeguarding training. Compliance rates are monitored and reported through the Quality and Safeguarding Committee, providing ongoing assurance to the Board. The plan to continue with Mental Health Capacity Training is planned throughout 2026.
Safeguarding supervision remains effective, achieving 100% attendance for one-to-one sessions in the last quarter, providing assurance of consistent reflective practice, professional support, and governance oversight.

2.3 All Age Continuing Care (AACC)

All Age Continuing Care (AACC) is a national reform programme covering NHS Continuing Healthcare (CHC) and Children and Young People's Continuing Care (CYPCC). The AACC Vision 2023–2028 aims to reduce unwarranted variation and improve experience, transparency and consistency across all continuing care pathways, including smooth transitions between services. The following summary provides the Board with an update on the delivery of some areas of the All Age Continuing Care (AACC) workplan, outlining progress against national key performance indicators.

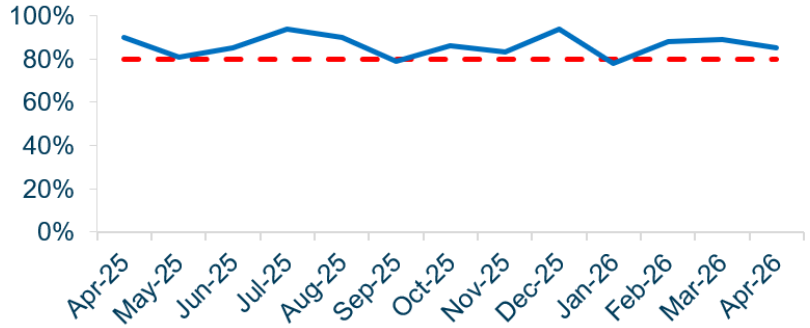
Delivery Model:

The Model ICB framework confirms ICBs as strategic commissioners and has prompted a review of AACC functions to explore opportunities to streamline delivery while maintaining statutory accountability. The Model ICB AACC / CHC Good Practice guidance (published in September 2025) sets out recommendations to support improved quality, efficiency and sustainability, while retaining AACC and CHC functions within ICBs. Delivery is structured around four High Impact Actions: Strategy & Leadership; People & Skills; Delivery Models & Commissioned Functions; and Strategic Enablers.

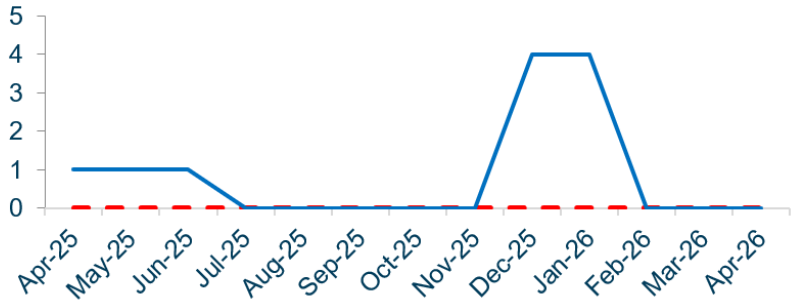
National Key Performance Indicators (Quality Premium Metrics):

The organisation is currently meeting national key performance indicators for the timeliness of eligibility decisions. As at March, 85% of assessments were completed within 28 days which is above the national standard of 80%. SEL ICB reported no incomplete referrals over 12 weeks again meeting the national standard.

CHC assessments completed within 28 Days:



Incomplete referrals over 12 weeks:



Funded Nursing Care:

There remains a significant number of reviews overdue particularly with Funded Nursing Care (FNC). Teams have continued to work to their locally agreed trajectories to improve performance.

Performance Monitoring:

Governance and oversight arrangements are monitored through routine monthly and quarterly reporting. These are in place to support Board and Committee scrutiny of performance information; however, assurance continues to be kept under review at all

times to ensure reporting remains sufficiently comprehensive and reflective of emerging requirements.

Independent Review Panels:
NHS England has highlighted SEL as an outlier for the number of Independent Review Panel decisions that have been overturned as the ICB had 50% of CHC decisions overturned by the NHSE led Independent Review Panel Process. The AACC team is developing an improvement plan following the review of the recommendations of all 2025/26 cases.

Complaints and MP Enquiries:
There are indicators of reduced assurance in relation to decision quality and consistency. This includes a sustained level of complaints and MP enquiries, together with the higher-than-average rate of Independent Review Panel overturns These indicators suggest a need for continued focus on quality assurance, learning, and communication. Actions are underway to review overturned decisions, strengthen feedback and learning processes, and further refine reporting to improve patient-level insight.

2.4 Local Maternity and Neonatal System (LMNS)

Both the National Maternity and Neonatal Investigation and the Ockenden Nottingham Review reports are due to be published in June (Ockenden June 24th and the National Investigation June 30th). National Expert Reference Groups will convene on the 7th of July to review the recommendations from both reports and then present their recommendations to the Secretary of State for Health and Social Care on the 14th of July.

Maternity Incentive Scheme Year 8 Publication:
Year 8 guidance bridges the current framework to future reforms that will be informed by the National Maternity and Neonatal Investigation and aligns with the 10-Year Health Plan. The scheme has been streamlined with a less administrative burden on the trusts and no longer requires ICB CEO sign off. The scheme has transitioned from ten safety actions to six focused core areas:

- Workforce and Capacity
- Training
- Learning from Reviews and Investigations
- Service User Voice and Equity
- Care Bundles
- Board Oversight, Governance, Culture and Leadership

Maternal Care Bundles:

A new NHSE nationally mandated care bundle to reduce maternal mortality, severe morbidity and persistent inequalities was published in January 2026. The care bundle provides standardised best practice guidance across five elevated risk clinical areas:

- Reducing venous thromboembolism through early assessment
- Pre-hospital and acute care
- Epilepsy in pregnancy
- Maternal mental health
- Obstetric haemorrhage

The scheme mandates a 'whole system' approach coordinating care across primary care, GPs, maternity teams, early pregnancy services, Emergency Departments and specialised services such as abortion services. A toolkit to support implementation is available with the expectation that the bundle is fully implemented by March 2027. There are organisational responsibilities at provider, ICB, regional NHS teams and Maternal Medicine Network (MMN) level. LMNS engagement with key entire system level stakeholders has commenced.

Handover of Assurance to Regional Team:

A formal handover of LMNS assurance to the regional team has taken place and regional perinatal quality surveillance meetings have commenced. Reporting from providers to the regional team includes:

- Areas of risk/concern
- QI projects in progress
- Risks requiring escalation

The regional team will also be conducting insight visits to each trust as part of their assurance processes.

Temporary Capping of Maternity Bookings:

A trust within SEL decided to put in place a temporary cap on maternity bookings. The measure was introduced due to increasing demand across the service and a backlog in booking and scan availability. They continue to accept women and birthing people from all SEL postcodes and those with complex obstetric medical needs requiring specialist input. To date the plan is showing positive results with bookings by 10 weeks of pregnancy now at 54% compared to 20+% just months ago.

LMNS Improvement and Transformation:

Current improvement and transformation work taking place within the LMNS includes:

- Data review to support strategic commissioning and triangulation of multiple sources to best support planning and delivery.
- Neonatal mortality review meetings have commenced to enable timely sharing and learning and response to potential avoidable harm

- LMNS wide career clinics have commenced for midwives, with good uptake, and a plan to roll out to support workers and neonatal nurses.
- There is a significant LMNS wide training offer for maternity and neonatal staff with a focus on unit culture and civility, as well as a labour ward coordinator development programme and a matron development programme.
- Maternity and neonatal system level meetings are currently under review with consideration from a strategic commissioning perspective and LMNS transformation and improvement needs.
- With the increase in women and birthing people requesting care outside of trust guidance and the implications this has for safety and outcomes as well as workforce implications the LMNS are working on SEL guidance. This is also being reviewed nationally.
- A pilot is currently in progress to deliver Pregnancy Circles which are an alternative to standard antenatal/postnatal appointments. They are small community-based support and antenatal groups that bring together approximately 8-12 pregnant women/birthing people and partners and families at a similar stage of pregnancy and combine clinical checks with education, preparation for birth and parenting and peer bonding. The circles are also shown to save clinician time, and we hope for this pilot to roll out across the whole of the LMNS.
- Feedback from women and birthing people regarding their infant feeding journey has highlighted that the postnatal information available could be better. This informed an SEL ICS wide project creating an infant feeding information leaflet and new and sensitive updated information that is now available within the child health record for all SEL residents that have had a baby. The project was co-produced with service users, clinicians across maternity, neonatal and health visiting, public health, and local authority colleagues.
- The LMNS will be working with perinatal mental health colleagues to run a webinar for primary care and GP colleagues regarding perinatal mental health services and referral pathways, raising awareness and sharing learning.
- LMNS equality and equity work in progress includes a project collaborating with the Gypsy, Roma, and Traveller communities across SEL to support engagement with maternity and neonatal services, ensuring that this is culturally sensitive and based on the communities' needs. The LMNS is also developing with TED Learning a culture resource for maternity and neonatal staff to support them to have culturally appropriate conversations and deliver culturally sensitive care. Again, this is being co-produced with service users, people with lived experience and clinicians from across SEL.
- The LMNS led preconception health campaign as part of the Women's and Girl's Health Hub work is in its second phase. There will be a data review on reach.
- Future campaigns will include mental health, VTE and a focus on boys and men and their role in planning a pregnancy.

2.5 Infection Prevention and Control (IPC)

The NHS Standard Contract 2025/26 includes quality requirements for NHS foundation trusts and ICBs to minimise rates of both Clostridium difficile (C. difficile) and Gram-negative bloodstream infections to threshold levels set by NHS England. The SEL position to March 2026 shows SEL ICS is above trajectory for cases of E. coli, C. difficile, Klebsiella spp. and P. aeruginosa. This is in line with the overall London position at year end. There were 51 methicillin-resistant staphylococcus aureus (MRSA) bacteraemia cases across all settings against a threshold of zero. Twenty-two cases were recorded as community associated.

Local improvement projects continue within individual trust organisations and in partnership with SEL ICB IPC team. The South-East London Forum on Antimicrobial Stewardship continue to meet bi-monthly to update and review progress on systemwide workstreams. Other activities include ongoing general practice audits, outbreak management support and ensuring a prompt cascade to SEL primary care staff and trust IPC leads including updated guidance for managing suspected Ebola and hantavirus cases. SEL IPC team, working with other IPC community leads across London, facilitated the 5th annual pan-London IPC conference for primary care and adult social care staff. Interest was strong and over 600 staff registered to attend. Presentations included antimicrobial stewardship, hydration and sustainability.

2.6 Learning Disabilities and Autism (LDA)

Reducing reliance on inpatient beds:

During Q4, the increase in the number of admissions slowed in January and February, going up again in March. For adult admissions the majority (11 of 16) had previous admissions, often due to non-compliance with medication for a mental illness such as psychosis. While admissions appear to be appropriate, the use of the Dynamic Support Register to review the risk of admission and undertake Care Education Treatment Reviews (CETRs) remains essential to preventing admissions by intervening in the community and putting in place the right care and support needed. During March the inpatient target number was not achieved but there were some key discharges of long stay patients planned for the end of March. The end of year position was 51 adult inpatients against a target of 47.

There was a small reduction in admissions for children and young people (CYP) in Q4 2025/26, there were eight CYP admissions and at year end there were seven against a target of five. New admissions to hospital continue to be of new diagnoses of autism

during an inpatient stay and often characterised by complex social and family circumstances. At time of this report the number of CYP in hospital reduced to three.

Given the level of unpredictability around admission and discharges. Boroughs continue to work on achieving complex discharges which may for example involve the Court of Protection and Ministry of Justice restrictions. One of the three discharges expected of people in hospital over ten years was achieved during March and an achievement towards providing the least restrictive environment and quality of life in the community.

Annual Health Checks (AHCs):
By the end of January 2026 (Q4), all SEL boroughs had exceeded the operational trajectory, delivering 6,332 checks against a plan of 5,190 and remaining on course to surpass the 75% year-end target. By the end of March 2026, SEL achieved an overall completion rate of 82%, exceeding the 75% target, with a total of 7,771 checks delivered against a target of 6,830, demonstrating sustained strong performance across the year.

There are new operational planning targets for AHCs in 2026/27 where a percentage target will no longer be sought, but evidence of AHCs completed with a Health Action Plan (HAP). Completing and delivering a HAP is supportive of good quality AHCs if these are shared with people with a learning disability and their families as well as utilised by health and care professionals.

LeDeR:
Learning from the lives and deaths of people with a learning disability and autistic people (LeDeR) continues to identify key themes and opportunities for improvement. In Q4, 12 reviews were completed (5 initial and 7 focused), with three initial reviews escalated to focused reviews due to additional learning identified or the individual being from a global majority background; no reviews were deemed out of scope.

Of the completed reviews, six related to people with a learning disability only, four to people with both a learning disability and autism, and two to autistic adults without a learning disability. Among those with a learning disability, 90% were recorded on the GP Learning Disability Register and 70% had received an Annual Health Check in the year prior to death.

Pneumonia and other respiratory infections were the most common underlying cause of death in Q4 (5 of 12 cases), followed by cancer (4 cases), with other causes including cardiovascular disease, thromboembolic events, multi-organ failure and overdose. This indicates that pneumonia continues to represent a significant and persistent cause of death within this population.

STOMP/STAMP:

The SEL One Stop STOMP Clinic (Stopping Overmedication in People with a Learning Disability or Autism) continues to operate with triaged referrals and clinical oversight from SLaM, Oxleas and SEL ICB. During Q4 this service was rolled out across SEL.

By stopping the over medication of people, the STOMP Clinic made a significant impact on the lives of people in the community, effectively improving the quality of lives. Medication review is key to annual health checks (AHCs). These reviews are recognised and supported by colleagues in General Practice thanks to the ongoing work of the LDA Specialist Prescribing team.

Oliver Mc Gowan Mandatory Training:
The target of 30% for both Tier 1 and 2 was achieved in the last month of the year, making provider Trusts eligible to claim funding per person trained in 2026-27. In total SEL trained 14,699 Tier 2 staff and just shy of 6000 Tier 1 staff, which is a massive achievement. At the end of the year SEL achieved Tier 1 at 31%, Tier 2 at 30%.

2.7 Special Education Needs and Disabilities (SEND)

Each borough in SEL commenced work on their Maturity Assessments and Local SEND Reform Plans which boroughs had to produce for May and June 2026. The newly published Schools White paper which includes SEND Reforms was the focus of the SEL SEND Network and Local SEND Partnerships and Boards. SEL ICB needed to jointly agree Local Area SEND Reform plans, which at time of this report were all signed off by the ICB Chief Executive for submission on 19 June 2026.

3. Conclusion

The Quality and Nursing directorate continue to work with partners across the system to improve patient safety and service quality. Progress has been made across each of the directorate's functions.

South East London Integrated Care System

NHS South East London

Board Meeting in Public

Title	**Overview of Ockenden and Amos Reviews**						
Meeting date	22 July 2022		Agenda item Number	**12**	Paper Enclosure Ref		**K**
Author	Jacqui Kempen						
Executive lead	Elaine Clancy Chief Nurse and Quality Officer SEL ICB and SWL ICB						
Paper is for:	Update	x	Discussion	x	Decision		
Purpose of paper	To provide an overview of: • A summary of key findings from the Nottingham University Hospitals Maternity Review (Ockenden Report) published 24th June 2026. • A summary of key findings from the National Maternity and Neonatal Investigation (Baroness Amos Report) published 30th June 2026. • Expectation of Integrated Care Boards as Strategic Commissioners • SEL Maternity and Neonatal Overview (Appendix 1) • Current SEL LMNS activities addressing Ockenden Immediate and Essential actions and Amos recommendations (Appendices 2 and 3) • Proposed next steps for SEL LMNS and the NHS England 10-point plan (Appendix 3) • Both reports collectively represent a significant call for accelerated improvement in maternity and neonatal safety, culture, leadership and accountability.						
Summary of main points	Both reports set out the need for ICBs to take a more proactive approach on system leadership and accountability, actively testing whether services are safe, appropriately staffed and delivering on improvements through early intervention particularly if there are repeated concerns. There is an expectation that ICB maternity and neonatal governance is fit for purpose, even with responsibility for assurance now sitting with NHSE/DHSC regional maternity and neonatal teams. Listening to and receiving feedback from women, birthing people and families remains a priority via funded Maternity and Neonatal Voices Partnerships (MNVPs) and other engagement mechanisms. **Proposed Local Maternity and Neonatal Governance** Historically the LMNS has led oversight and assurance of maternity and neonatal services. As assurance responsibilities transition to the regional team, a revised strategic commissioning approach is required that maintains a focus on collaboration, shared learning, service improvement and transformation, informed by intelligence from the LMNS and NHS England. While further guidance is awaited, the SEL LMNS model provides a strong foundation for ongoing partnership working, supporting the implementation of recommendations and continuous improvement across maternity and neonatal services.						
Potential conflicts of Interest	None advised						
	Bexley	x	Bromley	x	Lewisham		x

Relevant to these boroughs	Greenwich	x	Lambeth	x	Southwark	x
Equalities Impact	Significant impact on equalities are highlighted by the reports.					
Financial Impact	Not the focus of this paper.					
Public Patient Engagement	The reports are published and received input from patients.					
Committee engagement	The executive group and quality committee have been kept informed of developments.					
Recommendation	That the Board note the findings of the reports, the expecation of strategic commissioners the next steps for the LMNS.					

South East London Integrated Care System

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Overview of Ockenden and Amos Reviews

SEL LMNS

July 2026

Purpose

To provide an overview of:

- A summary of key findings from the **Nottingham University Hospitals Maternity Review (Ockenden Report)** published 24th June 2026.
- A summary of key findings from the **National Maternity and Neonatal Investigation (Baroness Amos Report)** published 30th June 2026.
- Expectation of Integrated Care Boards as Strategic Commissioners
- SEL Maternity and Neonatal Overview (Appendix 1)
- Current SEL LMNS activities addressing Ockenden Immediate and Essential actions and Amos recommendations (Appendices 2 and 3)
- Proposed next steps for SEL LMNS and the NHS England 10-point plan (Appendix 3)

Both reports collectively represent a significant call for **accelerated improvement in maternity and neonatal safety, culture, leadership and accountability**.

Ockenden Overview

The review was commissioned in June 2022 and considers the provision of maternity and neonatal care at Nottingham University Hospitals Trust (NUH) between 2012 and 2025 after significant concerns raised by local families and a lack of confidence in a previous, regionally-led review

Over 2500 families and 800 current and former members of staff shared their stories and experiences.

An independent clinical review was completed for 2026 cases of families who had experienced loss, injury or harm during their maternity and neonatal journey. Of those 2026 cases, 520 mothers and babies were seriously harmed, and the deaths of 156 children and 6 mothers could have been avoided with better care.

The report sets out 18 National Immediate and Essential Actions (supported by 72 sub-actions), organised across 8 key national themes, alongside 32 Local Actions for Learning (LAFLs) for Nottingham University Hospitals NHS Trust.

The Review Team identified multiple accounts and evidence that the NUH Board should have been fully aware that the culture of maternity at NUH was toxic and had failed in three key respects:

- To understand the significance of a dysfunctional department with personality and cultural issues
- To recognise and support staff raising concerns
- To recognise and escalate safety and patient experience concerns

Ockenden Key Clinical, Culture and Governance Themes

- **Recognition, Escalation and Clinical Decision-Making:** Delays in recognising risk, responding to abnormal findings and escalating maternal and neonatal deterioration, with staff not always feeling able to raise concerns or challenge decisions.
- **Communication, Collaboration and Continuity of Care:** Fragmented pathways, poor information sharing and ineffective MDT working reduced shared understanding and contributed to missed or delayed care.
- **Leadership, Workforce and Clinical Oversight:** Staffing, capacity and service pressures, alongside inconsistent and insufficiently visible clinical leadership, affected the delivery of safe care.
- **Governance, Learning and Accountability:** Weak incident reporting, investigations and assurance processes, combined with poor implementation of learning, guidance and best practice, allowed known safety issues to persist.
- **Culture, Family Partnership and Openness:** Families were not always listened to, informed or involved, while concerns around staff voice, transparency, accountability and workplace culture hindered improvement.

Amos National Review

The investigation was commissioned in June 2025 to understand why avoidable harm and deaths continue to occur in maternity and neonatal care, despite multiple previous reviews and national recommendations.

Baroness Valerie Amos was appointed Chair in August 2025, with the investigation taking a system-wide approach to identifying the underlying issues affecting safety, quality, equity and accountability across England.

Over 450 families were listened to directly, more than 10,500 women, birthing people and families responded to the public Call for Evidence, and over 9,000 staff shared their experiences through surveys, trust visits and interviews. The Investigation also visited 12 NHS trusts, interviewed over 40 national leaders, and reviewed more than 9,500 pieces of evidence

The investigation aimed to develop one set of national recommendations, reduce health inequalities and promote health equity and help bereaved and harmed families receive justice and accountability

The report concluded that the maternity and neonatal system is fragmented, overly complex, too slow to learn and not consistently designed to deliver safe, equitable and compassionate care.

Key Concerns Identified:

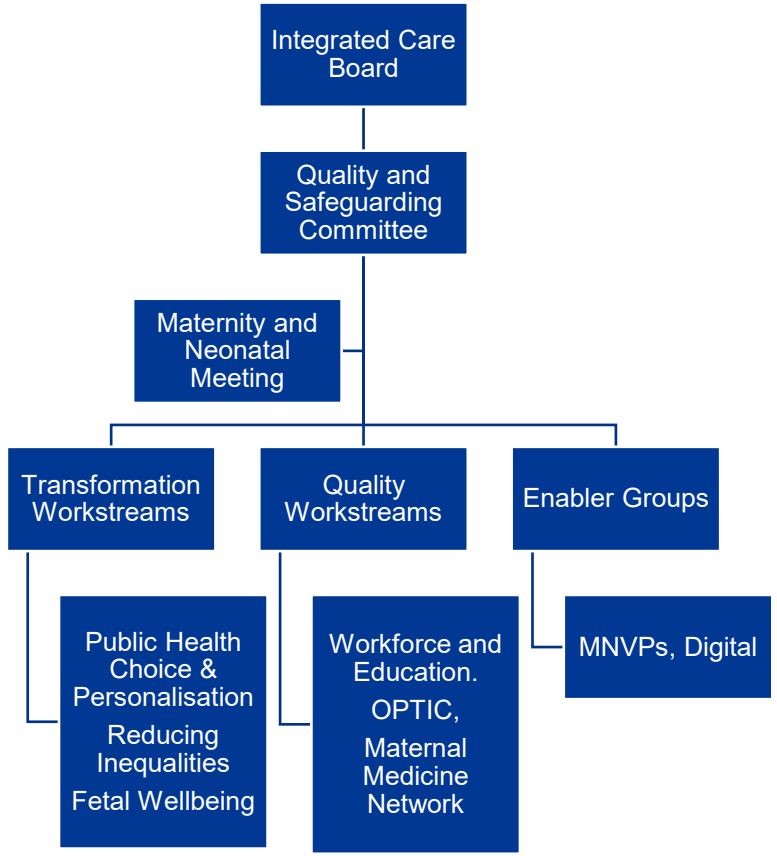
- **Women and families not being listened to** – Concerns dismissed, poor communication, lack of informed consent, and experiences of avoidable harm, trauma and loss of trust.
- **Racism, discrimination and inequality** – Unequal treatment, stereotyping and discrimination affecting care quality, outcomes, staff wellbeing and safety culture.
- **Services not designed for consistent safety** – Reactive, outdated service models, unsafe environments, lack of accountability when harm occurs, and care not keeping pace with changing needs.
- **Fragmented pathways and inconsistent care** – Poor integration across maternity, neonatal, primary, mental health and community services, leading to variation in care and repeated patient stories.
- **Workforce, training and leadership pressures** – Staffing shortages, rota gaps, insufficient protected learning time, limited support in clinical areas and fragile education arrangements.
- **Poor organisational culture** – Fear of blame, hierarchical behaviours, weak accountability, limited speaking up, and racism undermining teamwork, learning and improvement.
- **Weak governance, infrastructure and learning systems** – Fragmented oversight, variable incident reviews, poor digital interoperability, inadequate estates, and slow identification of emerging risks.
- **A system no longer designed for current needs** – Rising complexity of maternity care (older maternal age, higher rates of pre-existing conditions and clinical interventions), alongside widening health inequalities and societal discrimination, has placed increasing pressure on services that have not been redesigned to meet the needs of women, babies and families today.

Expectation of ICBs

Both reports set out the need for ICBs to take a more proactive approach on system leadership and accountability, actively testing whether services are safe, appropriately staffed and delivering on improvements through early intervention particularly if there are repeated concerns.
There is an expectation that ICB maternity and neonatal governance is fit for purpose, even with responsibility for assurance now sitting with NHSE/DHSC regional maternity and neonatal teams.
Listening to and receiving feedback from women, birthing people and families remains a priority via funded Maternity and Neonatal Voices Partnerships (MNVPs) and other engagement mechanisms.

Proposed Local Maternity and Neonatal Governance
Historically the LMNS has led oversight and assurance of maternity and neonatal services. As assurance responsibilities transition to the regional team, a revised strategic commissioning approach is required that maintains a focus on collaboration, shared learning, service improvement and transformation, informed by intelligence from the LMNS and NHS England.
While further guidance is awaited, the SEL LMNS model provides a strong foundation for ongoing partnership working, supporting the implementation of recommendations and continuous improvement across maternity and neonatal services.

LMNS Escalation



Next Steps for SEL LMNS

1. Following a letter from NHS England, Trusts and systems are expected to take immediate action against a 10-point plan (**Appendix 3**), with particular focus on listening to women and birthing people, tackling inequalities, strengthening governance, improving triage, ensuring safe staffing and demonstrating Board-level assurance against the findings of both reviews.

2. The National Maternity and Neonatal Taskforce group will meet on the 7th and 14th of July to review findings, actions and recommendations before presenting to the Health Secretary. A national action plan is expected in December.

3. SEL LMNS have undertaken a benchmarking exercise against the 18 Ockenden Immediate and Essential Actions and the 8 Amos Recommendations evidencing ongoing work to address these (**Appendix 1 and 2**)

4. Regional reflections sessions have been facilitated by the London Maternity team which highlighted a collective need for a coordinated regional response

5. Findings and Actions from both reviews to be discussed with LMNS leaders at the SEL LMNS Board to identify and prioritise system opportunities from the 10-point plan

6. The LMNS will work with its system to address gaps, support implementation and respond to further DHSC guidance once available. Significant work is already completed or underway at system and Trust level, as outlined in the appendices. The LMNS will continue to support transformation and improvement and evaluate interventions where possible.

7. Following recent changes to ICB/LMNS responsibilities, the LMNS is no longer directly responsible for formal assurance. However, future LMNS meetings will continue to provide a strategic forum for reviewing data, sharing learning and identifying opportunities for improvement.

South East London Integrated Care System

NHS South East London

Appendix 1: SEL LMNS Overview

SEL Overview (2025)

Ethnicity
Asian 15%
Black 20%
White 50%



19,000 babies born

> 82% breastfeeding initiation

7.85% Preterm birth rate

45-60% Emergency Caesarean Sections

6% Homebirth rate

Birth rates have been declining year on year in SEL, but pregnancy and birth complexity is increasing.

Contributing factors include rising maternal age (with women aged 30-39 years now representing the largest maternity cohort), increasing rates of obesity, gestational diabetes and hypertension as well as pre-existing conditions.

Mental health needs and socioeconomic challenges are more prevalent, increasing the complexity of maternity and neonatal care.

Data quality has improved but requires further work to ensure a more accurate picture and population health oversight.

South East London Integrated Care System

NHS South East London

Appendix 2: SEL LMNS Current Position July 2026

Current system level activities relating to the 18 National Ockenden Immediate and Essential Actions and the 8 Amos Recommendations.

N.B This does not reflect all trust activities, local benchmarking is ongoing

SEL High Level Oversight

SEL has established Maternity and Neonatal Voices Partnerships (MNVPs) aligned to each provider. Although MNVP Leads are volunteers rather than employed staff, most national requirements have been met, and a strong service user infrastructure is in place. The LMNS continues to engage extensively with underrepresented communities, respond to issues, and has supported reducing inequalities across its programme.

Despite trust board escalation and business case submissions, some services remain below national workforce standards. This is affecting the ability to improve services such as continuity of care and maternity triage.

Alongside mandatory trust training, the LMNS has delivered targeted staff development focused on outcomes, leadership and teamworking, reflecting previous themes and reports. All training has received positive feedback and utilisation within clinical areas.

Continuity of care is recognised as one of the more effective models for improving outcomes, building trust and reducing inequalities. While workforce and resource constraints currently limit wider implementation, two trusts are delivering some targeted antenatal and postnatal continuity for women and birthing people who have the greatest need.

The LMNS historically has maintained robust safety oversight through its Quality Surveillance Group. We will work with the regional team to support assurance and local improvement needs.

Women and families in SEL have reported racism and discrimination. Providers and the LMNS are improving staff training, cultural sensitivity and access to information but recognise that further action is needed to achieve equitable care for all.

Improved joining up of services is required to ensure women and birthing people receive consistent advice wherever they are cared for. This includes reducing variation in support services available across boroughs.

SEL LMNS - Ockenden IEAs

1. Listening to Women & Families

IE1 - Strengthening women-centred communication and informed choice

- SEL LMNS supports local Maternity and Neonatal Voices Partnership (MNVPs) to hear from all women and birthing people, including seldom-heard groups, through targeted outreach with trusted community organisations.
- SEL LMNS is supporting the review and development of the nationally led Healthier Together website, which will include local SEL information on labour and birth, pain relief and anaesthetic care.
- Pocket Guide's have been published to facilitate staff to have consistent balanced conversations about intervention and risk with women and birthing people across the system
- SEL-wide Personalised Care and Support Plans, led by service users, are nearing completion and will strengthen communication by clearly documenting women and birth people's preferences, needs and choices, keeping them at the centre of their care.
- The BRAINS shared decision-making tool was co-produced with service users across SEL. The tool aims to improve communication, support trusted relationships, and help women and birthing people ask questions to make informed care decisions.
- SEL LMNS has rolled out parent education classes in different languages, we are now working up a potential south London offer
- Development of an interactive e-learning resource is in progress to help staff in SEL understand, respect and respond to the cultural backgrounds, beliefs and individual needs of women and birthing people, supporting personalised and inclusive care
- Use of the SEL ICB 'Let's Talk Healthcare' platform is operational within the LMNS, this provides an accessible forum for service users to feedback and allows the LMNS team to update on improvement projects
- SEL LMNS has built strong relationships with local VCSE organisations, supporting meaningful engagement with local communities.

SEL LMNS - Ockenden IEAs

2. Workforce Planning & Safe Staffing

IE2 : Support a nationally agreed perinatal workforce planning methodology as a critical enabler of perinatal improvement at pace and scale

- All providers have completed Birthrate + and BAPM Nursing and Medical workforce assessments as part of Year 7 of the Maternity Incentive Scheme (NHS Resolution), oversight for this now sits within the regional team but continues to be on the LMNS board agenda.

3. Training & Multi-Professional Learning

IE3: National IEA for Labour Ward Coordinator Role
IE4: All trusts must support Training for Midwives in the use of Speculum Examination
IE5: Enhanced Maternal Care

- SEL LMNS-funded *Leading Labour Ward* programme is developing confident, compassionate and accountable Labour Ward Coordinators across SEL, equipping them with the leadership skills needed to strengthen safety culture, support their teams and drive service improvement. Cohorts 1 and 2 have now been successfully completed, with Cohort 3 scheduled to begin later in 2026.
- All SEL Labour Ward Coordinators are completing NHS Leadership Academy 360° appraisals to support self-awareness, reflective practice and ongoing leadership development.
- SEL LMNS has invested in SEL-wide training covering human rights, trauma-informed care, community childbirth emergencies, language barriers, support for refugee, migrant and asylum-seeking women, and advanced fetal wellbeing.
- SEL LMNS is supporting the implementation of the NHS England Maternal Care Bundle (aims to reduce maternal mortality and morbidity) at provider level by engaging with system stakeholders from primary care, pharmacy and early pregnancy

SEL LMNS - Ockenden IEAs

4. Risk Assessment Throughout Pregnancy

IE6: Delivering Safe, Personalised and Equitable Maternity Care Through Early Risk Recognition, Coordinated Care and Responsive Services
IE7: National standard for standardisation and recording of fetal growth risk assessment
IE8: There must be a national standard and documentation for maternity triage and record keeping in maternity care provision
IE9: Support the development and implementation of a structured assessment framework for the latent phase of labour, ensuring clarity when the 'latent phase of labour' becomes abnormal requiring escalation
IE10: All Trusts must define criteria for the safe use of telephone postnatal follow-up, indicating when telephone follow-up is acceptable or when face-to-face follow-up is mandatory
IE11: National standard for obstetric anaesthetic record-keeping
IE12: Safe, accessible and comprehensive maternity anaesthetic documentation
IE13: DHSC/NHSE should introduce and support access to coordinated multidisciplinary debrief and psychological support

- SEL LMNS has supported the pilot of Pregnancy Circles within one Trust. An evaluation is planned and will inform potential wider roll out
- SEL LMNS have developed Relational Care Promises that promote antenatal and postnatal continuity of care. Regular audits are in place to monitor compliance and improvement.
- SEL LMNS has delivered a series of trauma-informed care workshops, open to the multidisciplinary team. The Survivors Trust are also developing three videos for Trusts to incorporate into their mandatory training.
- SEL LMNS has provided Debrief training with 'Make Birth Better' to better equip midwives with the necessary skills to undertake effective debriefs.
- Implementation of the Maternity Disadvantage Tool is ongoing, which aids assessment of social complexities and improves communication of need between health care professionals.
- The LMNS Board has recently stepped up a sub-group to explore the provision of a system wide maternity telephone triage line,
- An LMNS Electronic Documentation improvement working group will be established to strengthen information sharing across specialties and organisational boundaries.
- LMNS led preconception health information is being delivered via the Women's and Girls Health Hubs and educational campaigns to support pregnancy planning and reduction of risk.

SEL LMNS - Ockenden IEAs

5. Incident Investigation & Family Involvement

IEA14: Funding for implementation of Maternity Patient Safety Incident Reporting Framework (PSIRF)

- PSIRF is currently operational across all three maternity providers, patient safety intelligence previously shared with the LMNS is now shared with the regional team as per the Perinatal Quality Oversight Model (PQOM).

6. Governance & Board Accountability

IEA15: Strengthened multidisciplinary governance and learning

- Following the recent publication of the Perinatal Quality Oversight Model (PQOM), there has been a shift to regional oversight and assurance from the LMNS. Work is ongoing to understand and embed processes to ensure high quality services are delivered through strategic commissioning.
- LMNS OPTiC workstream meets monthly as a multi-disciplinary team to discuss key learning from preterm birth and neonatal care. The ethos of the workstream is to share and learn at every opportunity and reduce system variation.
- SEL LMNS holds a quarterly network meeting to share current transformation and improvement work, this includes from the local MNVPs and service user feedback.
- Twice yearly safety events are held across the LMNS addressing themes highlighted through transformation and improvement work

SEL LMNS - Ockenden IEAs

7. Culture, Teamwork & Psychological Safety

IEA16: Foster a compassionate, psychologically safe, and learning culture
IEA17: DHSC/NHSE should recommend and support recruitment processes and implement a consistent onboarding package for new starters

- SEL LMNS has funded 'Civility Saves Lives' training for each Trust to promote improved interdisciplinary team working
- As highlighted previously, a key focus the Leading Labour Ward programme, and the bespoke matron programme is to foster psychologically safe work environments that support escalation

8. Mothers Who Have Died and Post Death Care

*IEA18:*All Trusts to ensure compliance, audited annually, with the NHS Records Management Code of Practice (2023).

- The Maternal Medicine Network is working to improve sharing and learning from maternal deaths
- The review has highlighted a need for further review and improvement of post death care within mortuaries

SEL LMNS – Amos Actions

1. **The Department of Health and Social Care (DHSC) must create a statutory Maternity and Neonatal Commissioner, introducing legislation into the Health Bill at the earliest possible opportunity, and appointing a Commissioner within six months of Royal Ascent.**

- DHSC have confirmed that they will appoint a Maternity and Neonatal Commissioner who will work with the national taskforce to create and deliver the national action plan

2. **DHSC, NHSE, Integrated Care Boards (ICBs) and NHS trusts must take action to listen to the voices of women, birthing people and families within 12 months.**

- SEL LMNS is supporting Maternity and Neonatal Voices Partnerships (MNVP) to ensure they engage and hear from all women and birthing people in their communities, including seldom-heard groups, by facilitating targeted outreach. Further work is required to facilitate action within the 12-month target via PREM
- SEL LMNS Board receives regular updates from each MNVP, including feedback from 15 Steps and Walk the Patch events to encourage system collaboration on improvement activities.
- Prior to the publication of the updated Perinatal Quality Oversight Model, trust complaints and service user feedback (FFT) were discussed regularly as part of the LMNS Quality Surveillance Group. This intelligence is now shared directly with the regional team.

SEL LMNS – Amos Actions

3. DHSC, NHSE and CQC must drive improvement, within 12 months, of the quality, transparency, oversight and accountability of investigations and ensure learning is captured and acted upon when things go wrong.

- Following the recent publication of the Perinatal Quality Oversight Model (PQOM), there has been a shift to regional oversight and assurance from the LMNS. Work is ongoing to understand and embed processes to ensure high quality services are delivered through strategic commissioning.

4. DHSC/NHSE must design a Modern Service Framework for maternity and neonatal services within 12 months and begin rollout within 18 months.

- The LMNS will utilise any opportunities to support the development of the Modern Service Framework

SEL LMNS – Amos Actions

5. DHSC, NHSE, ICBs, NHS trusts, the General Medical Council (GMC) and the Nursing and Midwifery Council (NMC) must treat racism, discrimination and inequality as a critical maternity safety issue – within 12 months, with work starting immediately.

- An interactive SEL e-learning resource is being developed to support staff to understand, respect and respond to the cultural backgrounds, beliefs and individual needs of women and birthing people. The resource includes a dedicated anti-racism module.
- Trusts in SEL are engaged with the Perinatal Equity and Anti-Discrimination Programme (PEADP) which is being delivered by the NHS Race and Health Observatory
- Key clinical leaders in the SEL Maternal Medicine Network have recently completed Above Difference training which aims to improve cultural intelligence and promote inclusive leadership

6. DHSC/NHSE must clarify existing system governance, oversight and accountability structures and improve the effectiveness of regulatory oversight within nine months.

- The LMNS is continuing to support the delivery of the Perinatal Quality Oversight Model by engaging with regional quality and safety forums

SEL LMNS – Amos Actions

7. DHSC, NHSE, ICBs and NHS trusts must work with colleges, universities, post graduate educators and others to improve culture and teamworking, and strengthen leadership at all levels of the system and across professions within 12 months.

- SEL LMNS-funded Leading Labour Ward programme supports improvements in culture, teamworking and leadership across SEL by developing confident, compassionate and accountable Labour Ward Coordinators. The programme equips coordinators with the skills to promote a positive safety culture, strengthen multidisciplinary working, support their teams and lead service improvement.
- A planned programme for matron development is due to commence in September 2026. It will focus on strengthening leadership capability, improving culture and teamworking, enhancing operational oversight, supporting professional development, and promoting system working across the senior midwifery workforce. This will help build leadership at all levels and support more consistent, collaborative practice across SEL.
- SEL LMNS has historically invested in staff development and training, work will be done to identify and facilitate training needs highlighted by both the Amos and Ockenden reports.
- SEL LMNS is piloting career clinics to support midwifery retention. Midwives are matched with senior leaders or specialist colleagues for supportive career conversations. The pilot will be evaluated, with potential to expand the offer to maternity support workers and neonatal colleagues.

SEL LMNS – Amos Actions

8. DHSC/NHSE must deliver estates and digital systems that are fit for modern maternity and neonatal care with 12-month, five-year and 10-year investment commitments and implementation deadlines.

- From Mid-2027, all three trusts in SEL will be using the EPIC Electronic patient record system which will ensure interoperability across organisational boundaries and promote information sharing.
- SEL LMNS and ICB have previously undertaken scoping work on maternity and neonatal estates, the team will await further guidance from DHSC/NHSE regarding future investment.

South East London Integrated Care System

NHS South East London

Appendix 3 – National 10 Point Plan

Letter from Sir Jim Mackey 30th June in response to the reviews

NHS England

NHS England Urgent Actions

Action	Description
Listening to women and their families by rolling out Martha's Rule	Commence roll out of Martha's Rule across all maternity and neonatal services in 2026/27. Building on the learning and insights from the recent pilot, this next phase will support implementation in all antenatal, intrapartum and postnatal inpatient maternity and obstetric settings, including maternity triage and assessment units, and neonatal settings. Implementation will be led by the national Martha's Rule programme team, working closely with patient safety collaboratives to provide coordinated local support, webinars, guidance and advice. We will contact provider organisations in the coming weeks to confirm expectations, resources and timelines.
Listen to women and their families using real time and transparently reported outcomes and experience and clinical audit data that are acted upon at board level	All trusts should implement a monthly cycle of collecting and analysing real-time patient outcomes and experience data at their public boards, using locally meaningful measures like the friends and family test, selected elements of PREM (when available) and insights from maternity and neonatal voices partnerships (MNVPs). This should include regularly listening to women, families and neonatal parents at different points in the pathway, publishing data and patient commentary for full transparency, and ensuring a clear focus on the experiences of people from minority backgrounds. This will ensure the voices and experiences of women and families are heard, scrutinised and acted upon by boards. As part of this, trusts should also reinvigorate clinical audits focusing on key and relevant recommendations identified in the Ockenden and Amos reviews in a focused and effective way, and act on the findings. The aim is to support this approach through national webinars.

NHS England Urgent Actions

Action	Description
Dual board-level accountability between medical directors and chief nursing officers for maternity and neonatal care	All trusts must establish clear joint accountability at board level for maternity and neonatal services, with medical directors and chief nurses holding shared responsibility for oversight, performance and improvement. This must include consistent medical director engagement alongside the chief nurse, and parity between obstetric, midwifery, neonatal and operational leadership. Directors of midwifery and clinical directors leading maternity and neonatal services should attend boards when maternity or neonatal matters are discussed. This should also be implemented across all levels in the NHS including at system, regional, and NHS England boards to eliminate siloed working and strengthen system-wide leadership.
Amos into action' staff listening exercise	Following the listening conversation undertaken by Baroness Amos to understand the nature of the issues in maternity and neonatal services, NHS England will launch a full, open conversation with NHS staff and leaders to identify how the recommendations can be implemented across the system. It will explore and help address the role of culture, attitudes, approaches to practice and ways of working – bringing staff into the single largest national conversation about what trusts and individual clinicians need to do differently to implement both the letter and spirit of the Amos recommendations. This will be done in a way that aligns to rather than duplicates the work of the taskforce and will report findings into it. It will focus on how we implement Baroness Amos's findings from the listening exercise and the findings of the Ockenden review of Nottingham University Hospitals NHS Trust (NUH).

South East London Integrated Care System

NHS England Urgent Actions

NHS South East London

Action	Description
Addressing inequalities in maternity and neonatal outcomes	We can now confirm the timeline for national rollout of the Perinatal Equity and Anti-discrimination Programme, which will be available to all trusts by the end of 2026. The programme has been developed to support maternity and neonatal teams to tackle racism and discrimination, improve the experiences and outcomes of ethnic minority groups and those from deprived communities, and support staff to work in environments free from discrimination and racism. All boards must ensure that provision is in place to allow enough staff to be released to complete the Perinatal Equity and Anti-discrimination Programme, with full participation expected across multi-disciplinary teams. Boards must also regularly review and act on trends identified by their inequalities data dashboards: Maternity and Neonatal Equalities dashboard - NHS England Digital All NHS trusts providing maternity services are responsible for fully implementing the Maternal Care Bundle by March 2027. This includes providing regular reports to the trust board on implementation.
Ensuring 24/7 safety and responsiveness of maternity and neonatal services	All boards must take accountability for ensuring safe and effective service arrangements, including reviewing staffing to ensure 24/7 availability and responsiveness across key workforce groups. Boards must also review internal resource deployment and consider whether roles not directly supporting frontline delivery can be redirected to strengthen service provision. Trusts must ensure that, where specialist posts exist (including in bereavement care and infant feeding), other staff have the knowledge and skills to ensure that care is not compromised when the specialist midwives are not immediately available. Commissioners also hold responsibility for ensuring that their maternity and neonatal service model aligns to local demographic needs. Boards and commissioners must address local gaps and eliminate siloed working to ensure services can consistently and safely respond to the needs of women, babies and families.

NHS England Urgent Actions

Action	Description
Trusts to review their homebirth services	Trusts have a continuing responsibility to offer homebirth as a choice for women and are responsible for ensuring that they manage their workforce to enable this. In November 2025, the Chief Midwifery Officer for England asked trusts to urgently review the safety and quality of their homebirth services. Trusts should ensure that this review has been undertaken and reported to their board and that any safety concerns requiring urgent attention have been actioned. They should ensure that improvement plans are in place where necessary and that risks have been communicated to their regional NHS England team. NHS England is working with partners to develop homebirth standards and a homebirth framework, to support women's autonomy, choice and personalised care, and to help services provide safe homebirth care.
Responding to patient safety incidents, complaints and concerns with humanity, candour and a trauma-informed approach	Trust boards should review how they are responding to patient safety incidents, complaints and concerns within maternity and neonatal services, with openness, humanity and candour. We will work with trusts, in alignment with the work of the national taskforce, to develop a blueprint for how organisations and leaders can respond with more humanity and compassion when things go wrong with a patient's care.
Post death care (already committed by Government)	All trusts must implement actions set out in the System letter following the Fuller and NUH reviews, including responding to the board assurance statement by 31 July and to the Human Tissue Authority requirement to review and assure completeness of incident records over the past 10 years. Every board should continue to review its local position and assure itself that all deceased people cared for in NHS settings are treated with the respect, dignity, security and compassion they deserve. This goes beyond compliance; it is fundamental to compassionate care.

NHS England Urgent Actions

Action	Description
Deliver safe and effective triage in maternity services (already committed by Government)	All trusts must commit to delivering safe and effective triage, starting by completing a board-level audit within 3 months, with a focus on ensuring that maternity triage services are consistently safe, responsive and appropriately resourced. This will be supported by new NHS England guidance, which will be published this week. This includes having dedicated midwifery staffing to answer calls and provide face-to-face assessments, separate from other services such as the labour ward. Services should also have enough clinical, antenatal and bed capacity, with clear escalation routes in place always, including overnight and at weekends. Boards should have clear oversight of triage quality and performance, supported by regular data on waiting times, assessment and review times, redeployment, delays and outcomes. Triage records should capture women's preferences, and services should use evidence-based standards, such as BSOTS, supported by rapid assessment training for triage midwives. This should enable trusts to identify risks, address delays and provide women with timely, consistent and high-quality triage care. The outcomes of this audit should be reported to regions and the Department of Health and Social Care / NHS England. Within 12 months, trusts must implement improvements in line with national triage guidance to ensure women have consistent access to high-quality, responsive triage across the NHS. National maternity triage principles and a supporting measurement framework will be provided to trusts this week to reduce unwarranted variation, strengthen consistency in how concerns are assessed and escalated, and provide a clearer basis for local, regional and national improvement.

South East London Integrated Care System

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Board Meeting in Public

Title	**Diagnostics in South East London: understanding the challenge, opportunities and strategic choices**					
Meeting date	22 July 2026	Agenda item Number	13	Paper Enclosure Ref	L	
Author	Carl Glenister, Associate Director – Cancer and Planned Care, NHS South East London ICB Harriet Agyepong, Associate Director – Performance, NHS South East London ICB Heather Gilmour, Managing Director, Acute Provider Collaborative					
Executive lead	Annabel Appleby, Acting Joint Director of Planning / Director – Acute and Specialised Services, NHS South East London ICB					
Paper is for:	Update	x	Discussion	x	Decision	
Purpose of paper	To support Board discussion on diagnostics as a strategic system issue, including the current position, improvement activity underway, CDCs as part of the future model, and the ICB strategic commissioning role.					
Summary of main points	Diagnostics is a critical enabler of cancer, elective recovery and wider system performance. The pack sets out that diagnostic performance for SEL ICB patients is now broadly in line with London/national comparators, while detailed provider recovery remains managed through Trust governance and NHS England assurance. APC/provider improvement work is underway, and CDCs are highlighted as part of the future diagnostic model. The paper frames the ICB role as strategic commissioning.					
Potential conflicts of Interest	None identified					
Relevant to these boroughs	Bexley	x	Bromley	x	Lewisham	x
	Greenwich	x	Lambeth	x	Southwark	x
Equalities Impact	Diagnostics access is relevant to health inequalities and timely access to care. The CDC section highlights a geographic access issue for Lambeth and Southwark. Equalities considerations should continue to be addressed as future diagnostic models and investment proposals develop.					
Financial Impact	No immediate financial decision is requested. Future diagnostic investment and CDC development have potential capital, revenue, workforce and affordability implications that will require further assessment before commitments are made.					
Public Patient Engagement	No specific public/patient engagement is proposed for this discussion item. Future diagnostic service changes or material access/pathway changes should include appropriate engagement as proposals develop.					
Committee engagement	Provider recovery is managed through Trust governance and NHS England assurance. Collaborative improvement is supported through APC governance and diagnostic networks. CDC proposals will require continued governance as business case and commissioning considerations develop.					

Recommendation	The Board is asked to note the current position and improvement activity; recognise diagnostics as a system-wide strategic commissioning issue; support the proposed framing of the ICB role; and discuss future opportunities and strategic choices, including CDC development and targeted investment.

South East London Integrated Care System

NHS South East London

Diagnostics in South East London

Understanding the challenge, opportunities and strategic choices

Purpose of today's discussion

A strategic discussion about diagnostics and the opportunities available to improve outcomes for South East London patients

Diagnostics is a critical enabler of cancer pathways, elective recovery and wider system performance.

1. **Understanding the challenge**
2. **Understanding the response**
3. **Understanding future opportunities**

Challenge

Current diagnostic position and what the data tells us

Response

Improvement activity underway across providers, APC and the wider system

Opportunities

CDCs, strategic commissioning and future investment considerations

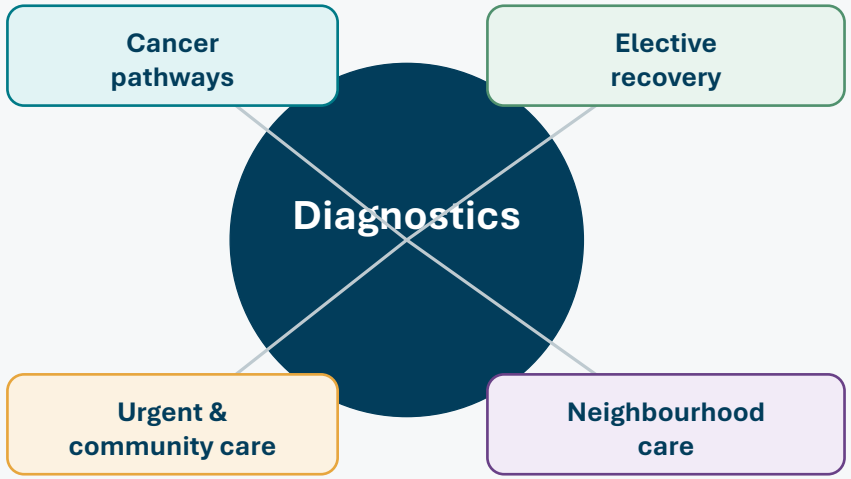
Today is not intended to duplicate detailed provider performance oversight, recovery plans or NHS England assurance processes.

Diagnostics is a system issue, not a single programme

Why this discussion is different

Diagnostics underpins multiple system priorities

It affects patients across many pathways — not a single service line.



Imaging • Pathology • Endoscopy • Physiological sciences

Different parts of the system play distinct roles

This is why today is not structured as a detailed provider recovery review.

Individual trusts

- Operational delivery
- Workforce & capacity
- Recovery plans
- Performance

Trusts working together as the APC

- Collaborative improvement
- Diagnostic networks
- Pathway & digital development
- Demand optimisation

ICB

- Strategic commissioning
- System priorities
- Investment decisions
- Future models

Today's focus

Understanding the challenge, current improvement activity and future opportunities for the system — not detailed provider recovery plans.

Detailed performance and recovery arrangements continue through established provider governance and NHS England assurance processes.

Structure of today's discussion

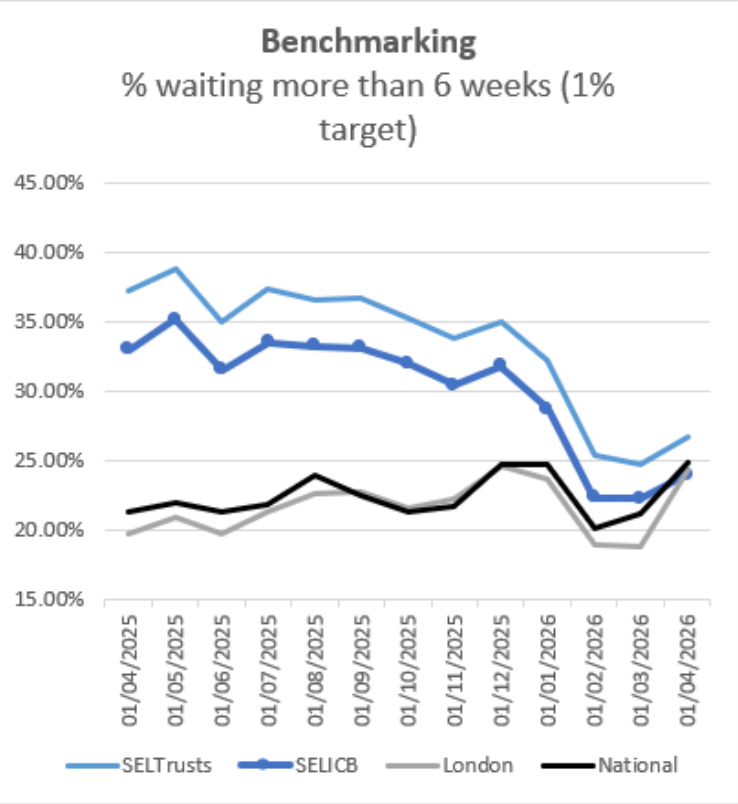
The discussion will move from understanding the challenge through to future opportunities and Board debate.



Today's discussion moves from understanding the challenge and current improvement activity, through future opportunities such as CDCs, before considering the strategic commissioning implications and wider Board discussion.

For SEL ICB patients, diagnostic performance is in line with the rest of London

- The NHS Constitutional standard for diagnostics is that only 1% of patients should be waiting more than 6 weeks for their diagnostic.
- Performance is assessed by reviewing waiting times for a basket of diagnostics covering imaging, endoscopies and physiological measurements, which is part of the statutory reporting made by trusts monthly (DM01), not all diagnostics are included in this data eg, x-rays are not included.
- Trust performance against this metric is now managed through NHSE assurance processes, with detailed provider performance and recovery actions risk assessed and reviewed and discussed with trusts on a regular basis by NHSE London.



- Published data shows that at the beginning of 2025/26, SEL acute trusts (aggregate) and SEL ICB (registered population*) were outliers on diagnostic performance when compared with London and the national performance picture.
- Data for April 26, however, shows that the gap has since been closed, with performance for SEL ICB patients now in line with the London and national picture. Aggregate SEL trusts while significantly improved are still above.
- To note however, that no system is performing in line with the expected 1% target. The expectation is that all organisations will return to the 1% constitutional target by March 2029.
- Currently Non obstetric ultrasound, adult audiology and MRI are the most challenged diagnostics.

* SEL ICB performance will include SEL patients waiting at non-SEL acute trusts.

Collaborative improvement complements and supports individual trust improvement and recovery

1



Collaboration through networks

- Clinically-led networks with multidisciplinary membership and effective programme support
- Reporting into APC governance groups
- **Imaging** Network, supported by a Clinical Reference Group
- **Endoscopy** Network
- **Cardiology** diagnostics group
- Collaborative support to primary care – guidelines, webinars etc

2



Working together to increase capacity

- Diagnostic demand growth
- Capital investment with national funding – Eltham & Sidcup CDCs, new endoscopy suites, new equipment: we work together to prioritise the highest impact investments
- Newer equipment allows for less downtime, faster and more reliable scans, more sophisticated scans – better care

3



Using digital tools to support GP decision-making – right test, right time

- ICE Order Comms for GPs replaced multiple referral routes
- Clinical Decision Support (CDS) supports GPs to refer for the right test for their patients' needs (RCR best practice)
- Moving towards consistent offer for GP Direct Access tests across SEL through ICE

4



Optimising referrals to manage growing demand

- Best practice testing – Right Test Right Time - Making best use of what we have
- Patient information and decision support – understanding the benefits and risks of testing
- Exploring clinical decision support for internal test requests –working with colleagues beyond the diagnostic networks

5

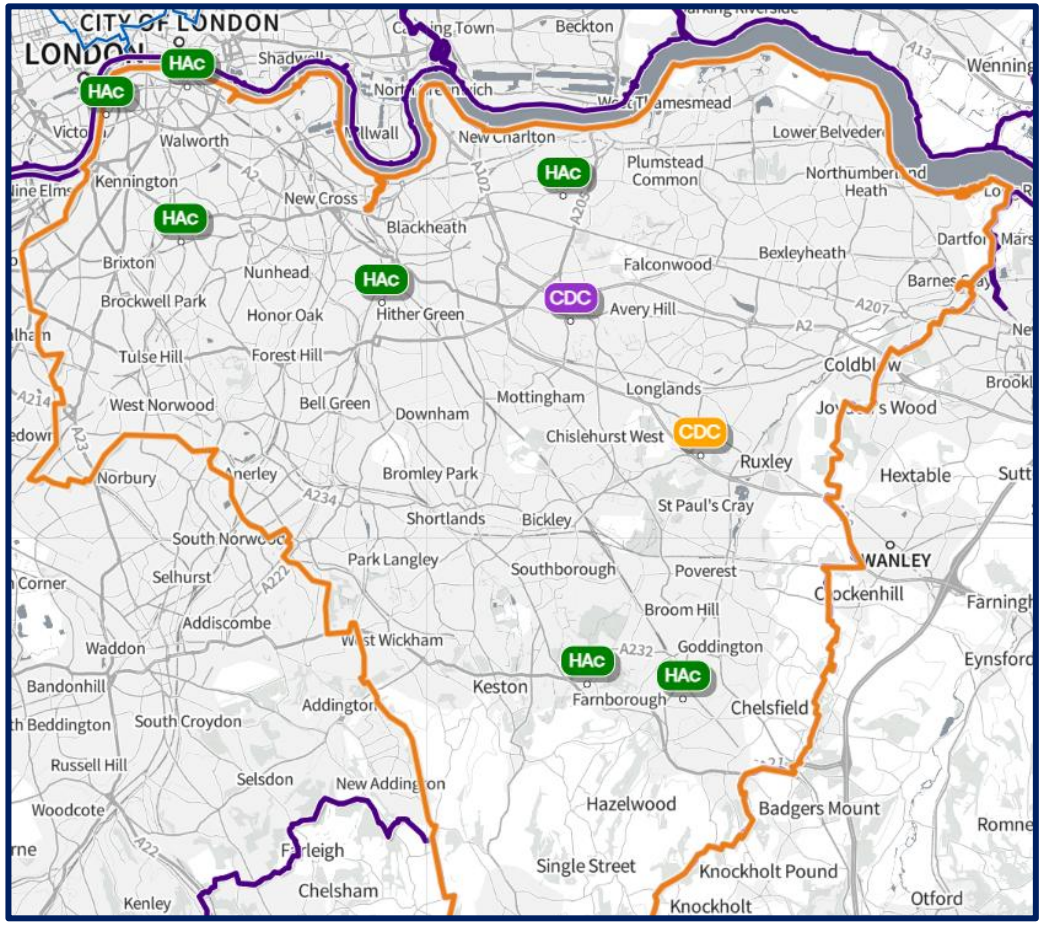


Growing capacity – equipment, workforce, operating hours

- Digital and AI opportunities – acquisition and reporting
- Flexible workforce models, eg VESP project
- System demand and capacity analysis – making sure we can accommodate future needs
- More capital investment in the pipeline across multiple modalities – including bidding for an additional CDC

Community Diagnostic Centres as part of the future diagnostic model

- GSTT have developed proposals for the development of a Community Diagnostic Centre to serve Lambeth and Southwark.
- There have been continuing challenges with diagnostic waiting times at Guy's & St Thomas' and King's College Hospital, both large tertiary centres located in the north-western part of SEL. Addressing this through existing SEL CDC capacity is limited by the challenges Lambeth and Southwark patients face in accessing these centres.
- The western half of the region lacks localised diagnostic capacity, creating a clear geographic inequality. Establishing a CDC in a central location easily accessible from across Lambeth and Southwark via public transport directly addresses this imbalance, ensuring patients are not required to travel across SEL to access essential diagnostic services.



Lambeth and Southwark CDC – Future Benefits

- The proposed CDC aims to deliver the following diagnostic modalities:
  - 1 x MRI Scanner
  - 5 x Multi-purpose rooms for a mix of NOUS and a Women and Girl's Health Hub
  - 5 x Echo / ECG rooms
  - Adult and Paediatric Phlebotomy
  - 2 x multi purpose rooms for either extra NOUS, WHH, clinical rooms to enable a cardiac one-stop or another service
- The CDC will significantly expand diagnostic capacity across multiple modalities, improving throughput and freeing up acute diagnostic space for specialist work. Key increases include:
  - NOUS: +1,000 scans per week
  - MRI: +200 scans per week
  - ECHO: +150–200 tests per week across 3 new rooms
  - Women and Girl's Health Hub: 64 new outpatient appointments weekly
  - Phlebotomy: 500 appointments weekly

A CDC in Lambeth and Southwark would deliver a clear and meaningful left shift by moving significant volumes of diagnostic activity (spanning both GP referrals and secondary care demand) out of acute hospitals and into a community-based setting, which is accessible outside working hours and on weekends. By situating the CDC between the main acute sites of STH, Guys and KCH, the scheme ensures that diagnostics are provided closer to where local residents live and work, but is accessible to a wide population cohort. This makes healthcare more convenient, reduces travel barriers, and supports higher attendance and engagement across Lambeth and Southwark.

The CDC would play a pivotal role in improving integration across local pathways. Fully interoperable digital connectivity, enabled by the shared electronic patient records system in place between GSTT, KCH and (by time of go live) LGT, and ≥90% pathway compliance will streamline patient journeys, reduce duplication, and support seamless care across primary, community, and acute settings. Alongside this, enhanced local access to imaging, cardiology and other diagnostics will mean ≥100,000 additional diagnostic appointments each year.

Strategic Commissioning: What this means for Diagnostics

The ICB role is not to operationally manage diagnostics; it is to commission for outcomes, align investment with system priorities, and work through partners with operational expertise.

1. Understand need

Population need, access, outcomes and variation
Using data and intelligence from providers, APC, networks and communities

2. Shape strategy

Diagnostics as an enabler of cancer, elective recovery, neighbourhood care and future models

3. Align resources

Commissioning levers, CDC development and strategic investment targeted where there is clear system benefit

4. Evaluate impact

Impact on access, equity, productivity, outcomes and value — not just activity

Applied to diagnostics

Work through expertise

Providers, Acute Provider Collaborative and diagnostic networks hold the operational expertise and delivery levers.

Move beyond demand alone

Demand optimisation matters, but the strategic question also includes productivity, pathways and the residual gap.

Commission future models

CDCs are a strategic commissioning opportunity — with revenue, workforce and pathway implications — not just a capital scheme.

From performance oversight alone → to a strategic view of the future diagnostic model

What should we take away from this discussion

Five key messages for the Board

1. No single solution

Demand, workforce, estate, equipment, productivity and pathways all influence performance.

2. Improvement already underway

Providers, Acute Provider Collaborative programmes, diagnostic networks and CDC developments are addressing different parts of the challenge.

3. Strategic choices matter

Future investment should focus on genuine residual gaps after optimisation, pathway redesign and productivity opportunities.

4. The ICB role is changing

Increasingly focused on strategic commissioning, prioritisation, investment decisions and outcomes.

5. Diagnostics requires a coordinated response

Success depends on providers, Acute Provider Collaborative, diagnostic networks and commissioners working together around a shared future diagnostic model.

Questions received in advance

Themes from Board questions - for discussion / Q&A

Questions broadly fall into four linked themes. We can answer some at headline level in the Board discussion; others require further demand/capacity, financial and business case work.

1 Drivers of pressure and future need

- What is driving the current position: demand, workforce, equipment, productivity, pathways?
- What happened in Q1 and how confident are we in plans?
- Have we modelled future need and the residual gap?

2 Impact of current response

- Which actions are having the greatest impact?
- Capacity vs optimisation, digital tools and collaboration — what moves the dial?
- Are resources focused on the highest-impact areas?

3 CDCs, access and inequalities

- Utilisation and effectiveness of current CDCs; access to capacity across providers.
- Is geography a barrier, especially for Lambeth and Southwark?
- Affordability, revenue/workforce implications and contribution to future need.

4 Strategic commissioning and Board role

- What are the ICB commissioning levers? Spend, investment choices, market-shaping?
- What is the actual ask of the Board?
- What choices are being put forward beyond CDC development?

These questions reinforce the central message: diagnostics needs to be considered as a system and strategic commissioning issue — not only as a performance metric.

Detailed answers can be picked up verbally at Board or through a short follow-up note where further analysis is required.